

ICD-11

International Classification of Diseases for Mortality and Morbidity Statistics

Eleventh Revision



World Health
Organization

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CHAPTER 01

Certain infectious or parasitic diseases

This chapter has 342 four-character categories.

Code range starts with 1A00

This chapter includes certain conditions caused by pathogenic organisms or microorganisms, such as bacteria, viruses, parasites or fungi.

Exclusions: Infection arising from device, implant or graft, not elsewhere classified (NE83.1)

Coded Elsewhere: Infections of the fetus or newborn (KA60-KA6Z)

Human prion diseases (8E00-8E0Z)

Pneumonia (CA40)

This chapter contains the following top level blocks:

- Gastroenteritis or colitis of infectious origin
- Predominantly sexually transmitted infections
- Mycobacterial diseases
- Certain staphylococcal or streptococcal diseases
- Pyogenic bacterial infections of the skin or subcutaneous tissues
- Certain zoonotic bacterial diseases
- Other bacterial diseases
- Human immunodeficiency virus disease
- Viral infections of the central nervous system
- Non-viral and unspecified infections of the central nervous system
- Dengue
- Certain arthropod-borne viral fevers
- Certain zoonotic viral diseases
- Certain other viral diseases
- Influenza
- Viral hepatitis
- Viral infections characterised by skin or mucous membrane lesions
- Mycoses
- Parasitic diseases
- Sepsis
- Sequelae of infectious diseases

Gastroenteritis or colitis of infectious origin (1A00-1A40.Z)

Coded Elsewhere: Intestinal fungal infections

Bacterial intestinal infections (1A00-1A0Z)

Any condition of the intestines, caused by an infection with a bacterial source.

Exclusions: Bacterial foodborne intoxications (1A10-1A1Z)

Coded Elsewhere: Abdominal actinomycosis (1C10.1)

Listerial gastroenteritis (1C1A.Y)

1A00

Cholera

Cholera is a potentially epidemic and life-threatening infection of the intestine, characterised by extreme watery (secretory) diarrhoea often accompanied by vomiting, with rapid depletion of body fluids and salt that may result in hypovolemic shock and acidosis. Cholera outbreaks are caused by toxigenic strains of *Vibrio cholerae* serogroups O1 and O139. Serogroup O1 has two biovars; classical and eltor. *Vibrio cholerae* O1, biovar cholerae is classical type. *Vibrio cholerae* O1, biovar eltor is eltor type.

Inclusions: cholera syndrome

1A01

Intestinal infection due to other *Vibrio*

1A02

Intestinal infections due to *Shigella*

A disease caused by an infection with the gram-negative bacteria genus *Shigella*. This disease is characterised by an acute onset of small volume diarrhoea, accompanied by fever and nausea. This disease may also present with toxemia, vomiting, cramps, and tenesmus. Transmission is by ingestion of contaminated food, or direct contact. Confirmation is by identification of *Shigella* in a faecal sample.

Coded Elsewhere: Sepsis due to shigella with septic shock (1C41)

1A03

Intestinal infections due to *Escherichia coli*

Any condition of the gastrointestinal system, caused by an infection with the gram-negative bacteria *Escherichia coli*.

1A03.0

Enteropathogenic *Escherichia coli* infection

An infection of the gastrointestinal system, caused by the gram-negative bacteria *Escherichia coli*. It is characterised by acute, profuse, watery diarrhoea. Transmission is by the faecal-oral route from contaminated food, water, or fomites. Confirmation is by identification of the *Escherichia coli* in a faecal sample.

1A03.1

Enterotoxigenic *Escherichia coli* infection

A condition of the gastrointestinal system, caused by an infection with the gram-negative bacteria *Escherichia coli*. This condition is characterised by acute, watery diarrhoea due to toxins released from the bacteria. Transmission is by the faecal-oral route from ingestion of contaminated food, water, or fomites. Confirmation is by identification of the *Escherichia coli* in faecal sample.

1A03.2	Enteroinvasive Escherichia coli infection	A condition of the gastrointestinal system, caused by an infection with the gram-negative bacteria Escherichia coli. This condition is characterised by acute and profuse diarrhoea (that may be haemorrhagic), fever, and abdominal cramps. Transmission is by the faecal-oral route from ingestion of contaminated food or water. Confirmation is by identification of the Escherichia coli in a faecal sample.
1A03.3	Enterohaemorrhagic Escherichia coli infection	
1A03.Y	Intestinal infections due to other specified Escherichia coli	
1A03.Z	Intestinal infections due to Escherichia coli, unspecified	
1A04	Intestinal infections due to Clostridioides difficile	A disease of the colon, caused by an infection with the gram-positive bacteria Clostridioides difficile (formerly known as Clostridium difficile). This disease is characterised by colitis, diarrhoea, abdominal pain, and fever. Transmission is commonly by direct or indirect contact, or from a disturbance of the normal bacterial flora of the colon. Confirmation is by identification of Clostridioides difficile in a faecal sample. Exclusions: Necrotising enterocolitis of newborn (KB88)
1A05	Intestinal infections due to Yersinia enterocolitica	A disease of the intestinal tract, caused by an infection with the gram-negative bacteria Yersinia enterocolitica. This disease commonly presents with a fever, diarrhoea, or abdominal pain. This disease may also lead to a systemic infection. Transmission is by the faecal-oral route from the ingestion of contaminated food or water, or direct contact with infected individuals or animal. Confirmation is by identification of Yersinia enterocolitica in a faecal sample. Exclusions: Extraintestinal yersiniosis (1B9A) Coded Elsewhere: Postinfectious arthropathy in enteritis due to Yersinia enterocolitica (1A05)
1A06	Gastroenteritis due to Campylobacter	
1A07	Typhoid fever	A condition caused by an infection with the gram-negative bacteria Salmonella typhi. This condition is characterised by an acute sustained fever. This condition may present with weakness, stomach pains, headache, loss of appetite, or flat, rose-coloured spots. Transmission is by the faecal-oral route from the ingestion of contaminated food or water. Confirmation is by identification of Salmonella typhi in a faecal or blood sample.
1A07.0	Typhoid peritonitis	
1A07.Y	Other specified typhoid fever	
1A07.Z	Typhoid fever, unspecified	

1A08**Paratyphoid fever**

A condition caused by an infection with the gram-negative bacteria *Salmonella* paratyphi. This condition is characterised by an acute sustained fever. The individual may feel weak, have stomach pains, headache, loss of appetite, or a rash of flat, rose-coloured spots. Transmission is by ingestion of contaminated food or water. Confirmation is by identification of *Salmonella* paratyphi in a faecal or blood sample.

1A09**Infections due to other *Salmonella***

Coding Note: Infection or foodborne intoxication due to any *Salmonella* species other than *S. typhi* and *S. paratyphi*

1A09.0***Salmonella* enteritis**

This refers to inflammation of the small intestine due to infection with bacteria of the genus *Salmonella*, a member of the family Enterobacteriaceae. Bacteria of the genus *Salmonella* are rod-shaped, Gram-negative, non-spore-forming and predominantly motile.

1A09.Y**Infections due to other *Salmonella* in other organs**

Coding Note: Infection or foodborne intoxication due to any *Salmonella* species other than *S. typhi* and *S. paratyphi*

1A09.Z***Salmonella* infection, unspecified**

Coding Note: Infection or foodborne intoxication due to any *Salmonella* species other than *S. typhi* and *S. paratyphi*

1A0Y**Other specified bacterial intestinal infections****1A0Z****Bacterial intestinal infections, unspecified**

Bacterial foodborne intoxications (1A10-1A1Z)

Any condition caused by an infection with a bacterial source. Transmission is by ingestion of contaminated food.

Exclusions:

salmonella foodborne intoxication and infection (1A09)

Listeriosis (1C1A)

Harmful effects of or exposure to noxious substances, Substances chiefly nonmedicinal as to source, Other noxious substances eaten as food (NE61)

Ichthyotoxism not specified as bacterial (NE61)

1A10**Foodborne staphylococcal intoxication****1A11****Botulism**

A disease caused by an infection with the gram-positive bacteria *Clostridium botulinum*. This disease commonly presents with abdominal pain, vomiting, acute paralysis, blurred vision, diplopia, and may be fatal. Transmission is by ingestion of contaminated food, direct contact, or from accidental overdose. Confirmation is by identification of *Clostridium botulinum* in a faecal or food sample.

1A11.0	Foodborne intoxication by botulinum toxin
1A11.1	Other forms of botulism
1A11.Z	Botulism, unspecified
1A12	Foodborne Clostridium perfringens intoxication
	<i>Inclusions:</i> enteritis necroticans foodborne Clostridium welchii intoxication
1A13	Foodborne Bacillus cereus intoxication
1A1Y	Other specified bacterial foodborne intoxications
1A1Z	Bacterial foodborne intoxications, unspecified

Viral intestinal infections (1A20-1A2Z)

Any condition of the intestines, caused by an infection with a viral source.

Exclusions: influenza with involvement of gastrointestinal tract (1E32)

Coded Elsewhere: Herpes simplex virus duodenitis (1F00.Y)
Human immunodeficiency virus disease enteritis (1C62.2)

1A20	Enteritis due to Adenovirus
	A disease of the intestinal tract, caused by an infection with adenovirus. This disease is characterised by a fever, diarrhoea, or vomiting. Transmission is by the faecal-oral route.
1A21	Gastroenteritis due to Astrovirus
1A22	Gastroenteritis due to Rotavirus
	A disease of the gastrointestinal tract, caused by an infection with rotavirus. This disease is characterised by acute onset of vomiting, non-haemorrhagic diarrhoea, and abdominal pain. Transmission is by ingestion of contaminated food or water, direct contact, or through fomites. Confirmation is by identification of rotavirus.
1A23	Enteritis due to Norovirus
	A disease of the gastrointestinal tract, caused by an infection with norovirus. This disease is characterised by acute onset of vomiting, non-haemorrhagic diarrhoea, and abdominal pain. Transmission is by ingestion of contaminated food or water, direct contact, or through fomites. Confirmation is by identification of norovirus.
1A24	Intestinal infections due to Cytomegalovirus
	A condition of the intestinal tract, caused by an infection with cytomegalovirus. The condition is characterised by diarrhoea, fever, abdominal pain, or haematochezia. Transmission is by direct contact with infected body fluids.
1A2Y	Other specified viral intestinal infections
1A2Z	Viral intestinal infections, unspecified

Protozoal intestinal infections (1A30-1A3Z)

Any condition of the intestines, caused by an infection with a protozoal parasitic source.

1A30

Infections due to *Balantidium coli*

Any condition caused by an infection with the protozoan parasite *Balantidium coli*.

1A31

Giardiasis

A condition caused by an infection with the protozoan parasite *Giardia*. This condition is characterised by gastroenteritis, or may be asymptomatic. Transmission is by the faecal-oral route from the ingestion of contaminated food or water. Confirmation is by identification of *Giardia* in a faecal sample.

1A32

Cryptosporidiosis

Any condition caused by an infection with the protozoan parasite *Cryptosporidium*.

1A33

Cystoisosporiasis

A disease caused by the protozoan parasite *Cystoisospora belli*. This disease is characterised by watery diarrhoea, fever, abdominal pain, nausea, or malaise. Transmission is by the faecal-oral route, commonly through the ingestion of contaminated food or water. Confirmation is by identification of *Cystoisospora belli* in a faecal sample.

1A33.0

Cystoisosporiasis of small intestine

Inclusions: Infection due to *Isospora belli*
Infection due to *Isospora hominis*

1A33.1

Cystoisosporiasis of colon

Isosporiasis of colon is a large intestinal inflammation caused by the protozoan *Isospora belli*.

1A33.Y

Other specified cystoisosporiasis

1A33.Z

Cystoisosporiasis, unspecified

1A34

Sarcocystosis

Any condition caused by an infection with the protozoan parasite *Sarcocystis*.

Inclusions: Sarcosporidiosis

1A35

Blastocystosis

1A36

Amoebiasis

Inclusions: infection due to *Entamoeba histolytica*

1A36.0

Intestinal infections due to *Entamoeba*

1A36.00	Acute amoebiasis A disease caused by an infection with the protozoan parasite <i>Entamoeba histolytica</i> . This disease is characterised by fever, abdominal pain, tenesmus, or diarrhoea containing blood. Transmission is by the faecal-oral route or ingestion of contaminated food or water. Confirmation is by identification of <i>Entamoeba histolytica</i> in a faecal or blood sample. Inclusions: amoebic dysentery
1A36.01	Amoeboma of intestine Coded Elsewhere: Amoeboma of large intestine (1A36.0Z)
1A36.0Z	Intestinal infections due to <i>Entamoeba</i> , unspecified
1A36.1	Extraintestinal infections due to <i>Entamoeba</i>
1A36.10	Amoebic liver abscess Inclusions: Hepatic amoebiasis
1A36.11	Amoebic lung abscess
1A36.12	Cutaneous amoebiasis
1A36.1Y	Amoebiasis of other specified sites
1A36.Z	Amoebiasis, unspecified
1A3Y	Other specified protozoal intestinal infections
1A3Z	Protozoal intestinal infections, unspecified
1A40	Gastroenteritis or colitis without specification of infectious agent Inclusions: enteritis septic gastroenteritis septic Exclusions: Noninfectious neonatal diarrhoea (KB8C) noninfective diarrhoea (ME05.1)
1A40.0	Gastroenteritis or colitis without specification of origin There is no mention whether the gastroenteritis or colitis is infectious or non-infectious.
1A40.Z	Infectious gastroenteritis or colitis without specification of infectious agent

Predominantly sexually transmitted infections (1A60-1A9Z)

Exclusions: Nonspecific and nongonococcal urethritis (GC02.1)
Arthropathy following genitourinary infection (FA11.2)

Coded Elsewhere: Sexually transmissible viral hepatitis
Herpes simplex labialis (1F00.01)
Herpes simplex gingivostomatitis (1F00.02)
Vulvovaginal candidosis (1F23.10)
Candida balanoposthitis (1F23.11)
Human immunodeficiency virus disease (1C60-1C62.Z)
Other infections with a predominantly sexual mode of transmission complicating pregnancy, childbirth or the puerperium (JB63.3)
Candidosis of external genitalia (1F23.1Y)
Anogenital molluscum contagiosum (1E76)

Syphilis (1A60-1A6Z)

A predominantly sexually transmitted infection caused by *Treponema pallidum* ssp. *pallidum*.

Coded Elsewhere: Syphilis complicating pregnancy, childbirth or the puerperium (JB63.1)

1A60	Congenital syphilis A disease caused by an infection with the gram-negative bacteria <i>Treponema pallidum</i> <i>pallidum</i> in utero. This disease may present with clinical signs depending on the stage of disease. Transmission is by vertical transmission.
1A60.0	Early congenital syphilis, symptomatic A disease affecting newborns or children up to 2 years of age, caused by an infection with the gram-negative bacteria <i>Treponema pallidum</i> <i>pallidum</i> in utero. This disease is characterised by premature birth, hepatosplenomegaly, skeletal abnormalities, and bullous skin disease. Transmission is by vertical transmission.
1A60.1	Early congenital syphilis, latent
1A60.2	Late congenital syphilitic oculopathy This is a late congenital, sexually transmitted infection caused by the spirochete bacterium <i>Treponema pallidum</i> subspecies <i>pallidum</i> . This diagnosis is with oculopathy.
1A60.3	Late congenital neurosyphilis Neurological sequelae of longstanding (> 2 years) untreated congenital neurosyphilis include mental delay, hydrocephalus, seizures, cerebral infarction and cranial nerve palsies.
1A60.4	Other late congenital syphilis, symptomatic
1A60.5	Late congenital syphilis, latent
1A60.Z	Congenital syphilis, unspecified

1A61**Early syphilis**

A disease caused by an infection with the gram-negative bacteria *Treponema pallidum pallidum*, including primary and secondary stages of syphilis, and early latent syphilis of less than 2 years duration. This disease is characterised by a single chancre in the primary stage, and diffuse rash in the secondary stage. Transmission is commonly by sexual contact.

Exclusions: Early congenital syphilis (1A60)

1A61.0**Primary genital syphilis**

A disease caused by an infection with the gram-negative bacteria *Treponema pallidum pallidum*. This disease is characterised by a single chancre in the genital region. Transmission is commonly by sexual contact.

1A61.1**Primary anal syphilis****1A61.2****Primary syphilis of other sites****1A61.3****Secondary syphilis of skin or mucous membranes**

A disease caused by an infection with *Treponema pallidum pallidum*. This disease is characterised by lesions of the skin and mucous membranes. Transmission is commonly by sexual contact.

1A61.4**Secondary syphilis of other sites**

A disease caused by an infection with the gram-negative bacteria *Treponema pallidum pallidum*. This disease is characterised by less common symptoms of syphilis, including hepatitis, kidney disease, arthritis, periostitis, optic neuritis, uveitis, or interstitial keratitis. Transmission is commonly by sexual contact.

1A61.5**Latent early syphilis**

A disease caused by an infection with the gram-negative bacteria *Treponema pallidum pallidum*. This disease is characterised by serologic proof of infection without symptoms of disease less than 1 year after secondary syphilis. Transmission is commonly by sexual contact.

1A61.Y**Other specified early syphilis****1A61.Z****Early syphilis, unspecified****1A62****Late syphilis**

A disease caused by an infection with the gram-negative bacteria *Treponema pallidum pallidum*. This disease is characterised by gummas, neurological abnormalities, or cardiac abnormalities. Clinical signs normally manifest approximately 3-15 years after initial infection. Transmission is commonly by sexual contact.

Exclusions: Late congenital syphilis (1A60)

1A62.0	Neurosyphilis A disease of the brain or spinal cord caused by an infection with the gram-negative bacteria <i>Treponema pallidum pallidum</i> . This disease is characterised by four different forms: meningovascular, tabes dorsalis, general paresis, or may be asymptomatic. Clinical signs normally manifest approximately 4-25 years after initial infection. Transmission is commonly by sexual contact.
1A62.00	Asymptomatic neurosyphilis
1A62.01	Symptomatic late neurosyphilis A diverse constellation of neuropsychiatric signs resulting from prolonged untreated or inadequately treated syphilis. The protean clinical manifestations include chronic, insidious meningeal inflammation with cranial nerve palsy, cognitive and/or behavioural impairment, ataxia, stroke, seizures and visual or auditory impairment. Coded Elsewhere: Dementia due to neurosyphilis (6D85.Y) Meningitis due to <i>Treponema pallidum</i> (1D01.0Y)
1A62.0Z	Neurosyphilis, unspecified
1A62.1	Cardiovascular late syphilis This is a late, sexually transmitted infection caused by the spirochete bacterium <i>Treponema pallidum subspecies pallidum</i> . This diagnosis is involving the cardiovascular area.
1A62.2	Symptomatic late syphilis of other sites
1A62.20	Ocular late syphilis This is a late, sexually transmitted infection caused by the spirochete bacterium <i>Treponema pallidum subspecies pallidum</i> . This diagnosis is with ocular.
1A62.21	Late syphilis involving the musculoskeletal system This is a late, sexually transmitted infection caused by the spirochete bacterium <i>Treponema pallidum subspecies pallidum</i> . This diagnosis is involving the musculoskeletal system.
1A62.22	Late syphilis of skin or mucous membranes This is a late, sexually transmitted infection caused by the spirochete bacterium <i>Treponema pallidum subspecies pallidum</i> . This diagnosis is involving the skin and mucous membranes.
1A62.2Y	Symptomatic late syphilis of other specified sites
1A62.2Z	Symptomatic late syphilis of unspecified site
1A62.Y	Other specified late syphilis
1A62.Z	Late syphilis, unspecified

1A63**Latent syphilis, unspecified as early or late**

A disease caused by an infection with the gram-negative bacteria *Treponema pallidum pallidum*. This disease is characterised by serologic proof of infection without symptoms of disease. Transmission is commonly by sexual contact.

Inclusions: Positive serological reaction for syphilis

1A6Z**Syphilis, unspecified**

Gonococcal infection (1A70-1A7Z)

A condition caused by an infection with the gram-negative bacteria *Neisseria gonorrhoeae*. Transmission is by sexual contact. Confirmation is by identification of *Neisseria gonorrhoeae*.

Coded Elsewhere: Gonorrhoea complicating pregnancy, childbirth or the puerperium (JB63.2)

1A70**Gonococcal genitourinary infection**

1A70.0 **Gonococcal infection of lower genitourinary tract without periurethral or accessory gland abscess**

Exclusions: Gonococcal infection of lower genitourinary tract with periurethral or accessory gland abscess (1A70.1)

1A70.00 Gonorrhoea of penis

1A70.0Y Other specified gonococcal infection of lower genitourinary tract without periurethral or accessory gland abscess

1A70.0Z Gonococcal infection of lower genitourinary tract without periurethral or accessory gland abscess, unspecified

1A70.1 **Gonococcal infection of lower genitourinary tract with periurethral or accessory gland abscess**

1A70.Y **Gonococcal infection of other specified genitourinary organ**

1A70.Z **Gonococcal genitourinary infection, unspecified**

1A71**Gonococcal pelviperitonitis**

This is an inflammation of the peritoneum, the thin tissue that lines the inner wall of the abdomen and covers most of the abdominal organs.

1A72**Gonococcal infection of other sites**

1A72.0 **Gonococcal infection of musculoskeletal system**

This is a species of Gram-negative coffee bean-shaped diplococci bacteria responsible for the sexually transmitted infection gonorrhoea. This diagnosis is of the musculoskeletal system.

1A72.1 **Gonococcal infection of rectum**

This is a species of Gram-negative coffee bean-shaped diplococci bacteria responsible for the sexually transmitted infection gonorrhoea of the rectum.

- 1A72.2 Gonococcal infection of anus**
This is a species of Gram-negative coffee bean-shaped diplococci bacteria responsible for the sexually transmitted infection gonorrhoea. This diagnosis is of the anus.
- 1A72.3 Gonococcal pharyngitis**
- 1A72.4 Gonococcal infection of eye**
This is a species of Gram-negative coffee bean-shaped diplococci bacteria responsible for the sexually transmitted infection gonorrhoea. This diagnosis is of the eye.
Coded Elsewhere: Neonatal conjunctivitis or dacryocystitis due to *Neisseria gonorrhoeae* (KA65.0)
- 1A72.Y Gonococcal infection of other specified sites**
- 1A73 Disseminated gonococcal infection**
Disseminated gonococcal infection occurs when there is bacteremic dissemination of *Neisseria gonorrhoeae* from its initial focus of infection in female pelvic organs. It manifests as pain and swelling around one or more joints, intermittent crops of erythematous papules and pustules on the limbs, fever and rigors. Blood cultures may be but are not always positive.
- 1A7Z Gonococcal infection, unspecified**

Sexually transmissible infections due to chlamydia (1A80-1A8Z)

An infection with the gram-negative bacteria *Chlamydia trachomatis*. This infection may be asymptomatic or characterised by fever, painful urination, urinary urgency, dyspareunia, vaginal bleeding or discharge, pain in the abdomen in females and by fever, urethritis, painful urination, discharge from the penis, swollen or tender testicles in males. Transmission is by anal, vaginal, or oral sex. Confirmation is by identification of *Chlamydia trachomatis*.

- 1A80 Chlamydial lymphogranuloma**
A disease of the inguinal lymph glands, caused by an infection with the gram-negative bacteria *Chlamydia trachomatis*. This disease is characterised by a genital ulcer, buboes, abscesses in the groin, blood in faeces, tenesmus, or proctocolitis. Transmission is by sexual contact. Confirmation is by identification of *Chlamydia trachomatis* in a blood sample or by polymerase chain reaction tests.

Inclusions: Durand-Nicolas-Favre disease

- 1A81 Non-ulcerative sexually transmitted chlamydial infection**

Exclusions: Neonatal chlamydial pneumonia (KB24)
Neonatal conjunctivitis due to *Chlamydia* (KA65.0)
Chlamydial lymphogranuloma (1A80)
Chlamydial peritonitis (1C21)
Trachoma (1C23)

- 1A81.0 Chlamydial infection of lower genitourinary tract**

1A81.1	Chlamydial infection of internal reproductive organs
1A81.Y	Non-ulcerative sexually transmitted chlamydial infection of other specified site
1A81.Z	Non-ulcerative sexually transmitted chlamydial infection of unspecified site
1A8Y	Other specified sexually transmissible infections due to chlamydia
1A8Z	Sexually transmissible infections due to chlamydia, unspecified
1A90	Chancroid A disease caused by an infection with the gram-negative bacteria <i>Haemophilus ducreyi</i> . This disease is characterised by painful ulcer(s) on the genitalia. Transmission is by sexual contact. Confirmation is by identification of <i>Haemophilus ducreyi</i> from the ulcer exudate. <i>Inclusions:</i> Ulcus molle
1A91	Granuloma inguinale A disease caused by infection with the gram-negative bacterium <i>Klebsiella granulomatis</i> . It commonly presents with painless genital ulceration following contact with an infected sexual partner. Small, painless nodules appear after an incubation period of about 10–40 days; later the nodules break down to create open, fleshy, oozing ulcers which gradually extend, mutilating the infected tissue. The lesions occur at the region of contact and are typically found on the shaft of the penis, the labia, or the perineum. <i>Inclusions:</i> Donovanosis
1A92	Trichomoniasis A disease caused by an infection with the protozoan parasite <i>Trichomonas</i> . This disease presents with symptoms depending on the site of infection. <i>Coded Elsewhere:</i> Intestinal trichomoniasis (1A3Y)
1A93	Sexually transmissible infestations <i>Coded Elsewhere:</i> Scabies (1G04) Pubic infestation by <i>Pthirus pubis</i> (1G03)
1A94	Anogenital herpes simplex infection A condition of the anogenital region, caused by an infection with herpes simplex virus type 1 or 2. This condition is characterised by vesicles, or may be asymptomatic. Transmission is by sexual contact. Confirmation is by identification of herpes simplex virus type 1 or 2.
1A94.0	Herpes simplex infection of genitalia or urogenital tract Herpes simplex infection affecting the vulva and vagina in women and the penis in men. It is more commonly due to infection with Herpes simplex type 2 virus than with type 1 virus.

1A94.1	Herpes simplex infection of perianal skin or rectum Herpes simplex infection of perianal skin and rectum. This is commonly due to Herpes simplex virus type 2 and acquired through anal sexual contact.
1A94.Z	Anogenital herpes simplex infection without further specification
1A95	Anogenital warts Anogenital warts are due to an infection of anogenital skin and mucous membranes by certain human papilloma viruses, most commonly HPV subtypes 6,11,16 and 18. Transmission is predominantly by sexual contact. They manifest typically as flat plaques or papillomatous, keratinous growths on and adjacent to the external genitalia and anus. Some HPV subtypes, including types 16 and 18, are oncogenic and predispose to the development of anogenital cancers. <i>Inclusions:</i> Condylomata acuminata <i>Coded Elsewhere:</i> Anogenital verrucous carcinoma of Buschke and Lowenstein (2C31.0)
1A95.0	Anal warts Infection of the anus or perianal skin by human papillomavirus (HPV). Although the majority of such infections are sexually transmitted and caused by HPV subtypes responsible for genital warts, autoinoculation from common warts, especially on the hands in children, may also cause perianal warts. <i>Inclusions:</i> Condylomata acuminata of anus
1A95.1	Genital warts Infection of anogenital mucosa or skin by the human papillomavirus. The infection is commonly asymptomatic but manifests typically as flat, papular or pedunculated growths depending on the site of infection. Transmission is by normally by sexual contact.
1A95.2	Extragenital condylomata acuminata Anogenital warts transmitted to extragenital sites (i.e. beyond the anogenital region). This may be through autoinoculation of anogenital wart virus to moist, intertriginous sites on the abdomen or under the breasts, or as a result of sexual activity, particularly to the lips and oral cavity. <i>Inclusions:</i> Anogenital warts affecting sites other than the anogenital area
1A9Y	Other specified predominantly sexually transmitted infections
1A9Z	Predominantly sexually transmitted infections, unspecified

Mycobacterial diseases (1B10-1B2Z)

Tuberculosis (1B10-1B1Z)

A disease caused by an infection with bacteria of the *Mycobacterium tuberculosis* complex. This disease presents with symptoms depending on the site of infection. Transmission is commonly by inhalation of infected respiratory secretions.

Inclusions: Infections due to *Mycobacterium tuberculosis* and *Mycobacterium bovis*

Exclusions: Pneumoconiosis associated with tuberculosis (CA60.3)

Coded Elsewhere: Congenital tuberculosis (KA61.0)

Tuberculosis complicating pregnancy, childbirth or the puerperium (JB63.0)

HIV disease clinical stage 1 associated with tuberculosis (1C60.0)

HIV disease clinical stage 2 associated with tuberculosis (1C60.1)

HIV disease clinical stage 3 associated with tuberculosis (1C60.2)

HIV disease clinical stage 4 associated with tuberculosis (1C60.3)

Human immunodeficiency virus disease associated with tuberculosis (1C60)

1B10

Tuberculosis of the respiratory system

This is a progressive or chronic disease resulting from infection with the bacterium *Mycobacterium tuberculosis* or other bacteria in the *M. tuberculosis* complex: *M. bovis*, *M. africanum*, *M. canetti*, *M. microti* and *M. pinnipedii*. The infection is limited to the respiratory system.

1B10.0

Respiratory tuberculosis, confirmed

A disease of the respiratory tract, caused by an infection with the bacteria *Mycobacterium tuberculosis*, which has been confirmed by laboratory testing. This disease is characterised by chronic cough, and sputum production that may be haemorrhagic. Transmission is commonly by inhalation of infected respiratory secretions. Confirmation is by identification of *Mycobacterium tuberculosis* in clinical samples.

1B10.1

Respiratory tuberculosis, not confirmed

A disease of the respiratory tract, caused by an infection with the bacteria *Mycobacterium tuberculosis*, which has not been confirmed. This disease is characterised by a chronic cough, and sputum production that may be haemorrhagic. Transmission is commonly by inhalation of infected respiratory secretions.

1B10.Z

Respiratory tuberculosis, without mention of bacteriological or histological confirmation

1B11

Tuberculosis of the nervous system

A disease of the central nervous system, caused by an infection with bacteria of the *Mycobacterium tuberculosis* complex. This disease is characterised by neurological deficits depending on the site affected. Transmission is through haematogenous spread to the nervous system after inhalation of infected respiratory secretions. Confirmation is by identification of bacteria of the *Mycobacterium tuberculosis* complex in the cerebrospinal fluid.

1B11.0	Tuberculous meningitis A disease of the meninges, caused by an infection with the bacteria <i>Mycobacterium tuberculosis</i> . This disease is characterised by fever, headache, or neurological deficits. Transmission is through haematogenous spread to the meninges after inhalation of infected respiratory secretions. Confirmation is by identification of <i>Mycobacterium tuberculosis</i> in the cerebrospinal fluid. <i>Inclusions:</i> Tuberculous leptomeningitis
1B11.1	Tuberculous meningoencephalitis
1B11.2	Meningeal tuberculoma Tuberculomas are conglomerate caseous foci within the substance of the brain, caused by dissemination of tuberculosis to the central nervous system. <i>Inclusions:</i> Tuberculoma of meninges
1B11.3	Tuberculous granuloma of brain
1B11.4	Tuberculous granuloma of the meninges
1B11.Y	Tuberculosis of other specified part of nervous system
1B11.Z	Tuberculosis of the nervous system, unspecified
1B12	Tuberculosis of other systems and organs
1B12.0	Tuberculosis of heart <i>Mycobacterium tuberculosis</i> infection involving the heart and pericardium
1B12.1	Tuberculosis of eye Tuberculosis involving the eye. This may manifest in multiple different ways including keratoconjunctivitis, episcleritis, anterior uveitis and posterior uveitis <i>Exclusions:</i> lupus vulgaris of eyelid (1B12.8)
1B12.2	Tuberculosis of ear This is a common, and in many cases lethal, infectious disease caused by various strains of mycobacteria, usually <i>Mycobacterium tuberculosis</i> . This diagnosis is of the ear. <i>Exclusions:</i> tuberculosis of skin of external ear (1B12.8) Tuberculous mastoiditis (1B12.40)
1B12.3	Tuberculosis of endocrine glands Infection of endocrine glands by <i>Mycobacterium tuberculosis</i> with resultant endocrine disturbances including adrenal or pituitary failure. <i>Coded Elsewhere:</i> Tuberculous Addison disease (1B12.3)
1B12.4	Tuberculosis of the musculoskeletal system <i>Coded Elsewhere:</i> Mycobacterial infection of vertebra (FA90.1)

1B12.40	Tuberculosis of bones or joints A disease of the bones and joints, caused by an infection with the bacteria <i>Mycobacterium tuberculosis</i>. This disease commonly presents with bone pain, joint inflammation, loss of movement or feeling in the affected bone or joint, and weak bones prone to fracture. Transmission is through haematogenous spread to the bones and joints after inhalation of infected respiratory secretions. Confirmation is by identification of <i>Mycobacterium tuberculosis</i> in biopsy samples of the affected site.
1B12.41	Tuberculous myositis
1B12.4Y	Tuberculosis of other specified part of the musculoskeletal system
1B12.4Z	Tuberculosis of the musculoskeletal system, unspecified
1B12.5	Tuberculosis of the genitourinary system Tuberculosis involving the urinary tract and/or reproductive organs. The primary site of infection is most commonly the kidney as a result of haematogenous spread from distant sites: infection may then spread further down the urinary tract and/or to the reproductive organs. Genital infection may be sexually transmitted.
1B12.6	Tuberculous peripheral lymphadenopathy A disease of the peripheral lymph nodes, caused by an infection with the bacteria <i>Mycobacterium tuberculosis</i>. This disease is characterised by inflammation of the peripheral lymph nodes, typically the cervical lymph nodes. Transmission is through haematogenous spread to the peripheral lymph nodes after inhalation of infected respiratory secretions. Confirmation is by identification of <i>Mycobacterium tuberculosis</i> from lymph node biopsies. Inclusions: Tuberculous adenitis Exclusions: Tuberculosis of intrathoracic lymph nodes, confirmed bacteriologically or histologically (1B10.0) Tuberculosis of intrathoracic lymph nodes, without mention of bacteriological or histological confirmation (1B10)
1B12.7	Tuberculosis of the digestive system Tuberculosis of the digestive tract or hepatobiliary system
1B12.8	Cutaneous tuberculosis Tuberculosis involving the skin and mucous membranes including lupus vulgaris, scrofuloderma and periorificial tuberculosis. Exclusions: Tuberculids (EA40-EA5Z) Skin complications of BCG immunisation (EA51) Coded Elsewhere: Acute miliary cutaneous tuberculosis (1B13.0)
1B12.Y	Tuberculosis of other specified organ or site
1B13	Miliary tuberculosis

1B13.0	Acute miliary tuberculosis of a single specified site A disease caused by an infection with the bacteria <i>Mycobacterium tuberculosis</i> that is disseminated through the body, and affecting a specific body site. This disease is characterised by numerous small lesions of 1-5 millimetre(s) in any organ, and fever. Transmission is commonly by inhalation of infected respiratory secretions. Confirmation is by radiography, CT, ultrasonography, and identification of <i>Mycobacterium tuberculosis</i> , depending on the site affected.
1B13.1	Acute miliary tuberculosis of multiple sites A disease caused by an infection with the bacteria <i>Mycobacterium tuberculosis</i> that is disseminated through the body, and affecting multiple body sites. This disease is characterised by numerous small lesions of 1-5 millimetre(s) in more than one organ, and fever. Transmission is commonly by inhalation of infected respiratory secretions. Confirmation is by radiography, advanced imaging, ultrasonography, and identification of <i>Mycobacterium tuberculosis</i> , depending on the sites affected.
1B13.2	Acute miliary tuberculosis, unspecified site
1B13.Y	Other specified miliary tuberculosis
1B13.Z	Miliary tuberculosis, unspecified
1B14	Latent tuberculosis
1B1Y	Other specified tuberculosis
1B1Z	Tuberculosis, unspecified
1B20	Leprosy A disease caused by an infection with <i>Mycobacterium leprae</i> . This disease commonly presents with a long asymptomatic period followed by granulomatous lesions of the skin, respiratory tract, and peripheral nerves. Transmission is commonly by droplet transmission. Confirmation is by identification of <i>Mycobacterium leprae</i> with skin biopsy. Inclusions: Infection due to <i>Mycobacterium leprae</i>
1B20.0	Paucibacillary leprosy
1B20.1	Multibacillary leprosy
1B20.2	Leprosy reactions
1B20.20	Type I leprosy reaction This phenomenon, also named “upgrading reaction,” occurs in borderline leprosy states and is associated with an increase in cell-mediated immunity. It occurs typically within the first 6 months of treatment in previously untreated patients but may be related to stress, intercurrent infections, or pregnancy. Clinical features include inflammatory swelling, erythema and occasionally ulceration of existing lesions, constitutional symptoms and neuritis. If the neuritis is not treated promptly permanent motor nerve damage may ensue.

1B20.21	Type II leprosy reaction This phenomenon, also named downgrading reaction, occurs in borderline leprosy states and is associated with a decrease in cell-mediated immunity with a shift towards the lepromatous end of the clinical spectrum.
1B20.3	Complications of leprosy
1B20.Z	Leprosy, unspecified
1B21	Infections due to non-tuberculous mycobacteria Any condition caused by an infection with Mycobacteria excluding infections due to Mycobacterium tuberculosis complex and Mycobacterium leprae. These conditions commonly present with lung disease; however, symptoms are dependent on the site of infection. Transmission is by direct contact with non-tuberculous Mycobacteria in the environment. Confirmation is by identification of non-tuberculous Mycobacteria from the affected site(s). <i>Exclusions:</i> Leprosy (1B20) Tuberculosis (1B10-1B1Z)
1B21.0	Pulmonary infection due to non-tuberculous mycobacterium A condition of the pulmonary system, caused by an infection with the bacteria Mycobacterium (excluding infections due to Mycobacterium tuberculosis and Mycobacterium leprae). This disease is characterised by cough, fever, weight loss, and fatigue. Transmission is by direct contact with Mycobacterium in the environment.
1B21.1	Non-tuberculous mycobacterial lymphadenitis
1B21.2	Cutaneous non-tuberculous mycobacterial infection <i>Exclusions:</i> Leprosy (1B20) Tuberculosis (1B10-1B1Z)
1B21.20	Mycobacterium ulcerans infection Mycobacterium ulcerans infection (Buruli ulcer) typically presents as a subcutaneous nodule which breaks down to form a deep painless ulcer which commonly reaches a size of 15 cm in diameter but may extend further to cause extensive tissue damage. The organism is found in wetlands of tropical and subtropical regions of the world, particularly Africa [Dermatology TAG].
1B21.2Y	Cutaneous infection due to other specified non-tuberculous mycobacteria
1B21.2Z	Cutaneous infection due to unspecified non-tuberculous mycobacteria
1B21.3	Disseminated non-tuberculous mycobacterial infection
1B21.4	Gastrointestinal non-tuberculous mycobacterial infection
1B21.Y	Non-tuberculous mycobacterial infection of other specified site
1B21.Z	Non-tuberculous mycobacterial infection of unspecified site
1B2Y	Other specified mycobacterial diseases

1B2Z**Mycobacterial diseases, unspecified****Certain staphylococcal or streptococcal diseases (1B40-1B5Z)**

Coded Elsewhere: Toxic shock syndrome (1C45)

Streptococcal tonsillitis (CA03.0)

Rheumatic myocarditis (BC42.3)

Chronic rheumatic pericarditis (BB21)

Acute staphylococcal tonsillitis (CA03.Y)

Acute rheumatic fever (1B40-1B42)

A disease of the connective tissue, caused by an infection with the gram-positive bacteria *Streptococcus pyogenes*; (the disease may also affect the heart, joints, central nervous system, subcutaneous tissues, or skin). This disease is characterised by fever, polyarthritides, carditis, subcutaneous nodules, or erythema marginatum. Transmission is through haematogenous spread to other parts of the body after direct or indirect contact. Confirmation is by electrocardiography, sedimentation rate, or identification of *Streptococcus pyogenes* in a blood sample.

Coded Elsewhere: Erythema marginatum rheumaticum (EA50.0)

1B40**Acute rheumatic fever without mention of heart involvement**

1B40.0 Rheumatic arthritis, acute or subacute

1B40.Y Other specified acute rheumatic fever without mention of heart involvement

1B40.Z Acute rheumatic fever without mention of heart involvement, unspecified

1B41**Acute rheumatic fever with heart involvement**

A disease of the cardiovascular system, caused as a result of rheumatic fever. Rheumatic heart disease is characterised by repeated inflammation with fibrinous repair. This disease may present with cardinal anatomic changes of the valve including leaflet thickening, commissural fusion, and shortening and thickening of the tendinous cords. Inflammation and valve scarring may also occur. Confirmation is by a thoracic radiography or echocardiography.

Exclusions:

Rheumatic mitral valve stenosis (BB60.0)
 Rheumatic mitral valve insufficiency (BB61.0)
 Rheumatic mitral valve prolapse (BB62.0)
 Rheumatic mitral stenosis with insufficiency (BB63.0)
 Rheumatic aortic valve stenosis (BB70.0)
 Rheumatic aortic valve insufficiency (BB71.0)
 Rheumatic aortic stenosis with insufficiency (BB72.0)
 Rheumatic tricuspid valve stenosis (BB80.0)
 Rheumatic tricuspid valve insufficiency (BB81.0)
 Rheumatic tricuspid valve stenosis with insufficiency (BB82.0)
 Rheumatic pulmonary valve stenosis (BB90.0)
 Rheumatic pulmonary valve insufficiency (BB91.0)
 Rheumatic pulmonary valve stenosis with insufficiency (BB92.0)

1B41.0**Acute rheumatic pericarditis**

A disease of the pericardium, caused by acute rheumatic fever. This disease is characterised by fever, dry cough, rapid heart rate, fatigue, or low blood pressure. Confirmation is by echocardiography, or thoracic radiography.

1B41.1**Acute rheumatic endocarditis**

A disease of the endocardium, caused as a result of acute rheumatic fever. This disease is characterised by a high fever, chills, shortness of breath, rapid or irregular heartbeat, coughing up of blood, abdominal pain or septicaemia. This disease commonly presents with valvular involvement. Confirmation is by echocardiography.

1B41.10

Rheumatic aortitis

1B41.1Y

Other specified acute rheumatic endocarditis

1B41.1Z

Acute rheumatic endocarditis, unspecified

1B41.2**Acute rheumatic myocarditis**

Acute rheumatic myocarditis is cardiac inflammation associated with acute rheumatic fever triggered by an autoimmune reaction to group A streptococci infection resulting in pancarditis involving inflammation of the myocardium, endocardium, and epicardium, usually with left-sided valvar involvement.

1B41.Y**Other acute rheumatic heart disease**

1B41.Z	Acute rheumatic heart disease, unspecified
1B42	Rheumatic chorea Sydenham chorea is a movement disorder that occurs with rheumatic fever heumaragic fever. It is characterised by rapid, uncoordinated jerking movements affecting primarily the face, feet and hands. Sydenham's chorea (SC) results from childhood infection with Group A beta-haemolytic Streptococci <i>Exclusions:</i> Huntington chorea (8A01.10)
1B50	Scarlet fever A disease caused by an infection with the gram-positive bacteria Streptococcus pyogenes. This disease is characterised by a sore throat, fever, and a red rash. Transmission is commonly by inhalation of infected respiratory secretions, direct skin contact, or indirect contact. <i>Inclusions:</i> Scarlatina NOS <i>Exclusions:</i> streptococcal sore throat (1B51) Staphylococcal scarlatina (EA50.3)
1B51	Streptococcal pharyngitis A disease of the pharynx, caused by an infection with the gram-positive bacteria Streptococcus pyogenes. This disease is characterised by fever, sore throat, tonsillar exudates, or large cervical lymph nodes. Transmission is commonly by inhalation of infected respiratory secretions, or indirect contact. Confirmation is by identification of Streptococcus pyogenes from a throat swab. <i>Inclusions:</i> Streptococcal sore throat <i>Exclusions:</i> Scarlet fever (1B50)
1B53	Meningitis due to Streptococcus A disease of the meninges, caused by an infection with the gram-positive bacteria genus Streptococcus. This disease commonly presents with nausea, vomiting, photophobia, and confusion. Transmission is through haematogenous spread to the meninges after inhalation of infected respiratory secretions. Confirmation is by identification of Streptococcus in the cerebrospinal fluid. <i>Inclusions:</i> Streptococcal meningitis
1B54	Meningitis due to Staphylococcus A disease of the meninges, caused by an infection with the gram-positive bacteria genus Staphylococcus. This disease commonly presents with acute inflammation of the meninges causing headache, fever, stiff neck, or neurological deficits. Confirmation is by identification of Staphylococcus in the cerebrospinal fluid. <i>Inclusions:</i> Staphylococcal meningitis
1B5Y	Other specified staphylococcal or streptococcal diseases
1B5Z	Staphylococcal or streptococcal diseases, unspecified

Pyogenic bacterial infections of the skin or subcutaneous tissues (1B70-1B7Y)

Coded Elsewhere: Acute bacterial paronychia (EE12.0)

1B70	Bacterial cellulitis, erysipelas or lymphangitis Diffuse, spreading infections of skin and soft tissues by a range of bacterial organisms, most commonly beta-haemolytic streptococci and <i>Staphylococcus aureus</i> . The clinical presentation is dependent not only on the organism but also on the manner in which it invades the tissues. Exclusions: Eosinophilic cellulitis (EB30)
1B70.0	Erysipelas Exclusions: postpartum or puerperal erysipelas (JB40)
1B70.00	Erysipelas of face
1B70.01	Erysipelas of external ear A rapidly expanding diffuse superficial dermal streptococcal infection involving the external ear. In contrast with infective otitis externa, the skin of the auricle is often initially healthy except at a point of entry for beta-haemolytic streptococci (commonly at a fissure behind the ear or where the ear lobule is attached to the side of the head) and systemic features including fever and malaise are common.
1B70.02	Erysipelas of lower limb
1B70.0Y	Erysipelas of other specified site
1B70.1	Streptococcal cellulitis of skin Exclusions: Orbital cellulitis (9A21.0) Cellulitis of external ear (AA01) anal cellulitis (DB70.00) vulvar cellulitis (GA00.0) Cellulitis of penis (GB06) Inflammatory disorders of scrotum (GB07.2) perirectal cellulitis (DB36.10) Superficial incisional site infection (NE81.20)
1B70.2	Staphylococcal cellulitis of skin Exclusions: Orbital cellulitis (9A21.0) Cellulitis of external ear (AA01) anal cellulitis (DB70.00) vulvar cellulitis (GA00.0) Cellulitis of penis (GB06) Inflammatory disorders of scrotum (GB07.2) perirectal cellulitis (DB36.10) Superficial incisional site infection (NE81.20)

1B70.3	Ascending bacterial lymphangitis A complication of a focal acute pyogenic bacterial infection in which the draining lymphatics become red, inflamed and tender as the result of ascending infection. It is most commonly caused by <i>Streptococcus pyogenes</i> .
1B70.Y	Bacterial cellulitis or lymphangitis due to other specified bacterium
1B70.Z	Bacterial cellulitis or lymphangitis due to unspecified bacterium
1B71	Necrotising fasciitis
1B71.0	Streptococcal necrotising fasciitis <i>Exclusions:</i> Neonatal necrotising fasciitis (1B71.2) <i>Coded Elsewhere:</i> Neonatal streptococcal necrotising fasciitis (1B71.2)
1B71.1	Polymicrobial necrotising fasciitis <i>Exclusions:</i> Neonatal necrotising fasciitis (1B71.2)
1B71.2	Neonatal necrotising fasciitis Neonatal necrotising fasciitis is a life-threatening acute necrotising infection of fascia, subcutaneous tissues, and overlying skin similar to the condition seen in adults. It is rare in neonates but, in contrast to the adult form, tends to affect otherwise healthy babies. It has followed omphalitis, mastitis and postoperative wound infection, though preceding sites of infection are not always found. It has more commonly been associated with <i>Staphylococcus aureus</i> than with streptococcal infection. Gram-negative organisms have also been implicated. It may cause extensive tissue destruction and mortality is high.
1B71.Y	Necrotising fasciitis due to other specified bacterial infection
1B71.Z	Necrotising fasciitis, unspecified
1B72	Impetigo A condition of the skin, commonly caused by a secondary infection with the gram-positive bacteria <i>Staphylococcus aureus</i> or group A beta haemolytic streptococci. This condition is characterised by bullous or non-bullous symptoms. Transmission is by direct contact with an infected individual. Confirmation is by identification of the infectious agent in a skin sample. <i>Exclusions:</i> impetigo herpetiformis (EA90.40) Staphylococcal scalded skin syndrome (EA50.2) <i>Coded Elsewhere:</i> Otitis externa in impetigo (AA3Y)

1B72.0	Bullous impetigo Bullous impetigo is a contagious superficial infection of the skin caused by certain strains of <i>Staphylococcus aureus</i> which release toxins into the local environment which are capable of cleaving desmoglein I, a protein involved in intercellular adhesion of epidermal keratinocytes. In contrast to the very superficial, rapidly shed and rarely observed blisters of non-bullous impetigo, the bullae of bullous impetigo are tense and well demarcated, sometimes reaching several centimetres in diameter before rupture. Coded Elsewhere: Neonatal bullous impetigo (EH11)
1B72.1	Non-bullous impetigo Non-bullous impetigo is due to superficial skin infection with either <i>Streptococcus pyogenes</i> or <i>Staphylococcus aureus</i> or both. The very superficial blisters which form in the upper epidermis are soon shed and rarely seen (cf. bullous impetigo) so that it normally presents with areas of superficial oozing and crusting on the skin surface. It often following minor skin injury or on skin damaged by a preexisting dermatosis such as atopic eczema or scabies.
1B72.2	Secondary impetiginisation of the skin Secondary infection of dermatoses such as atopic eczema by streptococci or staphylococci.
Coding Note:	Code also the causing condition
1B72.Y	Other specified impetigo
1B72.Z	Impetigo, unspecified
1B73	Ecthyma Ecthyma is a superficial ulcerative bacterial pyoderma. It is characterised by small, purulent, shallow, punched-out ulcers with thick, brown-black crusts and surrounding erythema. The commonest form is caused by beta-haemolytic streptococci, often in association with <i>Staphylococcus aureus</i>. It is associated with poor hygiene and malnutrition. Ecthyma gangrenosum is an uncommon severe variant caused by <i>Pseudomonas aeruginosa</i>.
1B73.0	Streptococcal ecthyma
1B73.1	Staphylococcal ecthyma Ecthyma due to a monoinfection with <i>Staphylococcus aureus</i>. It is less common than streptococcal ecthyma.
1B73.2	Ecthyma gangrenosum Ecthyma gangrenosum is a potentially life-threatening infection of skin in patients who are immunocompromised through disease or immunosuppressive therapy. It is most commonly caused by <i>Pseudomonas aeruginosa</i> though a variety of other organisms may be implicated. It is characterised by usually painless erythematous macules or plaques which progress to haemorrhagic blistering and necrosis of the skin. Lesions may be multiple and widely disseminated, though the anogenital area is a common site. <i>Pseudomonas</i> can frequently be cultured from the blood. Coded Elsewhere: Neonatal ecthyma gangrenosum (EH11)

1B73.Y	Other specified ecthyma
1B73.Z	Ecthyma, unspecified
1B74	Superficial bacterial folliculitis Bacterial infection of the follicular ostium manifested as follicular papules and pustules with perifollicular erythema. The most commonly isolated organisms are coagulase-negative staphylococci and <i>Staphylococcus aureus</i> . The infection may be acute but is more commonly subacute or chronic; individual lesions heal without scarring. Commonly affected sites include the scalp, beard area, thighs and buttocks.
1B74.0	Staphylococcus aureus superficial folliculitis Infection of the follicular ostium with <i>Staphylococcus aureus</i> . There is a predilection for hairy areas including the scalp, beard and thighs.
1B74.Y	Superficial bacterial folliculitis due to other specified organism
1B74.Z	Superficial bacterial folliculitis due to unspecified organism
1B75	Deep bacterial folliculitis or pyogenic abscess of the skin Single or multiple focal infections of skin and soft tissues most commonly centred on the hair follicle and most commonly due to <i>Staphylococcus aureus</i> . Pyogenic abscesses may develop in other locations in skin which has been injured as a result of either trauma or surgery.
1B75.0	Furuncle A localised infection of a hair follicle by <i>Staphylococcus aureus</i> . It manifests as a painful swollen purulent mass centred on a hair follicle.
1B75.1	Carbuncle A deep follicular pyogenic staphylococcal skin infection involving a group of adjacent hair follicles. It manifests as a painful boggy mass containing multiple purulent discharging sinuses.
1B75.2	Furunculosis The presence of multiple furuncles, this condition is associated with disorders such as malnutrition and diabetes mellitus. Treatment-resistant furunculosis may be associated with Panton-Valentine leucocidin-producing <i>Staphylococcus aureus</i> .
1B75.3	Pyogenic abscess of the skin A pus-producing abscess of the skin most commonly due to bacterial infection by <i>Staphylococcus aureus</i> . It is prone to develop where the normal anatomy is disturbed as in pilonidal disease, an epidermoid cyst or around foreign bodies such as surgical sutures. Coded Elsewhere: Sacrococcygeal pilonidal abscess (EG63.2) Infected epidermoid cyst (EK70.00)

1B75.4 Chronic deep bacterial folliculitis

A chronic pyogenic infection by *Staphylococcus aureus* involving the whole depth of the hair follicle. Sycosis occurs mostly in males after puberty, and commonly involves the follicles of the beard. Most cases begin in the third or fourth decade. Unknown host factors appear to be important in the chronicity of the infection. Extensive follicular destruction and scarring may ensue (lupoid sycosis).

Coded Elsewhere: Folliculitis cruris pustulosa atrophicans (ED81.0)

1B7Y Other specified pyogenic bacterial infection of skin and subcutaneous tissue

Certain zoonotic bacterial diseases (1B90-1B9Z)

This is a group of bacterial diseases that are transmitted to humans by contact with infected vertebrate animals.

1B90 Rat-bite fevers

Any disease caused by an infection with the gram-negative bacteria *Streptobacillus moniliformis* or gram-negative bacteria *Spirillum minus*. This disease presents with symptoms depending on the bacterial agent. Transmission is through the bite of an infected rat or rodent.

1B90.0 Spirillosis

A disease caused by an infection with the gram-negative bacteria *Spirillum minus*. This disease is initially characterised by local inflammation, followed by fever, lymphadenitis, and headache. Transmission is commonly by direct contact through the bite or scratch of an infected rat. Confirmation is by identification of *Spirillum* in blood or tissue samples.

Inclusions: Sodoku

1B90.1 Streptobacillosis

A disease caused by an infection with the gram-negative bacteria *Streptobacillus moniliformis*. This disease is characterised by systemic illness with fever, chills, rash, and polyarthralgias. Transmission is commonly by direct contact through the bite or scratch of an infected rat. Confirmation is by identification of *Streptobacillus* in blood or joint samples.

Inclusions: Epidemic arthritic erythema
Haverhill fever
Streptobacillary rat-bite fever

1B91 Leptospirosis

A disease caused by an infection with the gram-negative bacteria *Leptospira*. In the first phase, this disease is characterised by generalised illness (fever, chills, or myalgias) or individuals may be asymptomatic; in the second phase, the heart, liver, kidneys, or brain may be affected by the infection (symptoms are dependent on the site affected). Transmission is by ingestion of contaminated food or water, droplet transmission, or direct cutaneous contact. Confirmation is by identification of *Leptospira* in samples from the affected individual.

1B92	Glanders A disease caused by an infection with the gram-negative bacteria <i>Burkholderia mallei</i>. This disease presents with symptoms depending on the route of infection. Transmission is by contact with tissues or body fluids from infected animals (typically horses), or inhalation of infected aerosol. Confirmation is by identification of <i>Burkholderia mallei</i> in blood, sputum, urine, or skin samples. Inclusions: Infection due to <i>Pseudomonas mallei</i>
1B93	Plague A disease caused by an infection with the gram-negative bacteria <i>Yersinia pestis</i>. This disease presents with symptoms depending on the site of infection, and may be fatal. Transmission is through the bite of an infected flea, by direct contact, or by droplet transmission.
1B93.0	Bubonic plague A disease caused by an infection with the gram-negative bacteria <i>Yersinia pestis</i>. This disease commonly presents with an infection of the lymph nodes leading to swelling and pain. This disease may also present with gangrene of the extremities, chills, malaise, high fever, muscle cramps, or seizures. Transmission is through the bite of an infected flea, by direct contact, or by droplet transmission.
1B93.1	Cellulocutaneous plague Cellulocutaneous plague is a zoonotic disease caused by <i>Yersinia pestis</i> (formerly known as <i>Pasteurella pestis</i>) involving the skin around the flea bite which transmitted the pathogen.
1B93.2	Pneumonic plague Pneumonic plague is a zoonotic disease caused by <i>Yersinia pestis</i> (formerly known as <i>Pasteurella pestis</i>) involving the lung. The lungs are seeded by hematogenous spread or from inhalation of the pathogen.
1B93.3	Plague meningitis Plague meningitis is a zoonotic disease caused by <i>Yersinia pestis</i> (formerly known as <i>Pasteurella pestis</i>) that involves the central nervous system.
1B93.Y	Other specified plague
1B93.Z	Plague, unspecified

1B94**Tularaemia**

A disease caused by an infection with *Francisella tularensis*. This disease is characterised by fever, chills, headache, and weakness, as well as other symptoms depending on the route of infection. Transmission is through the bite of an infected tick or deer fly, by ingestion of contaminated water or food, airborne transmission, or by direct contact with infected animals. Confirmation is by identification of *Francisella tularensis*, or the presence of antibodies to *Francisella tularensis*, in a blood or sputum sample.

Inclusions:

- deer-fly fever
- rabbit fever
- infection due to *Francisella tularensis*

1B94.0**Ulceroglandular tularaemia****1B94.Y****Other specified tularaemia****1B94.Z****Tularaemia, unspecified****1B95****Brucellosis**

A disease caused by an infection with the gram-negative bacteria *Brucella*. This disease is characterised by fever, muscular pain, or sweating. Transmission is by ingestion of unpasteurized milk and soft cheeses made from infected animals. Confirmation is by identification of *Brucella* or antibodies to *Brucella*.

Inclusions:

- Malta fever
- Mediterranean fever
- undulant fever

Coded Elsewhere: Spondylitis in brucellosis (FA13)

1B96**Erysipeloid**

A disease caused by an infection with the gram-positive bacteria *Erysipelothrix rhusiopathiae*. This disease is characterised by localised cellulitis. Transmission is by direct cutaneous contact with *Erysipelothrix rhusiopathiae*, often in individuals handling seafood and raw meat.

1B97**Anthrax**

A disease caused by an infection with the gram-positive bacteria *Bacillus anthracis*. This disease presents with clinical signs depending on the route of infection. Transmission is by inhalation, ingestion, or cutaneous contact with *Bacillus anthracis* spores. Confirmation is by identification of *Bacillus anthracis* in a sample, or detection of antibodies or toxins.

1B98**Cat-scratch disease**

A disease commonly caused by an infection with the gram-negative bacteria *Bartonella henselae*. This disease is characterised by regional lymphadenopathy, or fever. Transmission is commonly from the scratch or bite of a cat infested with fleas infected with *Bartonella henselae*.

Inclusions:

- Cat-scratch fever
- Rochalimaea henselae* infection

1B99**Pasteurellosis**

A disease caused by an infection with the gram-negative bacteria *Pasteurella*. This disease is characterised by local cellulitis and may lead to other clinical signs depending on the route of infection. Transmission is commonly by direct contact through the bite, scratch, or lick from an infected animal, inhalation of infected respiratory secretions, or ingestion of contaminated meat. Confirmation is by identification of *Pasteurella* from the affected individual.

1B9A**Extraintestinal yersiniosis**

A disease caused by an infection with the gram-negative bacteria *Yersinia enterocolitica*, excluding infections in the intestinal tract. This disease presents with symptoms depending on the site of infection, and may lead to a systemic infection. Transmission is by the faecal-oral route from the ingestion of contaminated food or water, or direct contact with infected animals or humans. Confirmation is by identification of *Yersinia enterocolitica* from affected tissues.

Exclusions: Enteritis due to *Yersinia enterocolitica* (1A05)
Plague (1B93)

1B9Z**Unspecified zoonotic bacterial disease****Other bacterial diseases (1C10-1C4Z)**

Coded Elsewhere: Other bacterial infections of the fetus or newborn (KA61)

Bacterial duodenitis (DA51.6Y)

1C10**Actinomycosis**

A disease commonly caused by an infection with the gram-positive bacteria *Actinomyces*. This disease is characterised by painful abscesses in the mouth, lungs, and gastrointestinal tract. Transmission is by endogenous infection. Confirmation is by identification of *Actinomyces* in infected tissue or fluid samples.

Exclusions: Actinomycetoma (1C43)

Coded Elsewhere: Myelitis associated with actinomyces (1D02.0)

1C10.0**Pulmonary actinomycosis**

This is a pulmonary infectious bacterial disease caused by *Actinomyces* species such as *Actinomyces israelii* or *A. gerencseriae*. It can also be caused by *Propionibacterium propionicus*, and the condition is likely to be polymicrobial aerobic anaerobic infection.

1C10.1**Abdominal actinomycosis**

This is a cervicofacial infectious bacterial disease caused by *Actinomyces* species such as *Actinomyces israelii* or *A. gerencseriae*. It can also be caused by *Propionibacterium propionicus*, and the condition is likely to be polymicrobial aerobic anaerobic infection.

1C10.2	Cervicofacial actinomycosis Cervicofacial actinomycosis is the commonest clinical form of actinomycosis, a sporadically occurring endogenous polymicrobial inflammatory process in which fermentative actinomycetes of the genera <i>Actinomyces</i> (especially <i>A. israelii</i> and <i>A. gerencseriae</i>), <i>Propionibacterium</i> and <i>Bifidobacterium</i> act as the principal pathogens. The typical presentation is a slowly progressive development from painless tissue infiltration and induration of soft tissues of the face and neck to multiple abscesses and draining sinus tracts discharging pus. Actinomycosis is a major factor and indicator of poor prognosis in infected osteoradionecrosis of the jaws following head and neck cancer therapy.
1C10.3	Primary cutaneous actinomycosis
1C10.Y	Other specified forms of actinomycosis
1C10.Z	Actinomycosis, unspecified
1C11	Bartonellosis Any infection caused by the gram-negative bacteria <i>Bartonella</i> . Coded Elsewhere: Cat-scratch disease (1B98)
1C11.0	Carrion disease Infection by <i>Bartonella bacilliformis</i> which can present as a systemic illness, Oroya fever, or as a benign skin eruption, verruga peruana.
1C11.00	Oroya fever A disease commonly caused by an infection with the gram-negative bacteria <i>Bartonella bacilliformis</i> . This disease is characterised by severe haemolytic anaemia and transient immunosuppression. This disease may present with fever, malaise, or jaundice. Transmission is through the bite of infected sandflies from the genus <i>Lutzomyia</i> . Confirmation is by identification of <i>Bartonella bacilliformis</i> in a blood sample.
1C11.01	Verruga peruana A disease caused by an infection with the gram-negative bacteria <i>Bartonella bacilliformis</i> . This disease is characterised by multiple nodular and red-to-purple vascular skin lesions, subsequent to Oroya fever. Transmission is through the bite of infected sandflies from the genus <i>Lutzomyia</i> .
1C11.1	Trench fever A disease caused by an infection with the gram-negative bacteria <i>Bartonella quintana</i> . This disease is characterised by fever, headache, rash, bone pain, or may be asymptomatic. Transmission is through the bite of infected body lice. Confirmation is by identification of <i>Bartonella quintana</i> in a blood sample. <i>Bartonella quintana</i> was formerly known as <i>Rickettsia quintana</i> . Inclusions: Quintan fever
1C11.Y	Other forms of bartonellosis

1C12**Whooping cough**

A disease of the upper respiratory tract, caused by an infection with the gram-negative bacteria *Bordetella*. This disease typically presents with paroxysmal cough, inspiratory whoop, and fainting or vomiting after coughing. Transmission is by inhalation of infected respiratory secretions. Confirmation is by identification of *Bordetella* from nasopharyngeal samples or sputum, or detection of antibodies against *Bordetella*.

1C12.0**Whooping cough due to *Bordetella pertussis***

A disease of the upper respiratory tract, caused by an infection of the gram-negative bacteria *Bordetella pertussis*. This disease typically presents with paroxysmal cough, inspiratory whoop, and fainting or vomiting after coughing. Transmission is by inhalation of infected respiratory secretions. Confirmation is by identification of *Bordetella pertussis* from nasopharyngeal samples or sputum, or detection of antibodies against *Bordetella pertussis*.

1C12.1**Whooping cough due to *Bordetella parapertussis***

A disease of the upper respiratory tract, caused by an infection of the gram-negative bacteria *Bordetella parapertussis*. This disease typically presents with a mild clinical presentation of paroxysmal cough, inspiratory whoop, and fainting or vomiting after coughing. Transmission is by inhalation of infected respiratory secretions. Confirmation is by identification of *Bordetella parapertussis* from nasopharyngeal samples or sputum, or detection of antibodies against *Bordetella parapertussis*.

1C12.Y**Other specified whooping cough****1C12.Z****Whooping cough, unspecified****1C13****Tetanus**

A disease of the skeletal muscle fibres, caused by an infection with the gram-positive bacteria *Clostridium tetani*. This disease is characterised by muscle spasms. Transmission is by direct contact of an open wound.

Exclusions: Obstetrical tetanus (1C14)
 Tetanus neonatorum (1C15)

1C14**Obstetrical tetanus**

A disease caused by an infection with the gram-positive bacteria *Clostridium tetani*. This disease is characterised by a prolonged contraction of skeletal muscle fibres during pregnancy or within six weeks of termination of pregnancy. Transmission is by direct contact.

1C15**Tetanus neonatorum**

A disease affecting neonates, caused by an infection with the gram-positive bacteria *Clostridium tetani*. This disease is characterised by systemic muscle spasms that arise within the first few days after delivery. Transmission is commonly by direct contact or lack of maternal immunity.

1C16**Gas gangrene**

Gas gangrene or clostridial myonecrosis is a potentially fatal, rapidly progressive necrotizing infection of muscle and soft tissue resulting from bacterial invasion of healthy muscle from adjacent traumatized muscle or soft tissue. The infection originates in a wound contaminated with bacteria of the genus *Clostridium*. *C. perfringens* accounts for the majority of cases (over eighty percent), while *C. novyi*, *C. septicum*, and *C. histolyticum* cause most of the other cases.

1C17**Diphtheria**

A disease commonly of the respiratory system, caused by an infection of the gram-positive bacteria *Corynebacterium diphtheriae*. This disease is characterised by sore throat, fever, and a pseudomembrane on the tonsils, pharynx, or nasal cavity. Transmission is by inhalation of infected respiratory secretions, or direct cutaneous contact. Confirmation is by identification of *Corynebacterium diphtheriae* from a throat swab or infected tissue, and by clinical signs.

1C17.0**Pharyngeal or tonsillar diphtheria**

Inclusions: Diphtheritic membranous angina

1C17.00

Postdiphtheritic paralysis of uvula

1C17.0Y

Other specified pharyngeal or tonsillar diphtheria

1C17.0Z

Pharyngeal or tonsillar diphtheria, unspecified

1C17.1**Nasal diphtheria****1C17.2****Laryngeal diphtheria**

localised infection of mucous membranes of the larynx caused by toxigenic strains of *Corynebacterium diphtheriae*; it is characterised by the presence of a pseudomembrane at the site of infection; diphtheria toxin, produced by *C. diphtheriae*, can cause myocarditis, polyneuritis, and other systemic toxic effects.

Inclusions: Diphtheritic laryngotracheitis

1C17.3**Cutaneous diphtheria**

Exclusions: Erythrasma (1C44)

1C17.Y**Other specified diphtheria****1C17.Z****Diphtheria, unspecified****1C18****Brazilian purpuric fever**

A disease affecting children, caused by an infection with the gram-negative bacteria *Haemophilus aegyptius*. This disease is characterised by fever, nausea, vomiting, purpuric lesions, and sepsis, that is preceded by conjunctivitis. Transmission may be by mechanical transmission from infected eye gnats, contact with discharge from infected individuals, or fomites used near the eyes. Confirmation is by identification of *Haemophilus influenzae* from blood.

Inclusions: Systemic *Haemophilus aegyptius* infection

1C19**Legionellosis**

Legionellosis varies in severity from a mild febrile illness to a serious and sometimes fatal form of pneumonia and is caused by exposure to *Legionella* species found in water, and potting mixes.

Legionellosis is a generic term describing the pneumonic and non-pneumonic forms of infection with *Legionella*.

1C19.0**Nonpneumonic Legionnaires' disease**

The non-pneumonic form (Pontiac disease) is an acute, self-limiting influenza-like illness usually lasting 2–5 days. The incubation period is from a few and up to 48 hours. The main symptoms are fever, chills, headache, malaise and muscle pain (myalgia).

1C19.1**Legionnaires' disease**

Legionnaires' disease, the pneumonic form, has an incubation period of 2 to 10 days (but up to 16 days has been recorded in some outbreaks). Initially, symptoms are fever, loss of appetite, headache, malaise and lethargy. Some patients may also have muscle pain, diarrhoea and confusion. There is also usually an initial mild cough, but as many as 50% of patients can present phlegm. Blood-streaked phlegm or hemoptysis occurs in about one-third of the patients. The severity of disease ranges from a mild cough to a rapidly fatal pneumonia. Death occurs through progressive pneumonia with respiratory failure and/or shock and multi-organ failure.

1C1A**Listeriosis**

A disease caused by an infection with the gram-positive bacteria *Listeria*. This disease commonly presents with fever and muscle aches, followed by gastrointestinal symptoms.

Inclusions: listerial foodborne infection

Coded Elsewhere: Neonatal listeriosis (KA61.1)

1C1A.0**Cutaneous listeriosis**

This is a bacterial infection caused by a Gram-positive, motile bacterium, *Listeria monocytogenes*. Listeriosis occurs primarily in newborn infants, elderly patients, and patients who are immunocompromised.

1C1A.1**Listerial meningitis or meningoencephalitis**

A disease of the meninges or brain, caused by an infection with the gram-positive bacteria *Listeria*. This disease is characterised by fever, headache, or neurological deficits. Transmission is through haematogenous spread to the meninges from ingestion of contaminated food. Confirmation is by identification of *Listeria* from cerebrospinal fluid.

1C1A.Y**Other specified listeriosis****1C1A.Z****Listeriosis, unspecified**

1C1B**Nocardiosis**

A disease caused by an infection with the gram-positive bacteria *Nocardia*. This disease presents with symptoms depending on the site of infection (commonly lung, brain, or skin). Transmission is by inhalation of *Nocardia* from soil or water, or by direct cutaneous contact. Confirmation is by identification of *Nocardia* in samples from affected sites.

1C1B.0**Pulmonary nocardiosis**

A disease of the respiratory system, caused by an infection with the gram-positive bacteria *Nocardia*. This disease is characterised by chest pain, haemoptysis, fever, weight loss, and cough. Transmission is by inhalation of *Nocardia* from soil or water. Confirmation is by identification of *Nocardia* in sputum samples, or lung biopsy.

1C1B.1**Cutaneous nocardiosis**

Cutaneous nocardiosis may be due to direct infection of the skin where it presents either as a solitary cold abscess or as a lymphangitic process in which infection spreads up lymphatic channels to form a linear array of suppurative nodules. Skin involvement is also present in a third of cases of systemic nocardiosis.

Exclusions: Actinomycetoma due to *Nocardia* species (1C43)

1C1B.Y**Other specified forms of nocardiosis****1C1B.Z****Nocardiosis, unspecified****1C1C****Meningococcal disease**

This illness is severe and includes infections of the lining of the brain and spinal cord (meningitis) and generalised bloodstream infections (bacteraemia or septicaemia).

Meningococcus bacteria are spread through the exchange of respiratory and throat secretions like spit (e.g., by living in close quarters, kissing). Meningococcal disease can be treated with antibiotics, but quick medical attention is extremely important. Keeping up to date with recommended vaccines is the best defence against meningococcal disease.

Inclusions: Meningococcal infection

Coded Elsewhere: Meningococcal encephalitis (1D00.0)

1C1C.0**Meningococcal meningitis**

A condition of the meninges, caused by an infection with the gram-negative bacteria *Neisseria meningitidis*. This condition is characterised by high fever, stiff neck, severe headache, vomiting, purpura, photophobia, and sometimes chills, altered mental status, or seizures. Transmission is through haematogenous spread to the meninges after droplet transmission or direct contact. Confirmation is by identification of *Neisseria meningitidis* through a lumbar puncture, agglutination tests, or polymerase chain reaction.

Inclusions: Meningitis due to *Neisseria meningitidis*

1C1C.1	Waterhouse-Friderichsen syndrome A syndrome of the adrenal glands, commonly caused by an infection with meningococcus <i>Neisseria meningitidis</i> . This syndrome is characterised by bleeding into the adrenal glands due to the severe bacterial infection. This syndrome may present with fever, chills, vomiting, myalgia, or rash, with progression to disseminated intravascular coagulation. Transmission is by direct contact or droplet transmission. <i>Inclusions:</i> Meningococcal haemorrhagic adrenalitis
1C1C.2	Meningococcaemia A condition caused by an infection with the gram-negative bacteria <i>Neisseria meningitidis</i> that leads to a severe systemic inflammatory response. This condition is characterised by fever, rash, and myalgia. Transmission is by direct contact or droplet transmission. Confirmation is by identification of <i>Neisseria meningitidis</i> in blood samples.
1C1C.20	Acute meningococcaemia A condition caused by an infection with the gram-negative bacteria <i>Neisseria meningitidis</i> that leads to a severe systemic inflammatory response. This condition is characterised by fever, chills, myalgia, nausea, or petechial rash, with progression to shock and disseminated intravascular coagulation. Transmission is by direct contact or droplet transmission. Confirmation is by identification of <i>Neisseria meningitidis</i> in blood samples.
1C1C.2Y	Other specified meningococcaemia
1C1C.2Z	Meningococcaemia, unspecified
1C1C.Y	Other specified meningococcal disease
1C1C.Z	Meningococcal disease, unspecified
1C1D	Yaws An infectious disease caused by <i>Treponema pallidum</i> subsp <i>pertenue</i> which mainly affects children in rural communities in the humid tropics. It affects the skin and bones, is spread by skin to skin contact, and is not sexually transmitted, but cannot be distinguished serologically from syphilis.
1C1D.0	Primary yaws Primary yaws results from primary inoculation of <i>Treponema pallidum</i> subsp. <i>pertenue</i> into the skin, manifesting 2-12 weeks later as a localised papule (initial, primary or 'mother' yaw) before developing into a large non-tender ulcerating nodule, often resembling a raspberry (hence the name 'framboesia'). The primary lesion is most commonly located on the legs and ankles may also be found on the buttocks, arms, hands, and face. It usually heals after 3–6 months and is still present at the onset of the secondary stage in only a minority (9-15%). <i>Inclusions:</i> Chancre of yaws Primary framboesia

1C1D.1	Secondary yaws Secondary yaws results from lymphatic and haematogenous spread of <i>Treponema pallidum</i> subsp. <i>pertenue</i> spirochaetes from the initial inoculation site and appears from a few weeks to 2 years after the primary infection. The commonest initial symptoms are non-specific and include arthralgia and malaise. Secondary skin lesions consist of multiple papules and nodules similar to the initial lesion but smaller. They may be localised, regional or generalised; they may ulcerate and on moist areas may mimic syphilitic condylomata lata. Hyperkeratotic plaques on the palms and soles may develop painful fissures and secondary infection, resulting in a characteristic 'crab-like' gait.
1C1D.2	Tertiary yaws Tertiary yaws develops in <10% of untreated infected individuals after an interval of 5 years or more. The late stage skin lesions are characterised by gummatous nodules with necrotic tissue destruction, followed by debilitating scarring and contracture. Destructive osteitis can result in ulceration of the palate and nasopharynx ('gangosa'), or bowing of the tibia (sabre shins). Hypertrophic periostitis at periarticular sites can lead to exostosis of the paranasal maxillae ('goundou').
1C1D.3	Latent yaws Latent yaws is defined as yaws with no clinical signs and only serological evidence of infection (reactive treponemal and non-treponemal tests). Infectious relapses may occur in latent cases for up to 5 and, rarely, 10 years. The total duration of infectivity for an untreated yaws patient, including relapses, is thought to be about 12-18 months. Inclusions: Yaws without clinical manifestations, with positive serology
1C1E	Pinta A disease of the skin, caused by an infection with the gram-negative bacteria <i>Treponema pallidum</i> carateum. This disease is characterised by hyperkeratosis and hyperpigmentation. Transmission may be by direct contact.
1C1E.0	Primary lesions of pinta The primary stage of pinta is characterised by a sparse eruption of cutaneous papules and erythematous scaly plaques. This stage may last for months to years. Inclusions: Primary chancre of pinta
1C1E.1	Intermediate lesions of pinta The intermediate stage of pinta develops months to years after the primary stage and is characterised by more extensive lesions (known as pintids) which gradually change from pink to blue, black or grey and become atrophic. Inclusions: Pintids

1C1E.2 Late lesions of pinta

Late lesions of pinta are confined to the skin and are characterised by dyschromia and atrophy. They typically take between two and four years to develop following initial infection. The skin appears mottled and atrophic with numerous irregular and variegated hypermelanotic, hypomelanotic and amelanotic patches typically involving the wrists, palms, ankles, and elbows, as well as the skin around and within old lesions.

Inclusions: Hypomelanosis due to late pinta

1C1E.3 Mixed lesions of pinta**1C1F Endemic non-venereal syphilis**

Endemic non-venereal syphilis is caused by *Treponema pallidum* subspecies endemicum and is transmitted by skin-to-skin or mouth-to-mouth contact rather than sexual contact. Children are at greatest risk of infection. Clinical features are similar to venereal syphilis with a primary ulcer (usually in the mouth or on the nipples of breast-feeding women nursing infected children) and, in the secondary stage, a generalised papular rash, oral mucous patches, condylomata lata and generalised lymphadenopathy. Late stage infection is characterised by destructive gummata of the nasopharynx (gangosa), bones and skin.

Inclusions: Bejel
Endemic syphilis
Njovera

1C1G Lyme borreliosis

A tick-borne infection by the spirochaete *Borrelia burgdorferi*, Lyme borreliosis typically presents with a characteristic rash, erythema chronicum migrans, at an average of seven days after a bite from an infected tick. The rash may be accompanied by flu-like symptoms. Disseminated infection may cause meningitis, cranial neuropathies and carditis amongst other manifestations. Late disease, months to years after initial infection, may present with a pauciarticular arthritis or with encephalomyelitis.

Coding Note: Use additional code if desired, to identify any associated condition.

Use additional code, if desired, to identify any sequelae. The extension code 'Cause of late effect' is used in addition to both codes to show the relationship between the causative condition and the resulting sequelae.

1C1G.0 Early cutaneous Lyme borreliosis

Borrelia burgdorferi infection involving the skin, typically as erythema migrans, the commonest presentation of Lyme disease.

Coding Note: Use additional code if desired, to identify any associated condition.

1C1G.1 Disseminated Lyme borreliosis

Coding Note: Use additional code if desired, to identify any associated condition.

1C1G.10 Lyme neuroborreliosis

1C1G.11 Lyme carditis

1C1G.12 Ophthalmic Lyme borreliosis

1C1G.13	Lyme arthritis
1C1G.14	Late cutaneous Lyme borreliosis
1C1G.1Y	Other specified disseminated Lyme borreliosis
Coding Note:	Use additional code if desired, to identify any associated condition.
1C1G.1Z	Disseminated Lyme borreliosis, unspecified
Coding Note:	Use additional code if desired, to identify any associated condition.
1C1G.Y	Other specified Lyme borreliosis
Coding Note:	Use additional code if desired, to identify any associated condition. Use additional code, if desired, to identify any sequelae. The extension code 'Cause of late effect' is used in addition to both codes to show the relationship between the causative condition and the resulting sequelae.
1C1H	Necrotising ulcerative gingivitis Necrotising ulcerative gingivitis (NUG) is a condition affecting the gums that is caused by a bacterial infection. It is a form of periodontal (gum) disease. But unlike other forms, it typically develops quickly and causes moderate to severe pain. "Necrotising" means that the condition destroys tissue. "Ulcerative" refers to sores that can appear on the gums. Inclusions: Fusospirochaetal gangrene
1C1H.0	Other Vincent infections
1C1H.Y	Other specified necrotising ulcerative gingivitis
1C1H.Z	Necrotising ulcerative gingivitis, unspecified
1C1J	Relapsing fever
1C1J.0	Tick-borne relapsing fever A disease caused by an infection with the bacteria <i>Borrelia</i> . This disease is characterised by repeated episodes of fever, with the febrile episode lasting for approximately 3 days, followed by the afebrile state of approximately 7 days. Transmission is through the bite of an infected soft tick (from the genus <i>Ornithodoros</i>). Confirmation is by identification of spirochete bacteria from a blood smear, bone marrow, or cerebrospinal fluid. Inclusions: Relapsing fever due to any <i>Borrelia</i> species other than <i>Borrelia recurrentis</i>
1C1J.1	Louse-borne relapsing fever A spirochaetal infection caused by the human to human transmission of <i>Borrelia recurrentis</i> by the human body louse. Epidemics are associated with poor living conditions as may result from famine or war. Episodic fever may progress to severe jaundice, haemorrhage, confusion and death. Confirmation is by identification of <i>Borrelia</i> in blood films. Inclusions: Relapsing fever due to <i>Borrelia recurrentis</i>

Other diseases due to chlamydiae (1C20-1C2Z)

1C20

Chlamydial conjunctivitis

Chlamydial conjunctivitis is a sexually transmitted infection of conjunctiva caused by bacteria *Chlamydia trachomatis*. Bacteria can be passed from an infected mother to baby during vaginal childbirth. The symptoms include that one or both eyes will be red with a sticky discharge and swollen eyelids.

Inclusions: Paratrachoma

1C21

Chlamydial peritonitis

This is a sexually transmitted infection that causes an inflammation of the peritoneum, the thin tissue that lines the inner wall of the abdomen and covers most of the abdominal organs.

1C22

Infections due to *Chlamydia psittaci*

Any condition caused by an infection with the gram-negative bacteria *Chlamydia psittaci*. These conditions are characterised by variable clinical presentations such as fever, cough, headaches, chills, fatigue, nausea, vomiting, diarrhoea, or pneumonia. Transmission is commonly by inhalation of aerosol contaminated with body fluids from infected birds, or direct contact with infected birds. Confirmation is by identification of *Chlamydia psittaci*.

Inclusions: Psittacosis
Ornithosis
Parrot fever

Coded Elsewhere: Pneumonia in chlamydia psittaci infection (CA40.0Y)

1C23

Trachoma

A disease caused by an infection with the gram-negative bacteria *Chlamydia trachomatis*. This disease is characterised by a roughening of the inner surfaces of the eyes, and inflammation that may lead to superficial vascularization of the cornea (pannus) and scarring of the conjunctiva. Long term effects include blindness or other visual impairments. Transmission is by direct or indirect contact with the eyes or nose of an infected individual.

1C23.0

Initial stage of trachoma

This refers to the initial stage of an infectious disease caused by the *Chlamydia trachomatis* bacterium which produces a characteristic roughening of the inner surface of the eyelids.

Inclusions: Trachoma dubium

1C23.1

Active stage of trachoma

This refers to the active stage of an infectious disease caused by the *Chlamydia trachomatis* bacterium which produces a characteristic roughening of the inner surface of the eyelids.

1C23.Y

Other specified trachoma

1C23.Z

Trachoma, unspecified

1C2Y

Other specified diseases due to chlamydiae

1C2Z**Diseases due to chlamydiae, unspecified****Rickettsioses (1C30-1C3Z)**

Any disease caused by an infection with the gram-negative bacteria *Rickettsia*. These diseases commonly present with fever, malaise, and rash. Transmission is commonly through the bite of an infected flea, louse, mite, or tick.

Coded Elsewhere: Trench fever (1C11.1)

1C30**Typhus fever**

A disease caused by an infection with the gram-negative bacteria *Rickettsia*. This disease is characterised by fever, delirium, back pain, or arthralgia. Transmission is commonly through the bite of an infected flea, louse, mite, or tick.

Exclusions: Rickettsiosis due to *Ehrlichia sennetsu* (1C30-1C3Z)

1C30.0**Epidemic louse-borne typhus fever due to *Rickettsia prowazekii***

This is a form of typhus so named because the disease often causes epidemics following wars and natural disasters. The causative organism is *Rickettsia prowazekii*, transmitted by the human body louse (*Pediculus humanus corporis*). This diagnosis is due to is a species of gram negative, Alpha Proteobacteria, obligate intracellular parasitic, aerobic bacteria that is the aetiologic agent of epidemic typhus, transmitted in the faeces of lice.

1C30.1**Recrudescent typhus**

This is a form of typhus so named because the disease often causes epidemics following wars and natural disasters. The causative organism is *Rickettsia prowazekii*, transmitted by the human body louse (*Pediculus humanus corporis*).

Inclusions: Brill-Zinsser disease

1C30.2**Typhus fever due to *Rickettsia typhi*****1C30.3****Typhus fever due to *Orientia tsutsugamushi***

Inclusions: Tsutsugamushi fever

1C30.Y**Other specified typhus fever****1C30.Z****Typhus fever, unspecified****1C31****Spotted fever**

A disease caused by an infection with the gram-negative bacteria *Rickettsia*. This disease is characterised by fever, eschar, or rash. Transmission is commonly through the bite of an infected tick.

1C31.0**Spotted fever due to *Rickettsia rickettsii***

Inclusions: Rocky Mountain spotted fever
Sao Paulo fever

1C31.1 **Spotted fever due to *Rickettsia conorii***
Inclusions: Boutonneuse fever
 Mediterranean tick fever
 African tick typhus
 Kenya tick typhus

1C31.2 **Spotted fever due to *Rickettsia sibirica***
Inclusions: North Asian tick fever
 Siberian tick typhus

1C31.3 **Spotted fever due to *Rickettsia australis***
Inclusions: Queensland tick typhus

1C31.Y **Other specified spotted fever**

1C31.Z **Spotted fever, unspecified**

1C32 **Rickettsialpox**
 An acute febrile disease caused by *Rickettsia akari*, which is transmitted from its rodent host by the house-mouse mite *Liponyssoides sanguineus*. An initial skin lesion at the site of a mite bite, often associated with lymphadenopathy, is followed by fever; a disseminated skin rash appears, which generally does not involve the palms and the soles, and lasts only a few days. Death is uncommon.
Inclusions: Kew Gardens spotted fever

1C33 **Q fever**
 A disease caused by an infection with the gram-negative bacteria *Coxiella burnetii*. This disease is characterised by fever, or may be asymptomatic. Transmission is by inhalation of the bacteria, contact with contaminated milk, urine, faeces, vaginal mucus, or semen of infected animals, or through the bite of an infected tick.
Inclusions: Nine Mile fever
 Infection due to *Coxiella burnetii*
 Quadrilateral fever

1C3Y **Other specified rickettsioses**

1C3Z **Rickettsioses, unspecified**

1C40 **Campylobacteriosis**
 Campylobacteriosis is caused by *Campylobacter* bacteria (curved or spiral, motile, non-spore-forming, Gram-negative rods). The disease is usually caused by *C. jejuni*, a spiral and comma shaped bacterium normally found in cattle, swine, and birds, where it is nonpathogenic, but the illness can also be caused by *C. coli* (also found in cattle, swine, and birds), *C. upsaliensis* (found in cats and dogs) and *C. lari* (present in seabirds in particular).

1C41	Bacterial infection of unspecified site <i>Exclusions:</i> meningococcal infection NOS (1C1C) chlamydial infection NOS (1C20-1C2Z) rickettsial infection NOS (1C30-1C3Z) spirochaetal infection NOS (1C10-1C4Z) Infection arising from device, implant or graft, not elsewhere classified (NE83.1) <i>Coded Elsewhere:</i> Acute meningococcaemia (1C1C.20) Disseminated gonococcal infection (1A73)
1C42	Melioidosis A disease caused by the saprophytic environmental gram-negative bacterium <i>Burkholderia pseudomallei</i> which is found in soil or water in humid tropical regions of the world, especially South-East Asia and northern Australia. It has protean manifestations ranging from fulminant septicaemia with fatal outcome to chronic low grade infection.
1C43	Actinomycetoma Actinomycetoma is a chronic progressive subcutaneous infection caused by implantation of aerobic branching actinomycetes through a skin wound. These organisms are filamentous bacteria which live as saprophytes in soil or on plants; the commonest infecting agents are <i>Nocardia brasiliensis</i> , <i>Actinomadura madurae</i> and <i>Streptomyces somaliensis</i> . The earliest stage of infection is a firm painless nodule but with time the whole area becomes hard and swollen with multiple papules, pustules and draining sinuses on the skin surface. Extension to underlying bones and joints can result in gross deformity. <i>Inclusions:</i> Mycetoma due to filamentous bacteria <i>Exclusions:</i> Eumycetoma (1F29)
1C44	Non-pyogenic bacterial infections of the skin Skin infection by bacteria which do not characteristically induce pus formation.
1C45	Toxic shock syndrome <i>Exclusions:</i> endotoxic shock NOS (1G41)
1C45.0	Streptococcal toxic shock syndrome
1C45.1	Staphylococcal toxic shock syndrome
1C45.Y	Toxic shock syndrome due to other specified infectious agent
1C45.Z	Toxic shock syndrome without specified infectious agent
1C4Y	Other specified bacterial diseases
1C4Z	Unspecified bacterial disease

Human immunodeficiency virus disease (1C60-1C62.Z)

A case of HIV infection is defined as an individual with HIV infection irrespective of clinical stage including severe or stage 4 clinical disease, (also known as AIDS) confirmed by laboratory criteria according to country definitions and requirements.

Coded Elsewhere: Congenital human immunodeficiency virus infection (KA62.6)

1C60 Human immunodeficiency virus disease associated with tuberculosis

Coded Elsewhere: Human immunodeficiency disease complicating pregnancy, childbirth or the puerperium (JB63.7)

1C60.0 HIV disease clinical stage 1 associated with tuberculosis

Coded Elsewhere: Human immunodeficiency virus disease associated with generalised lymphadenopathy (1C62.0)

Acute human immunodeficiency virus infection syndrome (1C62.0)

1C60.1 HIV disease clinical stage 2 associated with tuberculosis

Coded Elsewhere: HIV-associated immune reconstitution inflammatory syndrome (4B23)

Herpes resulting from human immunodeficiency virus disease (1C62.1)

Human immunodeficiency virus disease associated with mycosis classified elsewhere (1C62.1)

1C60.2 HIV disease clinical stage 3 associated with tuberculosis

Coded Elsewhere: Human immunodeficiency virus disease associated with haematological or immunological abnormalities (1C62.2)

Human immunodeficiency virus disease associated with lymphoid interstitial pneumonitis (1C62.2)

Human immunodeficiency virus disease enteritis (1C62.2)

Gastritis due to human immunodeficiency virus disease (1C62.2)

Myelitis due to Human immunodeficiency virus (1C62.2)

Meningitis due to human immunodeficiency virus (1C62.2)

HIV disease resulting in candidosis classified elsewhere (1C62.2)

HIV disease resulting in cytomegaloviral disease (1C62.2)

1C60.3	HIV disease clinical stage 4 associated with tuberculosis <i>Coded Elsewhere:</i> Dementia due to human immunodeficiency virus (6D85.3) HIV retinitis (9B72.01) HIV - [human immunodeficiency virus] disease associated with Burkitt lymphoma (1C62.3) Human immunodeficiency virus disease associated with other types of non-Hodgkin lymphoma (1C62.3) Human immunodeficiency virus disease associated with other malignant neoplasms of lymphoid, haematopoietic or related tissue (1C62.3) Human immunodeficiency virus disease associated with multiple malignant neoplasms (1C62.3) Human immunodeficiency virus disease associated with encephalopathy (1C62.3) Human immunodeficiency virus disease associated with wasting syndrome (1C62.3) Oesophagitis associated with human immunodeficiency virus disease (DA24.Y) HIV or AIDS vacuolar myelopathy (8A45.0Y)
1C60.30	Kaposi sarcoma associated with human immunodeficiency virus disease associated with tuberculosis
1C60.3Y	Other specified HIV disease clinical stage 4 associated with tuberculosis
1C60.3Z	HIV disease clinical stage 4 associated with tuberculosis, unspecified
1C60.Z	Human immunodeficiency virus disease associated with tuberculosis, clinical stage unspecified
1C61	Human immunodeficiency virus disease associated with malaria <i>Coded Elsewhere:</i> Human immunodeficiency disease complicating pregnancy, childbirth or the puerperium (JB63.7)
1C61.0	HIV disease clinical stage 1 associated with malaria <i>Coded Elsewhere:</i> Human immunodeficiency virus disease associated with generalised lymphadenopathy (1C62.0) Acute human immunodeficiency virus infection syndrome (1C62.0)
1C61.1	HIV disease clinical stage 2 associated with malaria <i>Coded Elsewhere:</i> HIV-associated immune reconstitution inflammatory syndrome (4B23) Herpes resulting from human immunodeficiency virus disease (1C62.1) Human immunodeficiency virus disease associated with mycosis classified elsewhere (1C62.1)

1C61.2	HIV disease clinical stage 3 associated with malaria <i>Coded Elsewhere:</i> Human immunodeficiency virus disease associated with haematological or immunological abnormalities (1C62.2) Human immunodeficiency virus disease associated with lymphoid interstitial pneumonitis (1C62.2) Human immunodeficiency virus disease enteritis (1C62.2) Gastritis due to human immunodeficiency virus disease (1C62.2) Myelitis due to Human immunodeficiency virus (1C62.2) Meningitis due to human immunodeficiency virus (1C62.2) HIV disease resulting in candidosis classified elsewhere (1C62.2) HIV disease resulting in cytomegaloviral disease (1C62.2)
1C61.3	HIV disease clinical stage 4 associated with malaria <i>Coded Elsewhere:</i> Dementia due to human immunodeficiency virus (6D85.3) HIV retinitis (9B72.01) HIV - [human immunodeficiency virus] disease associated with Burkitt lymphoma (1C62.3) Human immunodeficiency virus disease associated with other types of non-Hodgkin lymphoma (1C62.3) Human immunodeficiency virus disease associated with other malignant neoplasms of lymphoid, haematopoietic or related tissue (1C62.3) Human immunodeficiency virus disease associated with multiple malignant neoplasms (1C62.3) Human immunodeficiency virus disease associated with encephalopathy (1C62.3) Human immunodeficiency virus disease associated with wasting syndrome (1C62.3) Oesophagitis associated with human immunodeficiency virus disease (DA24.Y) HIV or AIDS vacuolar myelopathy (8A45.0Y)
1C61.30	Kaposi sarcoma associated with human immunodeficiency virus disease associated with malaria
1C61.3Y	Other specified HIV disease clinical stage 4 associated with malaria
1C61.3Z	HIV disease clinical stage 4 associated with malaria, unspecified
1C61.Z	Human immunodeficiency virus disease associated with malaria, clinical stage unspecified
1C62	Human immunodeficiency virus disease without mention of tuberculosis or malaria <i>Coded Elsewhere:</i> Human immunodeficiency disease complicating pregnancy, childbirth or the puerperium (JB63.7)

1C62.0	HIV disease clinical stage 1 without mention of tuberculosis or malaria
1C62.1	HIV disease clinical stage 2 without mention of tuberculosis or malaria <i>Coded Elsewhere:</i> HIV-associated immune reconstitution inflammatory syndrome (4B23)
1C62.2	HIV disease clinical stage 3 without mention of tuberculosis or malaria
1C62.3	HIV disease clinical stage 4 without mention of tuberculosis or malaria <i>Coded Elsewhere:</i> Dementia due to human immunodeficiency virus (6D85.3) HIV retinitis (9B72.01) Oesophagitis associated with human immunodeficiency virus disease (DA24.Y) HIV or AIDS vacuolar myelopathy (8A45.0Y)
1C62.Z	Human immunodeficiency virus disease without mention of associated disease or condition, clinical stage unspecified

Viral infections of the central nervous system (1C80-1C8Z)

Any disease of the central nervous system, caused by an infection with a viral source.

Coded Elsewhere: Progressive multifocal leukoencephalopathy (8A45.02)

Enteroviral exanthematous fever (1F05.2)

Herpes simplex meningitis (1F00.20)

Subacute sclerosing panencephalitis (8A45.01)

West Nile virus infection (1D46)

Colorado tick fever (1D41)

Zoster meningitis (1E91.3)

1C80

Viral encephalitis not elsewhere classified

Coded Elsewhere: Western equine encephalitis (1C83)

Eastern equine encephalitis (1C84)

Venezuelan equine encephalitis (1C8C)

Argentinian haemorrhagic fever (1D61.0)

Bolivian haemorrhagic fever (1D61.1)

Lassa fever (1D61.2)

La Crosse encephalitis (1C8D)

Oropouche virus disease (1D43)

Marburg disease (1D60.1)

Ebola disease (1D60.0)

Japanese encephalitis (1C85)

St Louis encephalitis (1C86)

Rocio viral encephalitis (1C87)

Murray Valley encephalitis (1C88)

Far Eastern tick-borne encephalitis (1C89)

Central European tick-borne encephalitis (1C8A)

Encephalitis due to herpes simplex virus (1F00.21)

Encephalitis due to mumps virus (1D80.3)

Varicella encephalitis (1E90.2)

Measles complicated by encephalitis (1F03.1)

Sequelae of viral encephalitis (1G84)

California encephalitis (1C8B)

Encephalitis due to Arenavirus (1D6Y)

Encephalitis due to Rubella virus (1F02.0)

1C81	Acute poliomyelitis A disease of the nervous system, caused by human poliovirus. This disease commonly presents with a fever, sore throat, headache, vomiting, or stiffness of the neck and back. This disease may present with an acute onset of flaccid paralysis. Transmission is commonly by the faecal-oral route or direct contact. Confirmation is by identification of poliovirus in a faecal sample or by a lumbar puncture.
1C82	Rabies A disease caused by infection with the rabies virus. This disease is characterised by fever, and headache, followed by neurological symptoms dominated by a furious or paralytic form. Coded Elsewhere: Encephalitis due to classical rabies virus (1C80)
1C83	Western equine encephalitis
1C84	Eastern equine encephalitis
1C85	Japanese encephalitis A disease of the brain, caused by an infection with flavivirus. This disease is characterised by fever, headache, meningism, hyperexcitability, or decreased consciousness. This disease may also present with neurological signs such as cranial nerve palsies, tremor and ataxia, parkinsonism, or upper limb paralysis. Transmission is through the bite of an infected mosquito. Confirmation is by identification of flavivirus in a serum sample or cerebrospinal fluid.
1C86	St Louis encephalitis
1C87	Rocio viral encephalitis A disease of the brain, caused by an infection with Rocio virus. In the first phase, this disease is characterised by a fever, headache, vomiting, or conjunctivitis; in the second phase, this disease is characterised by neurological symptoms and muscle weakness. Transmission is through the bite of an infected mosquito. Confirmation is by identification of Rocio virus in a serum or cerebrospinal fluid sample.
1C88	Murray Valley encephalitis A disease of the brain, caused by an infection with Murray Valley encephalitis virus. This disease is characterised by fever, headache, nausea, vomiting, tiredness, or may be asymptomatic. Severe cases may present with confusion, fatigue, lack of coordination, or encephalitis. Transmission is through the bite of an infected mosquito. Confirmation is by detection of anti-Murray Valley encephalitis antibodies in a serum sample. Inclusions: Australian encephalitis
1C89	Far Eastern tick-borne encephalitis
1C8A	Central European tick-borne encephalitis
1C8B	California encephalitis Inclusions: California meningoencephalitis
1C8C	Venezuelan equine encephalitis

1C8D**La Crosse encephalitis****1C8E****Viral meningitis not elsewhere classified**

Any disease of the meninges, caused by an infection with a viral source.

Coded Elsewhere: Meningitis due to mumps virus (1D80.2)

Measles complicated by meningitis (1F03.2)

Herpes simplex meningitis (1F00.20)

Varicella meningitis (1E90.1)

Neonatal meningitis (KA65.4)

Meningitis due to human immunodeficiency virus (1C62.2)

Meningitis due to rubella virus (1F02.0)

Zoster meningitis (1E91.3)

Viral meningitis due to Epstein-Barr virus (1D81.0)

Viral meningitis due to Cytomegalovirus (1D82.Y)

1C8E.1**Enteroviral meningitis**

A disease of the meninges, caused by an infection with enterovirus. This disease is characterised by high fever, headache, vomiting, nausea, stiff neck, photophobia, drowsiness, skin rash, confusion, seizures, or loss of consciousness. This disease may be asymptomatic in older adults. Transmission is through haematogenous spread to the meninges. Confirmation is by identification of enterovirus through a lumbar puncture, by agglutination tests, or by polymerase chain reaction.

1C8E.2**Meningitis due to adenovirus**

A disease of the meninges, caused by an infection with adenovirus. This disease is characterised by high fever, headache, vomiting, nausea, stiff neck, photophobia, drowsiness, skin rash, confusion, seizures, or loss of consciousness. This disease may be asymptomatic in older adults. Transmission is through haematogenous spread to the meninges. Confirmation is by identification of adenovirus through a lumbar puncture, by agglutination tests, or by polymerase chain reaction.

1C8E.Y**Other specified viral meningitis not elsewhere classified****1C8E.Z****Viral meningitis not elsewhere classified, unspecified**

1C8F**Lymphocytic choriomeningitis**

A disease of the meninges, caused by an infection with lymphocytic choriomeningitis virus. This disease is characterised by fever, stiffness of the neck, malaise, lack of appetite, myalgia, headache, nausea, vomiting, or is asymptomatic. This disease may also present with cough, sore throat, arthralgia, testicular pain, or parotid pain. Transmission is by direct contact with body fluids from an infected rodent, through the bite of an infected rodent, or by droplet transmission. Confirmation is by identification of lymphocytic choriomeningitis virus in a blood or tissue sample.

Exclusions: Encephalitis due to Lymphocytic choriomeningitis virus (1D60-1D6Z)
meningoencephalitis due to Lymphocytic choriomeningitis virus (1D60-1D6Z)

1C8Y**Other specified viral infections of the central nervous system****1C8Z****Viral infections of the central nervous system, unspecified****Non-viral and unspecified infections of the central nervous system (1D00-1D0Z)**

Any condition of the nervous system, caused by an infection with a bacterial, fungal, parasitic or unspecified source.

Coded Elsewhere: Meningitis due to other and unspecified causes (1D01.Y)

1D00**Infectious encephalitis not elsewhere classified**

A disease of the brain, caused by an infection.

Coding Note: Code also the causing condition

1D00.0 Bacterial encephalitis

Coded Elsewhere: Tuberculous meningoencephalitis (1B11.1)
Tuberculous encephalitis (1B11.Y)

1D00.1 Fungal encephalitis**1D00.2 Parasitic or protozoal encephalitis**

A disease of the brain, caused by an infection with a parasitic or protozoal source.

Coded Elsewhere: Meningoencephalitis due to *Toxoplasma gondii* (1F57.1)
Encephalitis in African trypanosomiasis (1F51.Z)
Encephalitis due to malaria (1F40.0)
Encephalitis in Gambiense trypanosomiasis (1F51.0Y)
Encephalitis in Rhodesiense trypanosomiasis (1F51.1Y)
Encephalitis in Chagas disease (1F53.Y)

1D00.Y Other specified infectious encephalitis not elsewhere classified

Coding Note: Code also the causing condition

1D00.Z Infectious encephalitis not elsewhere classified, unspecified

Coding Note: Code also the causing condition

1D01 Infectious meningitis not elsewhere classified

A disease of the meninges, caused by an infection.

Coding Note: Code also the causing condition

1D01.0 Bacterial meningitis

Any disease of the meninges, caused by an infection with a bacterial source.

Coding Note: Code also the causing condition

Inclusions: arachnoiditis bacterial

leptomeningitis bacterial

pachymeningitis bacterial

Exclusions: bacterial: meningoencephalitis (1D00.0)

bacterial meningomyelitis (1D02.0)

Coded Elsewhere: Pachymeningitis (8E41)

1D01.00 Meningitis due to Haemophilus influenzae

1D01.0Y Other specified bacterial meningitis

Coding Note: Code also the causing condition

1D01.0Z Bacterial meningitis, unspecified

Coding Note: Code also the causing condition

1D01.1 Fungal meningitis

A disease of the meninges, caused by an infection with a fungal agent.

Coded Elsewhere: Coccidioides meningitis (1F25.12)

Candida meningitis (1F23.30)

Aspergillus meningitis (1F20.01)

1D01.10 Meningitis due to Cryptococcus neoformans

Inflammation of the pia and arachnoid and spinal fluid associated with the fungus cryptococcus neoformans. The respiratory tract is the usual portal of entry and meningitis may occur after dissemination to the meninges from the lungs. C neoformans meningitis tends to occur in patients with defective cellular immunity. The meningitis usually evolves sub acutely, but may be acute. Clinical features include headache, fever, nausea and vomiting, meningismus, visual disturbances, abnormal mental status, seizures, and raised intracranial pressure. Headache, fever, and stiff neck may be absent. The diagnosis is made by microscopic examination of the spinal fluid, culture of CSF and blood, and the latex agglutination test to detect the capsular polysaccharide antigen in CSF and blood. The organism may be seen on Gram stain or India ink stain of the CSF. The spinal fluid usually shows variable lymphocytic pleocytosis, a low glucose content, and a high protein level.

1D01.1Y Other specified fungal meningitis

1D01.1Z	Fungal meningitis, unspecified
1D01.2	Parasitic or protozoal meningitis Coded Elsewhere: Meningitis due to Cysticercosis (1F70.00) Eosinophilic meningitis due to Angiostrongylus cantonensis (1F60.0) Meningitis in Chagas disease (1F53.4) Meningitis in African trypanosomiasis (1F51.Z)
1D01.3	Benign recurrent meningitis
1D01.Y	Other specified infectious meningitis not elsewhere classified
Coding Note:	Code also the causing condition
1D01.Z	Infectious meningitis not elsewhere classified, unspecified
Coding Note:	Code also the causing condition
1D02	Infectious myelitis not elsewhere classified A disease of the spinal cord, caused by an infection.
Coding Note:	Code also the causing condition
1D02.0	Bacterial myelitis Inflammation of the spinal cord caused by a bacterial organism. Common agents causing bacterial myelitis include Mycoplasma pneumoniae, Mycobacterium Tuberculosis, Treponema Pallidum, and Brucella.
Coding Note:	Code also the causing condition
1D02.1	Viral myelitis
Coding Note:	Code also the causing condition
	Exclusions: Myelitis due to human immunodeficiency disease (1C60-1C62.Z)
1D02.2	Fungal myelitis Inflammation of the spinal cord caused by fungal agents. Primary pathogens include Cryptococcus neoformans, Coccidioides immitis, Blastomyces dermatitides, Histoplasma capsulatum, Candida, and Aspergillus.
Coding Note:	Code also the causing condition
1D02.3	Parasitic myelitis
Coding Note:	Code also the causing condition
1D02.Y	Other specified infectious myelitis not elsewhere classified
Coding Note:	Code also the causing condition
1D02.Z	Infectious myelitis not elsewhere classified, unspecified
Coding Note:	Code also the causing condition

1D03**Infectious abscess of the central nervous system**

A focal suppurative process of the brain parenchyma, the intracranial or spinal epidural or subdural space, and less commonly the spinal cord parenchyma. The suppurative process is most commonly associated with bacterial infection, and occasionally with fungal, protozoal, or parasitic infection. Brain abscesses develop most commonly by spread from a contiguous infected site (ear, paranasal sinuses, mastoid air cells, teeth), craniofacial osteomyelitis, and following open head trauma or previous neurosurgical procedure. Haematogenous spread from purulent pulmonary infections, bacterial endocarditis, or other sites of infection, can also cause brain abscess. Patients with intracranial abscess present with variable combinations of headache, altered mental status, focal deficits, and seizures. Fever may be present. Patients with intraspinal abscess present with variable degrees of paraparesis or quadriparesis, sensory impairment below the level of the lesion, altered sphincter function, and back pain. The diagnosis is made by CT or MRI imaging, and may be confirmed by histological examination and culture of the abscess material following neurosurgical drainage. Lumbar puncture is usually contraindicated.

Coding Note: Code also the causing condition

1D03.0 Intraspinal intramedullary abscess

Coding Note: Code also the causing condition

1D03.1 Intraspinal subdural abscess

Coding Note: Code also the causing condition

1D03.2 Intraspinal extradural abscess

Coding Note: Code also the causing condition

1D03.3 Intracranial abscess

A condition of the cranial cavity, caused by an infection with a bacterial, viral, or fungal source. This condition is characterised by a focal accumulation of purulent material within the cranial cavity. This condition may present with fever, headache, and focal neurological deficits.

Coding Note: Code also the causing condition

1D03.30 Deep cerebral hemispheric abscess

Coding Note: Code also the causing condition

1D03.31 Abscess of the corpus callosum

Coding Note: Code also the causing condition

1D03.32 Pituitary abscess

Coding Note: Code also the causing condition

1D03.33	Multiple or widespread intracranial abscess Multiple focal suppurative infections within the cranial cavity, including the epidural and subdural spaces, or in the brain, brainstem or cerebellum. The abscesses are typically surrounded by a vascularised capsule. Cerebritis describes nonencapsulated brain abscesses. The infective agent may be bacterial, fungal, or parasitic. The signs and symptoms are variable but typically present as an expanding mass lesion, over a variable period of time, with headache, fever, and a focal neurologic deficit. Seizures may occur. Diagnosis is made by neuroimaging. Microbiologic diagnosis is made by gram stain and culture of abscess material. Coding Note: Code also the causing condition
1D03.3Y	Other specified intracranial abscess Coding Note: Code also the causing condition
1D03.3Z	Intracranial abscess, unspecified Coding Note: Code also the causing condition
1D03.4	Intraspinal epidural abscess A condition of the epidural space, caused by an infection with a bacterial, viral, fungal, or parasitic source. This condition is characterised by a focal accumulation of purulent material within the epidural space. This condition presents with symptoms depending on the location of the abscess. Transmission is through haematogenous spread of the infectious agent commonly from a cutaneous or mucosal source. Coding Note: Code also the causing condition
1D03.5	Spinal cord abscess A condition of the spinal cord, caused by an infection with a bacterial, viral, or fungal source. This condition is characterised by a focal accumulation of purulent material within the spinal cord. This condition may present with fever, back pain, and neurological deficits. Transmission is through haematogenous spread of the infectious agent. Coding Note: Code also the causing condition
1D03.Y	Other specified site of infectious abscess of the central nervous system Coding Note: Code also the causing condition
1D03.Z	Infectious abscess of the central nervous system, site unspecified Coding Note: Code also the causing condition
1D04	Infectious granulomas of the central nervous system Coded Elsewhere: Tuberculous granuloma of brain (1B11.3) Tuberculous granuloma of the meninges (1B11.4)
1D04.0	Parasitic intracerebral granuloma

1D04.1	Intracranial granuloma	A condition of the cranial cavity, caused by an infection with a bacterial, viral, fungal, or parasitic source. This condition is characterised by an organised collection of macrophages within the cranial cavity. This condition may present with neurological deficits.
1D04.10	Fungal intracranial granuloma	
1D04.1Y	Other specified intracranial granuloma	
1D04.1Z	Intracranial granuloma, unspecified	
1D04.2	Intraspinal intramedullary granuloma	
1D04.3	Intraspinal subdural granuloma	
1D04.4	Intraspinal extradural granuloma	
1D04.5	Intraspinal epidural granuloma	A condition of the epidural space, caused by an infection with a bacterial, viral, fungal, or parasitic source. This condition is characterised by an organised collection of macrophages within the epidural space. This condition may present with neurological deficits.
1D04.Y	Other specified site of infectious granulomas of the central nervous system	
1D04.Z	Infectious granulomas of the central nervous system, site unspecified	
1D05	Infectious cysts of the central nervous system	
1D05.0	Epidural infectious cyst	A condition of the epidural space, caused by an infection with a bacterial, viral, fungal, or parasitic source. This condition is characterised by a membranous sac that may be filled with gas, fluid, or semi solid material within the epidural space. This condition may present with neurological deficits.
1D05.1	Subdural infectious cyst	A condition of the subdural space, caused by an infection with a bacterial, viral, fungal, or parasitic source. This condition is characterised by a membranous sac that may be filled with gas, fluid, or semi solid material between the dura mater and the arachnoid mater. This condition may present with neurological deficits.
1D05.Y	Other specified infectious cysts of the central nervous system	
1D05.Z	Infectious cysts of the central nervous system, unspecified	
1D0Y	Other specified non-viral and unspecified infections of the central nervous system	
1D0Z	Non-viral and unspecified infections of the central nervous system, unspecified	

Dengue (1D20-1D2Z)

Dengue is a viral disease transmitted by the bite of a mosquito infected by dengue viruses. It is one disease entity with different clinical presentations and often with unpredictable clinical evolution and outcome. Most patients recover following a self-limiting non-severe clinical course like nausea, vomiting, rash, aches and pains, but a small proportion progress to severe disease, mostly characterised by plasma leakage with or without haemorrhage, although severe haemorrhages or severe organ impairment can occur, with or without dengue shock.

Coded Elsewhere: Myelitis due to Dengue virus (1D02.1)

Encephalitis due to Dengue fever (1C80)

1D20

Dengue without warning signs

Inclusions: Dengue haemorrhagic fever Grade 1
Dengue fever without warning signs
Dengue haemorrhagic fever without warning signs

1D21

Dengue with warning signs

Clinical warning signs are: abdominal pain or tenderness, mucosal bleeding, lethargy and /or restlessness, rapid decrease in platelet count, increase in haematocrit. Other signs can include: persistent vomiting, visible fluid accumulation, liver enlargement more than 2 cm.

1D22

Severe dengue

Clinical signs include: 1. Severe plasma leakage leading to shock (Dengue shock syndrome - DSS) and/or fluid accumulation with respiratory distress. 2. severe bleeding as evaluated by clinician, 3.) severe organ involvement: Liver AST or ALT ≥ 1000 , CNS: impaired consciousness, involvement of other organs, as myocarditis or nephritis

1D2Z

Dengue fever, unspecified

Certain arthropod-borne viral fevers (1D40-1D4Z)

Coded Elsewhere: Dengue (1D20-1D2Z)

Far Eastern tick-borne encephalitis (1C89)

Central European tick-borne encephalitis (1C8A)

Western equine encephalitis (1C83)

Eastern equine encephalitis (1C84)

Japanese encephalitis (1C85)

St Louis encephalitis (1C86)

Rocio viral encephalitis (1C87)

Murray Valley encephalitis (1C88)

California encephalitis (1C8B)

Venezuelan equine encephalitis (1C8C)

La Crosse encephalitis (1C8D)

1D40

Chikungunya virus disease

1D41	Colorado tick fever
1D42	O'nyong-nyong fever
1D43	Oropouche virus disease A disease caused by an infection with Oropouche virus. This disease is characterised by fever, headache, neck and back pain, joint pain, or photophobia. This disease may also present with bronchitis, nausea, diarrhoea, abdominal pain or burning sensations all over the body. Transmission is through the bite of an infected mosquito or midge. Confirmation is by detection of the Oropouche virus specific antibodies in a serum sample. <i>Inclusions:</i> Oropouche fever
1D44	Rift Valley fever A disease caused by an infection with Rift Valley fever virus. This disease is commonly asymptomatic. This disease may also present with fever, liver abnormalities, weakness, back pain, or dizziness. Transmission is through the bite of an infected mosquito. Confirmation is commonly by detection of Rift Valley fever virus specific IgM or IgG antibodies in a blood sample.
1D45	Sandfly fever <i>Inclusions:</i> pappataci fever phlebotomus fever
1D46	West Nile virus infection A condition caused by an infection with West Nile Virus. This condition is common asymptomatic. This condition may present with fever, headache, stiffness of the neck, stupor, disorientation, coma, tremors, convulsions, muscle weakness, or paralysis. Transmission is through the bite of an infected mosquito. Confirmation is by detection of IgG or IgM anti-West Nile virus antibodies in a serum sample. <i>Inclusions:</i> West Nile fever
1D47	Yellow fever A condition caused by an infection with yellow fever virus. This condition is characterised by fever, chills, headache, myalgia, conjunctival congestion, or relative bradycardia. Severe conditions may also present with increasing fever, jaundice, renal failure, or bleeding. Transmission is through the bite of an infected mosquito. Confirmation is by detection of IgM anti-yellow fever virus antibodies in a serum sample.
1D48	Zika virus disease Zika virus infection is caused by the bite of an infected Aedes mosquito. The most common symptoms of Zika virus infection are mild fever and exanthema (skin rash), usually accompanied by conjunctivitis, muscle or joint pain, and general malaise that begins 2-7 days after the bite of the infected mosquito. <i>Coded Elsewhere:</i> Congenital Zika virus infection (KA62.0)

1D49**Crimean-Congo haemorrhagic fever**

A disease caused by an infection with Crimean-Congo haemorrhagic fever virus. The incubation period ranges from 2 to 9 days. Symptoms/signs typically include high fever, headache, malaise, arthralgia, myalgia, nausea, abdominal pain, and rarely diarrhoea. Early signs typically include fever, hypotension, conjunctivitis, and cutaneous flushing or a skin rash. Later, patients may develop signs of progressive haemorrhagic diathesis, such as petechiae, mucous membrane and conjunctival haemorrhage, haematuria, hematemesis, and melena. Lethality may reach 30%. Transmission occurs via bites of infected ticks, by direct contact with infected animal blood, or iatrogenic transmission. Laboratory diagnosis of the infection during the acute phase of illness consists of detection of viral nucleic acid or by isolation of the virus or by demonstration of viral antigen by enzyme-linked immunoassay from serum or plasma samples. In samples collected later during the illness, diagnosis is confirmed by demonstration of specific IgG and IgM antibodies.

1D4A**Omsk haemorrhagic fever**

A disease caused by an infection with the Omsk haemorrhagic fever virus. This disease is characterised by fever, chills, headache, gastrointestinal symptoms and bleeding, or muscle pain with vomiting. In severe cases, this disease may also present with encephalitis. Transmission is through the bite of an infected tick, by direct contact with an infected animal, by the faecal-oral route from an infected animal, or by ingestion of milk from infected goats or sheep. Confirmation is by detection of anti-Omsk haemorrhagic fever virus antibodies in a serum sample.

1D4B**Kyasanur Forest disease**

A disease caused by an infection with Kyasanur Forest disease virus. This disease commonly presents with fever, chills, headache, muscle pain and vomiting, or gastrointestinal symptoms and bleeding. This disease may also present with neurological manifestations such as a severe headache, mental disturbances, tremors, or vision deficits. Transmission is through the bite of an infected tick or by direct contact with an infected animal. Confirmation is by identification of Kyasanur Forest disease virus in a serum sample.

1D4C**Alkhurma haemorrhagic fever****1D4D****Ross River disease**

A disease caused by an infection with Ross River disease virus. This disease is characterised by arthralgia, with or without arthritis. This disease may also present with fever, fatigue, headache, swollen glands, arthralgia, or maculopapular rash commonly affecting the limbs and trunks. Transmission is through the bite of an infected mosquito. Confirmation is by detection of IgM or IgG anti-Ross River disease virus antibodies in a serum sample.

Inclusions: Epidemic polyarthritis and exanthema
Ross River fever

1D4E**Severe fever with thrombocytopenia syndrome****1D4Y****Other specified arthropod-borne viral fevers****1D4Z****Arthropod-borne viral fever, virus unspecified**

Certain zoonotic viral diseases (1D60-1D6Z)

Coded Elsewhere: COVID-19, virus identified (RA01.0)

1D60

Filovirus disease

A severe disease with high lethality caused by filovirus infection. Filovirus disease is typically characterised by acute onset of fever with non-specific symptoms/signs (e.g., abdominal pain, anorexia, fatigue, malaise, myalgia, sore throat) usually followed several days later by nausea, vomiting, diarrhoea, and occasionally a variable rash. Hiccups may occur. Severe illness may include haemorrhagic manifestations (e.g., bleeding from puncture sites, ecchymoses, petechiae, visceral effusions), encephalopathy, shock/hypotension, multi-organ failure, spontaneous abortion in pregnant women when infected. Common laboratory findings include thrombocytopenia, elevated transaminase concentrations, electrolyte abnormalities, and signs of renal dysfunction. Individuals who recover may experience prolonged sequelae (e.g., arthralgia, neurocognitive dysfunction, uveitis sometimes followed by cataract formation), and clinical and subclinical persistent infection may occur in immune-privileged compartments (e.g., CNS, eyes, testes). Person-to-person transmission occurs by direct contact with blood, other bodily fluids, organs, or contaminated surfaces and materials with risk beginning at the onset of clinical signs and increasing with disease severity. Family members, sexual contacts, healthcare providers, and participants in burial ceremonies with direct contact with the deceased are at particular risk. The incubation period typically is 7–11 days (range ≈2–21 days).

1D60.0

Ebola disease

A severe disease with high case fatality caused by infection with Ebola virus or a closely related virus. Ebola disease is typically characterised by acute onset of fever with non-specific symptoms/signs (e.g., abdominal pain, anorexia, fatigue, malaise, myalgia, sore throat) usually followed several days later by nausea, vomiting, diarrhoea, and occasionally a variable rash. Hiccups may occur. Severe illness may include haemorrhagic manifestations (e.g., bleeding from puncture sites, ecchymoses, petechiae, visceral effusions), encephalopathy, shock/hypotension, multi-organ failure, spontaneous abortion in infected pregnant women. Common laboratory findings include thrombocytopenia, elevated transaminase concentrations, electrolyte abnormalities, and signs of renal dysfunction. Individuals who recover may experience prolonged sequelae (e.g., arthralgia, neurocognitive dysfunction, uveitis sometimes followed by cataract formation), and clinical and subclinical persistent infection may occur in immune-privileged compartments (e.g., CNS, eyes, testes). Person-to-person transmission occurs by direct contact with blood, other bodily fluids, organs, or contaminated surfaces and materials with risk beginning at the onset of clinical signs and increasing with disease severity. Family members, sexual contacts, healthcare providers, and participants in burial ceremonies with direct contact with the deceased are at particular risk. The incubation period typically is 7–11 days (range ≈2–21 days).

1D60.00

Bundibugyo virus disease

Ebola disease caused by Bundibugyo virus.

1D60.01

Ebola virus disease

Ebola disease caused by Ebola virus.

1D60.02	Sudan virus disease Ebola disease caused by Sudan virus.
1D60.03	Atypical Ebola disease
Coding Note:	This code should be used in conjunction with codes that identify the causative virus. Unusual manifestations of disease include organ-specific (e.g. meningoencephalitis) or systemic inflammatory syndromes associated with viral recrudescence occurring after clinical recovery from acute disease. These manifestations may occur several months following infection. Additionally, this code may be used for unusual presentations of acute disease not included in the general description of Ebola disease.
1D60.0Y	Other specified Ebola disease
1D60.0Z	Ebola disease, virus unspecified
1D60.1	Marburg disease A severe disease with high case fatality caused by infection with Marburg virus or a closely related virus. Marburg disease is typically characterised by acute onset of fever with non-specific symptoms/signs (e.g., abdominal pain, anorexia, fatigue, malaise, myalgia, sore throat) usually followed several days later by nausea, vomiting, diarrhoea, and occasionally a variable rash. Severe illness may include haemorrhagic manifestations (e.g., bleeding from puncture sites, ecchymoses, petechiae, visceral effusions), encephalopathy, shock/hypotension, multi-organ failure. Common laboratory findings include thrombocytopenia, elevated transaminase concentrations, electrolyte abnormalities, and signs of renal dysfunction. Individuals who recover may experience prolonged sequelae (e.g., arthralgia, neurocognitive dysfunction, uveitis), and clinical and subclinical persistent infection may occur in immune-privileged compartments (e.g., CNS, eyes, testes). Person-to-person transmission occurs by direct contact with blood, other bodily fluids, organs, or contaminated surfaces and materials with risk beginning at the onset of clinical signs and increasing with disease severity. Family members, sexual contacts, healthcare providers, and participants in burial ceremonies with direct contact with the deceased are at particular risk. The incubation period typically is 7–11 days (range ≈2–21 days).
1D60.10	Marburg virus disease Marburg disease caused by Marburg virus or Ravn virus.
1D60.11	Atypical Marburg disease
Coding Note:	This code should be used in conjunction with codes that identify the causative virus. Unusual manifestations of disease include organ-specific (e.g. orchitis, uveitis) or systemic inflammatory syndromes associated with viral recrudescence occurring after clinical recovery from acute disease. These manifestations may occur several months following infection. Additionally, this code may be used for unusual presentations of acute disease not included in the general description of Marburg disease.
1D60.1Y	Other specified Marburg disease
1D60.1Z	Marburg disease, virus unspecified
1D60.Y	Other specified filovirus disease

1D60.Z **Filovirus disease, virus unspecified**

1D61 **Arenavirus disease**

Coded Elsewhere: Lymphocytic choriomeningitis (1C8F)

Encephalitis due to Arenavirus (1D6Y)

1D61.0 **Argentinian haemorrhagic fever**

A disease endemic to the Argentine Pampas that is caused by an infection with Junín virus and that is characterised by haemorrhagic and neurological manifestations and high lethality (10-30%). The disease begins with a 6-14 day-lasting prodromic phase. Argentinian haemorrhagic fever presents with fever, myalgia, erythema, conjunctival injection, non-menstrual uterine bleeding, epistaxis, haematemesis, melena, haematuria, or shock. Around 20-30% of patients advance to a neurological and haemorrhagic phase. Survivors have a long convalescence period. Transmission occurs by inhalation, consumption, or direct contact with excretions and bodily fluids from infected rodents. Diagnosis occurs by identification of Junín virus from blood or mucosal secretions samples.

1D61.1 **Bolivian haemorrhagic fever**

A disease endemic to Bolivia that is caused by an infection with Machupo virus. Early disease symptoms/signs include fever, mild hypertension, headache, bleeding gums, and fatigue. Advanced signs include mucous membrane haemorrhage, epistaxis, melena, and neurological damage such as tremors, seizures, loss of muscle control, and coma. Onset of disease symptoms occur usually within seven days of infection. The lethality ranges from 18% to 22%. Transmission occurs by inhalation, consumption, or direct contact with excretions and bodily fluids from infected rodents. Diagnosis occurs by identification of Machupo virus from blood or mucosal secretions samples.

1D61.2 **Lassa fever**

A disease endemic in large parts of sub-Saharan Western Africa caused by infection with Lassa virus. Infection is mild or asymptomatic in most cases, but can cause severe illness or death. After a prodromal period of 7-10 days (sometimes longer), initial symptoms/signs include fever, malaise, headache, sore throat, vomiting, abdominal pain, and diarrhoea. Subsequently, patients develop high fever, extreme lethargy, oedema of head/neck, encephalopathy, pleural effusion, and ascites. Bleeding into the skin, mucosae and underlying tissues occurs in the severest cases. Deafness occurs in many patients, and the disease is often particularly severe in pregnancy. The overall lethality can reach 15% even among hospitalized patients receiving supportive care. Transmission occurs by inhalation, consumption, or direct contact with excretions and bodily fluids from infected rodents. Diagnosis occurs by identification of Lassa virus in blood samples by molecular or serologic methods.

1D61.3	Venezuelan haemorrhagic fever
	A disease mainly found in rural areas of central Venezuelan that is caused by an infection with Guanarito virus. Symptoms/signs among patients include fever, malaise, headache, arthralgia, sore throat, vomiting, abdominal pain, diarrhoea, convulsions, and a variety of haemorrhagic manifestations. The majority of patients also develop leukopenia and thrombocytopenia. The overall lethality may reach 30% even in hospitalized patients receiving supportive care. Transmission occurs by by inhalation, consumption, or direct contact with excretions and bodily fluids from infected rodents. Diagnosis occurs by identification of Guanarito virus from blood or mucosal secretions samples.
1D61.Y	Other specified arenavirus disease
1D61.Z	Arenavirus disease, unspecified
1D62	Hantavirus disease
	An acute zoonotic viral disease characterised by abrupt onset of fever, influenza-like clinical signs (e.g., chills, headache, myalgia, dry cough), gastrointestinal signs (e.g., diffuse abdominal pain, vomiting, diarrhoea), transient troubled vision (acute myopia), lumbalgia due to renal swelling, haemorrhagic manifestations to various degrees sometimes followed by rapidly increasing dyspnoea due to not-cardiogenic acute lung oedema, and/or renal involvement. The latter is characterised by initial, often massive proteinuria and microhaematuria sometimes accompanied by transient renal function impediment. All hantavirus infections are heralded by varying degrees of transient thrombocytopenia, which may serve as an indicator of clinical severity.
1D62.0	Haemorrhagic fever with renal syndrome
	Acute zoonotic viral disease with abrupt onset of fever, lower back pain, varying degrees of haemorrhagic manifestations, and renal involvement caused by certain hantaviruses.
	Inclusions: Nephropathia epidemica
1D62.1	Hantavirus pulmonary syndrome
	A disease of the respiratory system, caused by infection with certain hantaviruses. This disease is characterised by fever, fatigue, myalgia, headache, chills, nausea, vomiting, diarrhoea, or abdominal pain. This disease may also present with coughing and dyspnoea. Transmission is by the faecal-oral route or airborne transmission.
1D62.2	Atypical hantavirus disease
1D62.Y	Other specified hantavirus disease
1D62.Z	Hantavirus disease, unspecified
1D63	Henipavirus encephalitis
	Acute bat-borne disease characterised by fever and headaches. The disease may progress to drowsiness, disorientation, mental confusion, and finally encephalitis (brain swelling) in less than a week. This progression may occur with or without an acute respiratory distress component. The incubation period ranges from 4 to 14 days. Lethality is high.

1D64**Middle East respiratory syndrome**

A disease caused by an infection with Middle East Respiratory Syndrome coronavirus (MERS-CoV). This disease is characterised by severe acute respiratory illness with fever, cough, and shortness of breath. Confirmation is by identification of Middle East Respiratory Syndrome coronavirus from genetic material.

1D65**Severe acute respiratory syndrome**

A disease of the respiratory system, caused by an infection with coronavirus. This disease is characterised by fever, headache, cough, myalgia, tachycardia, or diarrhoea. This disease may also lead to pneumonia. Transmission is by direct contact, inhalation of infected respiratory secretions, or airborne transmission. Confirmation is by identification of coronavirus in a blood, stool, respiratory secretions, or body tissue sample.

Exclusions: COVID-19, virus identified (RA01.0)
COVID-19, virus not identified (RA01.1)

1D6Y**Other specified zoonotic viral diseases****1D6Z****Zoonotic viral disease, virus unspecified**

Certain other viral diseases (1D80-1E1Z)

Coded Elsewhere: Congenital Varicella Zoster virus infection (KA62.2)

Other viral diseases complicating pregnancy, childbirth or the puerperium (JB63.5)

Viral duodenitis (DA51.6Y)

Tahyna fever (1D4Y)

1D80**Mumps**

A disease caused by an infection with mumps virus. This disease commonly presents with fever, headache, fatigue, or eventually parotitis. Transmission is by contact with respiratory secretions, directly or indirectly.

1D80.0**Mumps without complication****1D80.1****Orchitis due to mumps virus**

Within a few days of infection the mumps virus can attack the testicular glands leading to abrupt onset of fever ranging from 39-41 to 41°C, severe testicular pain, scrotal swelling and erythema. Mumps induced orchitis typically presents 1-2 weeks after parotitis. The virus' infiltration into the testicular glands can cause parenchymal inflammation, separation of seminiferous tubules and perivascular interstitial lymphocyte infiltration. Testicular atrophy can result from a rise in intratesticular pressure. Complications include oligospermia, azoospermia, and asthenospermia, which can contribute to subfertility. Infertility is more common in patients with bilateral mumps orchitis and is estimated to occur in approximately 13% of all patients. Sterility is rarely induced by mumps orchitis.

Inclusions: Mumps orchitis

1D80.2	Meningitis due to mumps virus A disease of the meninges, caused by an infection with mumps virus. This disease is characterised by photophobia, vomiting, fever, arthralgia, headache, stiff neck, convulsions, or seizures. This disease may also present with pale, blotchy skin or a distinctive rash. Transmission is by haematogenous spread to the meninges after inhalation of infected respiratory secretions or droplet transmission. Confirmation is by identification of mumps virus in a buccal swab or blood sample. <i>Inclusions:</i> mumps meningitis
1D80.3	Encephalitis due to mumps virus : An inflammatory process of the brain, frequently with evidence of meningeal involvement, due to infection by mumps virus. The clinical manifestations are usually acute, with fever and variable combinations of convulsions, impaired mental state, and focal deficits. The spinal fluid may show a cellular reaction and elevated protein. Diagnosis is by neuroimaging, spinal fluid analysis and culture, PCR, and serologic tests. <i>Inclusions:</i> Mumps encephalitis
1D80.4	Pancreatitis due to mumps virus <i>Inclusions:</i> mumps pancreatitis
1D80.Y	Other specified mumps
1D81	Infectious mononucleosis A disease typically caused by an infection with Epstein-Barr virus or cytomegalovirus. This disease commonly presents with extreme fatigue, fever, acute pharyngitis, body aches, or lymphadenopathy. Transmission is by direct contact with infected body fluids, commonly through saliva. <i>Inclusions:</i> Glandular fever Gammaherpesviral mononucleosis
1D81.0	Mononucleosis due to Epstein-Barr virus A disease typically caused by an infection with Epstein-Barr virus. This disease commonly presents with extreme fatigue, fever, acute pharyngitis, body aches, or lymphadenopathy. Transmission is by direct contact with infected body fluids, commonly through saliva.
1D81.1	Mononucleosis due to cytomegalovirus A disease typically caused by an infection with cytomegalovirus. This disease commonly presents with extreme fatigue, fever, acute pharyngitis, body aches, or lymphadenopathy. Transmission is by direct contact with infected body fluids, commonly through saliva.
1D81.Y	Other specified infectious mononucleosis
1D81.Z	Infectious mononucleosis, unspecified

1D82**Cytomegaloviral disease**

Any condition caused by an infection with cytomegalovirus. These conditions are commonly asymptomatic. Transmission is by direct contact with infected body fluids.

Coded Elsewhere: Mononucleosis due to cytomegalovirus (1D81.1)
 Congenital cytomegalovirus infection (KA62.3)
 Intestinal infections due to Cytomegalovirus (1A24)
 Cytomegaloviral retinitis (9B72.00)

1D82.0**Cytomegaloviral hepatitis**

A disease of the hepatic system, caused by an infection with human cytomegalovirus. This disease is characterised by fever, acute pharyngitis, fatigue, lymphadenopathy, or jaundice. Transmission is by direct contact with infected body fluids. Confirmation of an active infection is by identification of human cytomegalovirus in blood, saliva, urine, or other body tissue samples.

1D82.1**Cytomegaloviral pancreatitis****1D82.Y****Other specified cytomegaloviral disease****1D82.Z****Cytomegaloviral disease, unspecified****1D83****Epidemic myalgia**

A disease caused by an infection with the group B Coxsackie virus. This disease is characterised by pleuritic pain, fever, or muscle swelling. Transmission is by the faecal-oral route.

Inclusions: Bornholm disease

1D84**Viral conjunctivitis**

Inflammation, often mild, of the conjunctiva caused by a variety of viral agents. Conjunctival involvement may be part of a systemic infection.

Coding Note: Code also the causing condition

Exclusions: ocular disease herpesviral [herpes simplex] (1F00.1)
 Ophthalmic zoster (1E91.1)

Coded Elsewhere: Zoster conjunctivitis (1E91.1)

1D84.0**Conjunctivitis due to adenovirus**

A condition of the conjunctiva, caused by an infection with adenovirus. This condition is characterised by itchy eyes, tearing, redness, discharge, or photophobia (with corneal involvement). Transmission is by direct contact, indirect contact, or droplet transmission.

Inclusions: Acute adenoviral follicular conjunctivitis
 Swimming-pool conjunctivitis

1D84.1 Acute epidemic haemorrhagic conjunctivitis
A disease of the conjunctiva, caused by an infection with enterovirus. This disease is characterised by painful and red eyes, swollen lids, conjunctival follicles, chemosis, or subconjunctival haemorrhage. Transmission is by direct contact, or contact with contaminated water.

1D84.Y Other specified viral conjunctivitis

Coding Note: Code also the causing condition

1D84.Z Viral conjunctivitis, unspecified

Coding Note: Code also the causing condition

1D85

Viral carditis

A disease of the heart, caused by an infection with a viral source. This disease is characterised by fatigue, dyspnoea, palpitations, malaise, or atypical chest discomfort. This disease may also present with sinus tachycardia, cardiomyopathy, idiopathic ventricular arrhythmias, or cardiovascular collapse. Transmission is by endogenous spread or iatrogenic transmission. Confirmation is identification of the viral source in advanced imaging or cardiac biopsy.

Exclusions: Influenzal myocarditis, other influenza virus identified (1E30)

1D85.0 Dilated cardiomyopathy secondary to viral myocarditis

1D85.1 Acute viral carditis

1D85.2 Chronic viral carditis

1D85.3 Aseptic myocarditis of newborn

1D85.4 Coxsackie carditis

1D85.Y Other specified viral carditis

1D85.Z Viral carditis, unspecified

1D86

Viral haemorrhagic fever, not elsewhere classified

Exclusions: Certain arthropod-borne viral fevers (1D40-1D4Z)

Certain zoonotic viral diseases (1D60-1D6Z)

Viral infection of unspecified site (1D90-1D9Z)

Exclusions: Cytomegaloviral disease (1D82)

Herpes simplex infections (1F00)

1D90

Adenovirus infection of unspecified site

Adenovirus infections most commonly cause illness of the respiratory system; however, depending on the infecting serotype, they may also cause various other illnesses and presentations.

1D91**Enterovirus infection of unspecified site**

Coded Elsewhere: Congenital echovirus infection (KA62.4)
 Congenital enterovirus infection (KA62.5)

1D92**Coronavirus infection, unspecified site**

Exclusions: Severe acute respiratory syndrome (1D65)
 COVID-19, virus not identified (RA01.1)
 COVID-19, virus identified (RA01.0)

1D93**Parvovirus infection of unspecified site**

Coded Elsewhere: Congenital parvovirus syndrome (KA62.7)

1D9Y**Other viral infections of unspecified site****1D9Z****Unspecified viral infection of unspecified site****1E1Y****Other specified viral diseases****1E1Z****Unspecified viral disease****Influenza (1E30-1E32)**

Any disease of the respiratory system, caused by an infection with influenza virus. These diseases are characterised by fever, cough, headache, myalgia, arthralgia, or malaise. Transmission is by inhalation of infected respiratory secretions. Confirmation is by identification of influenza virus from a nasopharyngeal, nose, or throat swab.

1E30**Influenza due to identified seasonal influenza virus**

Exclusions: Haemophilus influenzae [H. influenzae] meningitis (1D01.00)
 Haemophilus influenzae [H. influenzae] pneumonia (CA40.02)

1E31**Influenza due to identified zoonotic or pandemic influenza virus**

Influenza, caused by influenza virus strains of special epidemiological importance with an animal-human or inter-human transmission.

For use of this category, reference must be made to the guidelines of the Global Influenza Programme (GIP, www.who.int/influenza/) of WHO.

Coding Note:

Influenza caused by influenza virus strains of special epidemiological importance with an animal-human or inter-human transmission limited to the inclusions

Exclusions: Haemophilus influenzae [H. influenzae] meningitis (1D01.00)
 Haemophilus influenzae [H. influenzae] pneumonia (CA40.02)

1E32**Influenza, virus not identified**

Any disease of the respiratory system, caused by an unidentified strain of influenza virus. These diseases are characterised by fever, cough, headache, myalgia, arthralgia, or malaise. Transmission is by inhalation of infected respiratory secretions.

- Inclusions:** Influenza specific virus not stated to have been identified
Viral influenza specific virus not stated to have been identified
- Exclusions:** Haemophilus influenzae [H. influenzae] meningitis (1D01.00)
Haemophilus influenzae [H. influenzae] pneumonia (CA40.02)

Viral hepatitis (1E50-1E5Z)

A group of liver diseases caused by infection with one or more of the five hepatitis viruses, hepatitis A virus, hepatitis B virus, hepatitis C virus, hepatitis D virus and hepatitis E viruses. The infection may be recent and present for less than 6 months (acute hepatitis) or present for more than 6 months (chronic hepatitis), in which case progression to cirrhosis and liver cancer can occur. Transmission is by the faecal-oral route including water contamination, sexual transmission, blood and body fluid contamination (parenteral spread) and from mother to baby at the time of birth (vertical transmission). Depending on the virus, diagnosis is confirmed by detection of specific viral antigens, anti-viral antibodies or viral nucleic acids in serum.

- Exclusions:** Herpes simplex hepatitis (1F00)
Autoimmune hepatitis (DB96.0)
Non-alcoholic steatohepatitis (DB92.1)
- Coded Elsewhere:** Viral hepatitis complicating pregnancy, childbirth or the puerperium (JB63.4)
Congenital viral hepatitis (KA62.9)

1E50**Acute viral hepatitis**

A group of liver diseases characterised by liver inflammation and fibrosis, caused by more than 6 months of infection with one or more of hepatitis B virus, hepatitis C virus and hepatitis D virus, with or without HIV. Even at stage of cirrhosis there are often no symptoms. Otherwise, clinical features include fatigue, hard liver edge and complications of cirrhosis (muscle wasting, ascites, splenomegaly/portal hypertension). Transmission of hepatitis B and C viruses is by blood and body fluid contamination, sexual transmission, and from mother to baby at the time of birth (vertical transmission). In addition to detection of specific antigens (HBsAg) and antibodies (anti-HCV), diagnostic assessment requires assay of viral nucleic acids (HBV DNA, HCV RNA etc).

- Exclusions:** Infectious liver disease (DB90)
Acute or subacute hepatic failure (DB91)
Chronic viral hepatitis (1E51)
- Coded Elsewhere:** Cytomegaloviral hepatitis (1D82.0)
Epstein-Barr viral hepatitis (DB90.Y)

1E50.0	Acute hepatitis A Acute liver injury and inflammation caused by recent and short-term (less than 6 months) infection with hepatitis A virus (HAV). Transmission is by the faecal-oral route. Diagnosis is confirmed by presence of IgM-anti-HAV in serum. Clinical features, if they occur, are characterised by anorexia, nausea and fever, with jaundice in severer cases. Exclusions: Infectious liver disease (DB90) Acute or subacute hepatic failure (DB91)
1E50.1	Acute hepatitis B Acute liver injury and inflammation caused by recent and short-term (less than 6 months) infection with hepatitis B virus (HBV). Transmission is by sexual, blood and body fluid contamination (parenteral spread), and from mother to baby at the time of birth (vertical transmission). Diagnosis is confirmed by presence of recent acquisition of HBsAg, ideally with IgM-anti-HBc in serum. Clinical features, if they occur, are characterised by anorexia, nausea and fever, with jaundice in severe cases.
1E50.2	Acute hepatitis C Acute liver injury and inflammation caused by recent and short-term (less than 6 months) infection with hepatitis C virus (HCV). Transmission is by blood and body fluid contamination (parenteral spread) in most cases, and rarely by sexual spread or from mother to baby at the time of birth (vertical transmission). Diagnosis is confirmed by presence of recent acquisition of anti-HCV with presence of HCV RNA in serum. Clinical features occur in a minority of cases and are characterised by anorexia, nausea and fever, rarely with jaundice. A high proportion of cases (>70%) develop chronic HCV infection, with liver disease of varying severity. Coded Elsewhere: Necrolytic acral erythema (EA20)
1E50.3	Acute hepatitis D Acute liver injury and inflammation caused by recent and short-term (less than 6 months) infection with hepatitis D virus (HDV). Transmission only occurs in someone with chronic HBV infection (super-infection) or at the same time as acute hepatitis B (co-infection), and is by blood and body fluid contamination (parenteral spread), and sexual spread. Diagnosis is confirmed by serum IgM-anti-HDV. Clinical features, if they occur, are characterised by anorexia, nausea and fever, with jaundice in severe cases. Acute liver failure occurs in some cases, and a high proportion of cases develops chronic HDV infection. Coded Elsewhere: Acute hepatitis B with Hepatitis D virus co-infection (1E50.1)
1E50.4	Acute hepatitis E A disease of the liver, caused by an acute infection with hepatitis E virus. This disease is characterised by nausea. Transmission is commonly by the faecal-oral route. Confirmation is by detection of anti-hepatitis E virus IgM antibodies in an individual's serum.
1E50.Y	Other specified acute viral hepatitis
1E50.Z	Acute viral hepatitis, unspecified

1E51**Chronic viral hepatitis**

A disease of the liver, caused by a chronic infection with a hepatotropic virus such as hepatitis B,C,D virus, with or without HIV (for six months or longer). This disease is characterised by fatigue, joint and muscle pain, jaundice, or urine of dark yellow colour. Transmission is by sexual contact, or direct contact with contaminated blood or body fluids. Confirmation is by detection of anti-hepatitis antibodies in the individual's serum.

Exclusions: Alcoholic liver disease (DB94)
 Autoimmune hepatitis (DB96.0)
 Non-alcoholic fatty liver disease (DB92)

1E51.0**Chronic hepatitis B**

A liver disease characterised by liver inflammation and fibrosis caused by more than 6 months of infection with the hepatitis B virus. Even at stage of cirrhosis there are often no symptoms. Otherwise, clinical features include fatigue, hard liver edge and complications of cirrhosis (muscle wasting, ascites, splenomegaly/portal hypertension). Transmission is by blood and body fluid contamination, sexual transmission, and from mother to baby at the time of birth (vertical transmission). Confirmation of the diagnosis is by detection of HBsAg, but assessment of severity, prognosis and indication for treatment requires quantification of HBV DNA in serum.

Coded Elsewhere: Chronic hepatitis B, co-infected with hepatitis D virus (1E51.2)
 Hepatitis B surface antigen [HBsAg] carrier (1E51.Y)

1E51.00**Chronic hepatitis B with human immunodeficiency virus co-infection**

A liver disease characterised by liver inflammation and fibrosis caused by more than 6 months of infection with the hepatitis B virus and with the human immunodeficiency virus (HIV). Clinical features include fatigue, hard liver edge and complications of cirrhosis (muscle wasting, ascites, splenomegaly/portal hypertension), and outcomes, including hepatocellular carcinoma are worse than for hepatitis B without HIV infection.

1E51.0Y**Other specified chronic hepatitis B****1E51.0Z****Chronic hepatitis B, unspecified**

1E51.1	Chronic hepatitis C
	A liver disease characterised by liver inflammation and fibrosis caused by more than 6 months of infection with the hepatitis C virus. Even at stage of cirrhosis there may be no symptoms. Otherwise, clinical features include fatigue and impaired quality of life, hard liver edge and complications of cirrhosis (muscle wasting, ascites, splenomegaly/portal hypertension). Chronic hepatitis C increases the risks of type 2 diabetes mellitus and cardiovascular disease, which contribute to increased all-cause mortality. Transmission is by blood and body fluid contamination, rarely by sexual transmission and from mother to baby at the time of birth (vertical transmission). Confirmation of the diagnosis is by detection of HCV RNA in the presence of a positive anti-HCV in serum.
	Exclusions: Non-alcoholic fatty liver disease (DB92)
	Coded Elsewhere: Chronic hepatitis B, co-infected with hepatitis C virus (1E51.0Y)
	Chronic hepatitis B, co-infected with hepatitis C virus and hepatitis D virus (1E51.0Y)
1E51.2	Chronic hepatitis D
	Coded Elsewhere: Chronic hepatitis B, co-infected with hepatitis C virus and hepatitis D virus (1E51.0Y)
1E51.3	Chronic hepatitis E
1E51.Y	Other specified chronic viral hepatitis
1E51.Z	Chronic viral hepatitis, unspecified
1E5Z	Viral hepatitis, unspecified

Viral infections characterised by skin or mucous membrane lesions (1E70-1F0Z)

Infections due to poxvirus (1E70-1E7Z)

1E70	Smallpox
	A disease caused by an infection with variola virus. This disease is characterised by a maculopapular rash that progresses to vesicles (commonly on the face, arms, and legs), and fever. Transmission is by direct contact. Confirmation is by identification of the variola virus in a skin sample of the rash.
	In 1980 the 33rd World Health Assembly declared that smallpox had been eradicated. The classification is maintained for surveillance purposes.
	Inclusions: Variola

1E71**Monkeypox**

A disease caused by an infection with monkeypox virus. In the first phase, this disease is characterised by lymphadenopathy, fever, headache, or malaise; in the second phase, this disease is characterised by a rash that starts as maculopapules and progresses to vesicles, then pustules, followed by crusts (may occur on the face, palms of the hands, soles of the feet, body, and mucous membranes). Transmission is by direct contact with infected animals (including body fluids or lesions), direct contact with body fluid from infected individuals, or through fomites. Confirmation is by identification of monkeypox virus.

Inclusions: mpox

1E72**Cowpox**

Cowpox is due to infection by an orthopoxvirus. Human disease is caused by cutaneous inoculation from an infected host. Cowpox is endemic in Europe amongst small rodents, particularly wood mice and wood voles. After a seven day incubation it causes a systemic febrile flu-like illness. Lesions are solitary or few, mainly affecting the face and

hands. An initial erythematous papula or blister later forms a crusted eschar which heals slowly leaving a deep pock-like scar.

1E73**Vaccinia**

A poxvirus which was formerly used to protect against smallpox. Its use as a vaccine can be complicated by a generalised rash secondary to viraemia, accidental infection of other sites or other individuals, progressive infection at the site of vaccination or, rarely, encephalomyelitis and myopericarditis.

1E74**Buffalopox**

Buffalopox is caused by an orthopox virus related to vaccinia virus. It is acquired in humans by direct inoculation from infected water buffalo. It is generally a mild illness similar to cowpox with just a few lesions on the hands and arms. It leaves minor pock-like scars.

1E75**Orf**

Orf is a virus infection of the skin contracted from sheep and goats. Orf is caused by a parapox virus which infects mainly young lambs and goats. Human lesions are caused by direct inoculation of infected material. Orf is not uncommon among sheep farmers, shearers, freezing workers, vets and farmers' wives or their children who bottle-feed lambs. They occur most commonly on the fingers, hands or forearms but can appear on the face.

1E76**Molluscum contagiosum**

A disease of the skin and mucous membranes, caused by an infection with molluscum contagiosum virus. This disease is characterised by papular skin eruptions, commonly 2-3 millimetres in diameter. Transmission is by direct contact.

Exclusions: Viral warts (1E80)

1E7Y**Other specified infections due to poxvirus****1E7Z****Infections due to poxvirus, unspecified**

Human papillomavirus infection of skin or mucous membrane (1E80-1E8Z)

Infection of the skin and mucous membranes by the human papillomavirus (HPV), the agent responsible for viral warts in humans. Clinical manifestations depend on the virus subtype and the anatomical site involved.

Coded Elsewhere: Anogenital warts (1A95)

1E80	Common warts Common warts are due to an infection of the epidermis by certain human papilloma viruses, most commonly HPV subtypes 1,2,4,27 and 57. They manifest typically as papillomatous, keratinous growths on the hands and feet but may affect any part of the skin (and also adjacent mucous epithelia). They are very common during childhood and adolescence. Exclusions: Warts of lips or oral cavity (1E82)
1E80.0	Digital or periungual warts Viral warts affecting the fingers, thumbs or non-plantar (or non-weight-bearing) skin of the toes. They are often difficult to eradicate, particularly if the nail folds are involved, but most will eventually resolve spontaneously. Exclusions: Plantar warts (1E80.1)
1E80.1	Plantar warts Viral warts affecting the plantar surfaces of the feet including the weight-bearing skin of the toes. They are often painful and are difficult to eradicate. In most cases, however, they do eventually resolve spontaneously.
1E80.Y	Other specified common warts
1E81	Plane warts Plane warts (flat warts) are clinically distinct from common warts and manifest as multiple small flat-topped, often lightly pigmented papules on the face or extremities. They are caused by human papillomavirus (HPV) subtypes 3 and 10.
1E82	Warts of lips or oral cavity Infection of the lips and/or oral cavity, particularly the keratinized surfaces of the gingiva and palate, with "skin" type human papillomavirus (types 2 and 4). Focal epithelial hyperplasia (Heck disease) is a specific form of oral human papillomavirus infection caused by types 13 or 32 and of high prevalence in certain communities in the Americas.
1E82.0	Focal epithelial hyperplasia of oral mucous membranes Otherwise known as Heck disease, this is due to infection of the oral mucosa by human papillomavirus types 13 or 32. It most commonly presents as multiple smooth mucosal papules, giving rise to a cobblestone appearance. It is particularly common in children from native communities of the Americas with incidence rates of up to 30% reported.

1E83**Wart virus proliferation in immune-deficient states**

Enhanced proliferation of human papillomavirus as a result of a failure of immune surveillance. This may be due to a genetic defect, disease or iatrogenic immunosuppression.

1E8Z**Viral warts, not elsewhere classified**

Varicella zoster virus infections (1E90-1E91.Z)

1E90**Varicella**

A disease caused by an infection with varicella zoster virus. This disease is characterised by a vesicular rash and fever. Transmission is by inhalation of infected respiratory secretions, or direct contact with fluid from vesicles.

Coded Elsewhere: Fetal varicella syndrome (KA62.2)

Congenital varicella (KA62.2)

1E90.0**Varicella without complication**

A disease caused by an infection with varicella zoster virus. This disease is characterised by a vesicular rash and fever. Transmission is by inhalation of infected respiratory secretions, or direct contact with fluid from vesicles.

1E90.1**Varicella meningitis**

A disease of the meninges, caused by an infection with varicella zoster virus. This infection is characterised by fever, stiff neck, headache, vomiting, photophobia, and sometimes an altered mental status or body aches. Transmission is through hematogenous spread to the meninges after inhalation of infected respiratory secretions, or direct contact with fluid from vesicles. Confirmation is by identification of varicella zoster viral DNA or anti-varicella zoster IgG in cerebrospinal fluid.

1E90.2**Varicella encephalitis**

Inclusions: Postchickenpox encephalitis

Varicella encephalomyelitis

1E90.Y**Varicella with other specified complication****1E91****Zoster**

A disease caused by the reactivation of a latent infection with varicella zoster virus. This disease commonly presents with a rash (typically within one or two adjacent dermatomes), cutaneous hyperaesthesia, or fever.

1E91.0**Zoster without complications**

A painful blistering skin eruption following a dermatomal distribution resulting from reactivation of Varicella zoster virus in dorsal nerve root ganglia.

1E91.1**Ophthalmic zoster**

A disease of the eyes, caused by the reactivation of a latent infection with varicella zoster virus in the trigeminal nerve. This disease is characterised by a periorbital rash (typically within one dermatome), and conjunctivitis.

1E91.2	Disseminated zoster Disseminated herpes zoster indicates the presence of widespread cutaneous involvement extending beyond the primarily affected and directly adjacent dermatomes. It may be associated with impaired immunity resulting from disease or from therapy.
1E91.3	Zoster with central nervous system involvement
1E91.4	Acute neuropathy of cranial nerve due to zoster
1E91.40	Acute trigeminal zoster neuropathy
1E91.41	Acute herpetic geniculate ganglionitis
1E91.4Y	Other specified acute neuropathy of cranial nerve due to zoster
1E91.5	Postherpetic polyneuropathy
1E91.Y	Zoster with other specified complications
1E91.Z	Zoster, unspecified
1F00	Herpes simplex infections Any condition caused by an infection with herpes simplex virus (human herpesviruses 1 and 2). Confirmation is by identification of herpes simplex virus type 1 or 2. Exclusions: Herpangina (1F05.1) Coded Elsewhere: Perinatal Herpes simplex infection (KA62.A) Anogenital herpes simplex infection (1A94) Sexually transmissible infections due to Herpes simplex virus (1A9Y)
1F00.0	Herpes simplex infection of skin or mucous membrane A disease of the skin and mucous membranes, caused by an infection with herpes simplex virus type 1 or 2. This disease is characterised by vesicles, or may be asymptomatic. Transmission is by direct contact. Confirmation is by identification of herpes simplex virus type 1 or 2. Coded Elsewhere: Anogenital herpes simplex infection (1A94)
1F00.00	Herpes simplex infection of skin Herpes simplex infection affecting skin and commonly arising from person-to-person inoculation of virus from contact sports such as Rugby football or wrestling.
1F00.01	Herpes simplex labialis Inclusions: Cold sore
1F00.02	Herpes simplex gingivostomatitis
1F00.03	Disseminated cutaneous herpes simplex infection complicating other skin diseases
1F00.0Y	Other specified herpes simplex infection of skin or mucous membrane

1F00.1	Herpes simplex infection of the eye A condition of the eye, caused by an infection with herpes simplex virus type 1 or 2. The condition is characterised by blepharoconjunctivitis or keratitis. Transmission is by direct contact. Confirmation is by identification of herpes simplex virus type 1 or 2. Coded Elsewhere: Herpes simplex conjunctivitis (1D84.Y)
1F00.10	Herpes simplex keratitis
1F00.11	Herpes simplex infection of eyelid
1F00.1Y	Other specified herpes simplex infection of the eye
1F00.1Z	Herpes simplex infection of the eye, unspecified
1F00.2	Herpes simplex infection of central nervous system A condition of the central nervous system, caused by an infection with herpes simplex virus (human herpesviruses 1 and 2). This condition is characterised by fever, headache, or other clinical symptoms depending on the site of infection. Confirmation is by identification of herpes simplex virus type 1 or 2. Coded Elsewhere: Herpes simplex myelitis (1D02.1)
1F00.20	Herpes simplex meningitis
1F00.21	Encephalitis due to herpes simplex virus Herpetic encephalitis is a cerebral infection caused by herpes simplex virus type 1 (HSV1). It presents as acute necrotising temporal encephalitis. Onset is rapid (less than 48 hours) with a fever of 40• C, headaches, and behavioural, language and memory problems. These initial manifestations are followed by numbness and coma, which may be accompanied by convulsions and paralysis. This disease, which affects only a small minority of HSV1-infected individuals, results from a primary immune deficiency. Inclusions: Simian B disease
1F00.3	Disseminated herpes simplex infection
1F00.Y	Other specified herpes simplex infections
1F00.Z	Herpes simplex infections, unspecified
1F01	Roseola infantum A disease caused by an infection with roseolovirus (human herpesvirus type 6 or 7). This disease is characterised by acute fever, followed by macular or maculopapular exanthem in some individuals. Transmission is by inhalation of infected respiratory secretions or direct contact.
1F02	Rubella A disease caused by an infection with the rubella virus. This disease commonly presents with lymphadenopathy, or an exanthem that starts on the face and spreads to the limbs and trunk. Transmission is commonly by inhalation of infected respiratory secretions, or direct contact. Coded Elsewhere: Congenital rubella syndrome (KA62.8)

1F02.0 Rubella with neurological complications

1F02.1 Rubella arthritis

A disease of the joints, caused by an infection with the rubella virus. This disease is characterised by inflammation of the joints leading to arthralgia or difficulties moving the joints. Transmission is by inhalation of infected respiratory secretions, or direct contact. Confirmation is by identification of rubella virus (in nasal or throat swab samples, blood, urine, or cerebrospinal fluid), or detection of rubella virus specific IgM antibodies.

1F02.2 Rubella without complication

Rubella was a common childhood viral infection until the advent of mass immunization programmes. It is characterised by a short-lived maculopapular exanthem, lymphadenopathy and mild fever: the majority of infections are not associated with significant morbidity. Transmission is by inhalation of infected respiratory secretions or by direct contact. Confirmation is by identification of rubella virus in nasal swab, throat swab or blood samples, or by detection of rubella virus specific IgM antibodies. Its public health importance lies in its potential to cause devastating harm to the fetus of an infected mother (congenital rubella syndrome).

1F02.Y Rubella with other specified complication

1F03

Measles

A disease of the respiratory system, caused by an infection with Morbillivirus. This disease is characterised by a blotchy rash, fever, cough, conjunctivitis, or malaise. This disease may also present with tiny white spots with bluish-white centres inside the mouth. Transmission is by inhalation of infected respiratory secretions, airborne transmission or direct contact. Confirmation is by detection of Morbillivirus RNA or measles-specific IgM antibodies.

Inclusions: morbilli

Coded Elsewhere: Subacute sclerosing panencephalitis (8A45.01)

1F03.0 Measles without complication

A disease caused by an infection with Morbillivirus. This disease is characterised by fever, cough, coryza, conjunctivitis, enanthema, or maculopapular rash, without any additional secondary pathological conditions. Transmission is by inhalation of infected respiratory secretions, or direct contact. Confirmation is by detection of Morbillivirus RNA, or detection measles-specific IgM antibodies.

1F03.1 Measles complicated by encephalitis

A disease caused by an infection with Morbillivirus that is complicated by an infection of the brain. This disease is characterised by symptoms of measles as well as inflammation of the brain. This disease may also present with fever, headache, poor appetite, vomiting, confusion, lethargy, or photophobia. Transmission is by haematogenous spread to the brain after inhalation of infected respiratory secretions, by airborne transmission, or by direct contact. Confirmation is by detection of Morbillivirus RNA or measles-specific IgM antibodies.

1F03.2	Measles complicated by meningitis A disease caused by an infection with Morbillivirus that is complicated by an infection of the meninges. This disease is characterised by symptoms of measles as well as inflammation of the meninges. This disease may also present with a fever, vomiting, lethargy, confusion, muscle pain, photophobia, or convulsions. Transmission is by haematogenous spread to the meninges after inhalation of infected respiratory secretions, airborne transmission, or direct contact. Confirmation is by detection of Morbillivirus RNA or measles-specific IgM antibodies. Inclusions: Postmeasles meningitis
1F03.Y	Measles with other complications
1F04	Erythema infectiosum A condition caused by infection with parvovirus B19 (member of the Erythrovirus genus). In children, this condition is characterised by fever and cold-like symptoms initially, followed by a skin rash typically in the facial region. In adolescents and adults, this condition may present with painful and swollen joints. Transmission is by droplet transmission, or vertical transmission. Inclusions: Slapped cheek syndrome
1F05	Picornavirus infections presenting in the skin or mucous membranes
1F05.0	Enteroviral vesicular stomatitis Enteroviral vesicular stomatitis, commonly called hand, foot and mouth disease, is a highly contagious enterovirus infection (usually Coxsackievirus A16 or Enterovirus 71). It typically causes a mild febrile illness with sore throat and loss of appetite followed by an eruption of vesicles on the lips, hands and feet. The majority of cases occur in children under the age of five. Inclusions: Hand, foot and mouth disease
1F05.1	Enteroviral vesicular pharyngitis
1F05.2	Enteroviral exanthematous fever An acute febrile, characteristically morbilliform exanthem due to infection by one of many different enteroviruses, especially Coxsackievirus and Echovirus. Exclusions: Enteroviral vesicular pharyngitis (1F05.1) Enteroviral vesicular stomatitis (1F05.0)
1F05.3	Foot and mouth disease A rare infection in humans due to the Aphthovirus Foot-and-mouth-disease virus (FMDV), which is responsible for a highly contagious epidemic infection of cloven-hoofed animals, particularly cattle. It manifests in humans with prodromal fever and malaise followed by vesiculation and ulceration of oral mucous membranes and lips. Vesicles may sometimes involve the digits and palmoplantar skin. Exclusions: Hand, foot and mouth disease (1F05.0)
1F05.Y	Other specified picornavirus infections presenting in the skin or mucous membranes

1F0Y**Other specified viral infections characterised by skin or mucous membrane lesions****1F0Z****Viral infections characterised by skin or mucous membrane lesions, unspecified****Mycoses (1F20-1F2Z)****Exclusions:**

Mycosis fungoides (2B01)

Hypersensitivity pneumonitis due to organic dust (CA70)

Coded Elsewhere:

Intestinal fungal infections

Fungal infection of fetus or newborn (KA63)

1F20**Aspergillosis**

Aspergillosis (ABPA) is a disease caused by fungi of the genus *Aspergillus* and occurs in people with lung diseases or weakened immune system. ABPA is most common in people with asthma or cystic fibrosis. The organism is ubiquitous, being found in soil and water or in other decaying vegetation. It enters the body through the lungs. The Symptoms of ABPA include wheezing and coughing of blood.

Inclusions:

aspergilloma

Coded Elsewhere: Aspergillus-induced allergic or hypersensitivity conditions (CA82.4)

Neonatal aspergillosis (KA63.1)

Myelitis associated with *Aspergillus* (1D02.2)**1F20.0****Invasive aspergillosis**

A disease caused by an infection with the fungi *Aspergillus*. This disease is characterised by colonization and invasion of tissue by *Aspergillus* in one part of the body and may spread to other parts of the body. Transmission is commonly by inhalation of *Aspergillus* spores.

1F20.00

Invasive aspergillosis of the digestive tract

1F20.01

Invasive cerebral aspergillosis

1F20.02

Disseminated aspergillosis

Invasive aspergillosis affecting three or more organs.

1F20.0Y

Invasive aspergillosis of other specified site

1F20.1**Non-invasive aspergillosis****1F20.10**

Aspergillus otomycosis

Chronic superficial fungal infection of the external auditory canal and auricle due to saprophytic fungi of the genus *Aspergillus*.

1F20.11

Chronic aspergillosis of the paranasal sinuses

Inclusions:Chronic granulomatous *Aspergillus* rhinosinusitis

1F20.12	Chronic pulmonary aspergillosis Nodular or cavitory lesion or lesions in the lung, of at least three months duration in a non-immunocompromised patient (or one whose immunocompromising condition has remitted or is trivial), caused by <i>Aspergillus</i> spp. as demonstrated on tissue section by staining, by positive culture of a percutaneous biopsy or positive <i>Aspergillus</i> IgG antibodies. Inclusions: - Pulmonary <i>Aspergillus</i> nodule - Simple pulmonary aspergilloma - Chronic cavitory pulmonary aspergillosis - CFPA - [chronic fibrosing pulmonary aspergillosis] - CNPA - [chronic necrotising pulmonary aspergillosis]
1F20.13	Tonsillar aspergillosis
1F20.14	<i>Aspergillus</i> bronchitis
1F20.15	Obstructing <i>aspergillus</i> tracheobronchitis
1F20.1Y	Other specified non-invasive aspergillosis
1F20.Z	Aspergillosis, unspecified
1F21	Basidiobolomycosis Basidiobolomycosis is characterised by a slowly spreading, painless, non-pitting subcutaneous swelling without other obvious clinical signs. It may be single, or there may be multiple satellite lesions. The disc-shaped masses have a uniform hard consistency. It usually involves the limbs or limb-girdle areas and the infection is most often seen in children. Inclusions: Subcutaneous mucoromycosis due to <i>Basidiobolus ranarum</i>
1F22	Blastomycosis A disease caused by an infection with the fungi <i>Blastomyces dermatitidis</i>. This disease is characterised by fever, chills, cough, myalgia, arthralgia, or chest pain. This disease may also present in the skin and bones. Transmission is by inhalation of fungal spores. Confirmation is by identification of <i>Blastomyces dermatitidis</i> in a urine, cerebrospinal fluid, or blood sample. Exclusions: - Brazilian blastomycosis (1F2E) - keloidal blastomycosis (1F2B)
1F23	Candidosis Candidosis is an infection caused by yeasts of the genus <i>Candida</i>. Superficial infections of the mucous membranes and skin are common, but deep invasive disease including fungal septicaemia, endocarditis and meningitis may also occur. Inclusions: - moniliasis - candidiasis Coded Elsewhere: - Neonatal candidosis (KA63.2) - Invasive neonatal candidosis (KA63.2)

1F23.0	Candidosis of lips or oral mucous membranes A disease of the lips and oral mucous membranes, caused by an infection with the fungi <i>Candida</i> . This disease commonly presents with white patches or plaques on the oral mucous membranes, angular cheilitis, or dysphagia. Transmission is by opportunistic transmission. Confirmation is by identification of <i>Candida</i> in an oral or skin sample. Exclusions: Neonatal candidosis (KA63.2)
1F23.1	Candidosis of skin or mucous membranes Coded Elsewhere: Neonatal mucocutaneous candidosis (EH12)
1F23.10	Vulvovaginal candidosis A disease caused by an infection with the fungi <i>Candida</i> . This disease is characterised by genital itching, burning, or vaginal discharge. Transmission is by endogenous spread, or sexual contact. Confirmation is commonly by identification of <i>Candida</i> in a vaginal swab.
1F23.11	Candida balanoposthitis A disease caused by an infection with the fungi <i>Candida</i> (commonly <i>Candida albicans</i>). This disease is characterised by inflammation of the glans or prepuce. This disease may also present with eroded white papules, or white discharge. Transmission is by sexual contact. Confirmation is by identification of <i>Candida</i> in a sub-preputial swab or urine sample. Inclusions: Candidosis of penis Penile thrush
1F23.12	Flexural or intertriginous candidosis Candidosis of flexural and intertriginous skin, where the warm, moist conditions favour the growth of <i>Candida</i> yeasts.
1F23.13	Candidosis of nail or paronychium Infection of the nail and/or paronychium (nail fold) with <i>Candida</i> yeasts
1F23.14	Chronic mucocutaneous candidosis Chronic Mucocutaneous Candidiasis is a primary immune deficiency characterised by persistent and/or recurrent infections of skin, nails and mucous membranes, caused by organisms of the genus <i>Candida</i> , mainly <i>C. Albicans</i> .
1F23.15	Disseminated cutaneous candidosis
1F23.16	Candida otomycosis Infection of the external auditory canal with <i>Candida</i> yeasts, especially <i>Candida parapsilosis</i> . The infection may present with whitish greasy debris in, or discharge from the external auditory canal, or with erythema, oedema and pain. <i>Candida</i> otomycosis is less common than otomycosis due to <i>Aspergillus</i> . Chronic infection may be associated with perforation of the eardrum.
1F23.1Y	Candidosis of skin or mucous membrane of other specified site
1F23.1Z	Candidosis of skin or mucous membranes, unspecified

1F23.2	Candidosis of gastrointestinal tract
1F23.3	Systemic or invasive candidosis Invasion of internal organs by <i>Candida</i> yeasts. Risk factors include acute leukemia, haematopoietic stem cell or solid organ transplantation, and acute critical illness. <i>Candida</i> species other than <i>Candida albicans</i> are commonly implicated. Coded Elsewhere: Invasive neonatal candidosis (KA63.2) Candidaemia (MA15.1)
1F23.30	Candida meningitis
1F23.31	Pulmonary candidosis A disease of the pulmonary system, caused by an infection with the fungi <i>Candida</i>. This disease is characterised by fever, chills, cough, nausea, vomiting, tachypnoea, tachycardia, or dyspnoea. Transmission is by opportunistic transmission. Confirmation is by identification of <i>Candida</i> from a sputum sample.
1F23.3Y	Other specified systemic or invasive candidosis
1F23.3Z	Systemic or invasive candidosis, unspecified
1F23.Y	Other specified candidosis
1F23.Z	Candidosis, unspecified
1F24	Chromoblastomycosis Chromoblastomycosis is a chronic fungal infection of the skin and subcutaneous tissues caused by a variety of pigmented fungi including <i>Phialophora verrucosa</i>, <i>Fonsecaea pedrosoi</i>, <i>Fonsecaea compacta</i> and <i>Cladophialophora carrionii</i>, which can be found in soil and wood. The infection usually follows trauma, such as a puncture from a splinter of wood and tends to affect exposed sites such as the feet and ankles. Chromoblastomycosis manifests initially as a warty papule which slowly enlarges to form a hypertrophic, warty plaque. Eventually, after months or many years, large hyperkeratotic masses may form, sometimes with secondary ulceration.
1F25	Coccidioidomycosis A disease caused by an infection with the fungi <i>Coccidioides</i>. This disease presents with symptoms depending on the site of infection, or may be asymptomatic. Transmission is commonly by inhalation of fungal spores. Confirmation is by identification or culture of <i>Coccidioides</i> from affected tissue or samples, or detection of antibodies against coccidioides in serum or cerebrospinal fluid.
1F25.0	Pulmonary coccidioidomycosis A disease of the pulmonary system, caused by an infection with the fungi <i>Coccidioides</i>. This disease is characterised by cough, myalgia, fatigue, chest pain, pneumonia, or pleural effusion. Transmission is commonly by inhalation of fungal spores. Confirmation is by direct examination or culture of <i>Coccidioides</i> in a sputum, bronchoalveolar lavage fluid, or tissue sample.

1F25.00	Acute pulmonary coccidioidomycosis Forty per cent of coccidioidal infections result in symptomatic pulmonary disease that may be indistinguishable from a bacterial community-acquired pneumonia. Radiographically these are usually focal alveolar infiltrates. Infection may be associated with the development of a rash, particularly erythema nodosum and erythema multiforme. Occasionally, there may be symmetric arthralgias or arthritis. Peripheral blood eosinophilia is not uncommon. Acute primary pulmonary coccidioidomycosis, particularly when associated with erythema nodosum and/or erythema multiforme, is frequently called "Valley fever." When associated with arthralgias or arthritis, it has been termed "desert rheumatism."
1F25.01	Chronic pulmonary coccidioidomycosis A chronic form of pulmonary coccidiomycosis. Pulmonary sequelae occur in approximately 5% of all cases of acute pulmonary coccidioidomycosis
1F25.1	Extrathoracic coccidioidomycosis Coccidioidomycosis involving sites other than the lungs and thoracic cavity. Recognised sites include lymph nodes, bones, joints, central nervous system and skin. Transmission is through haematogenous spread to other body sites after inhalation of fungal spores or by direct inoculation. Coded Elsewhere: Myelitis associated with Coccidioides infection (1D02.2)
1F25.10	Disseminated coccidioidomycosis Diffuse pulmonary coccidioidomycosis occurs either when there is inhalation of a massive number of arthroconia, such as may occur during archeological excavations, or among individuals with severely depressed cellular immunity (e.g., late HIV-1 infection [AIDS]; cancer chemotherapy; allogeneic transplant recipients, treatment with corticosteroids; and during the second, third trimesters and postpartum pregnancy). The radiographic appearance often is a mixture of small nodules and interstitial findings, sometimes called "reticulonodular" or, because of it may resemble overwhelming pulmonary tuberculosis, "miliary."
1F25.11	Primary cutaneous coccidioidomycosis Coccidioidomycosis may rarely result from direct inoculation, usually through a puncture of the skin by a thorn or other vegetative structure. The infection generally remains confined to this area with local lymphangitic spread and is not considered indicative of disseminated disease. Coccidioidal serology tests may be positive.
1F25.12	Coccidioides meningitis An uncommon but often lethal form of coccidioidomycosis due to dissemination of Coccidioides fungi from the primary site of infection, principally the lungs, to the central nervous system. Inclusions: Coccidioidomycosis meningitis
1F25.1Y	Other specified extrathoracic coccidioidomycosis
1F25.Z	Coccidioidomycosis, unspecified

1F26**Conidiobolomycosis**

Conidiobolomycosis is a subcutaneous infection involving nasal mucosa and paranasal sinuses, leading to formation of firm, subcutaneous nodules or polyps. The infection may be acquired via inhalation of spores or a minor trauma such as an insect bite. The infected host is frequently an otherwise healthy individual working outdoors in tropical areas. Conidiobolomycosis can, however, cause major facial disfigurement. In individuals with impaired immune responses more invasive and potentially fatal infections may occur: such infections are not usually associated with skin lesions.

Inclusions: Rhinomentomophthoromycosis

1F27**Cryptococcosis**

A disease caused by an infection with the fungi *Cryptococcus neoformans* or *Cryptococcus gattii*. This disease commonly presents with shortness of breath, cough, fever, fatigue, or headache. Transmission is by inhalation of fungal spores. Confirmation is by identification of *Cryptococcus neoformans* or *Cryptococcus gattii* in a blood, sputum, or cerebrospinal fluid sample.

1F27.0**Pulmonary cryptococcosis**

The pattern of cryptococcal pulmonary disease ranges from asymptomatic airway colonization to pneumonia to acute respiratory distress syndrome. If present, symptoms include cough, dyspnoea or chest pain. Common chest X-ray appearances include nodules or infiltrates. In the immunocompetent host, focal lesions are more commonly seen with infection due to *C. gattii*.

1F27.1**Cerebral cryptococcosis**

A disease of the central nervous system, caused by an infection with the fungi *Cryptococcus neoformans* or *Cryptococcus gattii*. This disease is characterised by fever, headache, lethargy, or neurological deficits. Transmission is by inhalation of fungal spores. Confirmation is by identification of *Cryptococcus neoformans* or *Cryptococcus gattii* in a blood, sputum, or cerebrospinal fluid sample.

Coded Elsewhere: Meningitis due to *Cryptococcus neoformans* (1D01.10)

1F27.2**Disseminated cryptococcosis**

Disseminated cryptococcosis is most common in immunocompromised hosts, with involvement with any organ and predilection for the central nervous system. It may manifest as systemic illness with fever, night sweats and malaise. Blood cultures may be positive (cryptococcaemia).

1F27.Y**Other specified cryptococcosis****1F27.Z****Cryptococcosis, unspecified**

1F28**Dermatophytosis**

Dermatophytosis (tinea, ringworm) is a superficial infection of the skin, hair or nails by dermatophyte fungi of the genera *Trichophyton*, *Epidermophyton* or *Microsporum*. These fungi normally invade only the outer keratinous layer of the epidermis (stratum corneum), the hair shaft and the nail. They count amongst the commonest infections in man. Some species (e.g. *Trichophyton rubrum*) are essentially anthropophilic and infect only man whereas others are zoophilic (e.g. *Trichophyton verrucosum*) but may cause human infection from contact with infected animals.

Inclusions: Infections due to species of *Epidermophyton*, *Microsporum* and *Trichophyton*

Exclusions: Tinea nigra (1F2D.4)
Tinea versicolor (1F2D.0)

1F28.0**Dermatophytosis of scalp**

Dermatophytosis (tinea) affecting scalp and scalp hair. Clinical features range from limited patchy alopecia and scaling to widespread inflammation and suppuration with occipital lymphadenopathy. The scalp is a typical site for a kerion (q.v.), often due to a zoophilic dermatophyte acquired from an infected animal.

Inclusions: Tinea capitis
Scalp ringworm

1F28.1**Dermatophytosis of nail**

Fungal infection of the nail plate due to dermatophyte fungi (tinea unguium). Infection results in a range of clinical signs including white or yellow discolouration, detachment of the plate from the nail bed (onycholysis), keratinous thickening under the nail plate (subungual hyperkeratosis) and fragility and fragmentation of the abnormal nail plate.

Inclusions: Onychomycosis due to dermatophyte
Tinea unguium
Ringworm of nails

Exclusions: Onychomycosis due to non-dermatophyte mould (1F2D.5)
Candida onychomycosis (1F23.13)

1F28.2**Dermatophytosis of foot**

Dermatophytosis of the skin of the foot (tinea pedis). The lateral interdigital toe clefts are the most common initial site of infection. Longstanding infection with *Trichophyton rubrum*, the most commonly implicated organism in Europe and North America, characteristically causes dry scaling over the sole of the foot. Other species which regularly invade the skin of the foot include *Epidermophyton floccosum* and *Trichophyton interdigitale*.

Inclusions: Moccasin foot
Tinea pedis
Athlete's foot
Ringworm of foot

1F28.3	Genitocrural dermatophytosis
	Dermatophyte infection of the inguinocrural folds and adjacent external genitalia (tinea cruris). It presents as erythema and inflammation of affected skin with an advancing scaly edge. It is typically itchy and affects adult men much more commonly than women or children. Dermatophyte infection of the toe clefts commonly co-exists.
	Inclusions: Tinea cruris Ringworm of groin Dermatophytosis of groin
1F28.4	Kerion
	Kerion results from a severe host inflammatory response to dermatophyte infection of the hair follicles of the scalp or beard. It typically presents as a single painful, severely inflammatory, suppurating boggy mass and is most commonly a reaction to a zoophilic dermatophyte infection especially <i>Trichophyton verrucosum</i> (cattle ringworm) or <i>Trichophyton mentagrophytes</i> .
1F28.5	Disseminated dermatophytosis
	Extensive and invasive dermatophyte infection due either to a specific genetic anergy to dermatophytes or to profound immunosuppression. Dermal nodules, abscesses or draining sinuses may occur; rarely bone, central nervous system and lymph nodes may be involved.
1F28.Y	Other specified dermatophytosis
1F28.Z	Dermatophytosis, unspecified
1F29	Eumycetoma
	A localised chronic infection caused by various species of fungi and characterised by the formation of aggregates of the causative organisms (grains) within abscesses. This results in severe damage to skin, subcutaneous tissues and bones of the feet, hands and other parts of the body, with draining sinuses which discharge grains to the surface. Recognised agents include <i>Madurella mycetomatis</i> , <i>Madurella grisea</i> , <i>Leptosphaeria senegalensis</i> , <i>Curvularia lunata</i> , <i>Scedosporium apiospermum</i> , <i>Neotestudina rosatii</i> , <i>Acremonium</i> spp. and <i>Fusarium</i> spp.
	Inclusions: Mycetoma due to fungal infection
	Exclusions: Actinomycetoma (1C43)
1F2A	Histoplasmosis
	Histoplasmosis is a disease caused by the fungus <i>Histoplasma</i> that exists worldwide with two significant variants: <i>Histoplasma capsulatum</i> and <i>Histoplasma duboisii</i> .
	Coded Elsewhere: Meningitis due to <i>Histoplasma capsulatum</i> (1D01.1Y) Histoplasmosis-related fibrosing mediastinitis (CB22.0)

1F2A.0	Pulmonary histoplasmosis capsulati A disease of the pulmonary system, caused by an infection with the fungi <i>Histoplasma capsulatum</i> . This disease is characterised by fever, chest pains, or a dry, nonproductive cough. Transmission is by inhalation of fungal spores, commonly from contaminated soil, or bat or bird faeces. Confirmation is by identification of <i>Histoplasma capsulatum</i> from affected tissue or body fluids, detection of antibodies against <i>Histoplasma capsulatum</i> , or detection of <i>Histoplasma capsulatum</i> antigen.
1F2A.1	Histoplasmosis due to <i>Histoplasma duboisii</i> This form of histoplasmosis is endemic to Sub-Saharan Africa and is generally less virulent than histoplasmosis due to <i>H. capsulatum</i> , the classical form which occurs predominantly in tropical and subtropical regions of the Americas but is also seen in Africa and Asia. Otherwise known as African histoplasmosis, histoplasmosis due to <i>Histoplasma duboisii</i> usually involves the skin and subcutaneous tissue, lymph nodes and bones and rarely the lungs and other internal organs.
1F2A.Y	Other specified histoplasmosis
1F2A.Z	Histoplasmosis, unspecified
1F2B	Lobomycosis A disease of the skin, caused by an infection with the fungi <i>Lacazia loboi</i> . This disease commonly presents with dermal nodules (either lenticular or in plaques), keloids, subcutaneous mycoses, or malignant tumours. Transmission is commonly by direct contact with contaminated water, soil, vegetation, or may be by direct contact with an infected dolphin. Confirmation is by identification of <i>Lacazia loboi</i> in a lesion exudate or tissue sample. <i>Inclusions:</i> Lobo disease
1F2C	Mucormycosis A disease caused by an infection with the fungi from the order Mucorales. This disease presents with symptoms depending on the site of the infection. Transmission is by direct contact with infected soil or decaying matter. Confirmation is by identification of fungi from the order Mucorales from a tissue sample.
1F2D	Non-dermatophyte superficial dermatomycoses Any condition of the skin and mucous membranes, caused by an infection with fungi other than <i>Candida</i> and dermatophytes. <i>Exclusions:</i> Candidosis (1F23) Dermatophytosis (1F28)
1F2D.0	Pityriasis versicolor A disease of the skin, caused by an infection with the fungi <i>Malassezia</i> . This disease is characterised by white, pink, fawn, brown, or often coalescing lesions that may be covered with thin furfuraceous scales. This disease commonly presents on the trunk, shoulders and arms, or neck and face. Transmission is by opportunistic transmission. Confirmation is by identification of <i>Malassezia</i> in a skin sample.

1F2D.1	Malassezia folliculitis
	Malassezia folliculitis is caused by the invasion of the hair follicle by Malassezia yeasts. Although Malassezia yeasts are a part of the normal human microflora, under certain conditions they can cause superficial dermatological conditions. The invasion results in the development of erythematous papules, and sometimes pustules, which may be either asymptomatic or itchy. Usually Malassezia yeasts are present along with staphylococci and propionibacteria in the follicles.
	Inclusions: Seborrhoeic folliculitis
	Exclusions: Seborrhoea (ED91.2)
1F2D.2	White piedra
	A disease of the hair shaft, caused by an infection with the fungi Trichosporon beigelii. This disease is characterised by irregular, soft, white, or light brown nodules which adhere to the hair follicle. Transmission is by direct contact with contaminated soil or water, or by airborne transmission. Confirmation is by identification of Trichosporon beigelii in a hair follicle sample.
	Inclusions: Trichosporosis nodosa
1F2D.3	Black piedra
	Inclusions: Trichomycosis nodularis
1F2D.4	Tinea nigra
	A disease of the skin, caused by an infection with the fungi Tinea nigra. This disease is characterised by brown to black macules; small, flat circumscribed changes in the colour of skin. This disease commonly presents on the palmar surfaces, soles, or other skin surfaces. Transmission is by direct contact with contaminated soil, wood, or vegetation. Confirmation is identification of Tinea nigra in a skin sample.
	Inclusions: Keratomycosis nigricans palmaris
1F2D.5	Onychomycosis due to non-dermatophyte mould
	Fungal nail infection due to organisms other than Candida and dermatophytes. These include Scopulariopsis brevicaulis, Neoscytalidium dimidiatum, Fusarium spp., and Aspergillus spp., which may not respond to therapies directed at the more common causes of onychomycosis.
	Exclusions: Candidosis of nail or paronychium (1F23.13)
1F2D.Y	Other specified non-dermatophyte superficial dermatomycoses
1F2E	Paracoccidioidomycosis
	A disease caused by an infection with the fungi Paracoccidioides brasiliensis. This disease commonly presents with fever, toxemia, weight loss, adenopathy, hepatosplenomegaly, anaemia, or eosinophilia. This disease may present with symptoms similar to tuberculosis, leukaemia, or lymphoma. Transmission is by inhalation of fungal spores. Confirmation is by identification of Paracoccidioides brasiliensis in a blood, sputum, or skin sample.

1F2E.0	Pulmonary paracoccidioidomycosis A disease of the pulmonary system, caused by an infection with the fungi <i>Paracoccidioides brasiliensis</i> . This disease is characterised by fever, cough, dyspnoea, or malaise. Transmission is by inhalation of fungal spores. Confirmation is by identification of <i>Paracoccidioides brasiliensis</i> in a blood or sputum sample.
1F2E.1	Disseminated paracoccidioidomycosis Disseminated paracoccidioidomycosis results from haematogenous and lymphatic dissemination of yeasts from the lungs and aerodigestive tract. Cutaneous involvement, seen in 25% of cases, presents as crusted papules, ulcers, nodules, and verrucous plaques. Lymphadenopathy occurs commonly in the cervical region, but all lymph node chains can be involved. Adrenal glands are commonly affected with a significant risk of adrenal insufficiency and Addisonian crisis. Long bones such as ribs, humeri, and clavicles can be involved. Mesenteric lymph node involvement can lead to bowel obstruction. Meningoencephalitis occurs in up to one quarter of cases.
1F2E.Y	Other specified paracoccidioidomycosis
1F2E.Z	Paracoccidioidomycosis, unspecified
1F2F	Phaeohyphomycosis
1F2G	Pneumocystosis <i>Coded Elsewhere:</i> HIV disease resulting in <i>Pneumocystis jirovecii</i> pneumonia (1C62.2)
1F2G.0	Pulmonary pneumocystosis An opportunistic pulmonary infection by the fungus <i>Pneumocystis jirovecii</i> . It is strongly associated with HIV and AIDS. <i>Coded Elsewhere:</i> Pneumonia due to pneumocystis (CA40.20)
1F2G.Z	Pneumocystosis, unspecified
1F2H	Scedosporiosis An opportunistic infection caused by fungal species of the genus <i>Scedosporium</i> . The most common clinical presentation is disseminated infection, which is associated with underlying disease, especially haematological malignancy, or with organ transplantation, especially of the lung. Infections of lung, bones or joints are also well recognised.
1F2J	Sporotrichosis A disease caused by an infection with the fungi <i>Sporothrix schenckii</i> . This disease presents with symptoms depending on the site of infection. Transmission is by direct contact with infected thorny plants, sphagnum moss, soil, bales of hays, or infected plant material. Confirmation is by identification of <i>Sporothrix schenckii</i> from a tissue or skin sample.

1F2J.0	Lymphocutaneous sporotrichosis This is the most common type of sporotrichosis and follows implantation of <i>Sporothrix schenckii</i> spores into a cutaneous wound, most commonly on the upper extremity. In addition to a localised nodule or pustule, a chain of nodules develops along draining lymphatics. In longstanding cases regional lymph nodes may become involved.
1F2J.1	Fixed cutaneous sporotrichosis Cutaneous sporotrichosis which remains localised to the area of inoculation.
1F2J.2	Pulmonary sporotrichosis Pulmonary forms of infection, although uncommon, can occur when <i>Sporothrix schenckii</i> conidia are inhaled. [1] Symptoms of pulmonary sporotrichosis mimic those of tuberculosis including constitutional complaints of fever, night sweats, weight loss, and fatigue as well as respiratory complaints including dyspnoea, cough, purulent sputum, and haemoptysis.[2]
1F2J.3	Disseminated sporotrichosis
1F2J.Y	Other specified sporotrichosis
1F2J.Z	Sporotrichosis, unspecified
1F2K	Talaromycosis Talaromycosis is an infection due to <i>Talaromyces marneffe</i> , an ubiquitous saprophyte of soil and decomposing organic matter. This dimorphic fungus, formerly known as <i>Penicillium marneffe</i> , is endemic to Southeast Asia and the southern part of China. Once considered rare, its occurrence has increased due to AIDS. It is now the third most common opportunistic infection in HIV-positive individuals. The most common symptoms are fever, skin lesions, anaemia, generalised lymphadenopathy, and hepatomegaly.
1F2L	Emmonsiosis An opportunistic infection caused by a variety of <i>Emmonsia</i> and <i>Emmonsia</i> -like fungal species. It was historically seen as a rare lung pathogen but is now increasingly reported as a disseminated infection in persons immunosuppressed, particularly as the result of HIV infection.
1F2L.0	Disseminated adiaspiromycosis An increasingly reported fulminant fungal infection caused by <i>Emmonsia</i> and <i>Emmonsia</i> -like fungal species. It is seen in the context of profound immunosuppression, especially from HIV infection. This is in contrast with pulmonary adiaspiromycosis, which is also caused by <i>Emmonsia</i> species but typically affects immunocompetent individuals. Its clinical presentation is similar to those of histoplasmosis and blastomycosis. Skin and lung involvement is characteristic but the CNS and blood may also be affected. Exclusions: Pulmonary adiaspiromycosis (1F2L.1)

1F2L.1	Pulmonary adiaspiromycosis
	Pulmonary adiaspiromycosis is an infection of the lungs due to inhalation of spores of the saprophytic soil fungus <i>Chrysosporium parvum</i> (formerly <i>Emmonsia parva</i>). The fungus affects many species of rodents but may occasionally infect humans. It is characterised by the presence of huge spherules (adiaspores) in the lungs.
	Inclusions: Adiaspiromycosis
1F2L.Y	Other specified emmonsiosis
1F2L.Z	Emmonsiosis, unspecified
1F2Y	Other specified mycoses
1F2Z	Mycoses, unspecified

Parasitic diseases (1F40-1G2Z)

Coded Elsewhere: Parasitic diseases in the fetus or newborn (KA64)

Malaria (1F40-1F4Z)

A disease caused by an infection with a protozoan parasite from the *Plasmodium* genus. This disease commonly presents with fever, chills, headache, nausea and vomiting, or malaise. Transmission is through the bite of an infected mosquito. Confirmation is commonly by identification of the *Plasmodium* genus in a blood sample.

Coding Note: In cases of mixed malaria code all relevant types separately.

Coded Elsewhere: HIV disease clinical stage 4 associated with malaria (1C61.3)
HIV disease clinical stage 3 associated with malaria (1C61.2)
HIV disease clinical stage 2 associated with malaria (1C61.1)
HIV disease clinical stage 1 associated with malaria (1C61.0)
Human immunodeficiency virus disease associated with malaria (1C61)
Malaria complicating pregnancy, childbirth, or the puerperium (JB63.60)
Other congenital malaria (KA64.Y)

1F40	Malaria due to <i>Plasmodium falciparum</i>
	A disease caused by an infection with the protozoan parasite <i>Plasmodium falciparum</i> . This disease is characterised by fever, chills, headache, myalgia, arthralgia, weakness, vomiting, or diarrhoea. This disease may also present with splenomegaly, anaemia, thrombocytopenia, hypoglycaemia, pulmonary or renal dysfunction, or neurologic changes. Transmission is through the bite of an infected mosquito. Confirmation is by identification of <i>Plasmodium falciparum</i> in a blood sample.
	Coding Note: Includes mixed infections of <i>Plasmodium falciparum</i> with any other <i>Plasmodium</i> species.
	Coded Elsewhere: Congenital <i>falciparum</i> malaria (KA64.1)

1F40.0	Plasmodium falciparum malaria with cerebral complications A disease of the cerebrum, caused by an infection with the protozoan parasite <i>Plasmodium falciparum</i> . This disease commonly presents with retinal whitening, splenomegaly, anaemia, thrombocytopenia, hypoglycaemia, pulmonary dysfunction, renal dysfunction, or neurologic changes. This disease may also present with fever, chills, headache, myalgia, arthralgia, weakness, vomiting, or diarrhoea. Transmission is through the bite of an infected mosquito. Confirmation is by identification of <i>Plasmodium falciparum</i> in a blood sample.
1F40.Y	Other severe and complicated <i>Plasmodium falciparum</i> malaria
Coding Note:	Includes mixed infections of <i>Plasmodium falciparum</i> with any other <i>Plasmodium</i> species.
1F40.Z	Malaria due to <i>Plasmodium falciparum</i>, unspecified
Coding Note:	Includes mixed infections of <i>Plasmodium falciparum</i> with any other <i>Plasmodium</i> species.
1F41	Malaria due to <i>Plasmodium vivax</i> A disease caused by an infection with the protozoan parasite <i>Plasmodium vivax</i> . This disease is characterised by fever, chills, headache, nausea and vomiting, body aches, or general malaise. Transmission is through the bite of an infected mosquito. Confirmation is by identification of <i>Plasmodium vivax</i> in a blood sample. Exclusions: when mixed with <i>Plasmodium falciparum</i> (1F40)
1F41.0	<i>Plasmodium vivax</i> malaria with rupture of spleen
1F41.Y	Malaria due to <i>Plasmodium vivax</i> with other complications
1F41.Z	<i>Plasmodium vivax</i> malaria without complication
1F42	Malaria due to <i>Plasmodium malariae</i> A disease caused by an infection with the protozoan parasite <i>Plasmodium malariae</i> . This disease is characterised by fever, chills, headache, nausea and vomiting, body aches, or general malaise. Transmission is through the bite of an infected mosquito. Confirmation is by identification of <i>Plasmodium malariae</i> in a blood sample. Exclusions: when mixed with <i>Plasmodium vivax</i> (1F41) when mixed with <i>Plasmodium falciparum</i> (1F40)
1F42.0	<i>Plasmodium malariae</i> malaria with nephropathy Quartan malarial nephropathy is a rare complication of <i>malariae</i> (quartan) malaria, especially occurring in children; it is a glomerulonephritis, usually fatal.
1F42.Y	Malaria due to <i>Plasmodium malariae</i> with other complications
1F42.Z	<i>Plasmodium malariae</i> malaria without complication

1F43**Malaria due to Plasmodium ovale**

A disease caused by an infection with the protozoan parasite Plasmodium ovale. This disease is characterised by fever, chills, headache, nausea and vomiting, body aches, or general malaise. Transmission is through the bite of an infected mosquito. Confirmation is by identification of Plasmodium ovale in a blood sample.

Exclusions: when mixed with Plasmodium malariae (1F42)
when mixed with Plasmodium falciparum (1F40)
when mixed with Plasmodium vivax (1F41)

1F44**Other parasitologically confirmed malaria****1F45****Malaria without parasitological confirmation**

Clinically diagnosed malaria without parasitological confirmation

Inclusions: Clinically diagnosed malaria without parasitological confirmation

1F4Z**Malaria, unspecified**

Coding Note: In cases of mixed malaria code all relevant types separately.

Nonintestinal protozoal diseases (1F50-1F5Z)

Infections with unicellular organisms of the subkingdom Protozoa.

Exclusions: Protozoal intestinal infections (1A30-1A3Z)

Coded Elsewhere: Amoebiasis (1A36)

Malaria without parasitological confirmation (1F45)

Protozoal diseases complicating pregnancy, childbirth or the puerperium (JB63.6)

1F50**Acanthamoebiasis****1F51****African trypanosomiasis**

A disease caused by an infection with the protozoan parasite Trypanosoma brucei. This disease presents with symptoms depending on the form of the protozoan parasite (Trypanosoma brucei rhodesiense or Trypanosoma brucei gambiense). Transmission is through the bite of an infected tsetse fly. Confirmation is by identification of Trypanosoma brucei in a blood or tissue sample.

1F51.0**Gambiense trypanosomiasis**

A disease caused by an infection with the protozoan parasite Trypanosoma brucei gambiense. This disease is characterised by fever, headache, muscle and joint aches, or malaise. This disease may also present with lymphadenopathy, weight loss, or neurological deficits. Transmission is through the bite of an infected tsetse fly. Confirmation is by identification of Trypanosoma brucei gambiense in a biopsy of the lymph node.

Inclusions: West African sleeping sickness
Infection due to Trypanosoma brucei gambiense

1F51.00	Meningitis in Gambiense trypanosomiasis
1F51.0Y	Other specified gambiense trypanosomiasis
1F51.0Z	Gambiense trypanosomiasis, unspecified
1F51.1	Rhodesiense trypanosomiasis A disease caused by an infection with the protozoan parasite <i>Trypanosoma brucei rhodesiense</i>. This disease is characterised by a chancre at the site of the bite. This disease may also present with fever, headache, muscle and joint aches, or lymphadenopathy. Transmission is through the bite of an infected tsetse fly. Confirmation is by identification of <i>Trypanosoma brucei rhodesiense</i> in a blood sample, lymph node fluid, or biopsy of the chancre. Inclusions: East African sleeping sickness Infection due to <i>Trypanosoma brucei rhodesiense</i>
1F51.10	Meningitis in Rhodesiense trypanosomiasis
1F51.1Y	Other specified rhodesiense trypanosomiasis
1F51.1Z	Rhodesiense trypanosomiasis, unspecified
1F51.Y	Other specified african trypanosomiasis
1F51.Z	African trypanosomiasis, unspecified
1F52	Babesiosis A disease caused by the protozoan parasite <i>Babesia</i>. This disease is characterised by reproduction and lysis of erythrocytes leading to symptoms that depend on the level of parasitaemia and immune status of the infected individual. This disease may present with fever, chills, malaise, myalgia, haemolytic anaemia, shock, or may be asymptomatic. Transmission is through the bite of an infected tick (<i>Ixodes</i>), or vertical transmission. Confirmation is by identification of <i>Babesia</i> in a blood smear, or detection of antibodies against <i>Babesia</i>. Inclusions: Piroplasmosis
1F53	Chagas disease A disease caused by an infection with the protozoan parasite <i>Trypanosoma cruzi</i>. This disease is characterised by fever, headache, lymphadenopathy, pallor, muscle pain, dyspnoea, swelling, or abdominal or chest pain. This disease may also be asymptomatic. Transmission is by direct contact with faeces from an infected triatomine bug, vertical transmission, iatrogenic transmission, or ingestion of contaminated food or water. Confirmation is by identification of <i>Trypanosoma cruzi</i> in a blood sample. Inclusions: American trypanosomiasis infection due to <i>Trypanosoma cruzi</i>

1F53.1	Acute Chagas disease without heart involvement A disease caused by an acute infection with the protozoan parasite <i>Trypanosoma cruzi</i>. This disease is characterised by fever, headache, lymphadenopathy, pallor, muscle pain, dyspnoea, swelling, or abdominal or chest pain. This disease presents with no cardiac involvement. Transmission is by direct contact with faeces from an infected triatomine bug, vertical transmission, iatrogenic transmission, or ingestion of contaminated food or water. Confirmation is by identification of <i>Trypanosoma cruzi</i> in a blood sample.
1F53.2	Chronic Chagas disease with heart involvement A disease caused by a chronic infection with the protozoan parasite <i>Trypanosoma cruzi</i>. This disease commonly presents with severe malaise or cardiac involvement (such as cardiomyopathy, cardiac failure, thromboembolism, bradyarrhythmias, tachyarrhythmias, apical aneurysms, or cardiac arrest). Transmission is by direct contact with faeces from an infected triatomine bug, vertical transmission, iatrogenic transmission, or ingestion of contaminated food or water. Confirmation is by identification of <i>Trypanosoma cruzi</i> in a blood sample.
1F53.3	Chagas disease with digestive system involvement A disease caused by an infection with the protozoan parasite <i>Trypanosoma cruzi</i>. This disease is characterised by severe malaise or digestive system involvement (such as megaoesophagus or megacolon). Transmission is by direct contact with faeces from an infected triatomine bug, vertical transmission, iatrogenic transmission, or ingestion of contaminated food or water. Confirmation is by identification of <i>Trypanosoma cruzi</i> in a blood sample.
1F53.4	Meningitis in Chagas disease
1F53.Y	Other specified Chagas disease
1F53.Z	Chagas disease, unspecified
1F54	Leishmaniasis Leishmaniasis is due to infection by vector-borne protozoa from the genus <i>Leishmania</i>. These protozoa exist as obligate intracellular parasites in human and mammalian hosts and are transmitted from host to host by certain species of sandfly. Depending on the <i>Leishmania</i> species involved, the resultant disease picture may range from a localised cutaneous ulcer through extensive mucocutaneous destruction to severe systemic disease.
1F54.0	Visceral leishmaniasis A disease caused by an infection with the protozoan parasite <i>Leishmania</i>. This disease is characterised by biphasic fever, hepatosplenomegaly, pancytopenia, wasting, darkening of the skin, or may be asymptomatic. Transmission is through the bite of an infected female phlebotomine sandfly. Confirmation is by identification of <i>Leishmania</i> from a tissue or blood sample, or detection of antibodies against <i>Leishmania</i>. Inclusions: Kala-azar

1F54.1**Cutaneous leishmaniasis**

Cutaneous leishmaniasis results from bites by sandflies infected by protozoan parasites of the genus *Leishmania*. *Phlebotomus* is the principal vector in the Old World (Mediterranean, North Africa, Ethiopia and Asia), where *L. major*, *L. tropica*, *L. aethiopica* and *L. donovani infantum* predominate. Other sandflies are responsible for transmitting the New World species, *L. mexicana* and *L. brasiliensis*. The commonest presentation is with one or more crusted nodules or ulcers on exposed sites which gradually heal with scarring. Mexican and Ethiopian forms have a tendency to cause diffuse infiltration of the skin; South American forms frequently progress to mucocutaneous leishmaniasis.

Coded Elsewhere: Post-kala-azar dermal leishmaniasis (1F54.0)

1F54.2**Mucocutaneous leishmaniasis**

Mucocutaneous leishmaniasis is a secondary infection of nasal and oral mucosae, predominantly by *Leishmania braziliensis*. It usually first manifests within two years of initial cutaneous infection but often after the latter has healed. It results from lymphatic or haematogenous spread of infection and can cause severe local tissue destruction.

Inclusions: *Leishmania braziliensis* infection

1F54.Z**Leishmaniasis, unspecified****1F55****Naegleriasis**

Any condition caused by an infection with the protozoan parasite *Naegleria*.

Coded Elsewhere: Primary amoebic meningoencephalitis (1D00.2)

1F56**Rhinosporidiosis**

Rhinosporidiosis is a chronic, usually painless localised infection of the mucous membranes. Formerly believed to be a fungus, the causative agent, *Rhinosporidium seeberi*, has also never been cultured. With 18S r DNA sequencing, this organism has been shown to be a protistan parasite. Rhinosporidiosis occurs worldwide, and the greatest numbers of cases are found in southern India and Sri Lanka.

1F57**Toxoplasmosis**

A disease caused by an infection with the protozoan parasite *Toxoplasma gondii*. This disease is characterised by fever, lymphadenitis, sore throat, or rash. Transmission is by direct ingestion of contaminated food, indirectly by food or water contaminated with infected cat faeces, or vertical transmission. Confirmation is by detection of antibodies against *Toxoplasma gondii*, or identification of *Toxoplasma gondii* in tissue, cerebrospinal fluid, blood, or other body fluids.

Coded Elsewhere: Congenital toxoplasmosis (KA64.0)

1F57.0**Hepatitis due to *Toxoplasma gondii***

A disease of the liver, caused by an infection with the protozoan parasite *Toxoplasma gondii*. This disease is characterised by jaundice. Transmission is by haematogenous spread to the liver after direct ingestion of contaminated food, or indirect transmission by consumption of food or water contaminated with infected cat faeces. Confirmation is by detection of antibodies against *Toxoplasma gondii* in a blood sample or identification of *Toxoplasma gondii* in hepatic tissue.

1F57.1	Meningoencephalitis due to <i>Toxoplasma gondii</i> A disease of the meninges and brain, caused by an infection with the protozoan parasite <i>Toxoplasma gondii</i>. This disease is characterised by seizures, neck pain, neurological deficits, or alterations in behaviour, cognition, or consciousness. Transmission is by haematogenous spread to the meninges and brain after direct ingestion of contaminated food, or indirect transmission by consumption of food or water contaminated with infected cat faeces. Confirmation is by detection of antibodies against <i>Toxoplasma gondii</i> in cerebrospinal fluid or identification of <i>Toxoplasma gondii</i> in cerebrospinal fluid, and advanced imaging of the nervous system. Inclusions: <i>Toxoplasma</i> meningoencephalitis
1F57.2	Pulmonary toxoplasmosis due to <i>Toxoplasma gondii</i> In immunodeficient patients, toxoplasmosis most often occurs in persons with defects in T cell-mediated immunity such as those receiving corticosteroids, anti-tumour necrosis factor (TNF) therapies, or cytotoxic drugs and those with hematologic malignancies, organ transplants, or acquired immunodeficiency syndrome (AIDS). Pulmonary toxoplasmosis in the immunodeficient patient may appear in the form of interstitial pneumonitis, necrotizing pneumonitis, consolidation, pleural effusion, or empyema, or all of these.[1] AIDS patients with <i>Toxoplasma</i> pneumonia present with cough, dyspnoea, and fever. As toxoplasmosis is generally seen only in advanced HIV infection with CD4 counts below 100, the majority of AIDS patients who develop toxoplasma pneumonia already have had previous HIV-associated opportunistic infections. In solid organ transplant patients, this is most commonly due to transplantation of a toxoplasma-seropositive lung or heart into a seronegative recipient, resulting in primary pulmonary disease. In bone marrow transplant patients, pulmonary toxoplasmosis occurs in 0.28% to 0.45% of patients. Unlike solid organ transplant patients, most of these patients have reactivation, not primary disease.[2] Source: [1]Montoya JG, et al. <i>Toxoplasma gondii</i>. In: edited by Mandell GL, Bennett JE, Dolin R, Mandell, Douglas, and Bennett's principles and practice of infectious diseases 7th ed. Philadelphia: Churchill Livingstone, 2010.3495-3526 [2]McCarthy J, et al. PARASITIC LUNG INFECTIONS. In: Robert J. Mason BJ et al. editors. Murray & Nadel's textbook of respiratory medicine. 5th ed. Philadelphia: Saunders Elsevier: 2010. p.661-698 Inclusions: Pulmonary toxoplasmosis
1F57.3	Eye disease due to <i>Toxoplasma gondii</i> Chorioretinitis or ocular toxoplasmosis is a relatively common manifestation of <i>T. gondii</i> infection. Ocular toxoplasmosis occurs when cysts deposited in or near the retina become active, producing tachyzoites. Focal necrotizing retinitis is the characteristic lesion, but retinal scars from prior reactivation are typically present. Inclusions: <i>Toxoplasma</i> oculopathy
1F57.Y	Other specified toxoplasmosis
1F57.Z	Toxoplasmosis, unspecified

1F58**Microsporidiosis****1F5Z****Unspecified protozoal disease**

Helminthiases (1F60-1F9Z)

Coded Elsewhere: Parasitic duodenitis (DA51.6Y)**Diseases due to nematodes (1F60-1F6Z)****1F60****Angiostrongyliasis**

A disease caused by an infection with the parasitic worm *Angiostrongylus*. This disease commonly presents with fever, headache, stiffness of the neck and back, tingling or painful feelings in the skin, nausea and vomiting, or may be asymptomatic. Transmission is by ingestion of larvae in contaminated food.

1F60.0**Eosinophilic meningitis due to *Angiostrongylus cantonensis***

A disease of the meninges caused by an infection with *Angiostrongylus cantonensis*. This disease is characterised by fever, headache, stiffness of the neck, nausea, vomiting, muscular weakness, or paraesthesia. This disease may also present with abscesses, cerebral oedema, haemorrhage, diplopia, ataxia, or blindness. Transmission is by ingestion of infected undercooked snails, slugs, or transport hosts (such as frogs, fresh water shrimp, or land crabs). Confirmation is by identification of *Angiostrongylus cantonensis* from a cerebrospinal fluid sample, blood sample, or from a food history.

1F60.1**Intestinal angiostrongyliasis**

A disease of the intestines caused by an infection with the parasitic worm *Angiostrongylus costaricensis*. This disease is characterised by abdominal pain, fever, nausea, or vomiting. This disease may also present with intestinal obstruction or perforation. Transmission is by ingestion of infected undercooked slugs, or food contaminated by infected slugs or their slime. Confirmation is by identification of *Angiostrongylus costaricensis* from a cerebrospinal fluid sample, blood sample or from a food history.

1F60.Y**Other specified angiostrongyliasis****1F60.Z****Angiostrongyliasis, unspecified****1F61****Anisakiasis**

A disease caused by an infection with the parasitic worm *Anisakis*. This disease presents with severe abdominal pain, nausea, vomiting, or a hypersensitivity reaction. Transmission is by ingestion of undercooked contaminated fish or squid. Confirmation is by a history of consumption of undercooked fish or squid, or identification of *Anisakis* in the intestines or in a vomit sample.

Inclusions: Infection due to *Anisakis* larvae

1F62**Ascariasis**

A disease caused by an infection with the parasitic worm *Ascaris lumbricoides*. This disease presents with symptoms depending on the extent of the infection, ranging from asymptomatic to intestinal blockage. Transmission is by the faecal-oral route from the ingestion of *Ascaris* eggs in contaminated food or water. Confirmation is by identification of *Ascaris* eggs in a faecal sample.

1F63**Capillariasis**

Coded Elsewhere: Capillariasis due to *Capillaria hepatica* (DB90.0)

1F63.0**Capillariasis of the intestine**

A condition caused by an infection with the parasitic worm *Capillaria philippinensis*. This condition is characterised by abdominal pain, diarrhoea, nausea, vomiting, or weight loss. Transmission is by ingestion of infected undercooked fish, or autoinfection. Confirmation is by identification of *Capillaria philippinensis* in a tissue biopsy of the small intestines or faecal sample.

Exclusions: Capillariasis due to *Capillaria hepatica* (DB90.0)

1F63.Y**Other specified capillariasis****1F63.Z****Capillariasis, unspecified****1F64****Dracunculiasis**

A disease resulting from drinking water contaminated with water fleas infected with larvae of the nematode *Dracunculus medinensis*. It may take up to a year from ingestion of larvae for a mature gravid female worm to migrate to the skin and discharge immature larvae on contact with water. Dracunculiasis typically manifests as an intensely pruritic papule on a lower extremity from which part of the worm may emerge. Secondary pyogenic infection is common. This may be preceded by generalised symptoms such as fever, pruritus, urticaria and oedema.

Inclusions: Guinea worm infestation

1F65**Enterobiasis**

A disease of the intestine, caused by an infection with the parasitic worm *Enterobius*. This disease is characterised by inflammation of the anus, pruritus, rectal pain, or may be asymptomatic. Transmission is by the faecal-oral route or airborne transmission of the eggs from the parasitic worm. Confirmation is by identification of *Enterobius* eggs around the perianal region.

Inclusions: Pinworm infection
Threadworm infection
Oxyuriasis

1F66**Filariasis**

infections with nematodes of the superfamily Filarioidea; presence of living worms in the body is mainly asymptomatic but the death of adult worms leads to granulomatous inflammation and permanent fibrosis; organisms of the genus *Elaeophora* infect wild elk and domestic sheep causing ischaemic necrosis of the brain, blindness, and dermatosis of the face.

Exclusions: Onchocerciasis (1F6A)

1F66.0**Loiasis**

A disease caused by an infection with the parasitic worm *Loa loa*. This disease is characterised by Calabar swellings found anywhere on the body (commonly found near joints). This disease may also present with generalised itching, muscle pain, joint pain, fatigue or may be asymptomatic. Transmission is through the bite of an infected fly (genus *Chrysops*). Confirmation is by identification of adult *Loa loa* in the skin or eye, *Loa loa* microfilariae in a blood sample obtained in the day (1000 - 1400), or detection of antibodies against *Loa loa* in a blood sample.

Inclusions: Eye worm disease of Africa
Loa loa infestation
 Calabar swelling

1F66.1**Mansonelliasis**

A disease caused by an infection with the parasitic worm *Mansonella*. This disease is characterised by pruritus, dermal pigmentary changes, fever, or lymphadenopathy, or may be asymptomatic. Transmission is through the bite of an infected midge (genus *Culicoides*) or blackfly (genus *Simulium*). Confirmation is by identification of *Mansonella* microfilariae in a skin or blood sample.

1F66.2**Filariasis due to *Brugia* species**

Coded Elsewhere: Filariasis due to *Brugia timori* (1F66.32)
 Filariasis due to *Brugia malayi* (1F66.31)

1F66.3**Lymphatic filariasis**

Infestation by filarial nematodes of the genera *Wuchereria* and *Brugia*. It is acquired via transcutaneous injection of larvae by mosquitoes previously infested with microfilariae from the blood of a human host. The adult worms live in the lymphatics but release microfilariae into the bloodstream to complete the life cycle of the parasite. Clinical disease occurs in only a minority of those infected. In the acute stage this may present as an acute painful adenolymphangitis with fever. Chronic infestation causes progressive obstruction of lymphatic vessels and can result in disfiguring lymphoedema (elephantiasis), particularly of the genitalia and lower extremities.

Exclusions: Lymphoedema due to lymphatic filariasis (BD93.13)

1F66.30**Filariasis due to *Wuchereria bancrofti***

This is a parasitic disease (usually an infectious tropical disease) that is caused by thread-like nematodes (roundworms) belonging to the superfamily Filarioidea, also known as "filariae".

Inclusions: Bancroftian filariasis

1F66.31**Filariasis due to *Brugia malayi***

This is a parasitic disease (usually an infectious tropical disease) that is caused by thread-like nematodes (roundworms) belonging to the superfamily Filarioidea, also known as "filariae". This diagnosis is due to a nematode (roundworm), one of the three causative agents of lymphatic filariasis in humans.

1F66.32	Filariasis due to <i>Brugia timori</i> This is a parasitic disease (usually an infectious tropical disease) that is caused by thread-like nematodes (roundworms) belonging to the superfamily Filarioidea, also known as "filariae". This diagnosis is due to a human filarial parasitic nematode (roundworm) which causes the disease "Timor filariasis."
1F66.3Z	Lymphatic filariasis, unspecified
1F66.4	Subcutaneous dirofilariasis Subcutaneous dirofilariasis normally results from the transmission of microfilariae of <i>Dirofilaria repens</i> from the latter's natural animal host to man via a mosquito bite. The adult worm cannot develop fully in man but typically manifests as a subcutaneous nodule, commonly located on or around the eyelids.
1F66.Y	Other specified filariasis
1F66.Z	Filariasis, unspecified
1F67	Gnathostomiasis A disease caused by an infection with the parasitic worm <i>Gnathostoma</i>. This disease is characterised by painful, itchy swelling under the skin from movement of the parasite under the skin. This disease may also initially present with fever, lethargy, abdominal pain, vomiting, or diarrhoea, and may infect other parts of the body (lungs, bladder, eyes, ears, nervous system). Transmission is commonly by ingestion of undercooked contaminated freshwater fish, eels, frogs, birds, or reptiles, or ingestion of contaminated water. Confirmation is commonly by detection of antibodies against <i>Gnathostoma</i>, identification of migratory skin lesions, eosinophilia, and history of potential exposure. Inclusions: Wandering swelling
1F68	Hookworm diseases A disease caused by an infection with the parasitic worm <i>Ancylostoma</i>. This disease is characterised by pruritus at the site of larval penetration. In mild infections, this disease may be asymptomatic; in moderate to severe infections, this disease may present with cough, pharyngeal irritation during larval migration in airways, iron-deficiency anaemia, abdominal pain, nausea, bloody diarrhoea, fatigue, or delayed development (mental or physical). Transmission is by direct contact with larvae from soil or sand contaminated with dog or cat faeces (by percutaneous migration of larvae), ingestion of larvae, or vertical transmission. Confirmation is by identification of <i>Ancylostoma</i> in a faecal sample. Inclusions: Hook-worm infestation by <i>Ancylostoma</i> Coded Elsewhere: Eosinophilic enteritis due to <i>Ancylostoma</i> (1F9Z)

1F68.0**Ancylostomiasis**

A disease caused by an infection with the parasitic hookworm *Ancylostoma duodenale*. This disease is characterised by pruritus at the site of larval penetration. In mild infections, this disease may be asymptomatic; in moderate to severe infections, this disease may present with cough, pharyngeal irritation during larval migration in airways, iron-deficiency anaemia, abdominal pain, nausea, bloody diarrhoea, fatigue, or delayed development (mental or physical). Transmission is by direct contact with larvae from soil or sand contaminated with dog or cat faeces (by percutaneous migration of larvae), ingestion of larvae, or vertical transmission. Confirmation is by identification of *Ancylostoma duodenale* in a faecal sample.

1F68.1**Necatoriasis**

A disease caused by an infection with the parasitic worm *Necator americanus*. This disease is characterised by pruritus at the site of larval penetration. In mild infections, this disease may be asymptomatic; in moderate to severe infections, this disease may present with cough, pharyngeal irritation during larval migration in airways, iron-deficiency anaemia, abdominal pain, nausea, arthralgia, or delayed development (mental or physical). Transmission is by direct contact with larvae from soil or sand contaminated with dog or cat faeces (by percutaneous migration of larvae). Confirmation is by identification of *Necator americanus* in a faecal sample.

Inclusions: Infection due to *Necator americanus*

1F68.2**Cutaneous larva migrans**

A disease caused by an infection with the parasitic worm larvae, commonly *Ancylostoma braziliense*, *A. caninum*, or *Uncinaria stenocephala*. This disease is characterised by intense pruritus and erythematous, serpiginous lesions due to migration of parasitic larvae in the upper dermis where the larvae penetrate the skin. Transmission is by direct contact with larvae from soil or sand contaminated with dog or cat faeces (by percutaneous migration of larvae).

1F69**Oesophagostomiasis**

This refers to an inflammation of small intestine caused by infection due to a nematode called *Oesophagostomum bifurcum*.

1F6A**Onchocerciasis**

Any condition caused by an infection with the parasitic worm *Onchocerca volvulus*. These conditions are characterised by the presence of firm subcutaneous nodules filled with adult worms, pruritus, long-term corneal inflammation (keratitis), or thickening of the corneal stroma. If untreated, these infections will lead to blindness. Transmission is through the bite of an infected *Simulium* fly.

1F6A.0**Onchocerciasis of the eye**

A disease of the eye, caused by an infection with the parasitic worm *Onchocerca volvulus*. This disease is characterised by transient punctate keratitis, or potentially blinding conditions (such as sclerosing keratitis, iridocyclitis, or optic atrophy). Transmission is through the bite of an infected *Simulium* fly. Confirmation is by identification of *Onchocerca volvulus* from the anterior chamber of the eye.

Inclusions: Ocular onchocerciasis

1F6A.1**Onchocerciasis of the skin**

A disease of the skin, caused by an infection with the parasitic worm *Onchocerca volvulus*. This disease is characterised by subcutaneous nodules on the skin (commonly affecting the iliac crests, ribs, knees, or trochanters). Transmission is through the bite of an infected *Simulium* fly. Confirmation is by identification of *Onchocerca volvulus* in a skin sample.

Inclusions: Cutaneous onchocerciasis

1F6A.Y**Other specified onchocerciasis****1F6B****Strongyloidiasis**

A disease caused by the parasitic worm *Strongyloides*. This disease presents with symptoms depending on the site of infection (gastrointestinal tract, pulmonary system, dermis, or systemic), or may be asymptomatic. Transmission is by direct contact through penetration of the skin (generally the feet) with larvae from faecally contaminated soil, or autoinfection of an established infection. Confirmation is by identification of *Strongyloides* larvae in faecal samples, duodenal fluid samples, sputum, pleural fluid, or tissue samples.

Exclusions: Trichostrongyliasis (1F6F)

1F6C**Syngamosis**

A disease caused by an infection with the parasitic worm *Mammomonogamus*. This disease is characterised by chronic nonproductive cough, crawling sensation in the throat, wheezing, or difficulties breathing. Transmission may be by ingestion of adult worms or eggs in contaminated food or water. Confirmation is by identification of adult *Mammomonogamus* by direct visualization, or identification of *Mammomonogamus* eggs in sputum or faecal samples.

1F6D**Toxocariasis**

A condition caused by an infection with the parasitic worm *Toxocara*. In ocular infections, this condition is characterised by vision loss or inflammation of the eye; in visceral infections, this condition is characterised by fever, coughing, enlarged liver, or pneumonia. This condition may also be asymptomatic. Transmission is by the faecal-oral route through the ingestion of food, water, or soil that contains *Toxocara* eggs (contaminated by faeces from an infected dog or cat). Confirmation is by detection of antibodies against *Toxocara* in a blood sample.

Inclusions: Toxocara infestation

1F6E**Trichinosis**

A disease caused by an infection with the parasitic worm *Trichinella*. This disease is characterised by fever, nausea, diarrhoea, vomiting, fatigue, or abdominal discomfort. This disease may also present with headache, chills, cough, swelling of the face and eyes, or aching joints and muscle pains. Transmission is by ingestion of contaminated meat. Confirmation is by detection of antibodies against *Trichinella* in a blood sample.

1F6F**Trichostrongyliasis**

A disease caused by an infection with the parasitic worm *Trichostrongylus*. This disease is characterised by abdominal pain, diarrhoea, weight loss, or may be asymptomatic. Transmission is by ingestion of contaminated food or water. Confirmation is by identification of *Trichostrongylus* eggs in a faecal sample.

1F6G**Trichuriasis**

A disease of the small intestine, caused by an infection with the parasitic worm *Trichuris trichiura*. This disease is commonly asymptomatic. This disease may also present with painful diarrhoea (containing a mixture of mucus, water, or blood). Transmission is by the faecal-oral route. Confirmation is by identification of *Trichuris trichiura* eggs in a faecal sample.

Inclusions: Trichocephaliasis

1F6H**Uncinariasis**

A disease caused by an infection with the parasitic worm *Uncinaria stenocephala*. This disease is characterised by pruritus at the site of larval penetration. In mild infections, this disease may be asymptomatic; in moderate to severe infections, this disease may present with cough, pharyngeal irritation during larval migration in airways, iron-deficiency anaemia, abdominal pain, nausea, arthralgia, or delayed development (mental or physical). Transmission is by direct contact with larvae from soil or sand contaminated with dog or cat faeces (by percutaneous migration of larvae). Confirmation is by identification of *Uncinaria stenocephala* in a faecal sample.

1F6Y**Other specified diseases due to nematodes****1F6Z****Diseases due to nematodes, unspecified****Diseases due to cestodes (1F70-1F7Z)****1F70****Cysticercosis**

A disease caused by an infection of tissue with larval cysts from the parasitic worm *Taenia solium*. This disease presents with symptoms depending on the site of infection (central nervous system, eye, or muscle). Transmission is through haematogenous spread of larvae to affected tissue after ingestion of *Taenia solium* eggs (or proglottids) in contaminated food or water. Confirmation is commonly by detection of antibodies against *Taenia solium* in a blood sample, cerebrospinal fluid, or faeces, and by advanced imaging of affected tissue.

Inclusions: cysticerciasis infection due to larval form of *Taenia solium*

1F70.0	Cysticercosis of central nervous system A disease of the central nervous system, caused by an infection of tissue with larval cysts from the parasitic worm <i>Taenia solium</i> . This disease presents with symptoms depending on the site of infection, the number and size of cysts, and the individual's immune status. This disease may present with epilepsy, chronic headache, hydrocephalus, neurological deficits, or may be asymptomatic. Transmission is by haematogenous spread of larvae to the central nervous system after ingestion of <i>Taenia solium</i> eggs in contaminated food or water. Confirmation is by detection of antibodies against <i>Taenia solium</i> in a blood sample, or cerebrospinal fluid and advanced imaging of the brain. Coded Elsewhere: Encephalitis due to cysticercosis (1D00.2)
1F70.00	Meningitis due to Cysticercosis A disease of the meninges, caused by an infection with larval cysts from the parasitic worm <i>Taenia solium</i> . This disease is characterised by headache, fever, seizures, or neurological deficits. Transmission is through hematogenous spread of larvae to the meninges after ingestion of <i>Taenia solium</i> eggs (or proglottids) in contaminated food or water. Confirmation is by advanced imaging and detection of antibodies against <i>Taenia solium</i> from serum or cerebrospinal fluid.
1F70.1	Cysticercosis of eye A disease of the eye, caused by an infection of tissue with larval cysts from the parasitic worm <i>Taenia solium</i> . This disease is characterised by cysts floating in the vitreous humour of the eye leading to impaired vision. Transmission is by haematogenous spread of larvae to the eye after ingestion of <i>Taenia solium</i> eggs in contaminated food or water. Confirmation is commonly by history of travel in parasite endemic regions and advanced imaging of the eye.
1F70.Y	Other specified cysticercosis
1F70.Z	Cysticercosis, unspecified
1F71	Diphyllobothriasis A disease caused by an infection with the parasitic worm <i>Diphyllobothrium</i> . This disease is characterised by abdominal discomfort, diarrhoea, vomiting, or weight loss. This disease may be asymptomatic. Transmission is by ingestion of infected undercooked fish. Confirmation is by identification of <i>Diphyllobothriasis</i> eggs in a faecal sample. Exclusions: larval diphyllobothriasis (1F75)
1F72	Dipylidiasis A condition caused by an infection with the parasitic worm <i>Dipylidium caninum</i> . This condition commonly present with abdominal pain, diarrhoea, anal pruritus, or may be asymptomatic. Transmission is by ingestion of an infected flea. Confirmation is by identification of <i>Dipylidium caninum</i> eggs in a faecal sample.
1F73	Echinococcosis Inclusions: Hydatidosis
1F73.0	Echinococcus infection of liver

- 1F73.1** **Echinococcus infection of lung**
- 1F73.2** **Echinococcus infection of bone**
- 1F73.3** **Echinococcus infection of central nervous system**
 Coded Elsewhere: Encephalitis due to echinococcosis (1D00.2)
- 1F73.Y** **Other specified echinococcosis**
- 1F73.Z** **Echinococcosis, unspecified**

1F74 Hymenolepiasis

A disease caused by an infection with the parasitic worm *Hymenolepis*. This disease is commonly asymptomatic. This disease may present with nausea, weakness, abdominal pain, diarrhoea, or vomiting. Transmission is by the ingestion of eggs commonly in contaminated food or water, or ingestion of infected arthropods. Confirmation is by identification of *Hymenolepis* eggs in a faecal sample.

1F75 Sparganosis

A disease caused by an infection with the parasitic worm *Spirometra*. This disease presents with symptoms depending on the site of the infection. Transmission is by ingestion of contaminated water or ingestion of infected undercooked second intermediate hosts (such as fish, reptiles or amphibians). Confirmation is by identification of *Spirometra* eggs in a faecal sample.

Inclusions: Larval diphyllbothriasis
 Spirometrosis

1F76 Taeniasis

A disease of the intestines, caused by an infection with the adult parasitic worm *Taenia*. This disease is characterised by abdominal pain, weight loss, diarrhoea, constipation, or may be asymptomatic. Transmission is by ingestion of larval cysts in undercooked beef or pork. Confirmation is by identification of *Taenia* eggs or proglottids in faecal samples (samples from multiple days).

Exclusions: Cysticercosis (1F70)

1F76.0 Taeniasis due to *Taenia solium*

A disease of the intestines, caused by an infection with the adult parasitic worm *Taenia solium*. This disease is characterised by abdominal pain, weight loss, diarrhoea, constipation, or may be asymptomatic. Transmission is by ingestion of larval cysts in undercooked pork. Confirmation is by identification of *Taenia solium* eggs or proglottidis in faecal samples (samples from multiple days).

Inclusions: *Taenia solium* taeniasis

1F76.1	Taeniasis due to <i>Taenia saginata</i>
	A disease of the intestines, caused by an infection with the adult parasitic worm <i>Taenia saginata</i> . This disease is characterised by abdominal pain, weight loss, diarrhoea, constipation, or may be asymptomatic. Transmission is by ingestion of larval cysts in undercooked beef. Confirmation is by identification of <i>Taenia saginata</i> eggs or proglottids in faecal samples (samples from multiple days).
	Inclusions: Infection due to adult tapeworm <i>Taenia saginata</i> <i>Taenia saginata</i> taeniasis
1F76.Y	Other specified taeniasis
1F76.Z	Taeniasis, unspecified
1F7Y	Other specified diseases due to cestodes
1F7Z	Diseases due to cestodes, unspecified

Diseases due to trematodes (1F80-1F8Z)

1F80	Clonorchiasis
	A condition caused by an infection with the parasitic worm <i>Clonorchis sinensis</i> . This condition commonly presents with inflammation and obstruction of the biliary ducts. This condition may also present with abdominal pain, nausea, or diarrhoea. Transmission is commonly by ingestion of undercooked fish infected with parasitic cysts. Confirmation is by identification of <i>Clonorchis sinensis</i> eggs in a faecal sample.
	Inclusions: Chinese liver fluke disease Oriental liver fluke disease Infection due to <i>Clonorchis sinensis</i>
1F81	Dicrocoeliasis
	A disease caused by an infection with the parasitic worm <i>Dicrocoelium dendriticum</i> . This disease is commonly asymptomatic. This disease may present with cholecystitis, liver abscesses, or upper abdominal pain. Transmission is by ingestion of infected ants. Confirmation is by identification of <i>Dicrocoelium dendriticum</i> eggs in a faecal sample or duodenal fluid.
	Inclusions: Lancet fluke infection

1F82

Fascioliasis

A disease of the hepatic system, caused by an infection with the parasitic worm *Fasciola*. In the acute phase, this disease is characterised by upper abdominal pain, fever, urticaria, shortness of breath, nausea, or vomiting due to migration of the parasite from the intestines to the liver. In the chronic phase, this disease is characterised by cholestasis, cholangitis, pancreatitis, or gallstones. This disease may be asymptomatic. Transmission is by ingestion of undercooked contaminated aquatic plants or contaminated water. Confirmation is commonly by identification of *Fasciola* eggs in a faecal sample (after the individual has followed a liver-free diet prior to testing), or detection of antibodies against *Fasciola*.

Inclusions: Sheep liver fluke disease

1F83

Fasciolopsiasis

A disease caused by an infection with the parasitic worm *Fasciolopsis buski*. This disease is characterised by abdominal pain or diarrhoea, or may be asymptomatic. This disease may also present with oedema of the face, abdomen, or legs, vomiting, anorexia, or intestinal obstruction. Transmission is by ingestion of undercooked contaminated aquatic plants. Confirmation is by identification of *Fasciolopsis buski* eggs in a faecal sample.

1F84

Opisthorchiasis

A disease caused by an infection with the parasitic worm *Opisthorchis*. This disease is commonly asymptomatic. In mild cases, this disease may present with dyspepsia, abdominal pain, diarrhoea, or constipation; in severe cases, this disease may present with hepatomegaly and malnutrition; in rare cases, this disease may present with cholangitis, cholecystitis, and cholangiocarcinoma. Transmission is by ingestion of infected undercooked freshwater fish. Confirmation is by identification of *Opisthorchis* in a faecal sample.

1F85

Paragonimiasis

A disease caused by an infection with the parasitic worm *Paragonimus*. This disease is characterised by cough or haemoptysis, or may be asymptomatic. This disease may present with other symptoms depending on the site where the parasite migrates to. Transmission is commonly by ingestion of undercooked contaminated crustaceans (crab or crayfish). Confirmation is commonly by identification of *Paragonimus* eggs in a sputum or faecal sample.

Inclusions: lung fluke disease
infection due to *paragonimus* species
Infestation due to *Paragonimus* species

1F86

Schistosomiasis

An infestation caused by helminths of the genus *Schistosoma*. The clinical features vary according to the species involved but the principal organs affected are the gastrointestinal tract and bladder.

Inclusions: snail fever

1F86.0	Schistosomiasis due to <i>Schistosoma haematobium</i> A disease caused by an infection with the parasitic worm <i>Schistosoma haematobium</i> . This disease is characterised by haematuria, scarring, calcification, or squamous cell carcinoma. This disease may also present with embolic egg granulomas in the brain or spinal cord. Transmission is by direct contact with freshwater that has been contaminated with <i>Schistosoma haematobium</i> eggs or snails that carry <i>Schistosoma haematobium</i> .
1F86.1	Schistosomiasis due to <i>Schistosoma mansoni</i> A disease caused by an infection with the parasitic worm <i>Schistosoma mansoni</i> . This disease commonly presents with Katayama fever, hepatic perisinusoidal egg granulomas, Symmers' pipe stem periportal fibrosis, or portal hypertension. This disease may also present with embolic egg granulomas in the brain or spinal cord. Transmission is by direct contact with freshwater that has been contaminated with <i>Schistosoma mansoni</i> eggs or snails that carry <i>Schistosoma mansoni</i> . Confirmation is by identification of the <i>Schistosoma mansoni</i> eggs in a faecal sample.
1F86.2	Schistosomiasis due to <i>Schistosoma japonicum</i> A disease caused by an infection with the parasitic worm <i>Schistosoma japonicum</i> . This disease is characterised by Katayama fever, hepatic perisinusoidal egg granulomas, Symmers' pipe stem periportal fibrosis, or portal hypertension. This disease may also present with embolic egg granulomas in the brain or spinal cord. Transmission is by direct contact with freshwater that has been contaminated with <i>Schistosoma japonicum</i> eggs or snails that carry <i>Schistosoma japonicum</i> . Confirmation is by identification of the <i>Schistosoma japonicum</i> eggs in a faecal sample. <i>Inclusions:</i> Asiatic schistosomiasis
1F86.3	Other schistosomiasis
1F86.4	Cercarial dermatitis A disease caused by an infection with the parasitic worm <i>Schistosoma</i> . This disease is characterised by tingling, burning, itching of the skin, small reddish pimples, or small blisters. Transmission is by direct contact with contaminated water. Confirmation is by identification of <i>Schistosoma</i> eggs in a faecal, urine, or blood sample.
1F86.5	Schistosomal pneumonitis
1F86.Z	Schistosomiasis due to unspecified or unknown <i>Schistosoma</i> species
1F8Y	Other specified diseases due to trematodes
1F8Z	Diseases due to trematodes, unspecified
1F90	Other and unspecified infestation by parasitic worms
1F90.0	Mixed intestinal helminthiasis
1F90.1	Intestinal parasitic infestation not otherwise specified This concept should be used for parasitic infestation of the intestine only when no more precise details are available.

- 1F90.2** **Intestinal helminthiasis, unspecified**
- 1F90.Y** **Other specified other and unspecified infestation by parasitic worms**
- 1F90.Z** **Other and unspecified infestation by parasitic worms, unspecified**

1F91 **Diphyllobothriasis and sparganosis**

Diphyllobothriasis is defined as infection with the cestode *Diphyllobothrium latum* or other *Diphyllobothrium* species, which occurs accidentally in humans who ingest water containing infected cyclops, eating raw or inadequately cooked flesh. Manifestations may include abdominal discomfort, diarrhoea, vomiting and megaloblastic anaemia. Massive infections may result in intestinal obstruction.

1F9Z **Helminthiasis, unspecified**

Infestations by ectoparasites (1G00-1G0Z)

Diseases caused by parasitic organisms which normally live on the surface of the host.

Coded Elsewhere: Epidemic louse-borne typhus fever due to *Rickettsia prowazekii* (1C30.0)
 Recrudescent typhus (1C30.1)

1G00 **Pediculosis**

A condition of the skin, hair, or genital region caused by an infection with the parasite *Pediculus*. This disease is characterised by pruritus. This condition also presents with symptoms depending on the site of infection. Transmission is by direct or indirect contact with an infected individual or animal. Confirmation is by identification of *Pediculus*.

1G00.0 **Pediculosis capitis**

A condition of the scalp and hair shaft, caused by an infection with the parasite *Pediculus humanus capitis*. This condition is characterised by pruritus which may lead to sores or thickened discoloured skin. Transmission is by direct or indirect contact with an infected individual or animal. Confirmation is by identification of *Pediculus humanus capitis* eggs or *Pediculus humanus capitis*.

1G00.1 **Pediculosis corporis**

A condition of the skin, caused by an infection with the parasite *Pediculus humanus corporis*. This condition is characterised by pruritus which may lead to sores or thickened discoloured skin. Transmission is by direct or indirect contact with an infected individual or animal. Confirmation is by identification of *Pediculus humanus corporis* eggs or *Pediculus humanus corporis*.

1G00.Z **Pediculosis of unspecified site or type**

1G01**Myiasis**

A disease of the tissues, caused by an infection with fly larvae from the order Diptera. This disease is characterised by a lump developing in the tissue. Transmission is by ingestion of contaminated larvae, direct contact with an infected mosquito, tick, fly, or indirect contact with infected fly eggs. Confirmation is by identification of Diptera from a tissue sample.

Inclusions: infestation by larvae of flies

1G01.0**Ocular myiasis**

A disease of the eye, caused by an infection with fly larvae from the order Diptera. This disease is characterised by a lump developing in the tissue. Transmission is by ingestion of contaminated larvae, direct contact with an infected mosquito, tick, fly, or indirect contact with infected fly eggs. Confirmation is by identification of Diptera.

1G01.1**Nasopharyngeal myiasis****1G01.2****Laryngeal myiasis****1G01.3****Cutaneous myiasis**

The infestation of the skin or subcutaneous tissues by the larvae of certain flies (*Phormia regina*, *Cordylobia anthropophaga*, *Cochliomyia hominivorax*, *C. macellaria*, *Wohlfahrtia vigil*, *W. meigeni*, *W. opaca*, *Dermatobia hominis*, *Sarcophaga krameri*). characterised by a painful boil-like lesion containing one or more larvae with severe pruritus and local destruction of tissue.

1G01.Y**Other specified myiasis****1G01.Z****Myiasis, unspecified****1G02****External hirudiniasis**

Infestation of the skin by leeches. Sensitisation to antigenic substances deposited in the skin can result in urticarial weals and bullae.

Exclusions: Internal hirudiniasis (1F60-1F9Z)

1G03**Pthiriasis**

Infestation most commonly of pubic hair and less commonly of body hair or eyelashes by the crab louse, *Pthirus pubis*. Transmission is by direct, typically sexual contact with an infected individual. Confirmation is by identification of *Pthirus pubis* or its eggs.

Inclusions: Infestation by crab lice

1G04	Scabies A highly contagious infestation of the skin by the mite <i>Sarcoptes scabiei</i> var. <i>hominis</i>. It may result in epidemics when introduced into institutions such as schools and nursing homes. The mites burrow into the skin, favouring the extremities, genitalia and, in infants, the axillae. The characteristic widespread intensely pruritic papulovesicular rash results largely from the host response rather than directly to burrowing by mites. Where such a response is absent as in immunosuppressed or debilitated patients, unchecked proliferation of mites results in crusted scabies. Sarcoptic mites from other mammals such as dogs may cause a transient pruritic eruption.
1G04.0	Classical scabies
1G04.1	Crusted scabies Crusted scabies results from unchecked proliferation of the human scabies mite in individuals who are unable to mount an adequate immune response to infestation. Extensive thick crusts containing vast numbers of mites form over the skin, particularly of the extremities. Because itching is usually absent, the diagnosis is frequently overlooked. Patients with crusted scabies may serve as the source for widespread outbreaks of scabies in institutions such as hospitals and care homes.
1G04.Y	Other and unspecified scabies
1G05	Tungiasis A disease of the skin, caused by an infection with the parasite <i>Tunga penetrans</i>. This disease is characterised by lesions (white patch with a black dot in the middle), skin inflammation, or pruritus surrounding the lesion. This disease may also be asymptomatic. Transmission is through the bite of an infected flea, or by direct contact with an infected animal. Confirmation is by identification of <i>Tunga penetrans</i> or travel history.
1G06	Cimicosis Infestation by bedbugs, which are blood-sucking temporary ectoparasites. The most common species to attack humans is <i>Cimex lectularius</i>. In individuals who are not sensitized by previous exposure, there may be no symptoms or signs other than purpuric macules at the sites of bites. Weals, papules or bullae may occur in sensitized individuals.
1G07	Infestation by mites Coded Elsewhere: Scabies (1G04) Cutaneous reactions to zoonotic mites (NE61)
1G07.0	Infestation by Demodex Infestation with <i>Demodex</i> mites. <i>Demodex folliculorum</i> is a saprophytic mite of the human pilosebaceous unit with a predilection for facial skin and eyelashes. <i>Demodex brevis</i> is found in the sebaceous glands of the eyelash follicle and in the lobules of eyelid meibomian glands. Although infestation is very common and normally symptomless, the mites have been linked to papulopustular rosacea and chronic blepharitis.
1G07.Y	Infestation of the skin by other specified parasitic mites

1G0Y	Infestation by other specified ectoparasite
1G0Z	Infestation by unknown or unspecified ectoparasite
1G2Y	Other specified parasitic diseases
1G2Z	Unspecified parasitic diseases

Sepsis (1G40-1G41)

Coding Note: Any type of infection - bacterial, viral, fungal or protozoal, can cause sepsis and must be coded as well. When the site of infection is unknown, select a code for Infection of unspecified site by organism followed by the appropriate code for sepsis.

Exclusions:

- Plague (1B93)
- Acute or fulminant melioidosis (1C42)
- Tularaemia (1B94)
- Other infection during labour (JB0D)
- Injury or harm arising following infusion, transfusion or therapeutic injection, not elsewhere classified (NE80)
- Genital tract or pelvic infection following abortion, ectopic or molar pregnancy (JA05.0)
- Disseminated gonococcal infection (1A73)
- Extraintestinal yersiniosis (1B9A)
- Injury or harm arising from a procedure, not elsewhere classified (NE81)
- Meningococcal disease (1C1C)
- Systemic inflammatory response syndrome of noninfectious origin (MG46)

Coded Elsewhere:

- Puerperal sepsis (JB40.0)
- Sepsis of fetus or newborn (KA60)
- Obstetric pyaemic or septic embolism (JB42.3)

1G40 Sepsis without septic shock

Sepsis is defined as a life-threatening organ dysfunction caused by a dysregulated host response to infection.

Coding Note: Any type of infection - bacterial, viral, fungal or protozoal, can cause sepsis and must be coded as well. When the site of infection is unknown, select a code for Infection of unspecified site by organism followed by the appropriate code for sepsis.

Exclusions: Septicaemia (MA15.0)

1G41**Sepsis with septic shock**

Septic shock is a subset of sepsis in which circulatory, cellular and metabolic abnormalities are profound enough to substantially increase mortality.

Coding Note:

Any type of infection - bacterial, viral, fungal or protozoal, can cause sepsis and must be coded as well. When the site of infection is unknown, select a code for Infection of unspecified site by organism followed by the appropriate code for sepsis.

Exclusions: Bacteraemia (MA15.0)

1G60**Certain other disorders of infectious origin**

Miscellaneous disorders of infectious origin not classifiable elsewhere including those due to algae and oomycetes

1G60.0**Mycetoma of unknown or unspecified type**

Mycetoma is a destructive localised chronic infection of skin, subcutaneous tissue and bone, most commonly affecting the foot. It can be caused by either fungi (eumycetoma) or filamentous bacteria (actinomycetoma). Where possible it should be classified more precisely as either actinomycetoma, the commonest type, or eumycetoma

Exclusions: Actinomycetoma (1C43)

Eumycetoma (1F29)

1G60.1**Pythiosis**

Pythiosis is a life-threatening infection by the oömycete *Pythium insidiosum*. Although infection in animals occurs widely across the world, human pythiosis is largely confined to Thailand and, with the exception of ocular disease, is closely associated with underlying haematological disease, especially thalassaemia. There is a high mortality in patients with disseminated or vascular disease. In the latter form, invasion of arterial wall results in vascular occlusion and a frequent need for amputation. Patients with ocular pythiosis commonly require enucleation. A small proportion of infections are limited to the skin and subcutaneous tissues.

1G60.2**Protothecosis**

Protothecosis is a rare opportunistic infection in humans caused by achloric algae of the genus *Prototheca*. The infection is usually localised and may be associated with antecedent local trauma. It is generally located on exposed sites and remains confined to skin and subcutaneous tissues. In immunocompromised patients, however, widespread cutaneous, subcutaneous or deep infection may occur.

1G60.Y**Other specified disorders of infectious origin not elsewhere classified**

Sequelae of infectious diseases (1G80-1G8Y)

A sequela is a chronic condition resulting from an acute condition and begins during that acute condition. The acute condition is no longer present. The sequela continues after the acute phase of the condition is resolved. For infectious diseases, the original infection is no longer present.

The sequelae categories indicate infections as the cause of sequelae which are themselves classified elsewhere.

Not to be used for chronic infections. Code the chronic infection to chronic or active infectious disease as appropriate.

Use an additional code, if desired, to identify the specific sequelae.

Coded Elsewhere: Sequelae of inflammatory diseases of central nervous system (1D0Y)

1G80

Sequelae of tuberculosis

Sequela of tuberculosis is a chronic condition resulting from an acute tuberculosis. *Mycobacterium tuberculosis* is no longer present. The sequela continues after the acute phase is resolved.

1G81

Sequelae of trachoma

This refers to a pathological condition resulting from an infectious disease caused by the *Chlamydia trachomatis* bacterium which produces a characteristic roughening of the inner surface of the eyelids.

1G82

Sequelae of leprosy

This refers to a pathological condition resulting from a chronic disease caused by the bacteria *Mycobacterium leprae* and *Mycobacterium lepromatosis*.

1G83

Sequelae of poliomyelitis

Sequelae of poliomyelitis refers to the residuals of acute poliomyelitis as well as other disorders that have an etiological link to either the acute polio infection or to chronic deficits resulting from the acute infection. Disorders that may manifest late in the lives of polio survivors include early advanced degenerative arthritis, sleep disorders, respiratory insufficiency, and a variety of mental disorders.

Exclusions: Post polio progressive muscular atrophy (8B62)

1G84

Sequelae of viral encephalitis

1G85

Sequelae of diphtheria

This refers to conditions that develop as a consequence of a bacterial infection of the respiratory tract with *Corynebacterium diphtheriae*.

1G8Y

Sequelae of other specified infectious diseases

1H0Z

Infection, unspecified

CHAPTER 02

Neoplasms

This chapter has 325 four-character categories.

Code range starts with 2A00

An abnormal or uncontrolled cellular proliferation which is not coordinated with an organism's requirements for normal tissue growth, replacement or repair.

Coded Elsewhere: Inherited cancer-predisposing syndromes

This chapter contains the following top level blocks:

- Neoplasms of brain or central nervous system
- Neoplasms of haematopoietic or lymphoid tissues
- Malignant neoplasms, except primary neoplasms of lymphoid, haematopoietic, central nervous system or related tissues
- In situ neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues
- Benign neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues
- Neoplasms of uncertain behaviour, except of lymphoid, haematopoietic, central nervous system or related tissues
- Neoplasms of unknown behaviour, except of lymphoid, haematopoietic, central nervous system or related tissues
- Inherited cancer-predisposing syndromes

Neoplasms of brain or central nervous system (2A00-2A0Z)

A benign or malignant neoplasm that affects the brain, meninges, or spinal cord. Representative examples of primary neoplasms include astrocytoma, oligodendroglioma, ependymoma, and meningioma.

2A00	Primary neoplasms of brain
2A00.0	Gliomas of brain
2A00.00	Glioblastoma of brain Glioblastomas are malignant astrocytic tumours (grade IV according to the WHO classification). They represent the most frequent brain tumours in adults. They may occur at any age, but 70% of cases are seen in patients between 45 and 70 years of age. The tumours are usually located in the brain hemispheres, but can be found anywhere in the central nervous system. Inclusions: glioblastoma NOS
2A00.0Y	Other specified gliomas of brain

2A00.0Z	Gliomas of brain, unspecified
2A00.1	Embryonal tumours of brain
2A00.10	Medulloblastoma of brain A malignant, invasive embryonal neoplasm arising from the cerebellum. It occurs predominantly in children and has the tendency to metastasize via the cerebrospinal fluid pathways. Signs and symptoms include truncal ataxia, disturbed gait, lethargy, headache, and vomiting. There are four histologic variants: anaplastic medulloblastoma, desmoplastic/nodular medulloblastoma, large cell medulloblastoma, and medulloblastoma with extensive nodularity.
2A00.11	Central primitive neuroectodermal tumour A malignant neoplasm that originates in the neuroectoderm. The neuroectoderm constitutes the portion of the ectoderm of the early embryo that gives rise to the central and peripheral nervous systems and includes some glial cell precursors.
2A00.1Y	Other specified embryonal tumours of brain
2A00.1Z	Embryonal tumours of brain, unspecified
2A00.2	Tumours of neuroepithelial tissue of brain
2A00.20	Tumours of the pineal gland or pineal region
2A00.21	Mixed neuronal-glial tumours
2A00.22	Choroid plexus tumours
2A00.2Y	Other specified tumours of neuroepithelial tissue of brain
2A00.2Z	Tumours of neuroepithelial tissue of brain, unspecified
2A00.3	Central neurocytoma of brain Central neurocytoma is a very rare brain tumour of young adults. It is typically found in the lateral ventricles and occasionally in the third ventricle. Symptoms are those of increased intracranial pressure.
2A00.4	Astroblastoma of the brain A rare glial neoplasm more commonly found in young adults. It is characterised by tumour cells with characteristics suggestive of an astrocytic origin (positive for GFAP), arranged perivascularly. The cells have broad, non-tapering processes radiating towards a central blood vessel. The biologic behaviour of astroblastomas is variable, so no WHO grade has been established, yet. (Adapted from WHO.)
2A00.5	Primary neoplasm of brain of unknown or unspecified type
2A01	Primary neoplasms of meninges
2A01.0	Meningiomas
2A01.00	Primary malignant meningioma
2A01.0Y	Other specified meningiomas

2A01.0Z	Meningiomas, unspecified
2A01.1	Mesenchymal tumours of meninges
2A01.2	Primary neoplasm of meninges of unknown or unspecified type
2A02	Primary neoplasm of spinal cord, cranial nerves or remaining parts of central nervous system
2A02.0	Gliomas of spinal cord, cranial nerves or other parts of the central nervous system
2A02.00	Glioblastoma of spinal cord, cranial nerves or other parts of central nervous system
2A02.0Y	Other specified gliomas of spinal cord, cranial nerves or other parts of the central nervous system
2A02.0Z	Gliomas of spinal cord, cranial nerves or other parts of the central nervous system, unspecified
2A02.1	Tumours of cranial or paraspinal nerves
2A02.10	Malignant peripheral nerve sheath tumour of cranial or paraspinal nerves Malignant schwannoma is a tumour of the peripheral nervous system that arises in the nerve sheath. Exclusions: Malignant nerve sheath tumour of peripheral nerves or autonomic nervous system, primary site (2B5E)
2A02.11	Paraspinal neuroblastoma
2A02.12	Malignant neoplasm of the optic nerve
2A02.1Y	Other specified tumours of cranial or paraspinal nerves
2A02.1Z	Tumours of cranial or paraspinal nerves, unspecified
2A02.2	Primary neoplasm of spinal cord of unknown or unspecified type
2A02.3	Benign neoplasm of cranial nerves This is a tumour of cranial nerves having none of the characteristics of a malignant neoplasm.
2A02.4	Benign neoplasm of spinal cord
2A0Z	Other and unspecified neoplasms of brain or central nervous system

Neoplasms of haematopoietic or lymphoid tissues (2A20-2B3Z)

A neoplasm arising from hematopoietic cells found in the bone marrow, peripheral blood, lymph nodes and spleen (organs of the hematopoietic system). Hematopoietic cell neoplasms can also involve other anatomic sites (e.g. central nervous system, gastrointestinal tract), either by haematogenous spread, direct tumour infiltration, or neoplastic transformation of extranodal lymphoid tissues. The commonest forms are the various types of leukaemia, Hodgkin and non-Hodgkin lymphomas, myeloproliferative neoplasms and myelodysplastic syndromes.

Coded Elsewhere: Symptoms of blood, blood-forming organs, or the immune system (MA00-MA0Y)

Myeloproliferative neoplasms (2A20-2A22)

2A20	Non mast cell myeloproliferative neoplasms Coded Elsewhere: Acquired thrombocytosis (3B63.1)
2A20.0	Chronic myelogenous leukaemia, BCR-ABL1-positive Exclusions: Atypical chronic myeloid leukaemia, BCR-ABL1- negative (2A41) Chronic myelomonocytic leukaemia (2A40) Other and unspecified myeloproliferative neoplasms (2A22) Chronic myeloid leukaemia, not elsewhere classified (2B33.2)
2A20.00	Chronic myelogenous leukaemia with blast crisis
2A20.01	Chronic myelogenous leukaemia, Philadelphia chromosome (Ph1) positive
2A20.02	Chronic myelogenous leukaemia, t(9:22)(q34; q11)
2A20.03	Naegeli-type monocytic leukaemia
2A20.0Y	Other specified chronic myelogenous leukaemia, BCR-ABL1-positive
2A20.0Z	Chronic myelogenous leukaemia, BCR-ABL1-positive, unspecified
2A20.1	Chronic neutrophilic leukaemia A rare chronic myeloproliferative neoplasm characterised by sustained peripheral blood neutrophilia, bone marrow hypercellularity due to neutrophilic granulocyte proliferation, and hepatosplenomegaly. The neutrophils lack dysplasia and often show toxic granulations. There is no detectable Philadelphia chromosome or BCR/ABL1 fusion gene.
2A20.2	Primary myelofibrosis Inclusions: chronic idiopathic myelofibrosis Exclusions: Acute panmyelosis with myelofibrosis (2A60.38)

2A20.3	Chronic eosinophilic leukaemia, not elsewhere classified A chronic myeloproliferative neoplasm characterised by persistent eosinophilia in the blood, bone marrow and peripheral tissues. Organ damage occurs as a result of leukaemic infiltration or the release of cytokines, enzymes or other proteins by the eosinophils. Chronic eosinophilic leukaemia, not otherwise specified excludes patients with a Ph chromosome, BCR-ABL1 fusion gene or rearrangement of PDGFRA, PDGFRB or FGFR1.
2A20.4	Polycythaemia vera
2A20.5	Non mast cell myeloproliferative neoplasm, unclassifiable Cases that have definite features of myeloproliferative neoplasms (MPN), but fail to meet the criteria of a specific MPN subtype.
2A20.Y	Other specified non mast cell myeloproliferative neoplasms
2A20.Z	Non mast cell myeloproliferative neoplasms, unspecified
2A21	Mastocytosis Mastocytosis is due to a clonal, neoplastic proliferation of mast cells that accumulate in one or more organ systems. Activating mutations of KIT are frequently found. It is characterised by the presence of multifocal compact clusters or cohesive aggregates/infiltrates of abnormal mast cells. The disorder is heterogeneous, ranging from skin lesions that may spontaneously regress to highly aggressive neoplasms associated with multiorgan failure and short survival. Subtypes of mastocytosis are recognised mainly by the distribution of the disease and clinical manifestations. In cutaneous mastocytosis (CM), the mast cell infiltration remains confined to the skin, whereas systemic mastocytosis (SM) is characterised by involvement of at least one extracutaneous organ with or without evidence of skin lesions. Mastocytosis should be strictly separated from mast cell hyperplasia or mast cell activation states without morphological and/or molecular abnormalities that characterize the neoplastic proliferation.
2A21.0	Systemic mastocytosis Systemic mastocytosis (SM) comprises a heterogeneous group of rare acquired and chronic haematological malignancies that are related to an abnormal proliferation of mast cells in tissue, including bone marrow, with or without skin involvement. SM can be divided into indolent SM (ISM) and aggressive SM (ASM).
2A21.00	Mast cell leukaemia
2A21.0Y	Other specified systemic mastocytosis
2A21.0Z	Systemic mastocytosis, unspecified
2A21.1	Cutaneous mastocytosis Cutaneous mastocytosis is characterised by abnormal accumulation and proliferation of cutaneous mast cells. Most types are isolated but cutaneous mastocytosis can occur in association with systemic disease. Clinical forms include cutaneous mastocytoma, urticaria pigmentosa (the most frequent form), pseudoxanthomatous nodular cutaneous mastocytosis, telangiectasia macularis eruptiva perstans and diffuse cutaneous mastocytosis.

2A21.10	Urticaria pigmentosa Inclusions: Maculopapular cutaneous mastocytosis
2A21.1Y	Other specified cutaneous mastocytosis
2A21.2	Mast cell sarcoma A rare entity characterised by localised but destructive growth of a tumour consisting of highly atypical, immature mast cells.
2A21.3	Extracutaneous mastocytoma A localised tumour consisting of mature mast cells.
2A21.Y	Other specified mastocytosis
2A21.Z	Mastocytosis, unspecified
2A22	Other and unspecified myeloproliferative neoplasms Exclusions: Chronic myelogenous leukaemia, BCR-ABL1-positive (2A20.0) Atypical chronic myeloid leukaemia, BCR-ABL1- negative (2A41)

Myelodysplastic syndromes (2A30-2A3Z)

Clonal hematopoietic disorders characterised by dysplasia and ineffective hematopoiesis in one or more of the hematopoietic cell lines. The dysplasia may be accompanied by an increase in myeloblasts, but the number is less than 20% in marrow and blood, which, according to the WHO guidelines, is the requisite threshold for the diagnosis of acute myeloid leukaemia.

Exclusions: Therapy-related myeloid neoplasms (2A60.2)
Drug-induced aplastic anaemia (3A70.10)

2A30	Refractory anaemia
2A31	Refractory neutropaenia A myelodysplastic syndrome characterised by the presence of at least 10% dysplastic neutrophils in the bone marrow or the peripheral blood.
2A32	Refractory thrombocytopenia A myelodysplastic syndrome characterised by the presence of at least 10% dysplastic megakaryocytes, found within at least 30 megakaryocytes examined in the bone marrow.
2A33	Refractory anaemia with ring sideroblasts A myelodysplastic syndrome characterised by an anaemia in which 15% or more of the erythroid precursors are ringed sideroblasts. The ring sideroblast is an erythroid precursor in which one third or more of the nucleus is encircled by granules which are positive for iron stain.

2A34

Refractory cytopenia with multi-lineage dysplasia

A myelodysplastic syndrome characterised by bi-cytopenia or pancytopenia and dysplastic changes in 10% or more of the cells in two or more of the myeloid cell lines.

2A35

Refractory anaemia with excess of blasts

A myelodysplastic syndrome characterised by bi-cytopenia or pancytopenia and dysplastic changes in one or multiple lineages, with 5-19% myeloblasts in the bone marrow, 2-19% blasts in the blood, or <20% blasts with the presence of Auer rods.

2A36

Myelodysplastic syndrome with isolated del(5q)

A myelodysplastic syndrome characterised by anaemia with or without other cytopenias and/or thrombocytosis and in which the sole cytogenetic abnormality is del(5q). Myeloblasts are <5% in the bone marrow and <1% in the blood.

Inclusions: 5 q- syndrome

2A37

Myelodysplastic syndrome, unclassifiable

A subtype of myelodysplastic syndrome which at disease presentation lacks findings appropriate for classification into any other MDS category, or has an MDS-associated cytogenetic abnormality and cytopenia, but lack sufficient dysplastic changes in any lineage and have <15% ring sideroblasts.

2A38

Refractory cytopenia of childhood

The most common subtype of the myelodysplastic syndromes affecting children. It is characterised by persistent cytopenia with less than 5% blasts in the bone marrow and less than 2% blasts in the peripheral blood.

2A3Y

Other specified myelodysplastic syndromes

2A3Z

Myelodysplastic syndromes, unspecified

Myelodysplastic and myeloproliferative neoplasms (2A40-2A4Z)

A category of clonal haematopoietic disorders that have both myelodysplastic and myeloproliferative features at the time of initial presentation.

2A40

Chronic myelomonocytic leukaemia

A myelodysplastic/myeloproliferative neoplasm which is characterised by persistent monocytosis, absence of a Philadelphia chromosome and BCR/ABL1 fusion gene, fewer than 20 percent blasts in the bone marrow and blood, often myelodysplasia, and absence of PDGFRA or PDGFRB rearrangement.

Inclusions: Chronic monocytic leukaemia

Exclusions: Myeloid/lymphoid neoplasm associated with PDGFRA rearrangement (2A50)

Myeloid neoplasm associated with PDGFRB rearrangement (2A51)

2A41**Atypical chronic myeloid leukaemia, BCR-ABL1- negative**

A myelodysplastic/myeloproliferative neoplasm characterised by the principal involvement of the neutrophil series with leukocytosis with circulating immature myeloid cells, fewer than 20 percent blasts in the bone marrow and blood, and severe dysgranulopoiesis. The neoplastic cells do not have a Philadelphia chromosome or the BCR/ABL1 fusion gene.

2A42**Juvenile myelomonocytic leukaemia**

A myelodysplastic/myeloproliferative neoplasm of childhood that is characterised by proliferation principally of the granulocytic and monocytic lineages. Myelomonocytic proliferation is seen in the bone marrow and the blood. The leukemic cells may infiltrate any tissue, however liver, spleen, lymph nodes, skin, and respiratory tract are the most common sites of involvement.

2A42.0**Juvenile myelomonocytic leukaemia in complete remission****2A42.Y****Other specified juvenile myelomonocytic leukaemia****2A42.Z****Juvenile myelomonocytic leukaemia, unspecified****2A43****Refractory anaemia with ring sideroblasts associated with marked thrombocytosis**

A provisional entity that encompasses cases with morphologic and clinical characteristics of refractory anaemia with ring sideroblasts, marked thrombocytosis, and abnormal megakaryocytes.

2A44**Myeloproliferative and myelodysplastic disease, unclassifiable**

This entity includes cases that have clinical, laboratory, and morphologic features that support the diagnosis of both a myelodysplastic syndrome and a myeloproliferative neoplasm, but do not meet the criteria for any of the other entities included in the myelodysplastic/myeloproliferative neoplasm category.

2A4Y**Other specified myelodysplastic and myeloproliferative neoplasms****2A4Z****Myelodysplastic and myeloproliferative neoplasms, unspecified**

Myeloid and lymphoid neoplasms with eosinophilia and abnormalities of PDGFRA, PDGFRB or FGFR1 (2A50-2A5Z)

A group of rare myeloid and lymphoid neoplasms characterised by rearrangement of the PDGFRA, PDGFRB, or FGFR1 genes, resulting in the formation of fusion transcripts and aberrant tyrosine kinase activity. Eosinophilia is a characteristic finding but it is not always present.

2A50**Myeloid/lymphoid neoplasm associated with PDGFRA rearrangement****2A51****Myeloid neoplasm associated with PDGFRB rearrangement**

A distinct type of myeloid neoplasm that occurs in association with rearrangement of PDGFRB gene at 5q32. Patients usually present with a picture resembling chronic myelomonocytic leukaemia and, less often atypical chronic myeloid leukaemia or chronic eosinophilic leukaemia.

2A52	Myeloid or lymphoid neoplasms with FGFR1 abnormalities Hematologic neoplasms characterised by the rearrangement of the FGFR1 gene, resulting in translocations with an 8p11 breakpoint. Patients may present with a myeloproliferative neoplasm, acute myeloid leukaemia, lymphoblastic lymphoma/leukaemia of T or B-cell lineage, or acute leukaemia of mixed phenotype.
2A5Z	Myeloid and lymphoid neoplasms with eosinophilia and abnormalities of PDGFRA, PDGFRB or FGFR1, unspecified
2A60	Acute myeloid leukaemias and related precursor neoplasms Acute myeloid leukaemia is characterised by clonal expansion of myeloid blasts in the peripheral blood and bone marrow. Clinical manifestations are fever, pallor, anaemia, hemorrhages and recurrent infections.
2A60.0	Acute myeloid leukaemia with recurrent genetic abnormalities
2A60.1	Acute myeloid leukaemia with myelodysplasia-related changes An acute myeloid leukaemia with at least 20% blasts in the bone marrow or blood, and either a previous history of myelodysplastic syndrome, multilineage dysplasia or typical myelodysplastic syndrome-related cytogenetic abnormalities. There is no history of prior cytotoxic therapy for an unrelated disorder, and there is absence of the genetic abnormalities that are present in acute myeloid leukaemia with recurrent genetic abnormalities.
2A60.2	Therapy-related myeloid neoplasms <i>Inclusions:</i> therapy-related myelodysplastic syndromes
2A60.20	Therapy related acute myeloid leukaemia or myelodysplastic syndrome
2A60.2Y	Other specified therapy-related myeloid neoplasms
2A60.2Z	Therapy-related myeloid neoplasms, unspecified
2A60.3	Acute myeloid leukaemia, not elsewhere classified by criteria of other types Acute myeloid leukaemias specified by morphological criteria should only be classified as such, if recurrent genetic abnormalities, prior history of a myelodysplastic syndrome or myelodysplastic/myeloproliferative neoplasm, or history of cytotoxic chemotherapy and/or radiotherapy are absent. <i>Exclusions:</i> - Acute myeloid leukaemia with recurrent genetic abnormalities (2A60.0) - Therapy-related myeloid neoplasms (2A60.2) - Acute myeloid leukaemia with myelodysplasia-related changes (2A60.1)
2A60.30	Acute myeloid leukaemia with minimal differentiation An acute myeloid leukaemia (AML) in which the blasts do not show evidence of myeloid differentiation by morphology and conventional cytochemistry.
2A60.31	Acute myeloid leukaemia without maturation An acute myeloid leukaemia (AML) characterised by blasts without evidence of maturation to more mature neutrophils.

2A60.32	Acute myeloid leukaemia with maturation An acute myeloid leukaemia (AML) characterised by blasts with evidence of maturation to more mature neutrophils.
2A60.33	Acute myelomonocytic leukaemia An acute leukaemia characterised by the proliferation of both neutrophil and monocyte precursors.
2A60.34	Acute monoblastic or monocytic leukaemia Acute monoblastic leukaemia and acute monocytic leukaemia are myeloid leukaemias in which 80% or more of the leukaemic cells are of monocytic lineage including monoblasts, promonocytes and monocytes; a minor neutrophil component, <20%, may be present.
2A60.35	Acute erythroid leukaemia Inclusions: Erythroleukaemia
2A60.36	Acute megakaryoblastic leukaemia An acute myeloid leukaemia in which at least 50% of the blasts are of megakaryocytic lineage. Inclusions: Acute myeloid leukaemia, M7 Acute megakaryocytic leukaemia
2A60.37	Acute basophilic leukaemia An acute myeloid leukaemia in which the immature cells differentiate towards basophils. This is a rare leukaemia.
2A60.38	Acute panmyelosis with myelofibrosis An acute myeloid leukaemia characterised by bone marrow fibrosis without preexisting primary myelofibrosis. Inclusions: Acute myelofibrosis Exclusions: Cases that meet criteria for AML with myelodysplasia related changes (2A60.1)
2A60.39	Myeloid sarcoma Myeloid sarcoma is a rare solid tumour of the myelogenous cells occurring in an extramedullary site. Inclusions: Chloroma Granulocytic sarcoma
2A60.3Y	Other specified acute myeloid leukaemia, not elsewhere classified by criteria of other types
2A60.3Z	Acute myeloid leukaemia, unspecified

2A60.4	Myeloid proliferation associated with Down syndrome Myeloid neoplasms occurring in individuals with Down syndrome. There is an increased risk of acute leukaemias in both children and adults with Down syndrome. In particular, the incidence of acute myeloid leukaemia in Down syndrome children of less than five years of age is particularly high, it is usually an acute megakaryoblastic leukaemia, and is associated with GATA1 gene mutation. This group of disorders also includes the entity transient abnormal myelopoiesis which occurs in neonates and is associated with GATA1 gene mutation.
2A60.40	Transient abnormal myelopoiesis A myeloid proliferation occurring in newborns with Down syndrome. It is clinically and morphologically indistinguishable from acute myeloid leukaemia and is associated with GATA1 mutations. The blasts display morphologic and immunophenotypic features of megakaryocytic lineage. In the majority of patients the myeloid proliferation undergoes spontaneous remission.
2A60.41	Myeloid leukaemia associated with Down syndrome Leukaemia of children with Down syndrome. Encompasses both MDS and AML
2A60.4Y	Other specified myeloid proliferation associated with Down syndrome
2A60.4Z	Myeloid proliferation associated with Down syndrome, unspecified
2A60.5	Blastic plasmacytoid dendritic cell neoplasm Blastic plasmacytoid dendritic cell neoplasm (BPDCN) is a clinically aggressive tumour derived from the precursors of plasmacytoid dendritic cells (PDCs, also called professional type I interferon-producing cells or plasmacytoid monocytes), with a high frequency of cutaneous and bone marrow involvement and leukaemic dissemination. There are currently no clues to the etiology of BPDCN, but its association with myelodysplastic syndrome (MDS) in some cases may suggest a related pathogenesis. Gene expression profiling studies have revealed that the neoplastic cells show a gene expression signature similar to that of resting normal PDCs and closer to that of myeloid than of lymphoid precursors. Inclusions: blastic NK-cell lymphoma
2A60.Y	Other specified acute myeloid leukaemias and related precursor neoplasms
2A60.Z	Acute myeloid leukaemias and related precursor neoplasms, unspecified
2A61	Acute leukaemias of ambiguous lineage An acute leukaemia in which the blasts lack sufficient evidence to classify as myeloid or lymphoid or they have morphologic and/or immunophenotypic characteristics of both myeloid and lymphoid cells.

Precursor lymphoid neoplasms (2A70-2A7Z)

Neoplasms of immature malignant lymphocytes (lymphoblasts) committed to the B-cell or T-cell lineage. Neoplasms involving the bone marrow and the peripheral blood are called precursor lymphoblastic leukaemias or acute lymphoblastic leukaemias. Neoplasms involving primarily lymph nodes or extranodal sites are called lymphoblastic lymphomas.

2A70	Precursor B-lymphoblastic neoplasms Neoplasms of lymphoblasts committed to the B-cell lineage.
2A70.0	B Lymphoblastic leukaemia or lymphoma, not elsewhere classified Precursor B cell neoplasm without defined recurrent genetic abnormality despite appropriate diagnostics
2A70.1	B lymphoblastic leukaemia or lymphoma with t(9:22) (q34;q11.2); BCR-ABL1 A precursor lymphoid neoplasm which is composed of B-lymphoblasts and carries a translocation between the BCR gene on chromosome 22 and the ABL1 gene on chromosome 9. It results in the production of the p190 kd or p210 kd fusion protein. It has an unfavorable clinical outcome.
2A70.Y	Other B-lymphoblastic leukaemia/lymphoma with recurrent genetic abnormalities
2A71	Precursor T-lymphoblastic neoplasms A neoplasm of lymphoblasts committed to the T-cell lineage, typically composed of small to medium-sized blast cells.
2A7Z	Precursor lymphoid neoplasms, unspecified

Mature B-cell neoplasms (2A80-2A8Z)

Non-Hodgkin lymphomas that originate from mature B lymphocytes. May reside in lymph nodes, lymphatic tissue of different organs or bone marrow and blood (then frequently called leukaemia).

2A80	Follicular lymphoma Follicular lymphoma (FL) is a neoplasm composed of follicle centre (germinal centre) B-cells (typically both centrocytes and centroblasts/large transformed cells), which usually has at least a partially follicular pattern. t(14;18) with BCL2 rearrangement is frequently observed. If diffuse areas of any size comprised predominantly or entirely of blastic cells are present in any case of follicular lymphoma, a diagnosis of diffuse large B-cell lymphoma is also made. Lymphomas composed of centro cytes and centroblasts with an entirely diffuse pattern in the sampled tissue may be included in this category. Inclusions: follicular lymphoma with or without diffuse areas Exclusions: Mature T-cell or NK-cell neoplasms (2A90-2B2Z)
2A80.0	Follicular lymphoma grade 1
2A80.1	Follicular lymphoma grade 2
2A80.2	Follicular lymphoma grade 3

2A80.3	Primary cutaneous follicle centre lymphoma A primary lymphoma of the skin composed of various numbers of small and large irregular neoplastic follicle center cells. Its morphologic pattern can be nodular, diffuse, or nodular and diffuse. It presents with solitary or grouped plaques and tumours, and it usually involves the scalp, forehead, or trunk. It rarely involves the legs. This type of cutaneous lymphoma tends to remain localised to the skin, and it has a favorable prognosis.
2A80.4	Paediatric type follicular lymphoma A variant of follicular lymphoma often involving cervical or other peripheral lymph nodes and the Waldeyer ring. It is frequently localised, and often lacks BCL-2 protein expression and never has a BCL2 translocation. It is usually but not exclusively seen in the pediatric population. The prognosis is usually favorable.
2A80.5	Follicular lymphoma in situ
2A80.6	Follicular lymphoma of small intestine
2A80.Y	Other specified follicular lymphoma
2A80.Z	Follicular lymphoma, unspecified
2A81	Diffuse large B-cell lymphomas Non-Hodgkin lymphomas are characterised by a proliferation of predominantly large neoplastic B lymphocytes. <i>Coded Elsewhere:</i> Diffuse large B-cell lymphoma of small intestine (2B33.5)
2A81.0	Primary mediastinal large B-cell lymphoma A large B-cell non-Hodgkin lymphoma arising in the mediastinum. Morphologically it is characterised by a massive diffuse lymphocytic proliferation associated with compartmentalizing fibrosis.
2A81.1	Intravascular large B-cell lymphoma
2A81.2	Plasmablastic lymphoma An aggressive diffuse large B-cell lymphoma frequently arising in the setting of HIV infection and characterised by the presence of large neoplastic cells resembling B-immunoblasts which have the immunophenotypic profile of plasma cells. Sites of involvement include the oral cavity and other extranodal sites
2A81.3	Lymphomatoid granulomatosis
2A81.4	T-cell/histiocyte rich large B-cell lymphoma A large B-cell lymphoma characterised by the presence of a limited number of scattered neoplastic large B-lymphocytes which are admixed with numerous non-neoplastic T-lymphocytes and frequently histiocytes.
2A81.5	Primary diffuse large B-cell lymphoma of central nervous system

2A81.6	Epstein-Barr Virus-positive diffuse large B cell lymphoma of the elderly An aggressive diffuse large B-cell lymphoma affecting patients older than 50 years. Epstein-Barr virus is present in all cases. There is no known history of immunodeficiency or prior lymphoma. The majority of patients present with extranodal disease.
2A81.7	Diffuse large B-cell lymphoma associated with chronic inflammation A diffuse large B-cell lymphoma arising in body cavities or narrow spaces of long standing chronic inflammation. The classic example is the pyothorax-associated lymphoma that arises in the pleural cavity of patients with a history of long standing pyothorax.
2A81.8	ALK-positive large B-cell lymphoma A usually aggressive large B-cell lymphoma characterised by the presence of monomorphic immunoblast-like neoplastic B-lymphocytes in a sinusoidal growth pattern. The neoplastic B-lymphocytes express the ALK kinase but they lack the 2;5 translocation.
2A81.9	Primary effusion lymphoma An aggressive non-Hodgkin B-cell lymphoma composed of large cells, presenting as a serous effusion without detectable tumour masses. It is universally associated with human herpes virus 8 (HHV-8)/Kaposi sarcoma herpes virus (KSHV) [HHV-8/KSHV]. It mostly occurs in the setting of immunodeficiency; most cases have been reported in HIV positive patients. The most common sites of involvement are the pleural, pericardial, and peritoneal cavities. The prognosis is extremely unfavorable.
2A81.A	Primary cutaneous diffuse large B-cell lymphoma, leg type An aggressive primary cutaneous B-cell lymphoma, usually involving the lower leg. It is composed of a generally monotonous proliferation of immunoblasts, or less frequently centroblasts, with few admixed reactive cells. This type of lymphoma occurs most often in the elderly who present with rapidly growing tumours, usually on one or both legs. Dissemination to extracutaneous sites is frequent.
2A81.Y	Other specified diffuse large B-cell lymphomas
2A81.Z	Diffuse large B-cell lymphoma, not otherwise specified
2A82	Mature B-cell neoplasm with leukaemic behaviour
2A82.0	Chronic lymphocytic leukaemia or small lymphocytic lymphoma An indolent, mature B-cell neoplasm composed of small, round B-lymphocytes. When the bone marrow and peripheral blood are involved, the term chronic lymphocytic leukaemia is used. The term small lymphocytic lymphoma is restricted to cases which do not show leukemic involvement of the bone marrow and peripheral blood. Inclusions: Small cell B-cell lymphoma

2A82.00	Chronic lymphocytic leukaemia of B-cell type Chronic lymphocytic leukaemia/small lymphocytic lymphoma (CLL/SLL) is a neoplasm composed of monomorphic small, round to slightly irregular B lymphocytes in the peripheral blood (PB), bone marrow (BM), spleen and lymph nodes, admixed with prolymphocytes and paraimmuno - blasts forming proliferation centres in tissue infiltrates. The CLL/SLL cells usually coexpress CD5 and CD23. In the absence of extramedullary tissue involvement, there must be $\geq 5 \times 10^9/L$ monoclonal lymphocytes with a CLL phenotype in the PB. The International Workshop on Chronic Lymphocytic leukaemia (IWCLL) report requires that the lymphocytosis be present for at least 3 months and also allows for the diagnosis of CLL to be made with lower lymphocyte counts in patients with cytopenias or disease-related symptoms {873A}. Whether patients who would have fulfilled the criteria in the past for CLL but who fulfill the criteria only for monoclonal B lymphocytosis (MBL) are better considered to have low stage CLL or MBL remains to be determined. Some may prefer to still consider many of these cases more like CLL. The term SLL is used for non-leukaemic cases with the tissue morphology and immunophenotype of CLL. The IWCLL definition of SLL requires lymphadenopathy, no cytopenias due to BM infiltration by CLL/SLL and $< 5 \times 10^9/L$ PB B-cells {873A}. Inclusions: Lymphoplasmacytic leukaemia Exclusions: Lymphoplasmacytic lymphoma (2A85.4) Coded Elsewhere: Richter syndrome (2A81.Y)
2A82.0Y	Other specified chronic lymphocytic leukaemia or small lymphocytic lymphoma
2A82.0Z	Chronic lymphocytic leukaemia or small lymphocytic lymphoma, unspecified
2A82.1	B-cell prolymphocytic leukaemia
2A82.10	B-cell prolymphocytic leukaemia in complete remission
2A82.1Y	Other specified b-cell prolymphocytic leukaemia
2A82.1Z	B-cell prolymphocytic leukaemia, unspecified
2A82.2	Hairy-cell leukaemia A neoplasm of small B-lymphocytes with hairy projections in bone marrow, spleen, and peripheral blood. Most patients present with splenomegaly and pancytopenia. Inclusions: Leukaemic reticuloendotheliosis
2A82.3	Splenic B-cell lymphoma or leukaemia, unclassifiable A small B-cell clonal lymphoproliferative disorder of the spleen that does not fall into any of the other categories of mature B-cell neoplasms.
2A82.Y	Other specified mature B-cell neoplasm with leukaemic behaviour
2A82.Z	Mature B-cell neoplasm with leukaemic behaviour, unspecified
2A83	Plasma cell neoplasms Plasma cells, usually secreting monoclonal immunoglobulin (M-protein) and/or immunoglobulin light chains.
2A83.0	Monoclonal gammopathy of undetermined significance

- 2A83.1 Plasma cell myeloma**
 A bone marrow-based plasma cell neoplasm usually characterised by a serum monoclonal protein and/or urinary light chains. "CRAB" criteria (osteolytic lesions, hypercalcaemia, renal failure, and anaemia) separate symptomatic plasma cell myeloma from asymptomatic (smoldering) myeloma.
- Inclusions:** Kahler disease
 Myelomatosis
 Medullary plasmacytoma
 multiple myeloma
- Exclusions:** Solitary plasmacytoma (2A83.2)
- 2A83.2 Solitary plasmacytoma**
 A single focus of clonal (malignant) plasma cells either in the bone or in another anatomic site without peripheral blood involvement. --2003
- Inclusions:** Solitary myeloma
- 2A83.3 Extraosseous plasmacytoma**
- 2A83.4 Plasma cell leukaemia**
 An aggressive plasma cell neoplasm. It is characterised by the presence of neoplastic plasma cells in the peripheral blood. The peripheral blood plasma cells comprise more than 20% of the peripheral blood white cells or the number of clonal plasma cells in the PB exceeds $2 \times 10^9/L$.
- 2A83.5 Monoclonal immunoglobulin deposition disease**
- 2A83.50 Heavy chain deposition disease**
 A disease of the kidney, caused by proliferation and deposition of pieces of truncated or abnormal alpha, gamma, delta, or mu immunoglobulin heavy chain segments of white blood cells. This disease is characterised by fibrillar or granular tissue deposits and renal dysfunction, which may lead to organ failure. Confirmation is by identifying heavy chain deposition tissue biopsy using immunofluorescence under a microscope.
- Exclusions:** Heavy chain diseases or malignant immunoproliferative diseases (2A84)
 Immunoglobulin heavy chain deficiency (4A01.04)
- 2A83.51 Light and heavy chain deposition disease**
 A disease of the kidney, caused by proliferation and deposition of pieces of truncated or abnormal light and heavy chain segments of white blood cells. This disease is characterised by fibrillar or granular tissue deposits and renal dysfunction, which may lead to organ failure. Confirmation is by identification of light and heavy chain deposition tissue biopsy under a microscope.

2A83.52	Light chain deposition disease A disease of the kidney, caused by the deposition of pieces of truncated or abnormal light chain segments of white blood cells. This disease is characterised by fibrillar or granular tissue deposits and renal dysfunction, which may lead to organ failure. Confirmation is by identification of light chain deposition tissue biopsy under an electron microscope. Exclusions: Immunodeficiencies with isotype or light chain deficiencies with normal number of B cells (4A01.04)
2A83.Y	Other specified multiple myeloma and plasma cell neoplasms
2A83.Z	Plasma cell neoplasm, unspecified
2A84	Heavy chain diseases or malignant immunoproliferative diseases A group of rare disorders of immunoglobulin synthesis associated with B-cell proliferative disorders that produce monoclonal heavy chains and typically no light chains.
2A84.0	Alpha heavy chain disease The small intestinal morphologic changes are consistent with a mucosa-associated lymphoid tissue lymphoma (MALT lymphoma).
2A84.1	Gamma heavy chain disease A clonal disorder characterised by the secretion of a truncated gamma chain. In most cases, it is associated with morphologic changes also seen in lymphoplasmacytic lymphomas, but the clinical course is typically more aggressive than in lymphoplasmacytic lymphoma/Waldenstrom's macroglobulinemia. Inclusions: Franklin disease
2A84.2	Mu heavy chain disease
2A84.Y	Other specified malignant immunoproliferative diseases
2A84.Z	Heavy chain diseases, unspecified
2A85	Other specified mature B-cell neoplasms or lymphoma
2A85.0	Nodal marginal zone lymphoma A primary nodal B-cell non-Hodgkin lymphoma which morphologically resembles lymph nodes involved by marginal zone lymphomas of extranodal or splenic types, but without evidence of extranodal or splenic disease. This is a rare entity, and most patients present with localised or generalised lymphadenopathy. The clinical course is indolent.
2A85.1	Extranodal marginal zone B-cell lymphoma of mucosa-associated lymphoid tissue of stomach A low grade, indolent B-cell lymphoma, usually associated with Helicobacter pylori infection. Morphologically it is characterised by a dense mucosal atypical lymphocytic (centrocyte-like cell) infiltrate with often prominent lymphoepithelial lesions and plasmacytic differentiation. Some of gastric MALT lymphomas carry the t(11;18)(q21;q21). Such cases are resistant to Helicobacter pylori therapy.

2A85.2	Extranodal marginal zone B-cell lymphoma, primary site skin A low-grade, extranodal marginal zone B-cell lymphoma of the mucosa-associated lymphoid tissue that arises from the skin. It usually presents with multifocal papular or nodular lesions in the arms or trunk. It rarely disseminates to internal organs or progresses to high grade lymphoma.
2A85.3	Extranodal marginal zone B-cell lymphoma, primary site excluding stomach or skin
2A85.4	Lymphoplasmacytic lymphoma Neoplasm of small B lymphocytes and plasma cells, mostly residing in the bone marrow. Frequently associated with the production of an IgM serum monoclonal protein, then called Waldenström macroglobulinemia (WM). Inclusions: primary macroglobulinaemia Waldenström macroglobulinaemia Waldenström macroglobulinaemia without mention of remission Exclusions: small cell B-cell lymphoma (2A82.0) Chronic lymphocytic leukaemia or small lymphocytic lymphoma (2A82.0)
2A85.5	Mantle cell lymphoma Mantle cell lymphoma is a rare form of malignant non-Hodgkin lymphoma affecting B lymphocytes in the lymph nodes in a region called the "mantle zone". It accounts for 2-10% of lymphomas. Inclusions: Small cell mantle cell lymphoma
2A85.6	Burkitt lymphoma including Burkitt leukaemia A highly aggressive lymphoma composed of monomorphic medium-sized B-cells with basophilic cytoplasm and numerous mitotic figures. It is often associated with the presence of Epstein-Barr virus (EBV) and is commonly seen in AIDS patients. Three morphologic variants are recognised: classical Burkitt lymphoma, Burkitt lymphoma with plasmacytoid differentiation, and atypical Burkitt/Burkitt-like lymphoma. All cases express the MYC translocation [t(8;14)]. Inclusions: "Burkitt-like" lymphoma Coded Elsewhere: HIV - [human immunodeficiency virus] disease associated with Burkitt lymphoma (1C62.3)
2A85.Y	Further specified mature B-cell neoplasms or lymphoma
2A86	B-cell lymphoma, mixed features
2A86.0	Malignant lymphoma of B cell type, not elsewhere classified Coding Note: If B-cell lineage or involvement is mentioned in conjunction with a specific lymphoma, code to the more specific description.
2A86.1	B-cell lymphoma unclassifiable with features intermediate between Burkitt lymphoma and diffuse large B-cell lymphoma

2A86.2	B-cell lymphoma unclassifiable with features intermediate between classical Hodgkin lymphoma and diffuse large B-cell lymphoma
2A86.Y	Other specified B-cell lymphoma, mixed features
2A86.Z	B-cell lymphoma, mixed features, unspecified
2A8Z	Mature B-cell neoplasms, unspecified

Mature T-cell or NK-cell neoplasms (2A90-2B2Z)

A group of neoplasms composed of T-lymphocytes with a mature (peripheral/post-thymic) immunophenotypic profile and/or NK-cells.

2A90	Mature T-cell lymphoma, specified types, nodal or systemic
2A90.0	T-cell prolymphocytic leukaemia An aggressive T-cell leukaemia, characterised by the proliferation of small to medium sized prolymphocytes with a mature T-cell phenotype, involving the blood, bone marrow, lymph nodes, liver, spleen, and skin.
2A90.1	T-cell large granular lymphocytic leukaemia A T-cell peripheral neoplasm characterised by a persistent (>6 months) increase in the number of peripheral blood large granular lymphocytes, without a clearly identified cause.
2A90.2	Chronic lymphoproliferative disorders of NK-cells Heterogeneous disorders with a chronic clinical course affecting predominantly adults and characterised by the proliferation of large granular lymphocytes with natural killer cell immunophenotype.
2A90.3	Aggressive NK cell leukaemia A rare, highly aggressive, Epstein-Barr virus-associated leukaemia, also known as aggressive NK-cell leukaemia/lymphoma; it may represent the leukemic counterpart of nasal type extranodal NK/T-cell lymphomas. It affects primarily teenagers and young adults. It is characterised by the systemic proliferation of NK cells in the peripheral blood, bone marrow, liver, and spleen.
2A90.4	Systemic Epstein-Barr Virus-positive T-cell lymphoma of childhood This neoplasm of childhood is characterised by a clonal proliferation of EBV-infected T-cells with an activated cytotoxic phenotype. It can occur shortly after primary acute EBV infection or in the setting of chronic active EBV infection (CAEBV).
2A90.5	Adult T-cell lymphoma or leukaemia, human T-cell lymphotropic virus type 1-associated A peripheral (mature) T-cell neoplasm linked to the human T-cell leukaemia virus type 1 (HTLV-1). Adult T-cell leukaemia/lymphoma is endemic in several regions of the world, in particular Japan, the Caribbean, and parts of Central Africa. Coded Elsewhere: Adult T-cell leukaemia or lymphoma, skin (2B0Y)

- 2A90.6 Extranodal NK/T-cell lymphoma, nasal type**
- An aggressive, predominantly extranodal, mature T-cell non-Hodgkin lymphoma. It is characterised by an often angiocentric and angiodestructive cellular infiltrate composed of EBV positive NK/T cells. The nasal cavity is the most common site of involvement. Patients often present with midfacial destructive lesions (lethal midline granuloma). The disease may disseminate rapidly to various anatomic sites including the gastrointestinal tract, skin, testis, and cervical lymph nodes. It is also known as angiocentric T-cell lymphoma. The term polymorphic reticulosis has been widely used to describe the morphologic changes seen in this type of lymphoma. However, the latter term may also apply to lymphomatoid granulomatosis, which is an angiocentric and angiodestructive EBV positive B-cell lymphoproliferative disorder.
- 2A90.7 Enteropathy associated T-cell lymphoma**
- An uncommon mature T-cell lymphoma of intraepithelial lymphocytes. It usually arises from the small intestine, most commonly the jejunum or ileum. Other less frequent primary anatomic sites include the duodenum, stomach, colon, or outside the gastrointestinal tract. Type II of this lymphoma may occur sporadically outside the context of celiac disease.
- Inclusions:** Enteropathy type intestinal T-cell lymphoma
 Intestinal T-cell lymphoma
- 2A90.8 Hepatosplenic T-cell lymphoma**
- An extranodal, mature T-cell non-Hodgkin lymphoma that originates from cytotoxic T-cells, usually of gamma/delta T-cell type. It is characterised by the presence of medium-size neoplastic lymphocytes infiltrating the hepatic sinusoids. A similar infiltrating pattern is also present in the spleen and bone marrow that are usually involved at the time of the diagnosis.
- 2A90.9 Angioimmunoblastic T-cell lymphoma**
- A mature T-cell non-Hodgkin lymphoma, characterised by systemic disease and a polymorphous infiltrate involving lymph nodes and extranodal sites. The clinical course is typically aggressive.
- Inclusions:** ALLD - [angioimmunoblastic lymphadenopathy with
 dysproteinaemia]
- 2A90.A Anaplastic large cell lymphoma, ALK-positive**
- A T-cell peripheral lymphoma composed of usually large, pleomorphic, CD30 positive T-lymphocytes with abundant cytoplasm characterised by the presence of a translocation involving the ALK gene and expression of ALK fusion protein. Most patients present with peripheral and/or abdominal lymphadenopathy, and often have advanced disease and extranodal involvement.
- Inclusions:** Anaplastic large cell lymphoma, CD30-positive

- 2A90.B Anaplastic large cell lymphoma, ALK-negative**
 A T-cell peripheral lymphoma morphologically indistinguishable from anaplastic large cell lymphoma, ALK-positive. It is characterised by the absence of the translocation involving the ALK gene and lacks expression of ALK fusion protein.
Exclusions: Primary cutaneous CD-30 positive T-cell lymphoproliferative disorders (2B03)
- 2A90.C Peripheral T-cell lymphoma, not otherwise specified**
 A heterogeneous category of nodal and extranodal mature T-cell lymphomas, which do not correspond to any of the specifically defined entities of mature T-cell lymphoma in the current classification.
Inclusions: T-zone variant Peripheral T-cell lymphoma
 Lymphoepithelioid lymphoma
 Follicular variant Peripheral T-cell lymphoma

Mature T-cell or NK-cell lymphomas and lymphoproliferative disorders, primary cutaneous specified types (2B00-2B0Z)

Primary cutaneous T-cell lymphomas (CTCL) and NK-cell lymphomas are malignant lymphoproliferative diseases of unknown cause that are thought to originate from T-lymphocytes in the lymphoid tissue of the skin and by definition are confined to the skin at initial diagnosis. Included in the class is also a small number of lymphoma-like primary cutaneous lymphoproliferative disorders which are not considered to be truly malignant.

- Inclusions:** Primary cutaneous peripheral T-cell lymphoma
Exclusions: Skin infiltration by nodal or non-cutaneous extranodal lymphoma (2E08)

2B00 Subcutaneous panniculitis-like T-cell lymphoma

Subcutaneous panniculitis-like T-cell lymphoma is a neoplasm of alpha/beta, usually CD8+ T-cells, mainly confined to the subcutis, presenting clinically as subcutaneous nodules which are usually not ulcerated.

2B01 Mycosis fungoides

A peripheral (mature) T-cell lymphoma presenting in the skin with patches/plaques or less commonly with tumours or erythroderma. It is characterised by epidermal and dermal infiltration of small to medium-sized T-cells with cerebriform nuclei.

2B02 Sézary syndrome

A generalised peripheral (mature) T-cell neoplasm characterised by the presence of erythroderma, lymphadenopathy, and neoplastic, cerebriform T-lymphocytes in the blood. Sézary syndrome is an aggressive disease.

2B03 Primary cutaneous CD-30 positive T-cell lymphoproliferative disorders

Primary skin disorders characterised immunohistologically by infiltration by neoplastic CD30+ lymphocytes.

- Inclusions:** Primary cutaneous CD30+large T-cell lymphoma

2B03.0	Primary cutaneous CD30 positive anaplastic large cell lymphoma An anaplastic large cell lymphoma limited to the skin at the time of diagnosis. Most patients present with solitary or localised skin lesions, which may be tumours, nodules or papules. The t(2;5) translocation that is present in many cases of systemic anaplastic large cell lymphoma, is not found in this disease.
2B03.1	Lymphomatoid papulosis Lymphomatoid papulosis is a proliferation of T-cells, often clonal, characterised clinically by the appearance of crops of dome-shaped papules and nodules which tend to ulcerate and then heal with scarring.
2B0Y	Other specified primary cutaneous mature T-cell or NK-cell lymphomas and lymphoproliferative disorders
2B0Z	Primary cutaneous T-cell lymphoma of undetermined or unspecified type
2B2Y	Other specified mature T-cell or NK-cell neoplasms
2B2Z	Mature T-cell or NK-cell neoplasms, unspecified
2B30	Hodgkin lymphoma Malignant lymphomas, previously known as Hodgkin's disease, characterised by the presence of large tumour cells in an abundant admixture of nonneoplastic cells. There are two distinct subtypes: nodular lymphocyte predominant Hodgkin lymphoma and classical Hodgkin lymphoma. Hodgkin lymphoma involves primarily lymph nodes.
2B30.0	Nodular lymphocyte predominant Hodgkin lymphoma Nodular lymphocyte predominant Hodgkin lymphoma (NLPHL) is characterised by a nodular, or a nodular and diffuse proliferation of scattered large neoplastic cells known as popcorn or lymphocyte predominant cells (LP cells) —formerly called L&H cells for lymphocytic and/or histiocytic Reed-Sternberg cell variants. At present an overlap between NLPHL and T-cell-rich large B-cell lymphoma cannot be excluded.
2B30.1	Classical Hodgkin lymphoma Classical Hodgkin lymphoma is a B-cell lymphoma characterised histologically by the presence of large mononuclear Hodgkin cells and multinucleated Reed-Sternberg (HRS) cells. A monoclonal B-cell lymphoproliferation in the vast majority of cases. It is characterised by a bimodal age distribution (15-30 years of life and late life) and is often associated with EBV infection. In less than 5% of cases it is a monoclonal proliferation of T-lymphocytes. Morphologically, it is characterised by the presence of Reed-Sternberg cells and mononuclear Hodgkin cells. The Reed-Sternberg and mononuclear Hodgkin cells are CD30 positive in nearly all cases and CD15 positive in the majority of cases. Inclusions: Classical Hodgkin lymphoma, type not specified

2B30.10	Nodular sclerosis classical Hodgkin lymphoma A subtype of classical Hodgkin lymphoma characterised by collagen bands surrounding lymphoid nodules. The lymphoid nodules contain lacunar and Reed-Sternberg cells. Mediastinal involvement occurs in 80% of patients. The prognosis of nodular sclerosis Hodgkin lymphoma is slightly better than that of mixed cellularity or lymphocyte depleted subtype.
2B30.11	Lymphocyte-rich classical Hodgkin lymphoma
2B30.12	Mixed cellularity classical Hodgkin lymphoma A subtype of classical Hodgkin lymphoma with a mixed inflammatory stroma containing Hodgkin and Reed-Sternberg cells.
2B30.13	Lymphocyte depleted classical Hodgkin lymphoma
2B30.1Z	Classical Hodgkin lymphoma, unspecified
2B30.Z	Hodgkin lymphoma, unspecified
2B31	Histiocytic or dendritic cell neoplasms True histiocytic malignancies are vanishing diagnoses due to improved understanding of the provenance of malignant cells.
2B31.0	Juvenile xanthogranuloma It is characterised by the presence of lipid-laden, foamy histiocytes and Touton-type giant cells in the dermis.
2B31.1	Histiocytic sarcoma Inclusions: Malignant Histiocytosis
2B31.2	Langerhans cell histiocytosis A neoplastic proliferation of Langerhans cells which contain Birbeck granules by ultrastructural examination. Three major overlapping syndromes are recognised: eosinophilic granuloma, Letterer-Siwe disease, and Hand-Schuller-Christian disease. The clinical course is generally related to the number of organs affected at presentation. Inclusions: Histiocytosis X
2B31.20	Langerhans cell histiocytosis involving the skin
2B31.2Y	Other specified Langerhans cell histiocytosis
2B31.3	Langerhans cell sarcoma A neoplastic proliferation of Langerhans cells with overtly malignant cytologic features. It can be considered a higher grade variant of Langerhans cell histiocytosis (LCH) and it can present de novo or progress from antecedent LCH.
2B31.4	Interdigitating dendritic cell sarcoma A neoplastic proliferation of spindle to ovoid cells which show phenotypic features similar to those of interdigitating dendritic cells. The clinical course is generally aggressive.

2B31.5	Follicular dendritic cell sarcoma A neoplasm composed of spindle to ovoid cells which have morphologic and immunophenotypic characteristics of follicular dendritic cells. It affects lymph nodes and other sites including the tonsils, gastrointestinal tract, spleen, liver, soft tissues, skin, and oral cavity. It usually behaves as a low grade sarcoma. Recurrences have been reported in up to half of the cases.
2B31.6	Indeterminate cell histiocytosis A very rare dendritic cell tumour composed of spindle to ovoid cells with a phenotype that is similar to the Langerhans cells. Patients usually present with cutaneous papules, nodules, and plaques. Systemic symptoms are usually absent. The clinical course is variable.
2B31.7	Fibroblastic reticular cell tumour A very rare dendritic cell tumour affecting the lymph nodes, spleen, and soft tissues. Morphologically it is similar to the interdigitating dendritic cell sarcoma or follicular dendritic cell sarcoma. The tumour cells are positive for cytokeratin and CD68. Clinical outcome is variable.
2B31.Y	Other specified histiocytic or dendritic cell neoplasms
2B31.Z	Histiocytic or dendritic cell neoplasms, unspecified
2B32	Immunodeficiency-associated lymphoproliferative disorders Post-transplant lymphoproliferative disorder (PTLD) is a polyclonal (benign) or clonal (malignant) proliferation of lymphoid cells that develops as a consequence of immunosuppression in a recipient of a solid organ or bone marrow allograft. PTLDs comprise a spectrum ranging from early, Epstein-Barr virus (EBV)-driven polyclonal lymphoid proliferations to EBV-positive or EBV- negative lymphomas of predominantly B-cell or less often T-cell type. In other Immunodeficiency-associated lymphoproliferative disorders, association with EBV is less pronounced. Inclusions: PTLD - [Post transplant lymphoproliferative disorder]
2B32.0	Post-transplant lymphoproliferative disorder, early lesion A lymphoproliferative disorder arising as a result of post-transplant immunosuppression therapy. It is characterised by the lack of tissue destruction and the architectural preservation of the involved tissues. It includes two morphologic variants: plasmacytic hyperplasia and infectious mononucleosis-like lymphoproliferative disorders.
2B32.1	Reactive plasmacytic hyperplasia
2B32.2	Post-transplant lymphoproliferative disorder, Infectious mononucleosis-like
2B32.3	Polymorphic post-transplant lymphoproliferative disorder
2B32.Y	Other specified immunodeficiency-associated lymphoproliferative disorders
2B32.Z	Immunodeficiency-associated lymphoproliferative disorders, unspecified
2B33	Malignant haematopoietic neoplasms without further specification Coding Note: Only to be designated in cases with incomplete diagnostics.

2B33.0	Acute leukaemia, not elsewhere classified
Coding Note:	Only to be designated in cases with incomplete diagnostics.
2B33.1	Myeloid leukaemia
2B33.2	Chronic myeloid leukaemia, not elsewhere classified
Coding Note:	Only to be designated in cases. with incomplete diagnostics
2B33.3	Lymphoid leukaemia, not elsewhere classified
Coding Note:	Only to be designated in cases with incomplete diagnostics.
2B33.4	Leukaemia, unspecified
Coding Note:	Only to be designated in cases with incomplete diagnostics.
2B33.5	Malignant lymphoma, not elsewhere classified
Coding Note:	Only to be designated in cases with incomplete diagnostics.
	Coded Elsewhere: Enteropathy associated T-cell lymphoma (2A90.7)
2B33.Y	Other malignant haematopoietic neoplasms without further specification
Coding Note:	Only to be designated in cases with incomplete diagnostics.
2B3Z	Neoplasms of haematopoietic or lymphoid tissues, unspecified

Malignant neoplasms, except primary neoplasms of lymphoid, haematopoietic, central nervous system or related tissues (2B50-2E2Z)

Coding Note: For use of this category, reference should be made to the mortality coding rules and guidelines in the Reference Guide.

Exclusions: Neoplasms of brain or central nervous system (2A00-2A0Z)
Neoplasms of haematopoietic or lymphoid tissues (2A20-2B3Z)

Malignant neoplasms, stated or presumed to be primary, of specified sites, except of lymphoid, haematopoietic, central nervous system or related tissues (2B50-2D3Z)

Malignant mesenchymal neoplasms (2B50-2B5Z)

A usually aggressive malignant mesenchymal cell tumour most commonly arising from muscle, fat, fibrous tissue, bone, cartilage, and blood vessels. Sarcomas occur in both children and adults. The prognosis depends largely on the degree of differentiation (grade) of the tumour. Representative subtypes are liposarcoma, leiomyosarcoma, osteosarcoma, and chondrosarcoma.

Exclusions: Mesenchymal tumours of meninges (2A01.1)
Tumours of cranial or paraspinal nerves (2A02.1)

2B50	Chondrosarcoma, primary site
	Exclusions: Osteosarcoma, primary site (2B51)

- 2B50.0** **Chondrosarcoma of bone or articular cartilage of limbs**
- 2B50.1** **Chondrosarcoma of bone or articular cartilage of pelvis**
- 2B50.2** **Chondrosarcoma of bone or articular cartilage of ribs, sternum or clavicle**
- 2B50.Y** **Chondrosarcoma of bone or articular cartilage of other specified sites**
- 2B50.Z** **Chondrosarcoma of bone or articular cartilage of unspecified sites**

2B51

Osteosarcoma, primary site

A usually aggressive malignant bone-forming mesenchymal tumour, predominantly affecting adolescents and young adults. It usually involves bones and less frequently extraosseous sites. It often involves the long bones (particularly distal femur, proximal tibia, and proximal humerus). Pain with or without a palpable mass is the most frequent clinical symptom. It may spread to other anatomic sites, particularly the lungs.

- 2B51.0** **Osteosarcoma of bone or articular cartilage of jaw**
- 2B51.1** **Osteosarcoma of bone or articular cartilage of limbs**
- 2B51.2** **Osteosarcoma of bone or articular cartilage of pelvis**
- 2B51.Y** **Osteosarcoma of bone and articular cartilage of other specified sites**
- 2B51.Z** **Osteosarcoma of bone and articular cartilage of unspecified sites**

2B52

Ewing sarcoma, primary site

A small round cell tumour that lacks morphologic, immunohistochemical, and electron microscopic evidence of neuroectodermal differentiation. It represents one of the two ends of the spectrum called Ewing's sarcoma/peripheral neuroectodermal tumour. It affects mostly males under age 20, and it can occur in soft tissue or bone. Pain and the presence of a mass are the most common clinical symptoms.

- 2B52.0** **Ewing sarcoma of bone or articular cartilage of limbs**
- 2B52.1** **Ewing sarcoma of bone or articular cartilage of pelvis**
- 2B52.2** **Ewing sarcoma of bone or articular cartilage of ribs**
- 2B52.3** **Ewing sarcoma of soft tissue**
A rare malignant neoplasm of the soft tissues. It is typically a disease of children and young adults. It is characterised by t(11:22) (q24: q12) resulting in the expression of EWS/FLI-1 chimeric transcript. Most commonly occurs in the paravertebral region, chest wall, pelvis and lower extremities.
- 2B52.Y** **Ewing sarcoma of bone and articular cartilage of other specified sites**
- 2B52.Z** **Ewing sarcoma of bone and articular cartilage of unspecified sites**

2B53

Fibroblastic or myofibroblastic tumour, primary site

- 2B53.0** **Myxofibrosarcoma, primary site**
- 2B53.1** **Fibroblastic or myofibroblastic tumour of skin**

2B53.Y	Other specified fibroblastic or myofibroblastic tumour, primary site
2B53.Z	Fibroblastic or myofibroblastic tumour, primary site, unspecified
2B54	Unclassified pleomorphic sarcoma, primary site A pleomorphic sarcoma characterised by the presence of fibrohistiocytic cells and spindle cells arranged in a storiform pattern.
2B54.0	Unclassified pleomorphic sarcoma of skin A rare malignant neoplasm arising from the skin. It is characterised by the presence of spindle cells in a storiform pattern and histiocytes with abundant cytoplasm. <i>Inclusions:</i> malignant fibrous histiocytoma of skin
2B54.1	Unclassified pleomorphic sarcoma of retroperitoneum or peritoneum
2B54.Y	Unclassified pleomorphic sarcoma, primary site, other specified site
2B54.Z	Unclassified pleomorphic sarcoma, primary site, unspecified site
2B55	Rhabdomyosarcoma, primary site Rhabdomyosarcoma is a malignant soft tissue tumour which develops from cells of striated muscle. It is the most common form of tumour found in children and adolescents.
2B55.0	Rhabdomyosarcoma of the oral cavity or pharynx
2B55.1	Rhabdomyosarcoma of respiratory or intrathoracic organs
2B55.2	Rhabdomyosarcoma of male genital organs
2B55.Y	Rhabdomyosarcoma, other specified primary site
2B55.Z	Rhabdomyosarcoma, unspecified primary site
2B56	Angiosarcoma, primary site
2B56.0	Angiosarcoma of heart
2B56.1	Angiosarcoma of skin A malignant tumour arising from the endothelial cells of the blood vessels. Microscopically, it is characterised by frequently open vascular anastomosing and branching channels. The malignant cells that line the vascular channels are spindle or epithelioid and often display hyperchromatic nuclei. Angiosarcomas most frequently occur in the skin and breast. Patients with long-standing lymphoedema are at increased risk of developing angiosarcoma.
2B56.2	Angiosarcoma of breast A malignant vascular neoplasm arising from the breast.
2B56.3	Angiosarcoma of liver A malignant vascular neoplasm arising from the liver. <i>Inclusions:</i> Kupffer cell sarcoma of liver

2B56.Y	Angiosarcoma, other specified primary site
2B56.Z	Angiosarcoma, unspecified primary site
2B57	Kaposi sarcoma, primary site A malignant neoplasm characterised by a vascular proliferation which usually contains blunt endothelial cells. Erythrocyte extravasation and hemosiderin deposition are frequently present. The most frequent site of involvement is the skin; however it may also occur internally. It generally develops in people with compromised immune systems including those with acquired immune deficiency syndrome (AIDS). Coded Elsewhere: Human immunodeficiency virus disease associated with Kaposi sarcoma (1C62.3)
2B57.0	Kaposi sarcoma of lung
2B57.1	Kaposi sarcoma of skin A Kaposi sarcoma arising from the skin. It presents with patches, plaques, or nodules.
2B57.2	Kaposi sarcoma of gastrointestinal sites
2B57.Y	Kaposi sarcoma of other specified primary sites
2B57.Z	Kaposi sarcoma of unspecified primary site
2B58	Leiomyosarcoma, primary site
2B58.0	Leiomyosarcoma of retroperitoneum or peritoneum
2B58.1	Leiomyosarcoma of uterus
2B58.2	Leiomyosarcoma of stomach This is a malignant nonepithelial tumour that arises from cells lining the stomach that develop into smooth-muscle.
2B58.Y	Leiomyosarcoma, other specified primary site
2B58.Z	Leiomyosarcoma, unspecified primary site
2B59	Liposarcoma, primary site Liposarcoma, a type of soft tissue sarcoma, describes a group of lipomatous tumours of varying severity ranging from slow-growing to aggressive and metastatic. Liposarcomas are most often located in the lower extremities or retroperitoneum, but they can also occur in the upper extremities, neck, peritoneal cavity, spermatic cord, breast, vulva and axilla.
2B59.0	Liposarcoma of soft tissue of limb
2B59.1	Liposarcoma of retroperitoneum or peritoneum
2B59.2	Liposarcoma of male genital organs
2B59.Y	Liposarcoma, other specified primary site

2B59.Z	Liposarcoma, unspecified primary site
2B5A	Synovial sarcoma, primary site A malignant neoplasm characterised by the chromosomal translocation t(X;18)(p11;q11). It can occur at any age, but mainly affects young adults, more commonly males. Although any site can be affected, the vast majority of the cases arise in the deep soft tissues of extremities, especially around the knee. Microscopically, synovial sarcoma is classified as monophasic (with a spindle or epithelial cell component) or biphasic (with both spindle and epithelial cell components). Synovial sarcomas can recur or metastasize to the lungs, bones, and lymph nodes.
2B5A.0	Synovial sarcoma of soft tissues of limb
2B5A.1	Synovial sarcoma of respiratory or intra-thoracic organs
2B5A.Y	Synovial sarcoma, other specified primary site
2B5A.Z	Synovial sarcoma, unspecified primary site
2B5B	Gastrointestinal stromal tumour, primary site This is the most common mesenchymal tumour that arises in the gastrointestinal tract. It is generally immunohistochemically positive for CD117 (KIT), phenotypically paralleling Cajal-cell differentiation, and most examples contain KIT- or PDGFRA-activating mutations. It is most frequent in the stomach and to a lesser degree in the small intestine. The prognosis depends on the tumour size and the mitotic activity.
2B5B.0	Gastrointestinal stromal tumour of stomach A gastrointestinal stromal tumour that arises from the stomach. It covers a spectrum of benign to malignant mesenchymal neoplasms and includes most gastric smooth muscle tumours, leiomyoblastomas, and tumours formerly called gastrointestinal autonomic nerve tumours.
2B5B.1	Gastrointestinal stromal tumour of small intestine A gastrointestinal stromal tumour that arises from the small intestine. It usually affects adults over fifty years of age. The majority of cases have spindle cell morphology. The prognosis depends on the tumour size and the mitotic activity.
2B5B.Y	Gastrointestinal stromal tumour of other gastrointestinal sites
2B5B.Z	Gastrointestinal stromal tumour of unspecified gastrointestinal sites
2B5C	Endometrial stromal sarcoma, primary site A malignant, infiltrating mesenchymal tumour arising from the uterine corpus, cervix, vagina, and the ovary. Based on its morphologic characteristics, it is classified as either a low grade or an undifferentiated (high grade) stromal sarcoma. The low grade endometrioid stromal sarcoma is characterised by the presence of oval to spindle-shape cells that resemble the cells of the endometrial stroma, without evidence of significant atypia and pleomorphism. Numerous small vessels are also present. The undifferentiated stromal sarcoma is characterised by an aggressive clinical course, the presence of significant cellular atypia, pleomorphism, and high mitotic activity.

2B5D	Malignant mixed epithelial mesenchymal tumour, primary site
2B5D.0	Malignant mixed epithelial mesenchymal tumour of ovary Malignant mixed epithelial mesenchymal tumour of the ovary is a rare and very aggressive neoplasm presenting most commonly in postmenopausal women and is composed of adenocarcinomatous and sarcomatous elements and, depending on the types of these elements, can be classified as homologous or heterologous. It often has a poor prognosis.
2B5D.1	Malignant mixed epithelial and mesenchymal tumour of corpus uteri A primary malignant neoplasm of the uterine corpus characterised by the presence of an epithelial and a mesenchymal component. This category includes carcinosarcoma, carcinofibroma, and adenosarcoma.
2B5D.Y	Malignant mixed epithelial mesenchymal tumour, other specified primary site
2B5D.Z	Malignant mixed epithelial mesenchymal tumour, unspecified primary site
2B5E	Malignant nerve sheath tumour of peripheral nerves or autonomic nervous system, primary site <i>Exclusions:</i> Malignant peripheral nerve sheath tumour of cranial or paraspinal nerves (2A02.10)
2B5F	Sarcoma, not elsewhere classified, primary site
2B5F.0	Sarcoma, not elsewhere classified of uterus <i>Coded Elsewhere:</i> Endometrial stromal sarcoma, primary site (2B5C) Leiomyosarcoma of uterus (2B58.1) Rhabdomyosarcoma of corpus uteri (2B55.Y)
2B5F.1	Sarcoma, not elsewhere classified of retroperitoneum or peritoneum
2B5F.10	Myosarcomas of omentum
2B5F.1Y	Other specified sarcoma, not elsewhere classified of retroperitoneum or peritoneum
2B5F.1Z	Sarcoma, not elsewhere classified of retroperitoneum or peritoneum, unspecified
2B5F.2	Sarcoma, not elsewhere classified of other specified sites
2B5F.3	Sarcoma, not elsewhere classified, primary site unknown
2B5G	Myosarcoma of uterus, part not specified <i>Coded Elsewhere:</i> Leiomyosarcoma of uterus (2B58.1) Rhabdomyosarcoma of corpus uteri (2B55.Y)
2B5H	Well differentiated lipomatous tumour, primary site
2B5J	Malignant miscellaneous tumours of bone or articular cartilage of other or unspecified sites <i>Exclusions:</i> Neoplasms of haematopoietic or lymphoid tissues (2A20-2B3Z)

2B5K **Unspecified malignant soft tissue tumours or sarcomas of bone or articular cartilage of other or unspecified sites**

2B5Y **Other specified malignant mesenchymal neoplasms**

2B5Z **Malignant mesenchymal neoplasm of unspecified type**

Malignant neoplasms of lip, oral cavity or pharynx (2B60-2B6Z)

Squamous cell carcinomas amount to more than 90% of malignant tumours of the oral cavity and oropharynx. As in other parts of the upper aerodigestive tract, there is a strong and synergistic association with tobacco smoking and alcohol abuse. In some regions, particularly the Indian subcontinent, oral cancer is among the most frequent malignancies, largely due to tobacco chewing.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2B60 **Malignant neoplasms of lip**

Malignant neoplasms originating from the transitional epithelium of the lip (excluding oral mucosa and skin of the outer lip) or from the underlying anatomical structures (e.g. orbicularis oris muscle).

Exclusions: Malignant neoplasm of skin of lip (2C30-2C3Z)

Malignant mesenchymal neoplasms (2B50-2B5Z)

2B60.0 **Basal cell carcinoma of lip**

A basal cell carcinoma arising from the lip.

2B60.1 **Squamous cell carcinoma of lip**

Squamous cell carcinoma located on or originating in the mucosa or vermillion of the lip, including the vermillion border but excluding the skin of the lip.

Exclusions: Squamous cell carcinoma of skin of lip (2C31)

2B60.Y **Other specified malignant neoplasms of lip**

2B60.Z **Malignant neoplasms of lip, unspecified**

2B61 **Malignant neoplasms of base of tongue**

A primary neoplasm involving the base of the tongue, often associated with chronic alcohol and tobacco use, older age, certain geographic locations, a family history of upper aerodigestive tract cancers and/or certain nutritional deficiencies and infectious agents.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2B61.0 **Squamous cell carcinoma of the base of the tongue**

A carcinoma that arises from the base of the tongue. Representative examples include squamous cell carcinoma, adenoid cystic carcinoma, and mucoepidermoid carcinoma.

2B61.Y **Other specified malignant neoplasms of base of tongue**

2B61.Z	Malignant neoplasms of base of tongue, unspecified
2B62	Malignant neoplasms of other or unspecified parts of tongue
	<i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2B62.0	Squamous cell carcinoma of other or unspecified parts of tongue
2B62.1	Malignant neoplasms of lingual tonsil
2B62.10	Squamous cell carcinoma of lingual tonsil
2B62.Y	Other specified malignant neoplasms of other and unspecified parts of tongue
2B62.Z	Malignant neoplasms of other or unspecified parts of tongue, unspecified
2B63	Malignant neoplasms of gum
	<i>Exclusions:</i> Benign osteogenic tumours of bone or articular cartilage of skull or face (2E83.0)
	Benign osteogenic tumours of bone or articular cartilage of lower jaw (2E83.1)
	Malignant mesenchymal neoplasms (2B50-2B5Z)
2B63.0	Squamous cell carcinoma of gum
2B63.Y	Other specified malignant neoplasm of gum
2B63.Z	Malignant neoplasms of gum, unspecified
2B64	Malignant neoplasms of floor of mouth
	<i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2B64.0	Squamous cell carcinoma of floor of mouth
2B64.Y	Other specified malignant neoplasm of floor of mouth
2B64.Z	Malignant neoplasms of floor of mouth, unspecified
2B65	Malignant neoplasms of palate
	<i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2B65.0	Adenocarcinoma of palate
2B65.1	Squamous cell carcinoma of palate
2B65.Y	Other specified malignant neoplasm of palate
2B65.Z	Malignant neoplasms of palate, unspecified
2B66	Malignant neoplasms of other or unspecified parts of mouth
	<i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2B66.0	Squamous cell carcinoma of other or unspecified parts of mouth

- 2B66.Y** **Other specified malignant neoplasms of other and unspecified parts of mouth**
- 2B66.Z** **Malignant neoplasms of other or unspecified parts of mouth, unspecified**

2B67

Malignant neoplasms of parotid gland

Salivary gland tumours can show a striking range of morphological diversity between different tumour types and sometimes within an individual tumour mass. In addition, hybrid tumours, dedifferentiation and the propensity for some benign tumours to progress to malignancy can confound histopathological interpretation. These features, together with the relative rarity of a number of tumours, can sometimes make diagnosis difficult, despite the abundance of named tumour entities. The increasing use of pre-operative fine needle aspiration biopsies also needs to be taken into account, as artifactual changes may be superimposed on the tumours. Unfortunately, the morphological variability of these tumours is mirrored by the immunocytochemical profiles, so that special stains are rarely useful in routine diagnosis of salivary gland epithelial neoplasms.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

- 2B67.0** **Adenocarcinoma of parotid gland**
- 2B67.1** **Squamous cell carcinoma of parotid gland**
- 2B67.Y** **Other specified malignant neoplasms of parotid gland**
- 2B67.Z** **Malignant neoplasms of parotid gland, unspecified**

2B68

Malignant neoplasms of submandibular or sublingual glands

Salivary gland tumours can show a striking range of morphological diversity between different tumour types and sometimes within an individual tumour mass. In addition, hybrid tumours, dedifferentiation and the propensity for some benign tumours to progress to malignancy can confound histopathological interpretation. These features, together with the relative rarity of a number of tumours, can sometimes make diagnosis difficult, despite the abundance of named tumour entities. The increasing use of pre-operative fine needle aspiration biopsies also needs to be taken into account, as artifactual changes may be superimposed on the tumours. Unfortunately, the morphological variability of these tumours is mirrored by the immunocytochemical profiles, so that special stains are rarely useful in routine diagnosis of salivary gland epithelial neoplasms.

Exclusions: Malignant neoplasms of parotid gland (2B67)
 Malignant mesenchymal neoplasms (2B50-2B5Z)

- 2B68.0** **Adenocarcinoma of submandibular or sublingual glands**
- 2B68.1** **Squamous cell carcinoma of submandibular or sublingual glands**
- 2B68.2** **Other specified malignant neoplasms of submandibular or sublingual glands**
- 2B68.Z** **Malignant neoplasms of submandibular or sublingual glands, unspecified**

2B69	Malignant neoplasms of tonsil
	<i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
	<i>Coded Elsewhere:</i> Malignant neoplasms of pharyngeal tonsil (2B6B.2) Malignant neoplasms of lingual tonsil (2B62.1)
2B69.0	Squamous cell carcinoma of tonsil
2B69.1	Other specified malignant neoplasms of tonsil
2B69.Z	Malignant neoplasms of tonsil, unspecified
2B6A	Malignant neoplasms of oropharynx
	Malignant neoplasms of the oral cavity and pharynx
	<i>Exclusions:</i> Malignant neoplasms of palate (2B65) Malignant neoplasms of tonsil (2B69) Malignant mesenchymal neoplasms (2B50-2B5Z)
2B6A.0	Squamous cell carcinoma of oropharynx
	A squamous cell carcinoma arising from the oropharynx. It predominantly affects adults in their fifth and sixth decades of life and is associated with alcohol and tobacco use. Human papillomavirus is present in approximately half of the cases. It is characterised by a tendency to metastasize early to the lymph nodes. When the tumour is small, patients are often asymptomatic. Physical examination may reveal erythematous or white lesions or plaques. The majority of patients present with locally advanced disease. Signs and symptoms include mucosal ulceration, pain, bleeding, weight loss, neck swelling, and difficulty speaking, chewing, and swallowing. Patients may also present with swollen neck lymph nodes without any symptoms from the oropharyngeal tumour. The most significant prognostic factors are the size of the tumour and the lymph nodes status.
2B6A.Y	Other specified malignant neoplasms of oropharynx
2B6A.Z	Malignant neoplasms of oropharynx, unspecified

2B6B**Malignant neoplasms of nasopharynx**

A wide variety of tumours can arise in the nasopharynx, but it is nasopharyngeal carcinoma that has fascinated generations of oncologists, pathologists, scientists and epidemiologists. It shows marked geographic differences, with highest incidence rates in Southern Chinese. In some endemic areas, the incidence has declined by about 30% over the past two decades, suggesting that environmental or lifestyle factors may play a major role and that the disease is, to some extent, preventable. Nasopharyngeal carcinoma shows a very strong association with Epstein-Barr virus (EBV) infection, irrespective of the ethnic origin of the patients. This association has pioneered a new paradigm of utilizing viral serological tests for the diagnosis of cancer and for screening in high-risk populations. Nasopharyngeal carcinoma is generally responsive to radiation therapy, and the clinical outcome has greatly improved over the years, due to refinements in staging and to improved therapy protocols. The unusual and often deceptive histological features of nasopharyngeal carcinoma have generated controversies over the nature of the tumour and still pose a challenge to surgical pathologists. There have possibly been more names invented for the various histological subtypes of nasopharyngeal carcinoma than any other tumour type.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2B6B.0**Squamous cell carcinoma of nasopharynx****2B6B.1****Malignant epithelial neoplasms of nasopharynx, unspecified type****2B6B.2****Malignant neoplasms of pharyngeal tonsil****2B6B.20**

Squamous cell carcinoma of pharyngeal tonsil

2B6B.21

Other or unspecified malignant epithelial neoplasm of pharyngeal tonsil

2B6B.2Y

Other specified malignant neoplasms of pharyngeal tonsil

2B6B.2Z

Malignant neoplasm of pharyngeal tonsil without mention of type

2B6B.Y**Other specified malignant neoplasms of nasopharynx****2B6B.Z****Malignant neoplasms of nasopharynx, unspecified****2B6C****Malignant neoplasms of piriform sinus**

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2B6C.0**Squamous cell carcinoma of piriform sinus****2B6C.Y****Other specified malignant neoplasms of piriform sinus****2B6C.Z****Malignant neoplasms of piriform sinus, unspecified****2B6D****Malignant neoplasms of hypopharynx**

A malignant neoplasm arising in the hypopharynx.

Exclusions: Malignant neoplasms of piriform sinus (2B6C)

Malignant mesenchymal neoplasms (2B50-2B5Z)

2B6D.0	Squamous cell carcinoma of hypopharynx and variants A squamous cell carcinoma arising from the hypopharynx. Signs and symptoms include dysphagia, hemoptysis, and the presence of a neck mass.
2B6D.Y	Other specified malignant neoplasms of hypopharynx
2B6D.Z	Malignant neoplasms of hypopharynx, unspecified
2B6E	Malignant neoplasms of other or ill-defined sites in the lip, oral cavity or pharynx <i>Exclusions:</i> Malignant neoplasm of oral cavity NOS (2B66) Malignant mesenchymal neoplasms (2B50-2B5Z)
2B6E.0	Squamous cell carcinoma of other or ill-defined sites in the lip, oral cavity or pharynx
2B6E.Y	Other specified malignant neoplasms of other or ill-defined sites in the lip, oral cavity or pharynx
2B6E.Z	Malignant neoplasms of other or ill-defined sites in the lip, oral cavity or pharynx, unspecified
2B6Y	Other specified malignant neoplasms of lip, oral cavity or pharynx
2B6Z	Malignant neoplasms of lip, oral cavity or pharynx, unspecified

Malignant neoplasms of digestive organs (2B70-2C1Z)

A primary malignant neoplasm involving any part of the gastrointestinal system.

Exclusions: Malignant neoplasm metastasis in digestive system (2D80-2D8Z)
Malignant mesenchymal neoplasms (2B50-2B5Z)
Malignant neoplasms of lip, oral cavity or pharynx (2B60-2B6Z)

2B70	Malignant neoplasms of oesophagus A primary malignant neoplasm involving the oesophagus <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
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2B70.0	Adenocarcinoma of oesophagus A malignant tumour with glandular differentiation arising predominantly from Barrett mucosa in the lower third of the esophagus. Grossly, esophageal adenocarcinomas are similar to esophageal squamous cell carcinomas. Microscopically, adenocarcinomas arising in the setting of Barrett esophagus are typically papillary and/or tubular. The prognosis is poor.
2B70.00	Barrett adenocarcinoma Barrett adenocarcinoma is defined as adenocarcinoma of lower oesophagus and gastroesophageal junction associated with Barrett's oesophagus.
2B70.0Y	Other specified adenocarcinoma of oesophagus

2B70.0Z	Adenocarcinoma of oesophagus, unspecified
2B70.1	Squamous cell carcinoma of oesophagus A squamous cell carcinoma arising from the esophagus. It can be associated with a long history of tobacco and alcohol abuse and is exceedingly rare before the age of 30. The median age is around 65 in both males and females, but the incidence in males is much higher than in females. It is located mostly in the middle and lower third of the esophagus. Grossly, polypoid, ulcerated, plaque-like and occult lesions have been described. The microscopic features are the same as in other squamous cell carcinomas. Any degree of differentiation may occur, and variation within a single tumour is common. The prognosis is poor.
2B70.Y	Other specified malignant neoplasms of oesophagus
2B70.Z	Malignant neoplasms of oesophagus, unspecified
2B71	Malignant neoplasms of oesophagogastric junction Malignant neoplasms that arise from the cells of the oesophagogastric junction (OGJ), which is defined as the point at which the oesophagus ends and the stomach begins. This mainly defines adenocarcinomas that straddle the junction of the oesophagus and stomach. This definition includes many tumours formally called cancers of the gastric cardia. Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)
2B71.0	Adenocarcinoma of oesophagogastric junction An adenocarcinoma that arises from and straddles the junction of the stomach and esophagus. The category of adenocarcinomas of the gastroesophageal junction also includes the majority of adenocarcinomas previously called gastric cardia adenocarcinomas. Squamous cell carcinomas that affect or cross the junction of the stomach and esophagus are classified as carcinomas of the distal esophagus. Adenocarcinoma of the gastroesophageal junction occurs more often in Caucasian middle aged and elderly males. Clinical signs and symptoms include dysphagia, abdominal pain, and weight loss. The prognosis depends on the completeness of the surgical resection, the number of lymph nodes involved by cancer, and the presence or absence of postoperative complications.
2B71.Y	Other specified malignant neoplasm of oesophagogastric junction
2B71.Z	Malignant neoplasms of oesophagogastric junction, unspecified
2B72	Malignant neoplasms of stomach A primary or metastatic malignant neoplasm involving the stomach. Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z) Coded Elsewhere: Extranodal marginal zone B-cell lymphoma of mucosa-associated lymphoid tissue of stomach (2A85.1) Leiomyosarcoma of stomach (2B58.2) Gastrointestinal stromal tumour of stomach (2B5B.0) Gastric malignant lymphoma (2B33.5)

- 2B72.0 Adenocarcinoma of stomach**
 An adenocarcinoma arising from the stomach glandular epithelium. Gastric adenocarcinoma is primarily a disease of older individuals. It most commonly develops after a long period of atrophic gastritis and is strongly associated with *Helicobacter pylori* infection. The lack of early symptoms often delays the diagnosis of gastric cancer. The majority of patients present with advanced tumours which have poor rates of curability. Microscopically, two important histologic types of gastric adenocarcinoma are recognised: the intestinal and diffuse type. The overall prognosis of gastric adenocarcinomas is poor, even in patients who receive a curative resection.
- 2B72.1 Malignant neuroendocrine neoplasm of stomach**
 A neoplasm with neuroendocrine differentiation that arises from the stomach. It includes well differentiated neuroendocrine tumours (low and intermediate grade) and poorly differentiated neuroendocrine carcinomas (high grade).
Inclusions: carcinoid and other malignant neuroendocrine neoplasms
Coded Elsewhere: Neuroendocrine neoplasm of duodenum (2B80.01)
 Neuroendocrine neoplasms of appendix (2B81.2)
- 2B72.Y Other specified malignant neoplasms of stomach**
- 2B72.Z Malignant neoplasms of stomach, unspecified**

Malignant neoplasms of intestine (2B80-2C0Z)

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

- 2B80 Malignant neoplasms of small intestine**
 A primary malignant neoplasm involving the small intestine.
Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)
Coded Elsewhere: Enteropathy associated T-cell lymphoma (2A90.7)
 Gastrointestinal stromal tumour of small intestine (2B5B.1)
 Malignant lymphoma of small intestine (2B33.5)
 Malignant neoplasm of jejunum (2B80.1Z)
 Malignant neoplasm of ileum (2B80.1Z)
 Small intestinal leiomyosarcoma (2B58.Y)

- 2B80.0 Malignant neoplasms of duodenum**
 A primary malignant neoplasm that affects the duodenum. Representative examples include carcinoma, lymphoma, and sarcoma.
Coded Elsewhere: Lymphoma of duodenum (2B33.5)
 Malignant mesenchymal tumour of the duodenum (2B5F.2)

- 2B80.00 Adenocarcinoma of duodenum**
 An adenocarcinoma that arises from the duodenum.

2B80.01	Neuroendocrine neoplasm of duodenum Neoplasms that arise from the cells of neuroendocrine system lining the duodenum.
2B80.0Y	Other specified malignant neoplasms of the duodenum
2B80.0Z	Malignant neoplasms of duodenum, unspecified
2B80.1	Malignant neoplasms of jejunum or ileum
2B80.10	Adenocarcinoma of jejunum or ileum
2B80.11	Neuroendocrine neoplasms of jejunum or ileum Neoplasms that arise from the cells of neuroendocrine system lining the jejunum or ileum including well-differentiated (low- to intermediated grade) neuroendocrine tumours. These include carcinoid tumour.
2B80.1Y	Other specified malignant neoplasms of jejunum or ileum
2B80.1Z	Malignant neoplasms of jejunum or ileum, unspecified
2B80.2	Malignant neoplasms of small intestine, site unspecified
2B80.20	Adenocarcinoma of small intestine, site unspecified A malignant tumour with glandular differentiation arising from epithelium of small intestine. Exclusions: Neuroendocrine neoplasms of small intestine, site unspecified (2B80.21)
2B80.21	Neuroendocrine neoplasms of small intestine, site unspecified Neoplasms that arise from the cells of neuroendocrine system lining the small intestine.
2B80.2Y	Other specified malignant neoplasms of small intestine, site unspecified
2B80.Y	Other specified malignant neoplasms of small intestine
2B80.Z	Malignant neoplasms of small intestine, unspecified
2B81	Malignant neoplasms of appendix A primary malignant neoplasm that affects the appendix. Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z) Coded Elsewhere: Lymphoma of the appendix (2B33.5)
2B81.0	Adenocarcinoma of appendix A malignant neoplasm arising from the glandular epithelium of the appendix. Most are mucinous adenocarcinomas.
2B81.00	Mucinous adenocarcinoma of appendix An adenocarcinoma, often cystic, with large amounts of extracellular mucin

2B81.2	Neuroendocrine neoplasms of appendix Malignant neoplasms with neuroendocrine differentiation that arise in the appendix. Most are well differentiated neuroendocrine tumours (low and intermediate grade), i.e. carcinoids. Poorly differentiated neuroendocrine carcinomas (high grade) are exceedingly rare.
2B81.Y	Other specified malignant neoplasms of appendix
2B81.Z	Malignant neoplasms of appendix, unspecified

Malignant neoplasms of large intestine (2B90-2B9Y)

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2B90	Malignant neoplasms of colon Primary malignant neoplasms arising in the colon. Exclusions: Malignant neoplasms of appendix (2B81) Coded Elsewhere: Gardner syndrome (LD2D.3) Malignant Lymphoma of colon (2B33.5) Gastrointestinal stromal tumour of colon (2B5B.Y) Kaposi sarcoma of colon (2B57.2) Leiomyosarcoma of colon (2B58.Y)
2B90.0	Malignant neoplasm of ascending colon and right flexure of colon
2B90.00	Adenocarcinoma of ascending colon or right flexure of colon A malignant tumour with glandular differentiation arising from epithelium of ascending colon and right flexure.
2B90.0Y	Other specified malignant neoplasm of ascending colon and right flexure of colon
2B90.0Z	Malignant neoplasm of ascending colon and right flexure of colon, unspecified
2B90.1	Malignant neoplasm of descending colon and splenic flexure of colon
2B90.10	Adenocarcinoma of descending colon or splenic flexure of colon A malignant tumour with glandular differentiation arising from epithelium of descending colon and splenic flexure.
2B90.1Y	Other specified malignant neoplasm of descending colon and splenic flexure of colon
2B90.1Z	Malignant neoplasm of descending colon and splenic flexure of colon, unspecified
2B90.2	Malignant neoplasm of transverse colon
2B90.20	Adenocarcinoma of transverse colon A malignant tumour with glandular differentiation arising from epithelium of transverse colon.
2B90.2Y	Other specified malignant neoplasm of transverse colon

2B90.2Z	Malignant neoplasm of transverse colon, unspecified
2B90.3	Malignant neoplasm of sigmoid colon <i>Exclusions:</i> Malignant neoplasms of rectosigmoid junction (2B91)
2B90.30	Adenocarcinoma of sigmoid colon A malignant tumour with glandular differentiation arising from epithelium of descending colon and splenic flexure.
2B90.3Y	Other specified malignant neoplasm of sigmoid colon
2B90.3Z	Malignant neoplasm of sigmoid colon, unspecified
2B90.Y	Other specified malignant neoplasms of colon
2B90.Z	Malignant neoplasms of colon, unspecified
2B91	Malignant neoplasms of rectosigmoid junction
2B91.0	Adenocarcinoma of rectosigmoid junction A malignant tumour with glandular differentiation arising from epithelium of rectosigmoid junction
2B91.Y	Other specified malignant neoplasms of rectosigmoid junction
2B91.Z	Malignant neoplasms of rectosigmoid junction, unspecified
2B92	Malignant neoplasms of rectum <i>Coded Elsewhere:</i> Malignant mesenchymal tumour of rectum (2B5F.2)
2B92.0	Adenocarcinomas of rectum An adenocarcinoma arising from the rectum. It is more frequently seen in populations with a Western type diet and in patients with a history of chronic inflammatory bowel disease.
2B92.1	Neuroendocrine neoplasms of rectum Malignant neoplasms with neuroendocrine differentiation that arise in the rectum. Most are well differentiated neuroendocrine tumours (low and intermediate grade), i.e. carcinoids. Poorly differentiated neuroendocrine carcinomas (high grade) are rare.
2B92.Y	Other specified malignant neoplasms of rectum
2B92.Z	Malignant neoplasms of rectum, unspecified
2B93	Malignant neoplasms of large intestine, site unspecified
2B93.0	Adenocarcinoma of large intestine, site unspecified
2B93.Y	Other specified malignant neoplasms of large intestine, site unspecified
2B93.Z	Malignant neoplasms of large intestine, site and type unspecified
2B9Y	Other specified malignant neoplasms of large intestine

2C00	Malignant neoplasms of anus or anal canal	A primary malignant neoplasm that arises in the anal canal up to the junction with perianal, hair-bearing skin. Representative examples include carcinomas and melanomas. Tumours of the anal margin are classified with skin tumours.
	Exclusions:	Malignant mesenchymal neoplasms (2B50-2B5Z)
2C00.0	Adenocarcinoma of anus or anal canal	An adenocarcinoma arising in the anal canal epithelium, including the mucosal surface, the anal glands, and the lining of fistulous tracts.
2C00.1	Melanoma of anus or anal canal	A form of cancer that develops in melanocytes, the cells that produce pigment melanin in the skin.
2C00.2	Neuroendocrine neoplasm of anus or anal canal	Neoplasms that arise from the cells of the neuroendocrine system lining the anus and anal canal.
2C00.3	Squamous cell carcinoma of anus or anal canal	A squamous cell carcinoma (SCC) arising from the anal canal up to the junction with the anal margin (perianal, hair-bearing skin). Human papillomavirus is detected in the majority of cases. Homosexual HIV-positive men have an increased risk of developing anal squamous cell carcinoma in comparison to the general male population. The prognosis is generally better for anal margin SCC than for anal canal SCC. The former are classified with skin tumours.
2C00.Y	Other specified malignant neoplasms of anus and anal canal	
2C00.Z	Malignant neoplasms of anus or anal canal, unspecified	
2C0Y	Other specified malignant neoplasms of intestine	
2C0Z	Malignant neoplasms of intestine, unspecified	
2C10	Malignant neoplasm of pancreas	A primary malignant tumour of the pancreas. Most are adenocarcinomas.
	Exclusions:	Malignant mesenchymal neoplasms (2B50-2B5Z)
2C10.0	Adenocarcinoma of pancreas	An adenocarcinoma which arises from the exocrine pancreas. Ductal adenocarcinoma and its variants are the most common types of pancreatic adenocarcinoma.
2C10.1	Neuroendocrine neoplasms of pancreas	A neoplasm with neuroendocrine differentiation that arises from the pancreas. It includes neuroendocrine tumours (low and intermediate grade) and neuroendocrine carcinomas (high grade).
2C10.Y	Other specified malignant neoplasms of pancreas	

2C10.Z	Malignant neoplasm of pancreas, unspecified
2C11	Malignant neoplasms of other or ill-defined digestive organs A primary malignant tumour involving a digestive organ or organs not coded elsewhere (including intestinal tract [part unspecified], overlapping lesions of the digestive tract and other ill-defined sites within the digestive system) <i>Exclusions:</i> Malignant neoplasms of retroperitoneum (2C50) Malignant neoplasms of peritoneum (2C51) Malignant mesenchymal neoplasms (2B50-2B5Z)
2C11.0	Adenocarcinoma of other or ill-defined digestive organs
2C11.1	Mucinous carcinoma of other or ill-defined digestive organs
2C11.2	Other specified malignant neoplasms of other or ill-defined digestive organs
2C11.Z	Malignant neoplasms of other or ill-defined digestive organs, unspecified
2C12	Malignant neoplasms of liver or intrahepatic bile ducts The most frequent and important hepatic neoplasm is the primary hepatocellular carcinoma (HCC). In many parts of the world, in particular Africa and Asia, it poses a significant disease burden. In these high incidence regions, chronic infection with hepatitis B virus (HBV) is the principal underlying cause, with the exception of Japan which has a high prevalence of hepatitis C infection. HBV vaccination has become a powerful tool in reducing cirrhosis and HCC, but implementation is still suboptimal in several high risk regions. In Western countries, chronic alcohol abuse is a major aetiological factor. Hepatic cholangiocarcinoma has a different geographical distribution, with peak incidences in Northern Thailand. Here, it is caused by chronic infection with the liver fluke, <i>Opisthorchis Viverrini</i> , which is ingested through infected raw fish. <i>Exclusions:</i> Secondary malignant neoplasm of liver (2D80) Malignant neoplasm of biliary tract NOS (2C17)
2C12.0	Malignant neoplasm of liver
2C12.00	Combined hepatocellular-cholangiocarcinoma Combined hepatocellular-cholangiocarcinoma is a tumour containing unequivocal, intimately mixed elements of both hepatocellular carcinoma and cholangiocarcinoma. <i>Inclusions:</i> Hepatocholangiocarcinoma
2C12.01	Hepatoblastoma A malignant liver neoplasm that occurs almost exclusively in infants, although isolated cases in older children and adults have been reported. Grossly, hepatoblastoma is solid, well circumscribed, and more often solitary than multiple. Microscopically, most of the tumours are composed exclusively of immature hepatocytic elements. About a fourth of hepatoblastomas contain a stromal component that may be undifferentiated or develop into bone or cartilage.

2C12.02	Hepatocellular carcinoma of liver A carcinoma that arises from the hepatocytes.
2C12.03	Mesothelial carcinoma of liver
2C12.0Y	Other specified malignant neoplasm of liver
2C12.1	Malignant neoplasm of intrahepatic bile ducts
2C12.10	Intrahepatic cholangiocarcinoma A carcinoma that arises from the intrahepatic bile duct epithelium in any site of the intrahepatic biliary tree. Grossly, the malignant lesions are solid, nodular, and grayish. Morphologically, the vast majority of cases are adenocarcinomas. Early detection is difficult and the prognosis is generally poor.
2C12.1Y	Other specified malignant neoplasms of intrahepatic bile ducts
2C12.Z	Malignant neoplasms of liver or intrahepatic bile ducts, unspecified
2C13	Malignant neoplasms of gallbladder A malignant tumour arising from the epithelium of the gallbladder. It is usually associated with the presence of gallstones. Clinical symptoms are not specific and usually present late in the course. Morphologically, adenocarcinoma is the most common type, however squamous cell carcinomas, adenosquamous carcinomas, signet ring carcinomas, and undifferentiated carcinomas can also occur.
2C13.0	Adenocarcinoma of the gallbladder An adenocarcinoma arising from the gallbladder. It is the most common malignant tumour of the gallbladder, typically in the fundus; it is usually well to moderately differentiated. The incidence is higher in patients with gallstones than in patients without gallstones. Signs and symptoms usually present late in the course of the disease and are reminiscent of those of chronic cholecystitis including right upper quadrant pain. Histologic variants include adenocarcinoma of the intestinal type, clear cell adenocarcinoma, mucinous adenocarcinoma, papillary adenocarcinoma, and signet ring adenocarcinoma.
2C13.Y	Other specified malignant neoplasm of gallbladder
2C13.Z	Malignant neoplasms of gallbladder, unspecified
2C14	Malignant neoplasms of proximal biliary tract, cystic duct
2C14.0	Adenocarcinoma of proximal biliary tract, cystic duct
2C14.1	Mucinous cystic neoplasm with associated invasive carcinoma of cystic duct
2C14.2	Neuroendocrine neoplasms of cystic duct
2C14.Y	Other specified malignant neoplasms of biliary tract, cystic duct
2C14.Z	Malignant neoplasms of proximal biliary tract, cystic duct, unspecified
2C15	Malignant neoplasms of biliary tract, distal bile duct

2C15.0	Adenocarcinoma of biliary tract, distal bile duct An adenocarcinoma that arises from the common bile duct distal to the insertion of the cystic duct.
2C15.1	Mucinous cystic neoplasm with associated invasive carcinoma of distal bile duct
2C15.2	Neuroendocrine neoplasms of distal bile duct
2C15.Y	Other specified malignant neoplasms of biliary tract, distal bile duct
2C15.Z	Malignant neoplasms of biliary tract, distal bile duct, unspecified
2C16	Malignant neoplasms of ampulla of Vater
2C16.0	Adenocarcinoma of ampulla of Vater A malignant glandular epithelial tumour arising in the ampulla of Vater
2C16.1	Neuroendocrine neoplasms of ampulla of Vater
2C16.Y	Other specified malignant neoplasms of ampulla of Vater
2C16.Z	Malignant neoplasms of ampulla of Vater, unspecified
2C17	Malignant neoplasms of other or unspecified parts of biliary tract <i>Exclusions:</i> Malignant neoplasm of intrahepatic bile duct (2C12) <i>Coded Elsewhere:</i> Malignant mesenchymal tumours of gallbladder or bile ducts (2B5F.2)
2C17.0	Adenocarcinoma of other or unspecified parts of biliary tract
2C17.1	Mucinous cystic neoplasm with associated invasive carcinoma of other or unspecified parts of biliary tract
2C17.2	Neuroendocrine neoplasms of other or unspecified parts of biliary tract
2C17.Y	Other specified malignant neoplasms of overlapping lesion of biliary tract
2C17.Z	Malignant neoplasms of other or unspecified parts of biliary tract, unspecified
2C18	Malignant neoplasms of perihilar bile duct "Includes left, right and common hepatic ducts to the origin of the cystic duct"
2C18.0	Hilar cholangiocarcinoma Klatskin tumour is an extra-hepatic cholangiocarcinoma arising in the junction of the main right or left hepatic ducts to form the common hepatic duct.
2C18.1	Mucinous cystic neoplasm with associated invasive carcinoma of perihilar bile duct
2C18.2	Neuroendocrine neoplasm of perihilar bile duct
2C18.Y	Other specified malignant neoplasms of perihilar bile duct
2C18.Z	Malignant neoplasms of perihilar bile duct, unspecified

2C1Z**Malignant neoplasms of digestive organs, unspecified****Malignant neoplasms of middle ear, respiratory or intrathoracic organs (2C20-2C2Z)****Exclusions:** Mesotheliomas of peritoneum (2C51.2)

Malignant mesenchymal neoplasms (2B50-2B5Z)

2C20**Malignant neoplasms of nasal cavity**

From the chapter on Nasal Cavity (H&N BB) Although the nasal cavity and paranasal sinuses occupy a relatively small anatomical space, they are the site of origin of some of the more complex, histologically diverse group of tumours in the entire human body. These include neoplasms derived from mucosal epithelium, seromucinous glands, soft tissues, bone, cartilage, neural/neuroectodermal tissue, haematolymphoid cells and the odontogenic apparatus. Many of the tumours are similar to those found elsewhere in the body but a few, such as the olfactory neuroblastoma, are unique to this site.

Exclusions: Malignant neoplasm of nose NOS (2C29)

Malignant neoplasm of olfactory bulb (2A02)

Malignant neoplasm of posterior margin of nasal septum and choana (2B6B)

Malignant neoplasm of skin of nose (2C30-2C3Z)

Malignant mesenchymal neoplasms (2B50-2B5Z)

2C20.0 Adenocarcinoma of nasal cavity**2C20.1 Malignant neuroepitheliomatous neoplasm of nasal cavity****2C20.2 Melanoma of nasal cavity****2C20.3 Olfactory neuroblastoma****2C20.4 Squamous cell carcinoma of nasal cavity****2C20.Y Other specified malignant neoplasm of nasal cavity****2C20.Z Malignant neoplasms of nasal cavity, unspecified****2C21****Malignant neoplasms of middle ear**

Malignant neoplasm originating in the middle ear.

Exclusions: malignant neoplasm of skin of (external) ear (2C30-2C3Z)

malignant neoplasm of bone of ear (meatus) (2B50-2B5Z)

malignant neoplasm of cartilage of ear (2B50-2B5Z)

Malignant mesenchymal neoplasms (2B50-2B5Z)

2C21.0 Adenocarcinoma of middle ear**2C21.1 Squamous cell carcinoma of middle ear**

2C21.2	Unspecified malignant epithelial neoplasm of middle ear
2C21.Y	Other specified malignant neoplasm of middle ear
2C21.Z	Malignant neoplasms of middle ear, unspecified
2C22	Malignant neoplasms of accessory sinuses
	<i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C22.0	Adenocarcinoma of accessory sinuses
2C22.1	Squamous cell carcinoma of accessory sinuses
2C22.10	Squamous cell carcinoma of sphenoidal sinus
2C22.1Y	Squamous cell carcinoma of other specified accessory sinuses
2C22.2	Malignant epithelial neoplasms of accessory sinuses, unspecified type
2C22.3	Melanomas of accessory sinuses
2C22.Y	Other specified malignant neoplasms of accessory sinuses
2C22.Z	Malignant neoplasms of accessory sinuses, unspecified
2C23	Malignant neoplasms of larynx
	<i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C23.1	Malignant neoplasms of glottis of larynx
2C23.10	Squamous cell carcinoma of larynx, glottis A squamous cell carcinoma of the larynx that arises from the glottic area. It may remain localised for a long period then in late disease stage, it may spread to the opposite true vocal cord, supraglottic and subglottic areas, and the soft tissues of the neck. Hoarseness is the presenting symptom.
2C23.1Y	Other specified malignant neoplasms of larynx, glottis
2C23.2	Malignant neoplasms of supraglottis of larynx
	<i>Exclusions:</i> Malignant neoplasm of anterior surface of epiglottis (2B6A)
2C23.20	Squamous cell carcinoma of larynx, supraglottis A squamous cell carcinoma of the larynx that arises from the supraglottic area. Signs and symptoms include dysphagia, a sensation of foreign body in the throat, and hemoptysis. It spreads to the space anterior to the epiglottis, pyriform sinus, and base of the tongue.
2C23.2Y	Other specified malignant neoplasms of larynx, supraglottis
2C23.3	Malignant neoplasms of subglottis of larynx A primary or metastatic malignant neoplasm involving the subglottis.

2C23.30	Squamous cell carcinoma of larynx, subglottis A squamous cell carcinoma of the larynx that arises from the subglottic area. Symptoms include dyspnea and stridor. It spreads to the hypopharynx, trachea, and thyroid gland.
2C23.31	Adenocarcinoma of larynx, subglottis
2C23.3Y	Other specified malignant neoplasms of larynx, subglottis
2C23.4	Malignant neoplasm of laryngeal cartilage
2C23.5	Malignant neoplasm of overlapping lesion of larynx
2C23.Z	Malignant neoplasms of larynx, unspecified
2C24	Malignant neoplasms of trachea A primary or metastatic malignant neoplasm involving the trachea Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z) trachea carina cancer (2C25)
2C24.0	Adenocarcinoma of trachea
2C24.1	Squamous cell carcinoma of trachea
2C24.2	Malignant epithelial neoplasms of trachea, unspecified type
2C24.Y	Other specified malignant neoplasms of trachea
2C24.Z	Malignant neoplasms of trachea, unspecified
2C25	Malignant neoplasms of bronchus or lung A primary or metastatic malignant neoplasm involving the lung. Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)
2C25.0	Adenocarcinoma of bronchus or lung A carcinoma that arises from the lung and is characterised by the presence of malignant glandular epithelial cells. There is a male predilection with a male to female ratio of 2:1. Usually lung adenocarcinoma is asymptomatic and is identified through screening studies or as an incidental radiologic finding. If clinical symptoms are present they include shortness of breath, cough, hemoptysis, chest pain, and fever. Tobacco smoke is a known risk factor.
2C25.1	Small cell carcinoma of bronchus or lung A highly aggressive subtype of lung carcinoma characterised by the presence of malignant small cells and necrosis. Metastatic disease is usually present at the time of diagnosis.

2C25.2	Squamous cell carcinoma of bronchus or lung A carcinoma arising from malignant squamous bronchial epithelial cells and characterised by the presence of keratinization and/or intercellular bridges. Cigarette smoking and arsenic exposure are strongly associated with squamous cell lung carcinoma.
2C25.3	Large cell carcinoma of bronchus or lung
2C25.4	Carcinoid or other malignant neuroendocrine neoplasms of bronchus or lung
2C25.5	Unspecified malignant epithelial neoplasm of bronchus or lung
2C25.Y	Other specified malignant neoplasms of bronchus or lung
2C25.Z	Malignant neoplasms of bronchus or lung, unspecified
2C26	Malignant neoplasms of the pleura A primary or metastatic malignant neoplasm affecting the pleura. A representative example of primary malignant pleural neoplasm is the malignant pleural mesothelioma. A representative example of metastatic malignant neoplasm to the pleura is when a metastatic carcinoma has spread to the pleura from another anatomic site. <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C26.0	Mesothelioma of pleura Malignant mesothelioma is a fatal asbestos-associated malignancy arising in the lining cells (mesothelium) of the pleural and peritoneal cavities, as well as in the pericardium and the tunica vaginalis.
2C26.Y	Other specified malignant neoplasms of the pleura
2C26.Z	Malignant neoplasms of the pleura, unspecified
2C27	Malignant neoplasms of thymus Primary malignant neoplasm involving the thymus. This category includes malignant thymomas, primary thymic carcinomas and carcinoid tumour or other neuroendocrine neoplasms of thymus. <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C27.0	Carcinoma of thymus A diverse group of carcinomas of the thymus gland, previously known as thymoma type C. It includes morphologic variants derived from purely epithelial cells, as well as from cells with neuroendocrine differentiation.
2C27.1	Carcinoid tumour or other neuroendocrine neoplasms of thymus Thymic neuroendocrine carcinoma is a type of thymic epithelial neoplasm displaying evidence of neuroendocrine differentiation.

2C27.2	Malignant thymoma A thymoma that has an aggressive clinical course (capsular invasion, infiltration of the surrounding tissues) and can metastasize. Although any morphologic subtype of thymoma may eventually have a malignant clinical course, this term is most often associated with thymoma types B3 and C.
2C27.Y	Other specified malignant neoplasms of thymus
2C27.Z	Malignant neoplasms of thymus, unspecified
2C28	Malignant neoplasms of heart, mediastinum or non-mesothelioma of pleura <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C28.0	Malignant germ cell neoplasms of heart, mediastinum or non-mesothelioma of pleura
2C28.1	Other specified malignant neoplasms of heart, mediastinum or non-mesothelioma of pleura
2C28.Z	Malignant neoplasms of heart, mediastinum or non-mesothelioma of pleura, unspecified
2C29	Malignant neoplasms of other or ill-defined sites in the respiratory system or intrathoracic organs <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C29.0	Squamous cell carcinomas of other and ill-defined sites in the respiratory system and intrathoracic organs
2C29.1	Other specified malignant neoplasms of other or ill-defined sites in the respiratory system or intrathoracic organs
2C29.Z	Malignant neoplasms of other or ill-defined sites in the respiratory system or intrathoracic organs, unspecified
2C2Y	Other specified malignant neoplasms of middle ear, respiratory or intrathoracic organs
2C2Z	Malignant neoplasms of middle ear, respiratory or intrathoracic organs, unspecified

Malignant neoplasms of skin (2C30-2C3Z)

A primary or metastatic tumour involving the skin. Primary malignant skin tumours most often are carcinomas (either basal cell or squamous cell carcinomas that arise from cells in the epidermis) or melanomas that arise from pigment-containing skin melanocytes. Metastatic tumours to the skin include carcinomas and lymphomas.

Exclusions: Carcinoma in situ of skin (2E64)
Metastatic malignant neoplasm involving skin (2E08)
Malignant mesenchymal neoplasms (2B50-2B5Z)

2C30

Melanoma of skin

A primary melanoma arising from atypical melanocytes in the skin. Precursor lesions include acquired and congenital melanocytic nevi, and dysplastic nevi. Several histologic variants have been recognised, including superficial spreading melanoma, acral lentiginous melanoma, nodular melanoma, and lentigo maligna melanoma.

Exclusions: Melanoma of penis (2C81.1)
Melanoma of vulva (2C70.1)

2C30.0

Superficial spreading melanoma, primary

The commonest form of melanoma, superficial spreading malignant melanoma accounts for about 70 per cent of all melanomas. It characteristically presents as an asymptomatic pigmented cutaneous macule which is asymmetrical in shape and displays variability in both hue and pigment intensity. It has a relatively long phase of progressive superficial extension (radial growth) before penetrating deeper into the dermis and entering an invasive vertical growth phase.

2C30.1

Nodular melanoma, primary

Variant of melanoma carrying a poor prognosis due to the fact that there is little or no prodromal radial (superficial) growth phase before deep invasion (vertical growth). The lesion presents as an elevated, reddish, bluish or dark brown coloured, dome-shaped tumour. Ulceration or bleeding from the lesion occurs frequently. It occurs most frequently in the fifth or sixth decade.

2C30.2

Lentigo maligna melanoma, primary

Lentigo maligna malignant melanoma is a form of melanoma which occurs within a lentigo maligna when neoplastic cells no longer remain confined to the epidermis (in situ radial growth) but invade the dermis (vertical growth). The lentigo maligna from which it arises may have been present as an irregularly pigmented macule on sun-exposed skin for many years before dermal invasion supervenes. Clinical differentiation from lentigo maligna may not be possible in the early stages of invasion but as the tumour progresses a focal thickening or nodule within the lentigo maligna will become apparent.

Exclusions: Lentigo maligna (2E63.00)

2C30.3	Acral lentiginous melanoma, primary Acral lentiginous malignant melanoma is a distinct and uncommon form of melanoma affecting either palmar and plantar skin or the nail apparatus. It is usually preceded by a slowly progressive in situ phase which may be overlooked. It typically presents either as an enlarging area of macular pigmentation on the palms and soles or as a longitudinal pigmented band within the nail plate. More aggressive tumours present as ulcerated nodules which, when involving the nail apparatus, can destroy the nail plate. Acral lentiginous malignant melanoma accounts for a high proportion of melanomas seen in dark-skinned people.
2C30.Y	Other specified melanoma of skin
2C30.Z	Melanoma of skin, unspecified
2C31	Squamous cell carcinoma of skin A carcinoma arising from the squamous cells of the epidermis. Skin squamous cell carcinoma is most commonly found on sun-exposed areas. The majority of the tumours are well-differentiated. Coded Elsewhere: Squamous cell carcinoma of penis (2C81.0)
2C31.0	Verrucous squamous cell carcinoma of skin Verrucous squamous cell carcinoma is a rare variant of well-differentiated squamous cell carcinoma with low malignant potential. It occurs principally on the glans and prepuce of the penis and on the sole of the foot. Exclusions: Verrucous squamous cell carcinoma of vulva (2C70.2)
2C31.1	Keratoacanthoma Keratoacanthoma is a relatively common keratinocytic epidermal tumour which shows resemblances to squamous cell carcinoma of the skin, from which it may be difficult to distinguish either clinically or histopathologically. It is characterised by rapid growth over a few weeks to months, followed by spontaneous resolution over 4-6 months. Because it is not possible to predict its benign behaviour with complete certainty during its initial growth phase, the designation "Well-differentiated squamous cell carcinoma (keratoacanthoma type)" is also used.
2C31.Z	Cutaneous squamous cell carcinoma
2C32	Basal cell carcinoma of skin Basal cell carcinoma or BCC is the most common malignancy in humans. It is believed that BCCs arise from pluripotential cells in the basal layer of the epidermis or, less commonly, hair follicle. BCCs typically occur in areas of chronic sun exposure and presents as slowly enlarging reddish papules, plaques or nodules on the head and neck, although the superficial variant is often located on the trunk. BCCs frequently ulcerate and become crusted. Although they rarely metastasize, they can cause significant local destruction and disfigurement if neglected or inadequately treated, particularly if of the sclerosing or infiltrative subtype.

2C32.0	Nodular basal cell carcinoma of skin This is the most common form of basal cell carcinoma and is typically located on the head or neck. It starts as a small translucent nodule which will frequently necrose and ulcerate as it enlarges. Telangiectatic blood vessels can often be detected just under the tumour surface. A minority may be pigmented and give rise to difficulty in distinguishing them from melanoma.
2C32.1	Sclerosing basal cell carcinoma of skin This form of basal cell carcinoma is composed of thin strands, cords and columns of malignant cells which infiltrate between collagen bundles of the dermis. It may infiltrate widely and deeply before it becomes clinically obvious. It typically starts as a pale, poorly-defined indurated plaque which may not come to medical attention until it starts to bleed and crust.
2C32.2	Superficial basal cell carcinoma of skin Superficial basal cell carcinomas are often multiple and appear as pink or red barely elevated patches varying in size from a few mm to over 10 cm in diameter. A fine pearly border can usually be seen on careful inspection. They occur most frequently on the trunk.
2C32.Y	Other specified basal cell carcinoma of skin
2C32.Z	Basal cell carcinoma of skin, unspecified
2C33	Adnexal carcinoma of skin A carcinoma arising from the sebaceous glands, sweat glands, or the hair follicles. Representative examples include sebaceous carcinoma, apocrine carcinoma, eccrine carcinoma, and pilomatrical carcinoma. <i>Inclusions:</i> - Primary cutaneous mucinous carcinoma - Primary cutaneous adenoid cystic carcinoma - Appendageal carcinoma of skin
2C34	Cutaneous neuroendocrine carcinoma Cutaneous neuroendocrine carcinoma is a primary cutaneous cancer arising from a subset of skin neuroendocrine cells (Merkel cells, giving the name Merkel cell carcinoma (MCC)).
2C35	Cutaneous sarcoma A group of generally rare malignant neoplasms arising from mesenchymal elements in the dermis including fibroblasts, pilar smooth muscle and vascular endothelium. <i>Coded Elsewhere:</i> - Angiosarcoma of skin (2B56.1) - Kaposi sarcoma of skin (2B57.1) - Cutaneous leiomyosarcoma (2B58.Y) - Dermatofibrosarcoma protuberans (2B53.Y)
2C3Y	Other specified malignant neoplasms of skin
2C3Z	Malignant neoplasm of skin of unknown or unspecified type

Malignant neoplasms of peripheral nerves or autonomic nervous system (2C40-2C4Z)

Exclusions: Malignant nerve sheath tumour of peripheral nerves or autonomic nervous system, primary site (2B5E)

Malignant mesenchymal neoplasms (2B50-2B5Z)

2C40 Malignant neuroepitheliomatous neoplasms of peripheral nerves or autonomic nervous system

2C41 Malignant perineurioma

2C4Y Other specified malignant neoplasms of peripheral nerves and autonomic nervous system

2C4Z Malignant neoplasms of peripheral nerves or autonomic nervous system, unspecified

Malignant neoplasms of retroperitoneum, peritoneum or omentum (2C50-2C5Z)

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2C50 Malignant neoplasms of retroperitoneum

A primary or metastatic malignant neoplasm involving the retroperitoneum. The vast majority of cases are carcinomas, lymphomas, or sarcomas.

Exclusions: Malignant neoplasms of omentum (2C52)

Malignant neoplasms of peritoneum (2C51)

Malignant mesenchymal neoplasms (2B50-2B5Z)

2C50.0 Cystic, mucinous or serous carcinoma of retroperitoneum

2C50.Y Other specified malignant neoplasms of retroperitoneum

2C50.Z Malignant neoplasms of retroperitoneum, unspecified

2C51 Malignant neoplasms of peritoneum

Exclusions: Malignant neoplasms of retroperitoneum (2C50)

Malignant neoplasms of omentum (2C52)

Malignant mesenchymal neoplasms (2B50-2B5Z)

2C51.0 Adenocarcinomas of peritoneum

2C51.1 Cystic, mucinous or serous carcinoma of peritoneum

2C51.2 Mesotheliomas of peritoneum

A benign or malignant mesothelial neoplasm that arises from the peritoneum.

2C51.20 Mesothelioma of mesocolon

2C51.21 Mesothelioma of mesentery

2C51.2Y Mesotheliomas of other specified sites of peritoneum

2C51.2Z	Mesotheliomas of peritoneum, site unspecified
2C51.Y	Other specified malignant neoplasms of peritoneum
2C51.Z	Malignant neoplasms of peritoneum, unspecified
2C52	Malignant neoplasms of omentum
	<i>Exclusions:</i> Malignant neoplasms of retroperitoneum (2C50) Malignant neoplasms of peritoneum (2C51) Malignant mesenchymal neoplasms (2B50-2B5Z)
	<i>Coded Elsewhere:</i> Myosarcomas of omentum (2B5F.10)
2C52.0	Cystic, mucinous or serous carcinoma of omentum
2C52.Y	Other specified malignant neoplasms of omentum
2C52.Z	Malignant neoplasms of omentum, unspecified
2C53	Malignant neoplasm involving overlapping sites of retroperitoneum, peritoneum or omentum
	<i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C53.0	Adenocarcinoma involving overlapping sites of retroperitoneum, peritoneum or omentum
2C53.1	Mesothelioma involving overlapping sites of retroperitoneum, peritoneum or omentum
2C53.Y	Other specified malignant neoplasm involving overlapping sites of retroperitoneum, peritoneum or omentum
2C53.Z	Malignant neoplasm involving overlapping sites of retroperitoneum, peritoneum or omentum, unspecified
2C5Y	Other specified malignant neoplasms of retroperitoneum, peritoneum or omentum
2C5Z	Malignant neoplasms of retroperitoneum, peritoneum or omentum, unspecified

Malignant neoplasms of breast (2C60-2C6Z)

The category refers to primary malignant neoplasms of parenchyma, connective, and soft tissue of the breast, including nipple and areola.

Inclusions: malignant neoplasm of connective tissue of breast

CA - [carcinoma] of breast

breast carcinoma NOS

Exclusions: Malignant neoplasm of skin of breast (2C30-2C3Z)

Malignant mesenchymal neoplasms (2B50-2B5Z)

2C60

Carcinoma of breast, specialised type

2C61

Invasive carcinoma of breast

2C61.0

Invasive ductal carcinoma of breast

2C61.1

Invasive lobular carcinoma of breast

An infiltrating lobular adenocarcinoma. The malignant cells lack cohesion and are arranged individually or in a linear manner (Indian files), or as narrow trabeculae within the stroma. The malignant cells are usually smaller than those of ductal carcinoma, are less pleomorphic, and have fewer mitotic figures.

2C61.2

Invasive pleomorphic lobular carcinoma of breast

A grade II invasive lobular carcinoma of the breast, characterised by the presence of neoplastic cells with large and atypical nuclei.

2C61.3

Invasive carcinoma of breast with mixed ductal and lobular features

An invasive ductal breast carcinoma associated with a lobular carcinomatous component. The lobular carcinomatous component may be in situ or invasive.

2C61.4

Invasive carcinoma of breast, unidentifiable type

A carcinoma that infiltrates the breast parenchyma and where the histopathological type could not be identified.

2C62

Inflammatory carcinoma of breast

An advanced, invasive breast adenocarcinoma characterised by the presence of distinct changes in the overlying skin. These changes include diffuse erythema, edema, peau d'orange (skin of an orange) appearance, tenderness, induration, warmth, enlargement, and in some cases a palpable mass. The skin changes are the consequence of lymphatic obstruction from the underlying invasive breast adenocarcinoma. Microscopically, the dermal lymphatics show prominent infiltration by malignant cells. The invasive breast adenocarcinoma is usually of ductal, NOS type. There is not significant inflammatory cell infiltrate present, despite the name of this carcinoma.

2C63**Malignant phyllodes tumour of breast**

A phyllodes tumour of the breast characterised by infiltrative margins and a sarcomatous stromal component. The sarcomatous stroma usually displays features of fibrosarcoma. Liposarcomatous, osteosarcomatous, or rhabdomyosarcomatous elements may also be present.

2C64**Solid papillary carcinoma of breast with evidence of invasion****2C65****Hereditary breast and ovarian cancer syndrome****2C6Y****Other specified malignant neoplasms of breast****2C6Z****Malignant neoplasms of breast, unspecified****Malignant neoplasms of female genital organs (2C70-2C7Z)**

A primary or metastatic malignant neoplasm involving the female reproductive system. Representative examples include endometrial carcinoma, cervical carcinoma, ovarian carcinoma, uterine corpus leiomyosarcoma, adenosarcoma, malignant mixed mesodermal (mullerian) tumour, and gestational choriocarcinoma.

Coding Note: Includes Malignant neoplasm of skin of female genital organs

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2C70**Malignant neoplasms of vulva**

Squamous cell carcinoma of the vulva occurs predominantly in the older age group. Although the incidence rate of vulvar intraepithelial neoplasia is increasing, that of squamous cell carcinoma of the vulva is declining, reflecting earlier detection and more successful treatment. In addition to human papillomavirus infection, cigarette smoking is a putative risk factor for vulvar squamous cell carcinoma. There are three known precursor lesions: vulvar intraepithelial neoplasia, lichen sclerosis and chronic granulomatous disease. Other important epithelial malignancies of the vulva are Paget disease and Bartholin gland carcinoma. They are much less

common than squamous lesions, and the risk factors are largely unknown. Prominent non-epithelial tumours are malignant melanoma and sarcoma botryoides.

Coding Note: Includes skin of vulva.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2C70.0**Basal cell carcinoma of vulva**

A slow growing, locally infiltrating carcinoma that arises from the vulva. It is characterised by the presence of malignant cells that resemble the basal cells that are present in the epidermis.

2C70.1**Melanoma of vulva**

2C70.2	Squamous cell carcinoma of vulva An invasive squamous cell carcinoma arising from the vulva. Risk factors include the human papilloma virus and cigarette smoking. Precursor lesions include the vulvar intraepithelial neoplasia, lichen sclerosus with associated squamous cell hyperplasia, and chronic granulomatous vulvar disease such as granuloma inguinale. Symptoms include vulvar pruritus or irritation, discharge, bleeding, and pain. The following morphologic variants have been identified: keratinizing, non-keratinizing, basaloid, warty, verrucous, keratoacanthoma-like, and squamous cell carcinoma with tumour giant cells. Risk factors for recurrence include advanced stage, tumour diameter greater than 2.5 cm, multifocality, capillary-like space involvement, associated vulvar intraepithelial neoplasia grades 2 or 3, and margins of resection involved by tumour. Coded Elsewhere: Verrucous squamous cell carcinoma of vulva (2C31.0)
2C70.Y	Other specified malignant neoplasms of vulva
Coding Note:	Includes skin of vulva.
2C70.Z	Malignant neoplasms of vulva, unspecified
Coding Note:	Includes skin of vulva.
2C71	Malignant neoplasms of vagina A primary or metastatic malignant tumour involving the vagina. Representative examples include carcinomas and sarcomas. Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)
2C71.0	Adenocarcinoma of vagina An adenocarcinoma arising from the vagina. Morphologic variants include the clear cell, endometrioid, mesonephric, and mucinous adenocarcinoma.
2C71.1	Melanoma of vagina A primary malignant neoplasm of the vagina composed of malignant melanocytes.
2C71.2	Squamous cell carcinoma of vagina A squamous cell carcinoma arising from the vagina. Human papillomavirus infection is associated with the development of vaginal intraepithelial neoplasia and invasive squamous cell carcinoma. Signs and symptoms include painless bleeding, postcoital bleeding, and urinary tract symptoms. Morphologically it resembles squamous cell carcinomas in other anatomic sites.
2C71.Y	Other specified malignant neoplasms of vagina
2C71.Z	Malignant neoplasms of vagina, unspecified
2C72	Malignant neoplasms of uterine ligament, parametrium, or uterine adnexa Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)
2C72.0	Adenocarcinoma of uterine ligament, parametrium, or uterine adnexa

- 2C72.1** **Mucinous or serous carcinoma of uterine ligament, parametrium, or uterine adnexa**
- 2C72.3** **Carcinosarcomas of uterine ligament, parametrium, or uterine adnexa**
- 2C72.Y** **Other specified malignant neoplasms of uterine ligament, parametrium, and uterine adnexa**
- 2C72.Z** **Malignant neoplasms of uterine ligament, parametrium, or uterine adnexa, unspecified**

2C73

Malignant neoplasms of ovary

A primary or metastatic malignant neoplasm involving the ovary. Most primary malignant ovarian neoplasms are either carcinomas (serous, mucinous, or endometrioid adenocarcinomas) or malignant germ cell tumours. Metastatic malignant neoplasms to the ovary include carcinomas, lymphomas, and melanomas.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

Coded Elsewhere: Hereditary breast and ovarian cancer syndrome (2C65)

2C73.0 **Carcinomas of ovary**

2C73.00 Clear cell adenocarcinoma of ovary

A malignant glandular epithelial tumour characterised by the presence of clear and hobnail cells. The tumour is highly associated with ovarian endometriosis, pelvic endometriosis and paraendocrine hypercalcemia.

2C73.01 Endometrioid adenocarcinoma of ovary

An endometrioid adenocarcinoma arising from the ovary. It comprises 10% to 25% of all primary ovarian carcinomas. Grossly, endometrioid carcinoma may present as a cystic or solid mass. Microscopically, the tumour greatly resembles the appearance of the ordinary type of endometrial adenocarcinoma. As a group, endometrioid carcinoma has a prognosis twice as good as that of serous or mucinous carcinoma.

2C73.02 Low grade serous adenocarcinoma of ovary

A slow-growing serous adenocarcinoma that arises from the ovary. It usually originates from borderline neoplastic processes or adenofibromas. It is characterised by the presence of low grade cytologic features and infrequent mitotic figures.

2C73.03 High grade serous adenocarcinoma of ovary

2C73.04 Mucinous adenocarcinoma of ovary

An invasive adenocarcinoma that arises from the ovary and is characterised by the presence of malignant epithelial cells that contain intracytoplasmic mucin. There is cellular atypia, increased layering of cells, complexity of glands, and papillary formations.

2C73.0Y Other specified carcinomas of ovary

2C73.0Z Carcinomas of ovary, unspecified

2C73.1	Dysgerminoma of ovary A malignant germ cell tumour arising from the ovary. Morphologically, it is identical to seminoma and consists of a monotonous population of germ cells with abundant pale cytoplasm and uniform nuclei. The stroma invariably contains chronic inflammatory cells, mostly T-lymphocytes. It responds to chemotherapy or radiotherapy and the prognosis relates to the tumour stage. <i>Inclusions:</i> Malignant dysgerminomatous germ cell tumour of ovary
2C73.2	Granulosa cell malignant tumour of ovary An aggressive granulosa cell tumour that arises from the ovary and metastasizes to other anatomic sites.
2C73.3	Malignant teratoma of ovary A malignant germ cell tumour arising from the ovary. It usually affects females in their first two decades of life. It contains variable amounts of immature embryonal tissues. Based on the amount of immature neuroepithelial component, immature teratomas are graded from 1 to 3. The stage and grade of the tumour and the grade of the metastatic tumour are the important factors that predict prognosis. The use of cisplatin-based combination chemotherapy has significantly improved the survival rates of the patients. <i>Coded Elsewhere:</i> Struma ovarii (5A02.Y)
2C73.4	Serous cystadenoma, borderline malignancy of ovary
2C73.5	Endodermal sinus tumour, unspecified site, female
2C73.Y	Other specified malignant neoplasms of the ovary
2C73.Z	Malignant neoplasms of ovary, unspecified
2C74	Malignant neoplasms of fallopian tube Tumours of the fallopian tube are much less common than the corresponding ovarian neoplasms; however, histologically the same surface epithelial-stromal tumour subtypes are recognised. Sex cord-stromal and germ cell tumours are rare. Hydatidiform moles and gestational choriocarcinoma are uncommon complications of tubal ectopic pregnancy. The wolffian adnexal tumour is also infrequent and typically occurs in the leaves of the broad ligament. The risk factors appear similar to those of the ovary. Fallopian tube carcinomas are a component of the hereditary breast-ovarian cancer syndrome caused by BRCA1 and BRCA2 germline mutations. <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C74.0	Adenocarcinoma of fallopian tube An adenocarcinoma that arises from the fallopian tube. Histologic subtypes include clear cell, endometrioid, serous, and mucinous adenocarcinoma. It spreads to adjacent organs, regional lymph nodes, and peritoneum.
2C74.Y	Other specified malignant neoplasms of fallopian tube
2C74.Z	Malignant neoplasms of fallopian tube, unspecified

2C75	Malignant neoplasms of placenta <i>Exclusions:</i> hydatidiform mole, NOS (JA02) Malignant mesenchymal neoplasms (2B50-2B5Z)
2C75.0	Malignant trophoblastic neoplasms of placenta A diverse group of pregnancy-related tumours characterised by excessive proliferation of trophoblasts. Representative examples include hydatidiform mole, gestational choriocarcinoma, and placental site trophoblastic tumour.
2C75.Y	Other specified malignant neoplasms of placenta
2C75.Z	Malignant neoplasms of placenta, unspecified
2C76	Malignant neoplasms of corpus uteri A malignant neoplasm that affects the uterine corpus. Representative examples include endometrial carcinoma, carcinosarcoma, leiomyosarcoma, and adenosarcoma. <i>Exclusions:</i> Endometrial stromal sarcoma, primary site (2B5C)
2C76.0	Endometrial endometrioid adenocarcinoma
2C76.1	Endometrial mucinous adenocarcinoma
2C76.2	Endometrial clear cell adenocarcinoma
2C76.3	Endometrial serous adenocarcinoma
2C76.4	Endometrial mixed adenocarcinoma
2C76.40	Endometrial squamous cell carcinoma
2C76.41	Endometrial small cell carcinoma
2C76.42	Endometrial undifferentiated carcinoma
2C76.43	Carcinosarcoma of uterus
2C76.Y	Other specified malignant neoplasms of corpus uteri
2C76.Z	Malignant neoplasms of corpus uteri, unspecified
2C77	Malignant neoplasms of cervix uteri Primary or metastatic malignant neoplasm involving the cervix.
2C77.0	Squamous cell carcinoma of cervix uteri A squamous cell carcinoma arising from the cervical epithelium. It usually evolves from a precancerous cervical lesion. Increased numbers of sexual partners and human papillomavirus (HPV) infection are risk factors for cervical squamous cell carcinoma. The following histologic patterns have been described: Conventional squamous cell carcinoma, papillary squamous cell carcinoma, transitional cell carcinoma, lymphoepithelioma-like carcinoma, verrucous carcinoma, condylomatous carcinoma and spindle cell carcinoma. Survival is most closely related to the stage of disease at the time of diagnosis.

2C77.1	Adenocarcinoma of cervix uteri
	An adenocarcinoma arising from the cervical epithelium. It accounts for approximately 15% of invasive cervical carcinomas. Increased numbers of sexual partners and human papillomavirus (HPV) infection are risk factors. Grossly, advanced cervical adenocarcinoma may present as an exophytic mass, an ulcerated lesion, or diffuse cervical enlargement. Microscopically, the majority of cervical adenocarcinomas are of the endocervical (mucinous) type.
2C77.2	Adenosquamous carcinoma of cervix uteri
2C77.3	Neuroendocrine carcinoma of cervix uteri
2C77.Y	Other specified malignant neoplasms of cervix uteri
2C77.Z	Malignant neoplasms of cervix uteri, unspecified
2C78	Malignant neoplasms of uterus, part not specified
2C79	Malignant neoplasm involving overlapping sites of female genital organs
	Inclusions: Malignant neoplasm of female genital organs whose point of origin cannot be classified to any other existing entity
2C7Y	Other specified malignant neoplasms of female genital organs
Coding Note:	Includes Malignant neoplasm of skin of female genital organs
2C7Z	Malignant neoplasms of female genital organs, unspecified
Coding Note:	Includes Malignant neoplasm of skin of female genital organs

Malignant neoplasms of male genital organs (2C80-2C8Z)

A primary or metastatic malignant neoplasm involving the male reproductive system. Representative examples include prostate carcinoma, penile carcinoma, testicular seminoma, and testicular embryonal carcinoma.

Coding Note: Includes malignant neoplasm of skin of male genital organs

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2C80	Malignant neoplasms of testis
	A primary or metastatic malignant neoplasm that affects the testis. Representative examples include seminoma, embryonal carcinoma, sarcoma, leukaemia, and lymphoma.
	Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)
2C80.2	Germ cell tumour of testis
	A germ cell tumour arising from the testis. Representative examples include teratoma, seminoma, embryonal carcinoma, and yolk sac tumour.
2C80.Y	Other specified malignant neoplasms of testis
2C80.Z	Malignant neoplasms of testis, unspecified

2C81**Malignant neoplasms of penis**

A primary or metastatic malignant neoplasm that affects the penis. Representative examples include penile carcinoma and penile sarcoma.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

Coded Elsewhere: Verrucous squamous cell carcinoma of penis (2C31.0)

2C81.0**Squamous cell carcinoma of penis**

A squamous cell carcinoma arising from the penis. It occurs chiefly in the squamous epithelium of the glans, coronal sulcus, and foreskin. Etiologic factors include phimosis, lichen sclerosus, smoking, ultraviolet irradiation, history of warts or condylomas, and lack of circumcision. Human papilloma virus is present in a subset of penile squamous cell carcinomas. Patients may present with an exophytic or flat ulcerative mass in the glans or a large primary tumour with inguinal nodal and skin metastases. Morphologic variants include the basaloid carcinoma, warty (condylomatous) carcinoma, verrucous carcinoma, and sarcomatoid (spindle cell) carcinoma.

Coded Elsewhere: Verrucous squamous cell carcinoma of penis (2C31.0)

2C81.1**Melanoma of penis**

Inclusions: Melanoma of skin of penis

Melanoma of mucocutaneous epithelium of penis

2C81.Y**Other specified malignant neoplasm of penis****2C81.Z****Malignant neoplasms of penis, unspecified****2C82****Malignant neoplasms of prostate**

Prostate cancer contributes significantly to the overall cancer burden, being the most frequent malignant neoplasia in men. The number of cases has continuously increased over the past decades, partly due to the higher life expectancy. An additional factor is the Western lifestyle, characterised by a highly caloric diet and lack of physical exercise. Epidemiological data indicates that black people are most susceptible, followed by white people, while Asian people have the lowest risk. The extent to which prostate cancer mortality can be reduced by PSA screening, is currently being evaluated. Histopathological diagnosis and grading play a major role in the management of prostate cancer

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2C82.0**Adenocarcinoma of prostate**

An adenocarcinoma arising from the prostate gland. It is one of the most common malignant tumours afflicting men. The majority of adenocarcinomas arise in the peripheral zone and a minority occurs in the central or the transitional zone of the prostate gland. Grading of prostatic adenocarcinoma predicts disease progression and correlates with survival. Several grading systems have been proposed, of which the Gleason system is the most commonly used. Gleason sums of 2 to 4 represent well-differentiated disease, 5 to 7 moderately differentiated disease and 8 to 10 poorly differentiated disease. Prostatic-specific antigen (PSA) serum test is widely used as a screening test for the early detection of prostatic adenocarcinoma.

2C82.Y	Other specified malignant neoplasms of prostate
2C82.Z	Malignant neoplasms of prostate, unspecified
2C83	Malignant neoplasms of scrotum
	<i>Inclusions:</i> malignant neoplasm of skin of scrotum
	<i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C83.0	Squamous cell carcinoma of scrotum
2C83.Y	Other specified malignant neoplasms of scrotum
2C83.Z	Malignant neoplasms of scrotum, unspecified
2C84	Malignant neoplasms of other specified male genital organs
	<i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C8Z	Malignant neoplasms of male genital organs, unspecified
Coding Note:	Includes malignant neoplasm of skin of male genital organs

Malignant neoplasms of urinary tract (2C90-2C9Z)

A primary or metastatic malignant tumour involving the urinary system. Common tumour types include carcinomas, lymphomas, and sarcomas.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2C90	Malignant neoplasms of kidney, except renal pelvis
	Cancer of the kidney amounts to 2% of the total human cancer burden, with approximately 190,000 new cases diagnosed each year. They occur in all world regions, with a preference for developed countries. Etiological factors include environmental carcinogens (tobacco smoking) and lifestyle factors, in particular obesity. Although renal tumours can be completely removed surgically, haematogeneous metastasis is frequent and may occur already at an early stage of the disease. The pattern of somatic mutations in kidney tumours has been extensively investigated and has become, in addition to histopathology, a major criterion for classification. Kidney tumours also occur in the setting of several inherited cancer syndromes, including von Hippel-Lindau disease.
	Exclusions: Malignant neoplasm of renal calyces (2C91)
	Malignant neoplasms of renal pelvis (2C91)
	Malignant mesenchymal neoplasms (2B50-2B5Z)

2C90.0	Renal cell carcinoma of kidney, except renal pelvis A carcinoma arising from the renal parenchyma. The incidence of renal cell carcinoma has increased by 35% from 1973 to 1991. There is a strong correlation between cigarette smoking and the development of renal cell carcinoma. The clinical presentation includes : haematuria, flank pain and a palpable lumbar mass. A high percentage of renal cell carcinomas are diagnosed when an ultrasound is performed for other purposes. Diagnostic procedures include: ultra sound, intravenous pyelography and computed tomography (CT).
2C90.Y	Other specified malignant neoplasms of kidney, except renal pelvis
2C90.Z	Malignant neoplasms of kidney, except renal pelvis, unspecified
2C91	Malignant neoplasms of renal pelvis Abnormal malignant growth of the cells within the renal pelvis. <i>Inclusions:</i> malignant neoplasm of pelviureteric junction malignant neoplasm of renal calyces <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C91.0	Urothelial carcinoma of renal pelvis
2C91.Y	Other specified malignant neoplasms of renal pelvis
2C91.Z	Malignant neoplasms of renal pelvis, unspecified
2C92	Malignant neoplasms of ureter A primary or metastatic malignant tumour involving the ureter. The majority are carcinomas. <i>Exclusions:</i> malignant neoplasm of ureteric orifice of bladder (2C94) Malignant mesenchymal neoplasms (2B50-2B5Z)
2C92.0	Urothelial carcinoma of ureter
2C92.Y	Other specified malignant neoplasms of ureter
2C92.Z	Malignant neoplasms of ureter, unspecified
2C93	Malignant neoplasms of urethra or paraurethral gland <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C93.0	Adenocarcinoma of urethra or paraurethral gland
2C93.1	Squamous cell carcinoma of urethra or paraurethral gland
2C93.2	Urothelial carcinoma of urethra or paraurethral gland
2C93.Y	Other specified malignant neoplasms of urethra and paraurethral gland
2C93.Z	Malignant neoplasms of urethra or paraurethral gland, unspecified

2C94	Malignant neoplasms of bladder A primary or metastatic malignant neoplasm involving the bladder. <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C94.0	Adenocarcinoma of bladder A rare adenocarcinoma arising from metaplastic bladder epithelium. It is frequently associated with long-standing local irritation. The majority of cases originate from the trigone and the posterior wall of the bladder.
2C94.1	Squamous cell carcinoma of bladder A squamous cell carcinoma of the bladder arising from metaplastic epithelium. It represents less than 10% of bladder carcinomas. The exception is the Middle East along the Nile Valley, where it represents the most common form of carcinoma because of the endemic nature of schistosomiasis. Bladder squamous cell carcinoma is often associated with long-standing chronic inflammation of the bladder and usually has a poor prognosis. The diagnosis of squamous cell carcinoma of the bladder should be reserved for those tumours that are predominantly keratin forming.
2C94.2	Urothelial carcinoma of bladder
2C94.Y	Other specified malignant neoplasms of bladder
2C94.Z	Malignant neoplasms of bladder, unspecified
2C95	Malignant neoplasm involving overlapping sites of urinary organs Malignant neoplasm of urinary organs whose point of origin cannot be classified to any other existing category <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2C95.0	Adenocarcinoma involving overlapping sites of urinary organs
2C95.1	Squamous cell carcinomas involving overlapping sites of urinary organs
2C95.2	Urothelial carcinoma involving overlapping sites of urinary organs
2C95.Y	Other specified malignant neoplasms of overlapping lesion of urinary organs
2C95.Z	Malignant neoplasm involving overlapping sites of urinary organs, unspecified
2C9Y	Other specified malignant neoplasms of urinary tract
2C9Z	Malignant neoplasms of urinary tract, unspecified

Malignant neoplasms of eye or ocular adnexa (2D00-2D0Z)

A malignant neoplasm affecting the structures of the eye.

Exclusions: Malignant neoplasm of optic nerve (2A02)
Malignant neoplasm of skin of eyelid (2C30-2C3Z)
Malignant mesenchymal neoplasms (2B50-2B5Z)

Coded Elsewhere: Melanoma of uvea (2D0Y)

2D00

Malignant neoplasm of conjunctiva

A malignant growth of cells within the conjunctiva of the eye.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2D00.0

Melanoma of conjunctiva

A malignant melanoma within the conjunctiva of the eye.

2D00.1

Malignant neoplasm of caruncle

This is a broad group of diseases involving unregulated cell growth of a small, red portion of the corner of the eye that contains modified sebaceous and sweat glands.

2D00.2

Squamous cell carcinoma of conjunctiva

2D00.Y

Other specified malignant neoplasm of conjunctiva

2D00.Z

Malignant neoplasm of conjunctiva, unspecified

2D01

Malignant neoplasm of cornea

A malignant growth of cells within the cornea of the eye.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2D01.0

Melanoma of cornea

A melanoma within the cornea of the eye.

2D01.1

Squamous cell carcinoma of cornea

2D01.Y

Other specified malignant neoplasms of cornea

2D01.Z

Malignant neoplasm of cornea, unspecified

2D02

Malignant neoplasm of retina

Abnormal growth of cells comprising the retina with malignant characteristics.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2D02.0	Adenocarcinoma of retinal pigment epithelium This is a cancer of an epithelium that originates in glandular tissue. Epithelial tissue includes, but is not limited to, the surface layer of skin, glands, and a variety of other tissue that lines the cavities and organs of the body. This diagnosis is with the pigmented cell layer just outside the neurosensory retina that nourishes retinal visual cells, and is firmly attached to the underlying choroid and overlying retinal visual cells.
2D02.1	Malignant neuroepithelial tumours of retina
2D02.2	Retinoblastoma Retinoblastoma is the most common intraocular malignancy in children. It is a life threatening condition but is potentially curable. It can be hereditary or non-hereditary, unilateral or bilateral (unilateral retinoblastoma, bilateral retinoblastoma, see these terms).
2D02.Y	Other specified malignant neoplasm of retina
2D02.Z	Malignant neoplasm of retina, unspecified
2D03	Malignant neoplasm of lacrimal apparatus <i>Exclusions:</i> Malignant mesenchymal neoplasms (2B50-2B5Z)
2D03.0	Adenocarcinoma of the lacrimal apparatus
2D03.1	Mucoepidermoid carcinoma of lacrimal apparatus
2D03.2	Squamous cell carcinoma of the lacrimal apparatus
2D03.Y	Other specified malignant neoplasm of lacrimal apparatus
2D03.Z	Malignant neoplasm of lacrimal apparatus, unspecified
2D04	Malignant neoplasm of orbit A primary or metastatic malignant neoplasm involving the orbit. <i>Exclusions:</i> Benign neoplasm of orbital bone (2E83.0) malignant neoplasm of orbital bone (2B50-2B5Z)
2D05	Malignant neoplasm of choroid
2D05.0	Melanoma of choroid
2D05.Y	Other specified malignant neoplasm of choroid
2D05.Z	Malignant neoplasm of choroid, unspecified
2D06	Malignant neoplasm of ciliary body <i>Inclusions:</i> Malignant neoplasm of eyeball
2D06.0	Adenocarcinoma of ciliary epithelium
2D06.1	Malignant medulloepithelioma of ciliary body

2D06.3	Malignant neuroepithelial tumours of ciliary body
2D06.4	Melanoma of ciliary body
2D06.Y	Other specified malignant neoplasm of ciliary body
2D06.Z	Malignant neoplasm of ciliary body, unspecified
2D07	Malignant neoplasm of iris
2D07.0	Adenocarcinoma of iris epithelium
2D07.1	Malignant neuroepithelial tumours of iris
2D07.2	Melanoma of iris
2D07.Y	Other specified malignant neoplasm of iris
2D07.Z	Malignant neoplasm of iris, unspecified
2D0Y	Other specified malignant neoplasms of eye and ocular adnexa
2D0Z	Malignant neoplasms of eye or ocular adnexa, unspecified

Malignant neoplasms of endocrine glands (2D10-2D1Z)

A malignant neoplasm affecting the endocrine glands. Representative examples include thyroid gland carcinoma, parathyroid gland carcinoma, pituitary gland carcinoma, and adrenal cortex carcinoma.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2D10 Malignant neoplasms of thyroid gland

A primary or metastatic malignant neoplasm affecting the thyroid gland.

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

Coded Elsewhere: Thyroid lymphoma (2B33.5)

2D10.0 Follicular carcinoma of thyroid gland

A differentiated adenocarcinoma arising from the follicular cells of the thyroid gland. The nuclear features which characterise the thyroid gland papillary carcinoma are absent. Radiation exposure is a risk factor and it comprises approximately 10% to 15% of thyroid cancers. Clinically, it usually presents as a solitary mass in the thyroid gland. It is generally unifocal and thickly encapsulated and shows invasion of the capsule or the vessels. Diagnostic procedures include: thyroid ultrasound and fine needle biopsy.

2D10.1	Papillary carcinoma of thyroid gland A differentiated adenocarcinoma arising from the follicular cells of the thyroid gland. Radiation exposure is a risk factor and it is the most common malignant thyroid lesion, comprising 75% to 80% of all thyroid cancers in iodine sufficient countries. Diagnostic procedures include: thyroid ultrasound, and fine needle biopsy. Microscopically, the diagnosis is based on the distinct characteristics of the malignant cells, which include enlargement, oval shape, elongation, and overlapping of the nuclei. The nuclei also display clearing or have a ground glass appearance.
2D10.2	Poorly differentiated carcinoma of thyroid gland
2D10.3	Undifferentiated carcinoma of thyroid gland A primary carcinoma of the thyroid gland composed of undifferentiated cells. The malignant cells demonstrate evidence of epithelial differentiation, either by immunohistochemistry or electron microscopic studies. Microscopically, in the majority of cases there is a mixture of spindle, epithelioid, and giant cells. The vast majority of the patients present with a rapidly enlarging neck mass. The clinical course is invariably aggressive. Inclusions: anaplastic carcinoma of thyroid gland
2D10.4	Medullary carcinoma of thyroid gland A neuroendocrine carcinoma arising from the C-cells of the thyroid gland. It is closely associated with multiple endocrine neoplasia syndromes. Approximately 10% to 20% of medullary thyroid carcinomas are familial. Patients usually present with a thyroid nodule that is painless and firm. In the majority of cases nodal involvement is present at diagnosis.
2D10.Y	Other specified malignant neoplasms of thyroid gland
2D10.Z	Malignant neoplasms of thyroid gland, unspecified
2D11	Malignant neoplasms of adrenal gland Tumours arising from the adrenal cortex include adenomas and carcinomas. These are rare neoplasms but may cause a variety of hormonal symptoms, including hyperaldosteronism, Cushing syndrome, and virilisation. A small fraction of adrenocortical tumours are associated with an inherited tumour syndrome, including Li-Fraumeni syndrome and Carney complex. Benign and malignant pheochromocytomas arise in the adrenal medulla and are derived from chromaffin cells of neural crest origin. Pheochromocytomas may occur in the setting of several hereditary conditions, including multiple endocrine neoplasia types 2a and 2b, von Hippel Lindau disease and neurofibromatosis. Extra adrenal paragangliomas arise from chromaffin cells in sympathoadrenal and parasympathetic paraganglia. They occur in many parts of the body and can pose a significant challenge to surgeons and oncologists. Some function as chemoreceptors, others are endocrinologically active. Familial paragangliomas are associated with mutations of the mitochondrial complex II genes. Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)
2D11.0	Adenocarcinoma of adrenal gland

2D11.1	Malignant phaeochromocytoma of adrenal gland
2D11.2	Neuroblastoma of adrenal gland Neuroblastomas are malignant tumours that form in certain types of the nerve tissue. It most often begins in the adrenal gland. About 1 out of 3 neuroblastomas start in the adrenal glands and about 1 out of 4 begin in sympathetic nerve ganglia in the abdomen. Most of the rest start in sympathetic ganglia near the spine in the chest or neck, or in the pelvis.
2D11.Y	Other specified malignant neoplasms of adrenal gland
2D11.Z	Malignant neoplasms of adrenal gland, unspecified
2D12	Malignant neoplasms of other endocrine glands or related structures <i>Exclusions:</i> - Malignant neoplasms of adrenal gland (2D11) - Malignant neoplasms of testis (2C80) - Malignant neoplasms of ovary (2C73) - Malignant neoplasm of pancreas (2C10) - Malignant neoplasms of thyroid gland (2D10) - Malignant neoplasms of thymus (2C27) - Malignant mesenchymal neoplasms (2B50-2B5Z)
2D12.0	Malignant epithelial neoplasms of other endocrine glands or related structures, unspecified type
2D12.1	Adenocarcinoma of other endocrine glands or related structures
2D12.Y	Other specified malignant neoplasms of other endocrine glands or related structures
2D12.Z	Malignant neoplasms of other endocrine glands or related structures, unspecified
2D1Z	Malignant neoplasms of endocrine glands, unspecified
2D3Y	Other specified malignant neoplasms, stated or presumed to be primary, of specified sites, except of lymphoid, haematopoietic, central nervous system or related tissues
2D3Z	Malignant neoplasms, stated or presumed to be primary, of specified sites, except of lymphoid, haematopoietic, central nervous system or related tissues, unspecified

Malignant neoplasms of ill-defined or unspecified primary sites (2D40-2D4Z)

Inclusions: metastatic cancer, unknown primary
metastatic cancer, unknown origin
metastatic cancer
metastatic disease

Exclusions: Malignant mesenchymal neoplasms (2B50-2B5Z)

2D40 Adenocarcinoma of unspecified site

A common cancer characterised by the presence of malignant glandular cells. Morphologically, adenocarcinomas are classified according to the growth pattern (e.g., papillary, alveolar) or according to the secreting product (e.g., mucinous, serous). Representative examples of adenocarcinoma are ductal and lobular breast carcinoma, lung adenocarcinoma, renal cell carcinoma, hepatocellular carcinoma (hepatoma), colon adenocarcinoma, and prostate adenocarcinoma.

2D41 Unspecified carcinoma of unspecified site

2D42 Malignant neoplasms of ill-defined sites

Malignant neoplasms of ill defined sites is used for cases where the documentation refers to a site that includes multiple organ systems and tissue types that should be coded separately.

2D43 Malignant neoplasms of independent, multiple primary sites

Coding Note: Use additional codes to identify individual neoplasms.

2D4Y Other specified malignant neoplasms of unspecified primary sites

2D4Z Unspecified malignant neoplasms of unspecified sites

Malignant neoplasm metastases (2D50-2E2Z)

Spread of a malignant neoplasm into secondary sites.

2D50 Malignant neoplasm metastasis in brain

A malignant neoplasm that has spread to the brain from another anatomic site or system. The majority are carcinomas (usually lung or breast carcinomas).

2D51 Malignant neoplasm metastasis in meninges

2D52 Malignant neoplasm metastasis in spinal cord, cranial nerves or remaining parts of central nervous system

Malignant neoplasm metastasis in lymph nodes (2D60-2D6Z)

Exclusions: Neoplasms of haematopoietic or lymphoid tissues (2A20-2B3Z)

2D60 Malignant neoplasm metastasis in lymph node of a single region

2D60.0 Malignant neoplasm metastasis in lymph nodes of head, face or neck

2D60.1	Malignant neoplasm metastasis in intrathoracic lymph nodes
2D60.2	Malignant neoplasm metastasis in intra-abdominal lymph nodes
2D60.3	Malignant neoplasm metastasis in axillary lymph nodes
2D60.4	Malignant neoplasm metastasis in inguinal lymph nodes
2D60.5	Malignant neoplasm metastasis in intrapelvic lymph nodes
2D60.Y	Other specified malignant neoplasm metastasis in lymph node of a single region
2D60.Z	Malignant neoplasm metastasis in lymph node of a single region, unspecified
2D61	Malignant neoplasm metastases in lymph nodes of multiple regions
2D6Z	Metastatic malignant neoplasm to unspecified lymph node

Malignant neoplasm metastasis in thoracic or respiratory organs (2D70-2D7Z)

2D70	Malignant neoplasm metastasis in lung <i>Exclusions:</i> Malignant neoplasms of bronchus or lung (2C25)
2D71	Malignant neoplasm metastasis in mediastinum The spread of cancer to the mediastinum from an adjacent or distant anatomic site.
2D72	Malignant neoplasm metastasis in pleura The spread of cancer to the pleura from an adjacent or distant anatomic site.
2D73	Malignant neoplasm metastasis in upper respiratory tract organs
2D7Y	Malignant neoplasm metastasis in other specified thoracic organs
2D7Z	Malignant neoplasm metastasis in thoracic or respiratory organs, unspecified

Malignant neoplasm metastasis in digestive system (2D80-2D8Z)

2D80	Malignant neoplasm metastasis in liver or intrahepatic bile duct Malignant neoplasms that have metastasized to the liver from extrahepatic primary tumours.
2D80.0	Malignant neoplasm metastasis in liver
2D80.1	Malignant neoplasm metastasis in intrahepatic bile duct
2D81	Malignant neoplasm metastasis in pancreas A malignant neoplasm that has spread to the pancreas from another anatomic site. Representative examples include metastatic carcinomas from the gastrointestinal tract, metastatic melanomas, and renal cell carcinomas.

2D82	Malignant neoplasm metastasis in extrahepatic bile ducts
2D83	Malignant neoplasm metastasis in ampulla of Vater
2D84	Malignant neoplasm metastasis in the small intestine The spread of cancer to the small intestine. This may be from a primary intestinal cancer, or from a cancer at a distant site.
2D85	Malignant neoplasm metastasis in large intestine The spread of cancer to the large intestine. This may be from a primary colon or rectal cancer, or from a cancer at a distant site.
2D86	Malignant neoplasm metastasis in anus Malignant tumour that metastasized in the anus and anal canal.
2D8Y	Malignant neoplasm metastasis in other specified digestive system organ
2D8Z	Malignant neoplasm metastasis in unspecified digestive system organ
Malignant neoplasm metastasis in retroperitoneum or peritoneum (2D90-2D91)	
2D90	Malignant neoplasm metastasis in retroperitoneum
2D91	Malignant neoplasm metastasis in peritoneum
Malignant neoplasm metastasis in other sites (2E00-2E0Y)	
2E00	Malignant neoplasm metastasis in kidney or renal pelvis The spread of the cancer to the kidney. This may be from a primary kidney cancer involving the opposite kidney, or from a cancer at a distant site.
2E01	Malignant neoplasm metastasis in bladder Tumours of the urinary bladder that originate from an extravesical, non-urothelial tract neoplasm
2E02	Malignant neoplasm metastasis in other or unspecified urinary system organs
2E03	Malignant neoplasm metastasis in bone or bone marrow The spread of a malignant neoplasm from a primary site to the skeletal system. The majority of metastatic neoplasms to the bone are carcinomas.
2E04	Malignant neoplasm metastasis in soft tissue
2E05	Malignant neoplasm metastasis in female reproductive system
2E05.0	Malignant neoplasm metastasis in ovary The spread of the cancer to the ovary. This may be from a primary ovarian cancer involving the opposite ovary, or from a cancer at a distant site.

2E05.Y	Malignant neoplasm metastasis in other female reproductive system organs
2E05.Z	Malignant neoplasm metastasis in female reproductive system, unspecified
2E06	Malignant neoplasm metastasis in male genital organs
2E07	Malignant neoplasm metastasis in adrenal gland A malignant tumour that has spread to the adrenal gland from an adjacent or distant anatomic site. The majority of cases are metastatic carcinomas, and less frequently lymphomas. (NCI05)
2E08	Metastatic malignant neoplasm involving skin Involvement of the skin by metastatic spread from a known or unknown primary malignant neoplasm. The secondary deposit may result from local migration of malignant cells, or from regional lymphatic or haematogenous spread from more distant sites.
2E09	Malignant neoplasm metastasis in peripheral nervous system
2E0Y	Malignant neoplasm metastasis in other specified sites
2E2Z	Malignant neoplasm metastasis, unspecified

In situ neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues (2E60-2E6Z)

2E60	Carcinoma in situ of oral cavity, oesophagus or stomach <i>Exclusions:</i> Melanoma in situ neoplasms (2E63)
2E60.0	Carcinoma in situ of lip, oral cavity or pharynx <i>Exclusions:</i> Carcinoma in situ of aryepiglottic fold, laryngeal aspect (2E62.0) Carcinoma in situ of epiglottis nos (2E62.0) Carcinoma in situ of epiglottis, suprahyoid portion (2E62.0) Carcinoma in situ of skin of lip (2E64)
2E60.1	Carcinoma in situ of oesophagus Stage 0 includes: For squamous cell carcinoma: Tis (HGD), N0, M0, G1, GX, tumour location: Any. For adenocarcinoma: Tis (HGD), N0, M0, G1, GX. Tis: High-grade dysplasia. N0: No regional lymph node metastasis. M0: No distant metastasis. G1: Well differentiated. GX: Grade cannot be assessed-stage grouping as G1. tumour location: Location of the primary cancer site is defined by the position of the upper (proximal) edge of the tumour in the esophagus. Coded Elsewhere: Oesophageal adenocarcinoma in situ (2E60.1)
2E60.2	Carcinoma in situ of stomach Stage 0 includes: Tis, N0, M0. Tis: Carcinoma in situ: intraepithelial tumour without invasion of the lamina propria. N0: No regional lymph node metastasis. M0: No distant metastasis.

2E61	Carcinoma in situ of other or unspecified digestive organs <i>Exclusions:</i> Melanoma in situ neoplasms (2E63)
2E61.0	Carcinoma in situ of colon Stage 0 includes: Tis, N0, M0. Tis: Carcinoma in situ: intraepithelial or invasion of lamina propria. N0: No regional lymph node metastasis. M0: No distant metastasis.
2E61.1	Carcinoma in situ of rectum Malignant epithelial tumour of rectum that has not invaded adjacent tissue of the large intestine.
2E61.2	Carcinoma in situ of anal canal Malignant epithelial tumour that has not invaded beyond the epithelium of the anal canal. <i>Exclusions:</i> Carcinoma in situ of anal margin (2E64.2) Carcinoma in situ of anal skin (2E64) Carcinoma in situ of perianal skin (2E64.2)
2E61.3	Carcinoma in situ of gallbladder, biliary tract or ampulla of Vater an early form of cancer without invasion of tumour cells into the surrounding tissue, usually before penetration through the basement membrane.
2E61.Y	Carcinoma in situ of other specified digestive organs
2E61.Z	Carcinoma in situ of unspecified digestive organs
2E62	Carcinoma in situ of middle ear or respiratory system <i>Exclusions:</i> Melanoma in situ neoplasms (2E63)
2E62.0	Carcinoma in situ of larynx <i>Exclusions:</i> Carcinoma in situ of aryepiglottic fold, hypopharyngeal aspect (2E60.0) Carcinoma in situ of aryepiglottic fold, marginal zone (2E60.0) Carcinoma in situ of aryepiglottic fold, NOS (2E60.0)
2E62.1	Carcinoma in situ of trachea
2E62.2	Carcinoma in situ of bronchus or lung
2E62.Y	Carcinoma in situ of other specified sites of middle ear and respiratory system
2E62.Z	Carcinoma in situ of unspecified sites of middle ear and respiratory system
2E63	Melanoma in situ neoplasms Stage 0 includes: Tis, N0, M0. Tis: Melanoma in situ. N0: No regional lymph node metastases. M0: No detectable evidence of distant metastases.

2E63.0	Melanoma in situ of skin Malignant melanoma confined to the epidermis and described as being in radial growth phase.
2E63.00	Lentigo maligna An atypical proliferation of atypical melanocytes in the dermal-epidermal junction, without infiltration of the papillary or reticular dermis. The melanocytic proliferation is associated with actinic damage and epidermal atrophy. It usually occurs in the sun-exposed skin of elderly people. It is a form of melanoma in situ and in approximately 5% of cases it progresses to lentigo maligna melanoma.
2E63.0Z	Melanoma in situ of skin, unspecified
2E63.1	Melanoma in situ of conjunctiva
2E63.Y	Melanoma in situ neoplasms, other specified site
2E63.Z	Melanoma in situ neoplasms, unspecified site
2E64	Carcinoma in situ of skin Stage 0 includes: Tis, N0, M0. Tis: Carcinoma in situ. N0: No regional lymph node metastasis. M0: No clinical or radiographic evidence of distant metastasis. Exclusions: Melanoma in situ neoplasms (2E63) Coded Elsewhere: Carcinoma in situ of vulva (2E67.1) Carcinoma in situ of penis (2E67.4)
2E64.0	Intraepidermal squamous cell carcinoma Malignant squamous neoplasia confined to the epidermis of extragenital skin and known commonly as Bowen disease. It arises most frequently on chronically sun-exposed glabrous skin of the head and neck or lower legs. It typically presents as single or multiple well-demarcated scaly erythematous patches, nodules or plaques which histologically show extensive keratinocytic atypia. It may develop from preexisting actinic keratosis (Actinic intraepidermal squamous cell carcinoma). Although it is most commonly associated with exposure to ultraviolet radiation, other carcinogens such as arsenic and tar may be implicated. Human papilloma virus may represent an additional risk factor in immunosuppressed patients.
2E64.00	Bowen disease of skin Intraepidermal squamous cell carcinoma due to predisposing factors including chronic human papilloma virus infection, arsenic ingestion, ionising radiation and chronic immunosuppression.
2E64.01	Actinic intraepidermal squamous cell carcinoma Intraepidermal squamous cell carcinoma attributable to chronic exposure to ultraviolet radiation and typically developing from a pre-existing actinic keratosis
2E64.0Y	Other specified intraepidermal squamous cell carcinoma
2E64.0Z	Intraepidermal squamous cell carcinoma, unspecified

2E64.1	Extramammary Paget disease of skin An intraepithelial adenocarcinoma of apocrine gland-bearing skin and mucous membrane. Clinically it presents as sharply demarcated erythematous plaques most commonly affecting the vulva in women and perianal skin in men. Coded Elsewhere: Vulvar Paget disease (2E67.11)
2E64.2	Carcinoma in situ of anal margin or perianal skin Carcinoma in situ of anal margin or perianal skin is most commonly squamous and related to oncogenic HPV strains, HIV infection or both. It may present as warty pigmented patches (Bowenoid papulosis).
2E64.Y	Other specified carcinoma in situ of skin
2E64.Z	Carcinoma in situ of skin, unspecified
2E65	Carcinoma in situ of breast Exclusions: carcinoma in situ of skin of breast (2E64) melanoma in situ of breast (skin) (2E63)
2E65.0	Lobular carcinoma in situ of breast
2E65.1	Lobular carcinoma in situ of breast, pleomorphic subtype
2E65.2	Ductal carcinoma in situ of breast Exclusions: Atypical ductal hyperplasia of breast (2F75)
2E65.3	Ductal carcinoma in situ of breast, comedo subtype
2E65.4	Mixed ductal and lobular carcinoma in situ of breast The co-existence of ductal and lobular carcinoma in situ in the breast, without evidence of stromal invasion.
2E65.5	Paget disease of nipple Paget disease of the nipple describes a rare presentation of breast cancer, seen most frequently in women aged 50-60, manifesting with nipple drainage and itching, erythema, crusty, excoriated nipple, thickened plaques and hyperpigmentation (less frequently). It is due to tumour cells invading the nipple-areola complex and represents 1%-3% of all new breast cancer diagnoses.
2E65.Y	Other specified carcinoma in situ of breast
2E65.Z	Carcinoma in situ of breast, unspecified
2E66	Carcinoma in situ of cervix uteri Exclusions: melanoma in situ of cervix (2E63) Low grade squamous intraepithelial lesion of cervix uteri (GA15.7)
2E66.2	High grade squamous intraepithelial lesion of cervix uteri
2E66.Y	Other specified carcinoma in situ of cervix uteri

2E66.Z	Carcinoma in situ of cervix uteri, unspecified
2E67	Carcinoma in situ of other or unspecified genital organs
	Exclusions: Melanoma in situ neoplasms (2E63)
2E67.0	Carcinoma in situ of endometrium
2E67.1	Carcinoma in situ of vulva
	Exclusions: Low grade squamous intraepithelial lesion of vulva (GA13.1)
2E67.11	Vulvar Paget disease An uncommon intraepithelial malignant neoplasm of eccrine or apocrine origin, arising from the vulva. It usually affects post-menopausal women. In approximately 10-20% of the cases there is an associated anorectal, or urothelial carcinoma or a skin appendage adenocarcinoma identified. It presents as a red, eczematous lesion. Microscopically, it is characterised by the presence of the typical Paget cells which are large, round cells with abundant cytoplasm and prominent nuclei.
2E67.12	Vulvar intraepithelial neoplasia, HPV-independent Vulvar intraepithelial neoplasia (VIN), HPV-independent, is a non-invasive precursor of HPV-independent squamous cell carcinoma of the vulva, characterized by atypia of the basal and parabasal keratinocytes in an otherwise well-differentiated epithelium. Exclusions: Low grade squamous intraepithelial lesion of vulva (GA13.1)
2E67.13	High grade squamous intraepithelial lesion of vulva , HPV-associated Squamous intraepithelial lesions (SILs) of the vulva (also known as vulvar intraepithelial neoplasia [VIN]), HPV-associated, are proliferations of squamous cells driven by HPV infection, showing maturation abnormalities and nuclear hyperchromasia that do not extend beyond the basement membrane. Coded Elsewhere: Bowenoid papulosis of the vulva (1A95.1)
2E67.2	Carcinoma in situ of vagina
2E67.22	High grade squamous intraepithelial lesion of vagina
2E67.2Y	Other specified carcinoma in situ of vagina
2E67.2Z	Carcinoma in situ of vagina, unspecified
2E67.3	Carcinoma in situ of other or unspecified female genital organs

2E67.4	Carcinoma in situ of penis This comprises both squamous carcinoma in situ and extramammary Paget disease of the penis. The former is an uncommon precancerous disease of penile skin. Lesions usually appear on the glans or inner aspect of the foreskin and are almost always found in uncircumcised men. If left untreated, 10-30% of cases develop into invasive squamous cell carcinoma of the penis. When it affects the skin of the shaft or prepuce it is commonly called Bowen disease. If it affects the glans or inner surface of the prepuce it may also be referred to as penile intraepithelial neoplasia (or in the past as erythroplasia of Queyrat). Extramammary Paget disease of penis is a rare form of carcinoma in situ involving penile skin or glans penis. Coded Elsewhere: Extramammary Paget disease of penis (2E64.1)
2E67.40	Squamous cell carcinoma in situ of skin of penis Squamous cell carcinoma affecting the skin of the prepuce or of the shaft of the penis and commonly called Bowen disease. HPV infection and chronic exposure to psoralen photochemotherapy are predisposing factors. Inclusions: Bowen disease of skin of penis
2E67.41	Squamous cell carcinoma in situ of mucocutaneous epithelium of penis Inclusions: Penile intraepithelial neoplasia of inner preputial epithelium Penile intraepithelial neoplasia of glans penis
2E67.5	High grade intraepithelial lesion of prostate High grade prostatic intraepithelial neoplasia characterised by the presence of severe architectural and cytologic abnormalities. Inclusions: high grade prostatic intraepithelial neoplasia Exclusions: low grade dysplasia of prostate (GA91.6)
2E67.6	Carcinoma in situ of other or unspecified male genital organs
2E68	Carcinoma in situ of bladder Stage 0is includes: Tis, N0, M0. Tis: Carcinoma in situ: flat tumour. N0: No regional lymph node metastasis. M0: No distant metastasis.
2E69	Carcinoma in situ of other or unspecified urinary organs
2E6A	Carcinoma in situ of the eye or ocular adnexa
2E6A.0	Carcinoma in situ of the conjunctiva Exclusions: Melanoma in situ of conjunctiva (2E63.1)
2E6A.1	Carcinoma in situ of the cornea
2E6A.Y	Carcinoma in situ of other and unspecified part of the eye and adnexa
2E6B	Carcinoma in situ of thyroid and other endocrine glands Exclusions: Carcinoma in situ of ovary (2E67.3) Carcinoma in situ of testis (2E67.6)
2E6Y	Carcinoma in situ of other specified site

2E6Z**Carcinoma in situ of unspecified site**

Benign neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues (2E80-2F5Z)

A neoplasm which is characterised by the absence of morphologic features associated with malignancy (severe cytologic atypia, tumour cell necrosis, and high mitotic rate). Benign neoplasms remain confined to the original site of growth and only rarely metastasize to other anatomic sites.

Benign mesenchymal neoplasms (2E80-2E8Z)

Benign neoplasms of muscle, fat, fibrous tissue, bone, cartilage, and blood vessels.

2E80**Benign lipomatous neoplasm**

A benign tumour composed of adipose (fatty) tissue. The most common representative of this category is the lipoma.

2E80.0**Lipoma****2E80.00**

Superficial subcutaneous lipoma

A benign well-circumscribed mesenchymal neoplasm composed of mature adipocytes and commonly known as a lipoma.

2E80.01

Deep subfascial lipoma

Deep subfascial lipomata are benign neoplasms of adipose tissue which arise deep to the deep fascia and have a tendency to infiltrate between and into muscle. They may occur at any body site and may cause diagnostic difficulty. They are well recognised to occur on the forehead beneath the frontalis muscle (frontalis-associated lipoma).

Inclusions:

Frontalis-associated lipoma

Infiltrating lipoma of soft tissue

Intramuscular lipoma of soft tissue

2E80.02

Deep internal or visceral lipoma

2E80.0Y

Lipoma, other specified site

2E80.0Z

Lipoma, unspecified site

2E80.1**Lipoblastoma****2E80.Y****Other specified benign lipomatous neoplasm****2E80.Z****Benign lipomatous neoplasm, unspecified****2E81****Benign vascular neoplasms****Exclusions:**

Blue naevus (2F20)

Pigmented naevus (2F20)

Coded Elsewhere: Lobular capillary haemangioma (2F26)

2E81.0	Neoplastic haemangioma <i>Exclusions:</i> Benign vascular neoplasms of infancy and childhood (2E81.2) Infantile haemangioma (2E81.2) <i>Coded Elsewhere:</i> Pulmonary sclerosing haemangioma (2F00.Y)
2E81.00	Umbilical cord haemangioma tumour composed of thin walled blood vessels lined by endothelium present within the cord
2E81.01	Conjunctival haemangioma or haemolymphangioma
2E81.0Y	Neoplastic haemangioma of other specified site
2E81.0Z	Neoplastic haemangioma, unspecified site
2E81.1	Benign lymphatic neoplasms Benign circumscribed or diffuse neoplasms of lymphatic vessels. They are much less common than lymphatic malformations and are distinguished from the latter by proliferative growth and the potential to become widely disseminated. <i>Exclusions:</i> Lymphatic malformations (LA90.1)
2E81.10	Disseminated lymphangiomatosis A rare disorder characterised by widespread proliferation of aberrant lymphatic vessels which typically infiltrate vital organs in the thorax and abdomen.
2E81.11	Acquired progressive lymphangioma Acquired progressive lymphangioma is a benign localised but slowly progressive tumour of lymphatic vessels that typically presents as reddish or bruise-like plaques on the abdominal wall, thigh or calf of young adolescents. <i>Exclusions:</i> Lymphatic malformations (LA90.1)
2E81.1Y	Other specified benign lymphatic neoplasms
2E81.1Z	Benign lymphatic neoplasms, unspecified
2E81.2	Benign vascular neoplasms of infancy and childhood The commonest benign vascular neoplasm of infancy is infantile haemangioma. Less common neoplasms are congenital haemangioma, spindle cell haemangioma, tufted angioma and kaposiform haemangioendothelioma.
2E81.20	Focal infantile haemangioma Infantile haemangioma is a common benign vascular neoplasm which develops in about 4% of infants. It appears within weeks of birth as a blanched, blushed, or telangiectatic area that then rapidly proliferates for several months before entering a prolonged process of involution lasting up to 12 years, leaving a residual variably prominent scar. A solitary focal tumour is seen in about 85% of cases. Over half of cases are located on the head and neck. Complications include bleeding, infection, ulceration and, in tumours situated close to the eye, amblyopia. <i>Inclusions:</i> Strawberry naevus

2E81.21	Multifocal infantile haemangioma Infantile haemangioma is multifocal in up to 25% of cases with numbers ranging from a few to many dozens. If more than 5 cutaneous tumours are present there is an increased risk of associated internal haemangiomatosis, especially of the liver.
2E81.2Y	Other specified benign vascular neoplasms of infancy and childhood
2E81.2Z	Benign vascular neoplasms of infancy and childhood, unspecified
2E81.Y	Other specified benign vascular neoplasms
2E81.Z	Benign vascular neoplasms, unspecified
2E82	Benign chondrogenic tumours
2E82.0	Benign chondrogenic tumours of bone or articular cartilage of limbs
2E82.1	Benign chondrogenic tumours of bone or articular cartilage of other specified sites
2E82.Z	Benign chondrogenic tumours, site unspecified
2E83	Benign osteogenic tumours A neoplasm arising from the bone or articular cartilage that does not invade adjacent tissues or metastasize to other anatomic sites. Representative examples include benign fibrous histiocytoma of bone, osteoma, osteoblastoma, chondroblastoma, and enchondroma.
2E83.0	Benign osteogenic tumours of bone or articular cartilage of skull or face
2E83.1	Benign osteogenic tumours of bone or articular cartilage of lower jaw
2E83.2	Benign osteogenic tumours of bone or articular cartilage of vertebral column <i>Exclusions:</i> Benign osteogenic tumour of sacrum (2E83.4)
2E83.3	Benign osteogenic tumours of bone or articular cartilage of ribs, sternum or clavicle
2E83.4	Benign osteogenic tumours of bone or articular cartilage of pelvic bones, sacrum or coccyx
2E83.5	Benign osteogenic tumours of bone or articular cartilage of limbs
2E83.Y	Benign osteogenic tumour of other specified site
2E83.Z	Benign osteogenic tumour of unspecified site
2E84	Benign fibrogenic or myofibrogenic tumour
2E84.0	Benign fibrogenic or myofibrogenic tumour of skin
2E84.Y	Benign fibrogenic or myofibrogenic tumour of other specified sites
2E84.Z	Benign fibrogenic or myofibrogenic tumour, site unknown

2E85	Benign fibrohistiocytic tumour
<i>Exclusions:</i>	Benign neoplasm of peripheral nerves or autonomic nervous system (2E90-2F3Z) Benign lymphatic neoplasms (2E81.1) Benign lipomatous neoplasm (2E80) Haemangioma (2E81) Benign neoplasm of uterine ligament, any (2F31) Benign vascular neoplasms (2E81) Leiomyoma of uterus (2E86.0) Benign neoplasm of connective tissue of breast (2F30)
2E85.0	Benign fibrohistiocytic tumour of soft tissues of limbs
2E85.1	Benign fibrohistiocytic tumour of retroperitoneum or peritoneum
<i>Exclusions:</i>	Benign lipomatous neoplasm (2E80) Benign neoplasm of mesothelial tissue (2F10)
2E85.2	Benign fibrohistiocytic tumour of skin
2E85.Y	Benign fibrohistiocytic tumour of other specified sites
2E85.Z	Benign fibrohistiocytic tumour, site unspecified
2E86	Benign smooth muscle or skeletal muscle tumour
2E86.0	Leiomyoma of uterus A well-circumscribed benign smooth muscle neoplasm characterised by the presence of spindle cells with cigar-shaped nuclei, interlacing fascicles, and a whorled pattern. <i>Exclusions:</i> Leiomyoma of ovary (2E86.1) Leiomyoma of fallopian tube (2E86.1) Leiomyoma of broad ligament (2E86.1) Leiomyoma of vagina (2E86.1) Leiomyoma of vulva (2E86.1) Benign non-mesenchymal neoplasms of uterus (2F31)
2E86.1	Leiomyoma of other or unspecified sites
2E86.2	Rhabdomyoma
2E86.Y	Other specified benign smooth muscle or skeletal muscle tumour
2E86.Z	Benign smooth muscle or skeletal muscle tumour, unspecified
2E87	Benign gastrointestinal stromal tumour
2E88	Benign endometrial stromal nodule
2E89	Benign mesenchymal tumours of uncertain differentiation

2E89.0	Benign tumours of uncertain differentiation, bone or cartilage
2E89.1	Benign tumours of uncertain differentiation, soft tissue
2E89.Y	Benign mesenchymal tumours of uncertain differentiation of other specified site
2E89.Z	Benign mesenchymal tumours of uncertain differentiation of unspecified site
2E8A	Other mixed or unspecified benign mesenchymal tumours
2E8Y	Other specified benign mesenchymal neoplasm
2E8Z	Benign mesenchymal neoplasms, unspecified

Benign non-mesenchymal neoplasms (2E90-2F3Z)

2E90	Benign neoplasm of lip, oral cavity or pharynx
2E90.0	Benign neoplasm of lip A neoplasm without malignant characteristics arising from the lip. <i>Exclusions:</i> Benign neoplasm of skin of lip (2F20-2F2Z)
2E90.1	Benign neoplasm of tongue Abnormal growth, without malignant characteristics, of the cells that comprise the tongue.
2E90.2	Benign neoplasm of floor of mouth
2E90.3	Benign neoplasm of other or unspecified parts of mouth <i>Exclusions:</i> Benign neoplasm of nasopharyngeal surface of soft palate (2E90.6) benign odontogenic neoplasms (2E83.0) mucosa of lip (2E90.0)
2E90.4	Benign neoplasm of tonsil <i>Exclusions:</i> benign neoplasm of pharyngeal tonsil (2E90.6) benign neoplasm of lingual tonsil (2E90.1) benign neoplasm of tonsillar pillars (2E90.5) benign neoplasm of tonsillar fossa (2E90.5)
2E90.5	Benign neoplasm of oropharynx A neoplasm of the oropharynx which is characterised by the absence of morphologic features associated with malignancy (severe cytologic atypia, tumour cell necrosis, and high mitotic rate). Benign neoplasms remain confined to the original site of growth and only rarely metastasize to other anatomic sites. <i>Exclusions:</i> Benign neoplasm of epiglottis, NOS (2F00) Benign neoplasm of epiglottis, suprahyoid portion (2F00)

2E90.6	Benign neoplasm of nasopharynx A neoplasm of the nasopharynx which is characterised by the absence of morphologic features associated with malignancy (severe cytologic atypia, tumour cell necrosis, and high mitotic rate). Benign neoplasms remain confined to the original site of growth and only rarely metastasize to other anatomic sites.
2E90.7	Benign neoplasm of hypopharynx
2E90.8	Benign neoplasm of pharynx, unspecified
2E91	Benign neoplasm of major salivary glands <i>Exclusions:</i> Benign neoplasms of minor salivary glands NOS (2E90.3)
2E91.0	Benign neoplasm of parotid gland
2E91.1	Benign neoplasm of other specified major salivary glands
2E91.Z	Benign neoplasm of unspecified major salivary glands
2E92	Benign neoplasm of digestive organs A neoplasm of other and/or ill-defined parts of the digestive system which is characterised by the absence of morphologic features associated with malignancy (severe cytologic atypia, tumour cell necrosis, and high mitotic rate). Benign neoplasms remain confined to the original site of growth and only rarely metastasize to other anatomic sites.
2E92.0	Benign neoplasm of oesophagus A non-metastasizing neoplasm arising from the esophageal wall. <i>Coded Elsewhere:</i> Benign mesenchymal tumour of oesophagus (2E8Y)
2E92.1	Benign neoplasm of stomach A non-metastasizing neoplasm arising from the gastric wall. <i>Coded Elsewhere:</i> Benign mesenchymal tumour of stomach (2E8Y)
2E92.2	Benign neoplasm of duodenum A non-metastasizing neoplasm arising from the wall of the duodenum. <i>Coded Elsewhere:</i> Benign mesenchymal tumour of duodenum (2E8Y)
2E92.3	Benign neoplasm of other or unspecified parts of small intestine <i>Exclusions:</i> Benign neoplasm of duodenum (2E92.2) <i>Coded Elsewhere:</i> Benign mesenchymal tumour of small intestine (2E8Y)
2E92.4	Benign neoplasm of the large intestine A non-metastasizing neoplasm arising from the wall of the colon and rectum. <i>Coded Elsewhere:</i> Benign mesenchymal tumour of large intestine (2E8Y)

2E92.40	Polyposis syndrome Intestinal polyposis syndromes can be divided, based on histology, into the broad categories of familial adenomatous polyposis (FAP), hamartomatous polyposis syndromes, and other rare polyposis syndromes, such as hereditary-mixed polyposis syndrome (HMPS). Coded Elsewhere: Gardner syndrome (LD2D.3) Peutz-Jeghers syndrome (LD2D.0) Cronkhite-Canada syndrome (LD27.01) Familial adenomatous polyposis (2B90.Y) Juvenile gastrointestinal polyposis (2B90.Y)
2E92.4Y	Other specified benign neoplasm of the large intestine
2E92.4Z	Benign neoplasm of the large intestine, unspecified
2E92.5	Benign neoplasm of anus or anal canal Primary benign tumour that forms in tissues lining the anus and anal canal. Exclusions: Benign neoplasm of perianal skin (2F20-2F2Z) Benign neoplasm of anal margin (2F20-2F2Z) Benign neoplasm of anal skin (2F20-2F2Z)
2E92.6	Benign neoplasm of gallbladder, extrahepatic bile ducts or ampulla of Vater Coded Elsewhere: Benign mesenchymal tumour of gallbladder, extrahepatic bile ducts or ampulla of Vater (2E8Y) Adenoma of bile ducts (2E92.6)
2E92.7	Benign neoplasm of liver or intrahepatic bile ducts Coded Elsewhere: Focal nodular hyperplasia of liver (DB99.Y) Angiomyolipoma of liver (2E80.02) Haemangioma of liver (2E81.0Y)
2E92.8	Benign neoplasm of pancreas A non-metastasizing neoplasm arising from the pancreas. Exclusions: Benign neoplasm of endocrine pancreas (2E92.9)
2E92.9	Benign neoplasm of endocrine pancreas Inclusions: Islet cell tumour benign neoplasm of islets of Langerhans
2E92.A	Benign neoplasm of ill-defined site within the digestive system
2E92.Y	Benign neoplasm of other specified digestive organs
2E92.Z	Benign neoplasm of unspecified digestive organs

Benign neoplasm of respiratory or intrathoracic organs (2F00-2F0Z)

2F00 Benign neoplasm of middle ear or respiratory system

2F00.0 Middle ear endocrine tumour

2F00.1 Recurrent respiratory papillomatosis

Recurrent respiratory papillomatosis is a rare respiratory disease characterised by the development of exophytic papillomas, affecting the mucosa of the upper aerodigestive tract (with a strong predilection for the larynx), caused by an infection with human papilloma virus. Symptoms at presentation may include hoarseness, chronic cough, dyspnoea, recurrent upper respiratory tract infections, pneumonia, dysphagia, stridor, and/or failure to thrive.

2F00.2 Laryngeal endocrine tumour

2F00.Y Other specified benign neoplasm of middle ear or respiratory system

2F00.Z Benign neoplasm of middle ear or respiratory system, unspecified

2F01 Benign neoplasm of intrathoracic organs

2F0Y Benign neoplasms of other specified respiratory and intrathoracic organs

2F0Z Benign neoplasms of unspecified respiratory and intrathoracic organs

2F10 Benign neoplasm of mesothelial tissue

A benign neoplasm arising from mesothelial cells. It is characterised by the formation of glandular and tubular patterns. It can occur in several anatomic sites including the pleura, peritoneum, and epididymis.

Benign cutaneous neoplasms (2F20-2F2Z)

Abnormal growth of the cells that comprise the tissues of the skin, without any evidence of malignancy.

Coded Elsewhere: Benign vascular neoplasms of infancy and childhood (2E81.2)

2F20 Benign cutaneous melanocytic neoplasms

Inclusions:

- Mole
- Pigmented naevus
- Benign melanocytic naevus

2F20.0 Common acquired melanocytic naevus

2F20.00 Multiple benign melanocytic naevi

The presence of multiple benign melanocytic naevi (often taken as more than 20 naevi >2mm in diameter), an independent risk factor for the development of melanoma with highest risk associated with highest numbers of naevi (>100).

2F20.0Y Other specified common acquired melanocytic naevus

2F20.1	Atypical melanocytic naevus Solitary or multiple, slightly raised, pigmented lesions with irregular borders, usually measuring more than 0.6cm in greatest dimension. Morphologically, there is melanocytic atypia and the differential diagnosis from melanoma may be difficult. Patients are at an increased risk for the development of melanoma. Inclusions: Dysplastic naevus, unspecified
2F20.2	Congenital melanocytic naevus Congenital melanocytic naevi are circumscribed areas of skin pigmentation present at birth as a result of abnormal intrauterine proliferation of melanocytes within the dermis, the epidermis or both. They may range in size from a few millimetres to many centimetres in diameter. If their projected or final adult maximal diameter is greater than 20 cm they are termed giant congenital melanocytic naevi.
2F20.20	Giant congenital melanocytic naevus A congenital melanocytic naevus (CMN) with a predicted or final adult maximal diameter of 400 mm or more. Giant CMNs are commonly centred on the dorsal surface of the body between the vertex and the buttocks but may occur elsewhere; they may be associated with multiple smaller satellite naevi (congenital or tardive), hypertrichosis, lipomas or benign proliferative nodules. There is a risk of pre-pubertal melanoma within giant CMN or the central nervous system (CNS). Leptomeningeal melanocytosis or focal neuromelanosis, found in 10-15% of cases, is often associated with other CNS tumours, hydrocephalus, epilepsy, arachnoid cysts, or Dandy-Walker malformation.
2F20.2Y	Other specified congenital melanocytic naevus
2F20.2Z	Congenital melanocytic naevus, unspecified
2F20.3	Generalised eruptive melanocytic naevi This phenomenon describes the rapid simultaneous appearance of multiple melanocytic naevi, often hundreds in number, on previously uninvolved sun-exposed skin. The phenomenon has been linked to immunosuppression, particularly in renal transplant recipients and in individuals receiving cancer chemotherapy, and may be considered a more advanced counterpart of generalised eruptive lentiginosis. Exclusions: Multiple benign melanocytic naevi (2F20.00) Generalized eruptive lentiginosis (ED61)
2F20.Y	Other specific types of melanocytic naevus
2F20.Z	Melanocytic naevus, unspecified
2F21	Benign keratinocytic acanthomas A group of benign discrete epidermal proliferative disorders including seborrhoeic keratosis and clear cell acanthoma.

2F21.0	Seborrhoeic keratosis
	Seborrhoeic keratoses are very common benign neoplasms of epidermal keratinocytes which increase in prevalence and number with age. They are commonly multiple and are very variable in shape and colour. Because of the sometimes intense pigmentation they are frequently mistaken for melanocytic tumours.
	<i>Inclusions:</i> Basal cell papilloma Seborrheic wart
2F21.Y	Other specified benign keratinocytic acanthomas
2F22	Benign neoplasms of epidermal appendages
	A range of benign neoplasms arising from the hair follicle, its associated glands or from sweat glands.
2F23	Benign dermal fibrous or fibrohistiocytic neoplasms
	Benign dermal neoplasms due to abnormal proliferation of fibroblasts, myofibroblasts or primitive mesenchymal cells.
2F23.0	Dermatofibroma
	A common benign skin tumour which presents as a firm dermal papule or nodule, most commonly on the lower limbs. Histologically it is characterised by coarse, haphazardly arranged collagen bundles and a variable cellular infiltrate including fibrocytes.
2F23.Y	Other specified benign dermal fibrous or fibrohistiocytic neoplasms
2F24	Benign cutaneous neoplasms of neural or nerve sheath origin
2F25	Cherry angioma
	<i>Inclusions:</i> Campbell de Morgan spot Senile angioma
2F26	Lobular capillary haemangioma
	Historically called pyogenic granuloma, this is a common benign proliferation of capillary blood vessels which may be induced by trauma or by certain drugs. It presents as one or more bright red papules or nodules often located around the mouth or on a terminal phalanx in relation to the nail. Bleeding, ulceration and crusting frequently occur. BRAF mutations within vascular endothelial cells may be present, indicating that this is, in at least a proportion of cases, a true neoplastic process.
	<i>Inclusions:</i> Lobular capillary haemangioma of skin
2F2Y	Other specified benign cutaneous neoplasms
2F2Z	Benign cutaneous neoplasm of unspecified type

2F30	Benign neoplasm of breast A non-metastasizing neoplasm arising from the breast parenchyma. <i>Exclusions:</i> Benign neoplasm of skin of breast (2F20-2F2Z) Lipoma (2E80.0)
2F30.0	Tubular adenoma of breast A benign, well circumscribed neoplasm that arises from the breast. It is composed entirely of tubular structures that contain epithelial and myoepithelial cells.
2F30.1	Lactating adenoma of breast A tubular type adenoma of the breast in which, during pregnancy and lactation, the epithelial cells show extensive secretory changes.
2F30.2	Intraductal papilloma of breast A benign papillary neoplasm that arises anywhere in the ductal system of the breast. It is characterised by fibrovascular structures lined by benign epithelial and myoepithelial proliferations. Intraductal breast papillomas are classified as central, when they arise in large ducts, or peripheral, when they arise in the terminal ductal lobular units.
2F30.3	Benign phyllodes tumour of breast A usually unilateral, benign and well circumscribed biphasic neoplasm that arises from the breast. It usually affects middle-aged women. It is characterised by the presence of a double layer of epithelial cells that are arranged in clefts, surrounded by a cellular, monomorphic spindle cell mesenchymal component. Mitoses are rare. Necrotic changes may be present in large tumours.
2F30.4	Fibromatosis of breast
2F30.5	Fibroadenoma of breast A benign tumour of the breast characterised by the presence of stromal and epithelial elements. It presents as a painless, solitary, slow growing, firm, and mobile mass. It is the most common benign breast lesion. It usually occurs in women of childbearing age. The majority of fibroadenomas do not recur after complete excision. A slightly increased risk of developing cancer within fibroadenomas or in the breast tissue of patients previously treated for fibroadenomas has been reported.
2F30.6	Extensive adenomatosis of nipple Rare benign nipple condition presenting as pruritus, burning or pain symptoms with clinical signs showing a nipple which appears ulcerated, crusting, scaling, indurated and erythematous. Differential diagnosis: Paget, psoriasis, etc
2F30.Y	Other specified benign neoplasm of breast
2F30.Z	Benign neoplasm of breast, unspecified
2F31	Benign non-mesenchymal neoplasms of uterus Other non-malignant tumours of the uterus not detailed elsewhere. <i>Exclusions:</i> Leiomyoma of uterus (2E86.0)

2F31.0	Benign non-mesenchymal neoplasm of uterus, cervix uteri <i>Exclusions:</i> Low grade squamous intraepithelial lesion of cervix uteri (GA15.7)
2F31.1	Benign non-mesenchymal neoplasm of uterus, corpus uteri
2F31.2	Benign non-mesenchymal neoplasms of uterus, other parts
2F32	Benign neoplasm of ovary A non-metastasizing neoplasm that arises from the ovary. Representative examples include serous cystadenoma, mucinous cystadenoma, clear cell adenofibroma, benign Brenner tumour, thecoma, and fibroma. <i>Coded Elsewhere:</i> Struma ovarii (5A02.Y)
2F32.0	Cystic teratoma A condition of the ovary, caused by abnormal proliferation due to genetic mutations, abnormal growth or division of germ cells. This condition is characterised by a benign ovarian neoplasm, and abdominal pain, mass or swelling, or abnormal uterine bleeding, and may lead to ovarian torsion or cystic rupture. Confirmation is by imaging.
2F32.1	Ovarian fibroma A condition of the ovary, caused by abnormal proliferation due to genetic mutations, abnormal growth or division of cells. This condition is characterised by a benign sex chord ovarian tumour. Confirmation is by imaging.
2F32.2	Meigs' Syndrome Benign Syndrome including ovarian fibroma, ascites and pleural effusion
2F32.3	Serous ovarian cystadenoma
2F32.Y	Other specified benign neoplasm of ovary
2F32.Z	Benign neoplasm of ovary, unspecified
2F33	Benign neoplasm of other or unspecified female genital organs A non-metastasizing neoplasm that arises from the female reproductive system. Representative examples include uterine corpus leiomyoma, endocervical polyp, and benign ovarian germ cell tumour.
2F34	Benign neoplasm of male genital organs A non-metastasizing neoplasm that arises from the male reproductive system. Representative examples include benign prostate phyllodes tumour, benign Sertoli cell tumour, seminal vesicle cystadenoma, and epididymal adenomatoid tumour.
2F35	Benign neoplasm of urinary organs A non-metastasizing neoplasm that arises from the organs that comprise the urinary system. Representative examples include renal oncocytoma, bladder inverted papilloma, and urothelial papilloma.

2F36	Benign neoplasm of eye or ocular adnexa <i>Exclusions:</i> Benign neoplasm of optic nerve (2A02.3) Benign neoplasm of skin of eyelid (2F20-2F2Z) <i>Coded Elsewhere:</i> Epibulbar choristoma (LA14.06) Seborrheic keratosis (2F21.0)
2F36.0	Benign neoplasm of choroid Abnormal growth of the cells of the choroid without malignant characteristics. <i>Coded Elsewhere:</i> Haemangioma of choroid (2E81.0Y)
2F36.1	Benign neoplasm of iris
2F36.2	Benign neoplasm of ciliary body
2F36.3	Teratoma of orbit This is an encapsulated tumour with tissue or organ components resembling normal derivatives of all three germ layers. This diagnosis is of the cavity or socket of the skull in which the eye and its appendages are situated.
2F36.4	Cysts of eyelid <i>Coded Elsewhere:</i> Epidermoid cyst (EK70.0)
2F36.Y	Other specified benign neoplasm of eye or ocular adnexa
2F36.Z	Benign neoplasm of eye or ocular adnexa, unspecified
2F37	Benign neoplasm of endocrine glands <i>Exclusions:</i> Benign neoplasm of endocrine pancreas (2E92.9) Benign neoplasm of thymus (2F01) Benign neoplasm of ovary (2F32) Benign neoplasm of testis (2F34) Benign neoplasm of hypothalamus (2A00.5)
2F37.0	Non-secreting pituitary adenoma
2F37.Y	Other specified benign neoplasm of endocrine glands
2F37.Z	Benign neoplasm of endocrine glands, unspecified
2F3Y	Benign non-mesenchymal neoplasms of other specified site
2F3Z	Benign non-mesenchymal neoplasms of unspecified site
2F5Y	Other specified benign neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues
2F5Z	Benign neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues, unspecified

Neoplasms of uncertain behaviour, except of lymphoid, haematopoietic, central nervous system or related tissues (2F70-2F7Z)

A neoplasm displaying morphologic, phenotypic, or genotypic characteristics that are clearly not benign but do not permit the establishment of a definitive diagnosis of malignancy. Such neoplasms may or may not eventually have a more aggressive clinical course. Representative examples include lymphoproliferations of uncertain malignant potential (e.g., lymphomatoid granulomatosis and lymphomatoid papulosis), borderline ovarian epithelial neoplasms (e.g., borderline ovarian endometrioid tumour and borderline ovarian mucinous tumour), borderline exocrine pancreatic neoplasm (e.g., pancreatic borderline intraductal papillary-mucinous neoplasm), and primary borderline peritoneal epithelial neoplasm.

Coded Elsewhere: Pathological fracture in neoplastic disease of uncertain behaviour (FB80.B)

2F70	Neoplasms of uncertain behaviour of oral cavity or digestive organs
2F70.0	Neoplasms of uncertain behaviour of lip, oral cavity or pharynx
2F70.1	Neoplasms of uncertain behaviour of stomach
2F70.2	Neoplasms of uncertain behaviour of small intestine
2F70.3	Neoplasms of uncertain behaviour of colon
2F70.4	Neoplasms of uncertain behaviour of rectum
2F70.5	Neoplasms of uncertain behaviour of liver, gallbladder or bile ducts
2F70.Y	Neoplasms of uncertain behaviour of oral cavity and digestive organs, other specified site
2F70.Z	Neoplasms of uncertain behaviour of oral cavity and digestive organs, unspecified site
2F71	Neoplasms of uncertain behaviour of middle ear, respiratory or intrathoracic organs
2F71.0	Neoplasms of uncertain behaviour of thymus
2F71.1	Neoplasms of uncertain behaviour of larynx
2F71.2	Neoplasms of uncertain behaviour of pleura
2F71.3	Neoplasms of uncertain behaviour of trachea, bronchus or lung
2F71.4	Neoplasms of uncertain behaviour of mediastinum
2F71.Y	Neoplasms of uncertain behaviour of middle ear, respiratory and intrathoracic organs, other specified site
2F71.Z	Neoplasms of uncertain behaviour of middle ear, respiratory and intrathoracic organs, unspecified site
2F72	Neoplasms of uncertain behaviour of skin

2F72.1	Spitzoid tumour of uncertain malignant potential
	A spindle cell and epithelioid cell melanocytic neoplasm in which there are sufficient features distinguishing it from a benign Spitz naevus to cast doubt on its benign nature. These atypical features include development in adult life, asymmetry, large diameter (>6 and especially >10 mm), significant thickness (particularly subcutaneous extension), lack of “maturation” and nodule formation, cytological atypia and a high mitotic rate.
2F72.2	Melanocytic naevus with severe melanocytic dysplasia
	Melanocytic naevus with severe melanocytic dysplasia is a histopathological diagnosis based on the presence of severe cytological atypia, defined as enlarged, spindle- and epithelioid-shaped melanocytes with hyperchromatic nuclei (typically at least twice the size of those of basal keratinocytes) and distinct nucleoli. Such naevi tend to be irregular in size and pigmentation and to have been excised because of concern that they may represent early melanoma.
2F72.Y	Other specified neoplasms of uncertain behaviour of skin
2F73	Neoplasms of uncertain behaviour of retroperitoneum
2F74	Neoplasms of uncertain behaviour of peritoneum
2F75	Neoplasms of uncertain behaviour of breast
2F76	Neoplasms of uncertain behaviour of female genital organs
2F77	Neoplasms of uncertain behaviour of male genital organs
2F78	Neoplasms of uncertain behaviour of urinary organs
2F79	Neoplasms of uncertain behaviour of eye or ocular adnexa
2F7A	Neoplasms of uncertain behaviour of endocrine glands
2F7A.0	Multiple polyglandular tumours
	<i>Coded Elsewhere:</i> Carney complex (5A70.Y) Von Hippel-Lindau disease (5A75)
2F7A.Y	Other specified neoplasms of uncertain behaviour of endocrine glands
2F7A.Z	Neoplasms of uncertain behaviour of endocrine glands, unspecified
2F7B	Neoplasms of uncertain behaviour of bone or articular cartilage
2F7C	Neoplasms of uncertain behaviour of connective or other soft tissue
2F7Y	Neoplasms of uncertain behaviour of other specified site
2F7Z	Neoplasms of uncertain behaviour of unspecified site

Neoplasms of unknown behaviour, except of lymphoid, haematopoietic, central nervous system or related tissues (2F90-2F9Z)

Coded Elsewhere: Pathological fracture in neoplastic disease of unknown behaviour (FB80.B)

2F90	Neoplasms of unknown behaviour of oral cavity or digestive organs
2F90.0	Neoplasms of unknown behaviour of colon
2F90.1	Neoplasms of unknown behaviour of rectum
2F90.Y	Neoplasms of unknown behaviour of oral cavity and digestive organs, other specified site
2F90.Z	Neoplasms of unknown behaviour of oral cavity and digestive organs, unspecified site
2F91	Neoplasms of unknown behaviour of middle ear, respiratory or intrathoracic organs
2F91.0	Neoplasms of unknown behaviour of larynx
2F91.1	Neoplasms of unknown behaviour of trachea, bronchus or lung
2F91.Y	Neoplasms of unknown behaviour of other specified middle ear, respiratory or intrathoracic organ
2F92	Neoplasms of unknown behaviour of skin
2F93	Neoplasms of unknown behaviour of retroperitoneum
2F94	Neoplasms of unknown behaviour of peritoneum
2F95	Neoplasms of unknown behaviour of breast
2F96	Neoplasms of unknown behaviour of female genital organs
2F97	Neoplasms of unknown behaviour of male genital organs
2F98	Neoplasms of unknown behaviour of urinary organs
2F99	Neoplasms of unknown behaviour of eye or ocular adnexa
2F9A	Neoplasms of unknown behaviour of endocrine glands
2F9B	Neoplasms of unknown behaviour of bone or articular cartilage
2F9C	Neoplasms of unknown behaviour of connective or other soft tissue
2F9Y	Neoplasms of unknown behaviour of other specified site
2F9Z	Neoplasms of unknown behaviour of unspecified site

CHAPTER 03

Diseases of the blood or blood-forming organs

This chapter has 56 four-character categories.

Code range starts with 3A00

This chapter includes diseases of the blood as well as diseases of blood forming organs.

- Exclusions:**
- Complications of pregnancy, childbirth or the puerperium (Chapter 18)
 - Diseases of the immune system (Chapter 04)
 - Certain conditions originating in the perinatal period (Chapter 19)
 - Injury, poisoning or certain other consequences of external causes (Chapter 22)
 - Human immunodeficiency virus disease (1C60-1C62.Z)
 - Endocrine, nutritional or metabolic diseases (Chapter 05)
 - Congenital malformations, deformations or chromosomal abnormalities (Chapter 20)
 - Other diseases of the blood or blood-forming organs or certain disorders involving the immune mechanism complicating pregnancy, childbirth or the puerperium (JB64.1)
- Coded Elsewhere:**
- Neoplasms of haematopoietic or lymphoid tissues (2A20-2B3Z)
 - Symptoms, signs or clinical findings of blood, blood-forming organs, or the immune system (MA00-MA3Y)

This chapter contains the following top level blocks:

- Anaemias or other erythrocyte disorders
- Coagulation defects, purpura or other haemorrhagic or related conditions
- Diseases of spleen

Anaemias or other erythrocyte disorders (3A00-3A9Z)

Inclusions: Anaemia, unspecified

Coded Elsewhere: Anaemia complicating pregnancy, childbirth or the puerperium (JB64.0)
Anaemia of prematurity (KA8B)

Nutritional or metabolic anaemias (3A00-3A03.Y)

3A00

Iron deficiency anaemia

A disease caused by chronic or acute bleeding, excessive menstrual bleeding, inadequate intake, substances (in diet or drugs) interfering with iron absorption, malabsorption syndromes, inflammation, infection or blood donation. This disease is characterised by decreased levels of iron present in the body. This disease may present with fatigue, pallor or dizziness. Confirmation is by identification of decreased levels of iron in a blood sample.

3A00.0

Acquired iron deficiency anaemia due to blood loss

Chronic blood loss is a possible cause in every case of iron-deficiency anaemia. Iron deficiency anaemia may be caused by acute bleeding in gastrointestinal tract, uterus or genitourinary system, copious menstrual blood losses (menorrhagia) and multiple blood donations. In many tropical countries, infestations with hookworms lead to intestinal blood losses that in some individuals can be considerable. Iron deficiency may also be caused by several circumstances related to "chronic posthaemorrhagic anaemia". A diagnosis of iron deficiency should always lead to a search for pathologic causes of blood loss (e.g. tumours in the gastrointestinal tract or uterus, especially if uterine bleedings have increased or changed in regularity).

Exclusions: congenital anaemia from fetal blood loss (KA8C)

3A00.01

Chronic posthaemorrhagic anaemia

Chronic iron-deficiency anaemia from bleeding may be caused by colon cancer, gastric cancer, peptic ulcer, Meckel diverticulum, hiatal hernia with linear erosions, colonic vascular ectasia, colonic polyps, haemangioma, inflammatory bowel disease, tumours in the gastrointestinal tract or uterus, and chronic menorrhagia. Some infants with severe iron deficiency have chronic intestinal blood loss induced by exposure to cow's milk protein. Repeated phlebotomy for blood tests is a cause of anaemia of prematurity.

3A00.0Z

Acquired iron deficiency anaemia due to blood loss, unspecified

3A00.1

Acquired iron deficiency anaemia due to low intake

Iron deficiency is probably the most common nutritional deficiency disorder in the world. Iron deficiency anaemia during pregnancy increases perinatal risks for mothers and neonates; and increases overall infant mortality. Severe anaemia is a major risk factor associated with greatly increased morbidity and mortality for young children and pregnant women. Prompt recognition of the condition, treatment and clinical follow-up of individuals, are crucial in avoiding complications such as high-output heart failure. Maternal iron deficiency during pregnancy increases the risk of iron deficiency in the infant. In less developed countries, the prevalence of iron deficiency during pregnancy is higher than in developed countries, and iron supplementation during pregnancy is beneficial.

- 3A00.2 **Acquired iron deficiency anaemia due to decreased absorption**
- 3A00.3 **Acquired iron deficiency anaemia due to increased requirement**
- 3A00.Y **Other specified iron deficiency anaemia**
- 3A00.Z **Iron deficiency anaemia, unspecified**

3A01

Megaloblastic anaemia due to vitamin B12 deficiency

A disease caused by inadequate dietary intake of vitamin B12, impaired absorption of vitamin B12, surgical removal of the small bowel, coeliac disease or inherited mutations affecting absorption of vitamin B12. This disease is characterised by decreased levels of vitamin B12 in the body presenting with or without anaemia. This disease may present with fatigue, pallor, dizziness, seizures, or symptoms of dementia. Confirmation is by identification of decreased levels of vitamin B12 in a blood sample.

3A01.0 Hereditary Vitamin B12 deficiency anaemia

This is a hereditary low blood level of vitamin B12. It can cause permanent damage to nervous tissue if left untreated long enough. Vitamin B12 itself was discovered through investigation of pernicious anaemia, which is an autoimmune disease that destroys parietal cells in the stomach that secrete intrinsic factor.

3A01.1 Neonatal vitamin B12 deficiency anaemia

A disease caused by a lack of vitamin B12 in the mother, which is passed onto the fetus in the antenatal period or to the neonate during breast feeding. This disease is characterised by decreased levels of vitamin B12. This disease may present with increased risk of birth defects or preterm delivery, anaemia, irritability, failure to thrive or apathy. Confirmation is by identification of low levels of vitamin B12 in a blood sample.

Exclusions: Hereditary Vitamin B12 deficiency anaemia (3A01.0)

3A01.2 Vitamin B12 deficiency anaemia due to low intake

A disease caused by insufficient intake of vitamin B12 into the body. This disease is characterised by low levels of vitamin B12 leading to low levels of red blood cells. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of low levels of vitamin B12 and red blood cell count in a blood sample.

3A01.3 Vitamin B12 deficiency anaemia due to intrinsic factor deficiency

Exclusions: Vitamin B12 deficiency anaemia due to congenital intrinsic factor deficiency (3A01)

3A01.30 Pernicious anaemia

Biermer's disease, also called acquired pernicious anaemia, is a disorder in vitamin B12 (cobalamin) absorption characterised by megaloblastic anaemia and gastrointestinal symptoms, and that can lead to neurological abnormalities.

3A01.3Y Other specified vitamin B12 deficiency anaemia due to intrinsic factor deficiency

3A01.4 Vitamin B12 deficiency anaemia due to intestinal disease

A number of intestinal disorders can also cause vitamin B12 (cobalamin) deficiency. These include severe pancreatic diseases and small bowel diseases such as malabsorption, ileal disease (including tuberculous ileitis, lymphoma, amyloid, long-term survivors of pelvic irradiation), extensive small bowel resection or bypass, gastric surgery/reconstruction for obesity (bariatric surgery) and Crohn's disease. When jejunal blind loops are present, bacterial overgrowth within the loops competes for cobalamin, leading to cobalamin deficiency. Although not as common currently, infestation with the fish tapeworm, *Diphyllobothrium latum*, was once a classic cause of cobalamin deficiency.

Coding Note: Code also the causing condition

3A01.5 Drug-induced vitamin B12 deficiency anaemia

3A01.Y Other specified megaloblastic anaemia due to vitamin B12 deficiency

3A01.Z Megaloblastic anaemia due to vitamin B12 deficiency, unspecified

3A02 Folate deficiency anaemia

3A02.0 Hereditary folate deficiency anaemia

3A02.1 Folate deficiency anaemia due to low intake

3A02.2 Folate deficiency anaemia due to increased requirements

3A02.3 Folate deficiency anaemia due to decreased intestinal absorption

A disease caused by determinants affecting intestinal absorption of folate arising after birth. This disease is characterised by low levels of folate in the body leading to incomplete formation of red blood cells resulting in large numbers of immature and incompletely developed red blood cells. This disease may present with fatigue, muscle weakness, loss of appetite, weight loss, diarrhoea, nausea, tachycardia or extremity paraesthesia. Confirmation is by identification of low folate levels in a blood sample.

Coding Note: Code also the causing condition

3A02.4 Drug-induced folate deficiency anaemia

3A02.Y Other specified folate deficiency anaemia

3A02.Z Folate deficiency anaemia, unspecified

3A03 Other nutritional or metabolic anaemias

A disease caused by nutritional and metabolic determinants leading to anaemia. This disease is characterised by decreased levels of red blood cells within the body. This disease may present with fatigue, pallor or dizziness. Confirmation is by identification of a decreased red blood cell count in a blood sample.

Coded Elsewhere: Disorders of pyrimidine metabolism (5C55.1)
Lesch-Nyhan syndrome (5C55.01)

- 3A03.0 Hereditary orotic aciduria**
Hereditary orotic aciduria is an extremely rare (less than 20 cases identified worldwide) autosomal recessive disorder characterised by retarded growth, anaemia and excessive urinary excretion of orotic acid. It is due to a severe deficiency in the activity of the pyrimidine pathway enzyme uridine 5'-monophosphate (UMP) synthase (bifunctional enzyme containing two activities: orotate phosphoribosyltransferase and orotidine 5'-monophosphate decarboxylase), coded by a single gene (UMPS) localised to chromosome 3q13.
- 3A03.1 Protein deficiency anaemia**
A disease caused by low levels of protein within the body. This disease is characterised by a low red blood cell count in the blood. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of a low red blood cell count in a blood sample.
Exclusions: Lesch-Nyhan syndrome (5C55.01)
- 3A03.2 Scorbutic anaemia**
Scorbutic anaemia, is a common finding in infants and young children with scurvy and is related to impaired iron absorption and coexistent haematopoietic nutrient deficiencies including iron, vitamin B12 and folate.
- 3A03.3 Copper deficiency anaemia**
Anaemia due to copper deficiency arises from impaired utilization of iron and is therefore a conditioned form of iron deficiency anaemia.
- 3A03.4 Acquired other vitamin B deficiency anaemia**
A disease caused by a lack of B vitamins in the body arising after birth. This disease is characterised by low levels of B vitamins leading to low levels of red blood cells in the body. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of low red blood cell count and low B vitamin counts in a blood sample.
- 3A03.40 Acquired pyridoxine deficiency anaemia**
A disease caused by determinants arising after birth. This disease is characterised by low levels of pyridoxine (vitamin B6) leading to low levels of red blood cells in the body. This disease may present with fatigue, muscle weakness, loss of appetite, weight loss, diarrhoea, nausea, fast heartbeat or numbness in extremities. Confirmation is by identification of low levels of pyridoxine and low red blood cell count in a blood sample.
- 3A03.41 Acquired riboflavin deficiency anaemia**
A disease caused by determinants arising after birth. This disease is characterised low levels of riboflavin (vitamin B2) leading to low levels of red blood cells in the body. This disease may present with fatigue, muscle weakness, loss of appetite, weight loss, diarrhoea, nausea, fast heartbeat or numbness in extremities. Confirmation is by identification of low levels of riboflavin and low red blood cell count in a blood sample.

- 3A03.42** **Acquired thiamine deficiency anaemia**
 A disease caused by a lack of thiamine arising after birth. This disease is characterised low levels of thiamine in the body leading to low levels of red blood cells. This disease may present with fatigue, muscle weakness, loss of appetite, weight loss, diarrhoea, nausea, fast heartbeat or numbness in extremities. Confirmation is by identification of low levels of thiamine and low red blood cell count in a blood sample.
- 3A03.4Y** **Other specified acquired other vitamin B deficiency anaemia**
- 3A03.5** **Acquired vitamin A deficiency anaemia**
- 3A03.6** **Acquired vitamin E deficiency anaemia**
Inclusions: Haemolytic anaemia due to vitamin E deficiency
- 3A03.Y** **Other and unspecified nutritional or metabolic anaemia**

Haemolytic anaemias (3A10-3A4Z)

A disease caused by determinants arising after birth, during the antenatal period or genetically inherited factors leading to premature haemolysis of red blood cells. This disease is characterised by low levels of red blood cells in the body due to abnormal breakdown of the cells. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of low red blood cell count in a blood sample.

Congenital haemolytic anaemia (3A10-3A1Y)

A disease caused by determinants arising in the antenatal period . This disease is characterised by low levels of red blood cells in the body due to abnormal destruction of the red blood cells. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of low red blood cell count in a blood sample.

Coded Elsewhere: Haemolytic disease of fetus or newborn (KA84)

3A10 **Hereditary haemolytic anaemia**

3A10.0 **Haemolytic anaemias due to hexose monophosphate shunt or glutathione metabolism anomalies**

This is a form of anaemia due to haemolysis, the abnormal breakdown of red blood cells (RBCs), either in the blood vessels (intravascular haemolysis) or elsewhere in the human body (extravascular). This diagnosis is due to is a process that generates NADPH and pentoses (5-carbon sugars) and glutathione metabolism anomalies.

- 3A10.00** **Haemolytic anaemia due to glucose-6-phosphate dehydrogenase deficiency**
 Glucose-6-phosphate dehydrogenase (G6PD) deficiency is the most common hereditary erythrocyte enzyme deficiency that can manifest with severe neonatal jaundice which can lead to serious neurological consequences, or, most often, with acute haemolytic anaemia following ingestion of certain foods (fava beans), common drugs (some antimalaria drugs, sulphamides, analgesics), or in the course of an infection, in otherwise asymptomatic individuals.

3A10.0Y	Other specified haemolytic anaemias due to hexose monophosphate shunt or glutathione metabolism anomalies
3A10.0Z	Haemolytic anaemias due to hexose monophosphate shunt or glutathione metabolism anomalies, unspecified
3A10.1	Haemolytic anaemia due to adenosine deaminase excess
3A10.2	Hereditary elliptocytosis Hereditary elliptocytosis is a group of rare conditions caused by abnormalities in the red cell cytoskeleton and marked by the presence on blood smears of numerous elliptical red blood cells, called elliptocytes. Clinical presentations are highly heterogeneous ranging from asymptomatic forms to more severe forms associated with variable anaemia, from moderate to severe and with pyropoikilocytosis including fragmented red cells, microelliptocytes and microspherocytes.
3A10.3	Familial pseudohyperkalaemia A disease caused by a genetically inherited mutation. This disease is characterised by a temperature-dependent defect in red cell membrane permeability to potassium that leads to high in vitro potassium levels in samples stored below 37°C leading to elevated potassium levels in the blood that does not reflect the true potassium level. Confirmation is by identification of genetic mutation through genetic testing.
3A10.Y	Other specified hereditary haemolytic anaemia
3A10.Z	Hereditary haemolytic anaemia, unspecified
3A1Y	Other specified congenital haemolytic anaemia

Acquired haemolytic anaemia (3A20-3A2Z)

A disease characterised by premature destruction of red blood cells arising after birth. This disease is further characterised by low levels of red blood cells in the body due to abnormal destruction of the cells. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of low red blood cell count in a blood sample.

3A20	Acquired haemolytic anaemia, immune A condition characterised by antibodies that are directed against red blood cells in an autoimmune reaction leading to low levels of red blood cells. This condition may present with pallor, fatigue, shortness of breath. Confirmation is by identification of antibodies in a blood sample and positive Coombs test result.
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- 3A20.0 Autoimmune haemolytic anaemia, warm type**
- Autoimmune haemolytic anaemia (AIHA) is an autoimmune disorder in which various types of auto-antibodies are directed against red blood cells causing their survival to be shortened and resulting in haemolytic anaemia. AIHA can be primary (idiopathic), secondary to infection or associated with diseases such as B-cell lymphomas, other systemic or organ-specific autoimmune diseases, Hodgkin's disease, hepatitis or primary immunodeficiencies, or, in the case of drug-induced AIHA, caused by a reaction to drugs.
- Exclusions:** Evans syndrome (3A20.5)
Haemolytic disease of fetus or newborn (KA84)
Paroxysmal cold haemoglobinuria (3A20.3)
- 3A20.1 Autoimmune haemolytic anaemia, cold type**
- Cold autoimmune haemolytic anaemia comprises two types of autoimmune haemolytic anaemia (AIHA) defined by the presence of cold autoantibodies (autoantibodies which are active at temperatures below 30°C): cold agglutinin disease (CAD), which is the more common, and paroxysmal cold haemoglobinuria (PCH). CAD is more common in people over the age of 55 years, while PCH typically presents in young children. CAD is caused by IgM autoantibodies while PCH is caused by an IgG immunoglobulin.
- Exclusions:** Immune thrombocytopenic purpura (3B64.10)
Haemolytic disease of fetus or newborn (KA84)
- 3A20.2 Autoimmune haemolytic anaemia, mixed type, cold and warm**
- Mixed autoimmune haemolytic anaemia is a type of autoimmune haemolytic anaemia (AIHA) defined by the presence of both warm and cold autoantibodies, which have a deleterious effect on red blood cells at either body temperature or at lower temperatures.
- 3A20.3 Paroxysmal cold haemoglobinuria**
- Paroxysmal cold hemoglobinuria is a very rare subtype of autoimmune haemolytic anaemia (AIHA), caused by the presence of cold-reacting autoantibodies in the blood and characterised by the sudden presence of hemoglobinuria, typically after exposure to cold temperatures. PCH is thought to account for at most 2-10% of cases of AIHA.
- 3A20.4 Alloimmune haemolytic anaemia**
- A disease caused by determinants such as a blood transfusion that lead to an immune response directed against the person's own red blood cells. This disease is characterised by low levels of red blood cells in the body due to abnormal destruction of the red blood cells. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of low red blood cell count in a blood sample.
- Exclusions:** Haemolytic disease of fetus or newborn (KA84)

3A20.5	Evans syndrome Evans syndrome is characterised by the association of autoimmune haemolytic anaemia with another haematological anomaly. The thrombocytopaenia may precede, occur concurrently with, or secondary to the autoimmune haemolytic anaemia.
3A20.Y	Other specified acquired haemolytic anaemia, immune
3A21	Acquired haemolytic anaemia, non-immune A disease caused by determinants such as infection, toxic chemicals, drugs and trauma arising after birth. This disease is characterised by haemolysis of red blood cells. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of decreased red blood cell count in a blood sample and negative Coombs test result.
3A21.0	Paroxysmal nocturnal haemoglobinuria Paroxysmal nocturnal hemoglobinuria (PNH) is an acquired clonal hematopoietic stem cell disorder characterised by corpuscular haemolytic anaemia, bone marrow failure and frequent thrombotic events. <i>Exclusions:</i> haemoglobinuria NOS (MF94) Aplastic anaemia with paroxysmal nocturnal haemoglobinuria (3A70.1)
3A21.1	Microangiopathic haemolytic anaemia This is a microangiopathic subgroup of haemolytic anaemia (loss of red blood cells through destruction) caused by factors in the small blood vessels. It is identified by the finding of anaemia and schistocytes on microscopy of the blood film.
3A21.2	Haemolytic uraemic syndrome <i>Exclusions:</i> Hereditary haemolytic uraemic syndrome (3A10)
3A21.Y	Other specified acquired haemolytic anaemia, non-immune
3A2Z	Acquired haemolytic anaemia, unspecified
3A4Z	Haemolytic anaemias, unspecified
3A50	Thalassaemias A disease caused by genetically inherited autosomal recessive mutations leading to abnormal production of haemoglobin. This disease is characterised by destruction of red blood cells leading to anaemia and abnormal production of haemoglobin. This disease may present with pallor, jaundice, iron overload, fatigue, or shortness of breath. Confirmation is by identification of mutations through genetic testing.
3A50.0	Alpha thalassaemia Alpha-thalassemia is an inherited haemoglobinopathy characterised by impaired synthesis of alpha-globin chains leading to a variable clinical picture depending on the number of affected alleles, and encompassing the alpha thalassaemia trait, haemoglobin H disease (HbH) and Bart's hydrops fetalis. <i>Exclusions:</i> Hydrops fetalis due to haemolytic disease (KA85)

3A50.00	Mild alpha thalassaemia diseases A disease caused by genetically inherited factors affecting the alpha chain of the haemoglobin molecule. This disease is characterised by structural abnormalities of the haemoglobin molecule. This disease may present with mild anaemia: pallor, fatigue, shortness of breath. Confirmation is by identification of changes to the alpha chain by genetic testing.
3A50.01	Thalassaemic alpha-chain variants
3A50.02	Haemoglobin H disease (– α/– – included) Haemoglobin H (HbH) disease is a moderate to severe form of alpha-thalassemia characterised by pronounced microcytic hypochromic haemolytic anaemia.
3A50.03	Homozygous or compound heterozygous alpha0 thalassaemia Hb Bart's hydrops fetalis is the most severe form of alpha-thalassemia and is almost always lethal. It is characterised by fetal onset of generalised oedema, pleural and pericardial effusions, and severe hypochromic anaemia.
3A50.0Y	Other specified alpha thalassaemia
3A50.0Z	Alpha thalassaemia, unspecified
3A50.1	Alpha thalassaemia related syndromes Alpha-thalassemia-related diseases refers to a group of diseases characterised by alpha-thalassemia and an associated disorder. Three conditions are included in this group: alpha-thalassemia-intellectual deficit syndrome, X-linked (or ATR-X syndrome), alpha-thalassemia-intellectual deficit syndrome and alpha-thalassemia-myelodysplastic disease or ATMDs.
3A50.2	Beta thalassaemia Beta-thalassemia (BT) is a haemoglobinopathy characterised by deficiency (Beta+) or absence (Beta0) of synthesis of the beta globin chains of haemoglobin (Hb). Three main types of BT have been described: minor, intermedia and major with clinical presentation ranging from asymptomatic forms to microcytic anaemia and splenomegaly due to defective erythropoiesis and haemolysis.
3A50.3	Delta, delta-beta or gamma-delta-beta thalassaemia Delta-beta-thalassemia is a form of beta-thalassemia characterised by decreased or absent synthesis of the delta- and beta-globin chains with a compensatory increase in expression of fetal gamma-chain synthesis.
3A50.4	Hereditary persistence of fetal haemoglobin Hereditary persistence of fetal haemoglobin (HPFH) associated with beta-thalassaemia is a haemoglobinopathy characterised by high haemoglobin (Hb)F levels and an increased number of fetal-Hb-containing-cells. The association of HPFH with beta-thalassaemia mitigates the clinical manifestations which vary from a normal state to beta-thalassaemia intermedia.
3A50.Y	Other specified thalassaemias
3A50.Z	Thalassaemias, unspecified

3A51**Sickle cell disorders or other haemoglobinopathies**

Any disorder caused by a HbS mutation in the haemoglobin gene. This disorder is characterised by abnormal rigid sickle-shaped red blood cells decreasing its ability to carry oxygen. This disorder may present with fatigue, shortness of breath, dizziness, headaches, pallor of skin or mucous membranes, and jaundice. This disorder is confirmed by identification of HbS mutation by genetic testing.

Coded Elsewhere: Osteonecrosis due to haemoglobinopathy (FB81.4)

Other sickle-cell disorders with retinopathy (9B71.Y)

3A51.0**Sickle cell trait**

A disease caused by genetic inheritance of one abnormal allele of the haemoglobin gene. This disease does not display the severe symptoms of sickle cell disease that occurs in homozygous individuals. Confirmation is by identification of mutation through genetic testing.

3A51.1**Sickle cell disease without crisis**

A disorder caused by a HbS mutation in the haemoglobin gene. This disorder is characterised by abnormal rigid sickle-shaped red blood cells decreasing its ability to carry oxygen. This disorder may present with fatigue, shortness of breath, dizziness, headaches, pallor of skin or mucous membranes, and jaundice. This disorder is confirmed by identification of HbS mutation by genetic testing.

3A51.2**Sickle cell disease with crisis**

Sickle cell crisis occurs when the sickle cells block blood flow, thus decreasing oxygen delivery to the tissues. This results in intense to severe pain in the extremities, lower back, abdomen, and chest. A crisis can be brought on by illness, stress, dehydration, exposure to temperature changes or high altitudes.

Inclusions: Hb-SS disease with crisis

3A51.3**Compound heterozygous sickling disorders without crisis**

A disease caused by genetic inheritance of two heterozygous recessive alleles of the haemoglobin gene leading to abnormal formation of haemoglobin molecule. This disease is characterised by rigid, sickle shaped red blood cells. Confirmation is by identification of mutations through genetic testing.

3A51.4**Compound heterozygous sickling disorders with crisis**

Compound heterozygous sickling disorders with crisis may present with acute chest syndrome, splenic sequestration, haemolytic crisis, and pain.

3A51.5**Haemoglobin C disease**

A disease caused by the bi-parental gene that encodes for haemoglobin C. This disease is characterised by abnormal structure of one of the globin chains of the haemoglobin molecule. This disease may present with mild haemolytic anaemia, increased risk for gallstones, enlarged spleen, episodes of joint pain, and increased risk of infection. This disease is confirmed by identification of the haemoglobin C gene by genetic testing.

Exclusions: Hereditary persistence of fetal haemoglobin (3A50.4)

3A51.6	Haemoglobin D disease Haemoglobin D (Hb D) disease is characterised by mild haemolytic anaemia and mild to moderate splenomegaly. Prevalence is unknown. Heterozygous forms of Hb D are clinically silent. Molecular testing can be useful to distinguish Hb D homozygosity from cases of heterozygous Hb D in association with beta-(0) thalassaemia.
3A51.7	High affinity haemoglobin A disease caused by determinants arising after birth, in the antenatal period or by genetically inherited factors leading to high oxygen affinity haemoglobin. This disease is characterised by abnormalities the globin chains that alter the affinity of the haemoglobin molecule for oxygen, affecting the normal loading of oxygen in the lungs and delivery of oxygen to the tissues.
3A51.8	Low affinity haemoglobin A disease caused by determinants arising after birth, in the antenatal period or by genetically inherited factors leading to low oxygen affinity haemoglobin. This disease is characterised by abnormalities the globin chains that alter the affinity of the haemoglobin molecule for oxygen, affecting the normal loading of oxygen in the lungs and delivery of oxygen to the tissues. This disease may present with fatigue, muscle weakness, loss of appetite, weight loss, diarrhoea, nausea, fast heartbeat or numbness in extremities.
3A51.9	Haemoglobin O disease A disease caused by the bi-parental inheritance of the gene that encodes for haemoglobin O. This disease is characterised by abnormal structure of one of the globin chains of the haemoglobin molecule. This disease may present with mild haemolytic anaemia, increased risk for gallstones, enlarged spleen, episodes of joint pain, and increased risk of infection. This disease is confirmed by identification of the haemoglobin O gene by genetic testing.
3A51.A	Haemoglobin E disease Haemoglobin E disease is characterised by the synthesis of an abnormal haemoglobin called haemoglobin E (HbE), instead of the normal haemoglobin A (HbA). Subjects heterozygous for HbE (AE) have an asymptomatic condition with no clinical relevance, except for the risk of transmitting E/beta thalassemia if the other parent carries beta thalassemia. The severity of these E/beta thalassemia forms is very variable, the clinical picture ranging from that of beta thalassemia minor through to thalassemia intermedia to thalassemia major. Subjects homozygous for HbE (EE) are asymptomatic.
3A51.B	Haemoglobin C/beta thalassaemia compound heterozygosity Haemoglobin C/beta thalassaemia is a condition resulting from coinheritance of haemoglobin C and beta thalassaemia, both beta globin genes being mutated.
3A51.Y	Other specified sickle cell disorders or other haemoglobinopathies
3A51.Z	Sickle cell disorders or other haemoglobinopathies, unspecified

Pure red cell aplasia (3A60-3A6Z)

A condition caused by determinates arising during the antenatal period, after birth or genetically inherited factors, leading to a change in the formation of erythrocytes. This condition is characterised by maturation arrest occurs in the formation of erythrocytes. This condition may present with severe anaemia. Confirmation is by identification of abnormally formed erythrocytes in a blood sample.

3A60	Congenital pure red cell aplasia A condition caused by determinants arising during the antenatal period, leading to a change in the formation of erythrocytes. This condition is characterised by maturation arrest occurs in the formation of erythrocytes. This condition may present with severe anaemia. Confirmation is by identification of decreased red blood cell count in a blood sample.
3A60.0	Congenital non-inherited pure red cell aplasia A condition caused by determinates arising during the antenatal period, leading to a change in the formation of erythrocytes. This condition is characterised by maturation arrest occurs in the formation of erythrocytes. This condition may present with severe anaemia. Confirmation is by identification of decreased levels of red blood cells in a blood sample.
3A60.1	Hereditary pure red cell aplasia A condition caused by determinates arising during the antenatal period, leading to a change in the formation of erythrocytes. This condition is characterised by maturation arrest occurs in the formation of erythrocytes. This condition may present with severe anaemia. Confirmation is by identification of decreased red blood cell count in a blood sample.
3A60.Z	Congenital pure red cell aplasia, unspecified
3A61	Acquired pure red cell aplasia A condition characterised by the near absence of red blood cell precursors in bone marrow, often associated with thymomas and autoimmune disorders Exclusions: Aplastic anaemia with paroxysmal nocturnal haemoglobinuria (3A70.1)
3A61.0	Acute acquired pure red cell aplasia This refers to transient (acute) and acquired type of anaemia affecting the precursors to red blood cells but not to white blood cells. In PRCA, the bone marrow ceases to produce red blood cells.
3A61.1	Chronic acquired pure red cell aplasia This refers to a chronic and acquired type of anaemia affecting the precursors to red blood cells but not to white blood cells. In PRCA, the bone marrow ceases to produce red blood cells.
3A61.Y	Other specified acquired pure red cell aplasia
3A61.Z	Acquired pure red cell aplasia, unspecified
3A6Z	Pure red cell aplasia, unspecified

3A70**Aplastic anaemia**

A disease caused by determinants arising after birth, in the antenatal period or genetically inherited factors leading to the inability of stem cells to generate new mature cells. This disease is characterised by low levels of red blood cells, white blood cells, and platelets. This disease may present with pallor, fatigue, dizziness, increased risk of infection or increased bruising or bleeding.

Inclusions: Medullary hypoplasia
Panmyelophthisis

3A70.0**Congenital aplastic anaemia**

A disease caused by determinants in the antenatal period leading to the inability of stem cells to generate new mature cells. This disease is characterised by low levels of red blood cells, white blood cells, platelets. This disease may present with pallor, fatigue, dizziness, increased risk of infection or increased bruising or bleeding.

Inclusions: familial hypoplastic anaemia
Constitutional medullar aplasia

Exclusions: Congenital amegakaryocytic thrombocytopenia (3B64.01)

Coded Elsewhere: Congenital hypoplastic anaemia (KA8C)
Noonan syndrome (LD2F.15)

3A70.1**Acquired aplastic anaemias**

A condition occurring secondary to other disorders or via an auto-immune response directed to the bone marrow arising after birth. This disease is characterised by an almost complete absence of hematopoietic stem cells resulting in low levels of red and white blood cells and platelets. This condition may present with fatigue, chronic infections, dizziness, weakness, headaches, and episodes of bleeding, usually in the skin and mucous membranes.

Inclusions: Acquired medullar aplasia

Coded Elsewhere: Paroxysmal nocturnal haemoglobinuria (3A21.0)
Myelofibrosis with myeloid metaplasia (2A20.2)

3A70.10**Drug-induced aplastic anaemia**

A disease caused by drug intake. This disease is characterised by inability of stem cells to generate new mature cells leading to low levels of red blood cells, white blood cells, platelets. This disease may present with pallor, fatigue, dizziness, increased risk of infection or increased bruising/bleeding.

3A70.11**Aplastic anaemia due to other external agents****3A70.12****Idiopathic aplastic anaemia****3A70.1Y****Other specified acquired aplastic anaemias****3A70.1Z****Acquired aplastic anaemias, unspecified****3A70.Z****Aplastic anaemia, unspecified**

3A71**Anaemia due to chronic disease**

A disease caused by chronic diseases such as chronic infection. This disease is characterised by inflammatory responses targeted at red blood cells leading to low levels of red blood cells in the body. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of low levels of red blood cells in a blood sample.

Coding Note: Code also the causing condition

3A71.0**Anaemia in neoplastic disease**

A disease caused by chronic neoplastic diseases. This disease is characterised by inflammatory responses targeted at red blood cell leading to low levels of red blood cells in the body. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of low levels of red blood cells in a blood sample

Coding Note: Code also the causing condition

3A71.1**Anaemia in chronic infectious diseases**

A disease caused by chronic infectious diseases leading to decreased levels of red blood cells in the blood. This disease is characterised by a low red blood cell count in the body. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of low red blood cell count in a blood sample.

Coding Note: Code also the causing condition

3A71.2**Anaemia in chronic kidney disease**

A disease caused by chronic kidney disease. This disease is characterised by a low red blood cell count in the blood. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of a low red blood cell count in a blood sample.

Coding Note: Code also the causing condition

3A71.Y**Anaemia due to other specified chronic disease**

Coding Note: Code also the causing condition

3A71.Z**Anaemia due to chronic disease, unspecified****3A72****Sideroblastic anaemia**

Sideroblastic anaemias are a group of disorders in which haemoglobin is insufficiently synthesized, because of defective use of iron (although plasmatic iron levels may be normal or elevated). They are said to be sideroblastic because of the presence of ringed sideroblasts in the blood due to accumulated ferritin in mitochondria. Anaemias may be microcytic hypochrome (in thalassemia and hereditary sideroblastic anaemias), or macrocytic (in idiopathic acquired sideroblastic anaemias).

3A72.0	Congenital sideroblastic anaemias A disease caused by determinants arising in the antenatal period leading to the production of ringed sideroblasts; abnormal nucleated erythroblasts. This disease is characterised by the inability of to incorporate haemoglobin, which red blood cells need to transport oxygen efficiently. This disease may present with pallor, fatigue, dizziness, and enlarged spleen and liver, heart disease, liver damage, or kidney failure.
3A72.00	Hereditary sideroblastic anaemias Inclusions: Sex-linked hypochromic sideroblastic anaemia
3A72.01	Hereditary syndromic sideroblastic anaemia Coded Elsewhere: Thiamine-responsive megaloblastic anaemia syndrome (5C63.Y)
3A72.0Y	Other specified congenital sideroblastic anaemias
3A72.0Z	Congenital sideroblastic anaemias, unspecified
3A72.1	Acquired sideroblastic anaemias A disease caused by determinants arising after birth such as myelodysplastic syndromes, antimicrobials, pyridoxine deficiency, lead poisoning, or copper deficiency. Zinc can indirectly cause sideroblastic anaemia by decreasing absorption and increasing excretion of copper. This disease is characterised by the inability of to incorporate haemoglobin, which red blood cells need to transport oxygen efficiently. This disease may present with pallor, fatigue, dizziness, and enlarged spleen and liver, heart disease, liver damage, or kidney failure. Coded Elsewhere: Refractory anaemia with ring sideroblasts (2A33)
3A72.Z	Sideroblastic anaemia, unspecified
3A73	Congenital dyserythropoietic anaemia Congenital dyserythropoietic anaemias (CDA) result from diverse erythropoietic disorders; they lead to the defective production of red blood cells (RBC) and often mild haemolysis that attests to a qualitative defect of these RBC released into the circulation. Three forms of CDA have been characterised: types I, II and III. The shared symptoms include anaemia of variable severity, intermittent jaundice, splenomegaly and hepatomegaly. Exclusions: Blackfan-Diamond syndrome (3A60.1) Di Guglielmo disease (2A60.35)

Polycythaemia (3A80-3A8Z)

Coded Elsewhere: Polycythaemia vera (2A20.4)

Polycythaemia neonatorum (KA8A)

3A80

Congenital polycythaemia

A disease caused by determinants occurring in the antenatal period leading to changes in the concentration of red blood cells. This disease is characterised by having a high concentration of red blood cells in the body leading to slow flow of blood. This disease may present with headaches, blurred vision, red skin, tiredness, high blood pressure, dizziness, periods of confusion, bleeding problems, gout or itchy skin. Confirmation is by identification of increased levels of red blood cells in a blood sample.

Coded Elsewhere: Polycythaemia neonatorum (KA8A)

3A80.0

Primary inherited erythrocytosis

A disease caused by genetically inherited factors leading to changes in the concentration of red blood cells. This disease is characterised by having a high concentration of red blood cells in the body leading to slow flow of blood. Confirmation is by identification of mutations by genetic testing.

3A80.Y

Other specified congenital polycythaemia

3A80.Z

Congenital polycythaemia, unspecified

3A81

Acquired polycythaemia

Secondary polycythaemia is acquired and caused by either natural or artificial increases in the production of erythropoietin, hence an increased production of erythrocytes.

3A81.0

Polycythaemia due to hypoxia, including high altitude

3A81.1

Polycythaemia due to over-transfusion or blood doping

3A81.2

Relative polycythaemia

A disease caused by loss of body fluids leading to apparent increased levels of red blood cells in the blood. This disease may present with headache, vertigo, abnormally enlarged spleen or liver, high blood pressure, or formation of blood clots. Confirmation is by identification of relative blood cell counts in a blood sample.

3A81.Y

Other specified acquired polycythaemia

3A81.Z

Acquired polycythaemia, unspecified

3A8Z

Polycythaemia, unspecified

3A90

Anaemia due to acute disease

Exclusions: Acute posthaemorrhagic anaemia (3A94)

3A91**Congenital methaemoglobinaemia**

A disease caused by determinants in the antenatal period leading to lack of the enzyme cytochrome b5 reductase. This disease is characterised by elevated levels of methemoglobin within the blood leading to haemoglobin ineffectively releasing oxygen to body tissues. This disease may present with shortness of breath, cyanosis, headache, fatigue, exercise intolerance, dizziness and loss of consciousness. Confirmation is by identification of mutation by genetic testing.

3A92**Hereditary methaemoglobinaemia**

Hereditary methemoglobinemia (HM) is a rare red cell disorder classified principally into two clinical phenotypes: autosomal recessive congenital (or hereditary) methemoglobinemia types I and II (RCM/RHM type 1; RCM/RHM type 2). In RCM type 1, well-tolerated cyanosis from birth is the only symptom. RCM type 2, with global loss of Cb5R function, is much more severe; the cyanosis is accompanied by neurological dysfunction (with intellectual deficit, microcephaly, growth retardation, opisthotonus, strabismus and hypertonia), which usually becomes evident during the first four months of life.

3A93**Acquired methaemoglobinaemia****3A94****Acute posthaemorrhagic anaemia**

A disease caused by blood loss such as subsequent to trauma. This disease is characterised by loss of blood from the body leading to low levels of red blood cells/blood in the body. This disease may present with pallor, fatigue, or shortness of breath. Confirmation is by identification of low levels of red blood cells in a blood sample.

Exclusions: congenital anaemia from fetal blood loss (KA8C)
Anaemia due to acute disease (3A90)

3A9Y**Other specified anaemias or erythrocyte disorders****3A9Z****Anaemias or other erythrocyte disorders, unspecified**

Coagulation defects, purpura or other haemorrhagic or related conditions (3B10-3B6Z)

A condition caused by determinants arising during the antenatal period, after birth or by genetically inherited factors, leading to coagulation defects. This condition is characterised by increased bruising and bleeding.

Exclusions: Postpartum coagulation defects (JA43.3)

Coagulation defects (3B10-3B4Z)

Congenital or constitutional haemorrhagic condition (3B10-3B1Z)

A condition caused by determinants arising during the antenatal period or genetically inherited factors, leading to defects in clotting mechanisms or abnormalities causing structural flaws in the blood vessels. This disease is characterised by spontaneous bleeding or bruising.

Coded Elsewhere: Congenital non-inherited haemorrhagic condition

3B10

Hereditary factor VIII deficiency

A disease caused by a genetically inherited mutation leading to a deficiency in clotting due to lack of factor VIII. This disease is characterised by increasing haemorrhaging and bruising. Confirmation is by identification of mutations by genetic testing.

3B10.0

Haemophilia A

Haemophilia A is the most common form of haemophilia characterised by spontaneous or prolonged haemorrhages due to factor VIII deficiency. Depending on the extent of the factor VIII deficiency, it can be severe (biological activity of factor VIII below 1%), moderately severe (activity of factor VIII between 1% and 5%), or mild (activity of factor VIII between 5 and 40%).

Exclusions: factor VIII deficiency with vascular defect (3B12)

3B10.1

Hereditary factor VIII deficiency with anti-factor VIII inhibitor

A disease caused by a genetically inherited mutation leading to a deficiency in clotting due to lack of factor VIII. This disease also causes anti-factor VIII inhibitor antibodies to be produced when receiving transfusions. Anti-factor VIII inhibitor antibodies develop as the body recognises the factor VIII as foreign, therefore deeming factor VIII infusions ineffective. This disease is characterised by increasing haemorrhaging and bruising. Confirmation is by identification of mutations by genetic testing.

3B10.Y

Other specified hereditary factor VIII deficiency

3B10.Z

Hereditary factor VIII deficiency, unspecified

3B11

Hereditary factor IX deficiency

A disease caused by a genetically inherited X-linked recessive trait leading to a defective gene located on the X chromosome. This disease is characterised by low levels of the protein factor IX in the body leading to increased haemorrhaging and bruising due to clotting abnormalities. Confirmation is by identification of recessive trait by genetic testing.

3B11.0	Haemophilia B Haemophilia B is a form of haemophilia characterised by spontaneous or prolonged haemorrhages due to factor IX deficiency. Depending on the extent of the factor IX deficiency, it can be severe (biological activity of factor IX below 1%), moderately severe (activity of factor IX between 1% and 5%), or mild (activity of factor IX between 5 and 40%). Inclusions: PTC - [plasma thromboplastin component] deficiency
3B11.Y	Other specified hereditary factor IX deficiency
3B11.Z	Hereditary factor IX deficiency, unspecified
3B12	Von Willebrand disease A disease caused by inherited genetic mutations. This disease is characterised by quantitative, structural or function abnormalities of Von Willebrand factor leading to abnormalities in coagulation of the blood. This disease may present with prolonged bleeding, easy bruising or, bleeding gums. Confirmation is by identification of mutation through genetic testing. Inclusions: Factor VIII deficiency with vascular defect Vascular haemophilia Angiohaemophilia Exclusions: factor VIII deficiency with functional defect (3B10) factor VIII deficiency NOS (3B10) Acquired von Willebrand disease or syndrome (3B20-3B2Y)
3B13	Haemophilia C A disease caused by genetically inherited mutations. This disease is characterised by decreased levels of factor XI leading to abnormalities in coagulation of the blood. This disease may present with prolonged bleeding, easy bruising or, bleeding gums. Confirmation is by identification of mutation through genetic testing.
3B14	Other inherited coagulation factor deficiency with bleeding tendency Any disease caused by genetically inherited mutations leading to lack of coagulation factors in the blood not elsewhere classified. These diseases are characterised by increased haemorrhaging and bruising as the blood cannot clot properly to control bleeding. Confirmation is identification of mutations by genetic testing.
3B14.0	Hereditary deficiency of factor I Congenital deficiencies of fibrinogen are coagulation disorders characterised by bleeding symptoms ranging from mild to severe resulting from reduced quantity and/or quality of circulating fibrinogen. Afibrinogenaemia (complete absence of fibrinogen) and hypofibrinogenaemia (reduced plasma fibrinogen concentration) correspond to quantitative anomalies of fibrinogen while dysfibrinogenaemia corresponds to a functional anomaly of fibrinogen. Hypo- and dysfibrinogenaemia may be frequently combined (hypodysfibrinogenaemia).

- 3B14.1 Hereditary factor X deficiency**
 Congenital factor X deficiency is an inherited bleeding disorder with a decreased antigen and/or activity of factor X (FX) and characterised by mild to severe bleeding symptoms.
- 3B14.2 Combined deficiency of vitamin K-dependent clotting factors**
 Hereditary combined vitamin K-dependent clotting factors deficiency (VKCFD) is a congenital bleeding disorder resulting from variably decreased levels of coagulation factors II, VII, IX and X, as well as natural anticoagulants protein C, protein S and protein Z.
- 3B14.Z Other inherited coagulation factor deficiency with bleeding tendency, unspecified**
- 3B15 Inherited coagulation factor deficiency without bleeding tendency**
 A disease caused by a genetically inherited mutation leading to decreased levels of coagulation factor. This disease is characterised by decreased levels of coagulation factor without leading to increased haemorrhaging. Confirmation is by identification of decreased levels of coagulation factor in a blood sample.
- 3B1Z Congenital or constitutional haemorrhagic condition, unspecified**

Haemorrhagic diseases due to acquired coagulation factor defects (3B20-3B2Y)

Any disease caused by determinants arising after birth. These diseases are characterised by abnormal coagulation of the blood.

Exclusions: vitamin K deficiency of newborn (KA8F.0)

- 3B20 Disseminated intravascular coagulation**
 A disorder that is characterised by the systemic intravascular activation of the coagulation system, simultaneously leading to intravascular thrombi, compromising an adequate blood supply to the organs, and to bleeding as the consequence of consumption of platelets and coagulation factors. It may be provoked by a wide range of disorders including infections, inflammatory disorders and malignancy.
- Coded Elsewhere:** Disseminated intravascular coagulation of fetus or newborn (KA88)

- 3B21 Haemorrhagic disorder due to circulating anticoagulants and coagulation factors**
 A disease caused by anticoagulants present in the body that prevent the blood from clotting normally. This disease is characterised by abnormalities in blood clotting. This disease may present with prolonged bleeding, easy bruising or, bleeding gums. Confirmation is by identification of anticoagulants present in a blood sample.
- Exclusions:** long-term use of anticoagulants without haemorrhage (QC48.0)

3B21.0	Haemorrhage due to thrombin inhibitor other than heparin A disease caused by any thrombin inhibitors other than heparin that affects normal coagulation of the blood. This disease is characterised by inability of the blood to coagulate leading to bleeding. Confirmation is by identification of thrombin inhibitors in a blood sample.
3B21.1	Haemorrhage due to factor Xa inhibitor A disease caused by factor Xa inhibitor that affects normal coagulation of the blood. This disease is characterised by inability of the blood to coagulate leading to bleeding. Confirmation is by identification of factor Xa inhibitor in a blood sample.
3B21.Y	Haemorrhagic disorder due to other specified circulating anticoagulants
3B21.Z	Haemorrhagic disorder due to unspecified circulating anticoagulants
3B22	Acquired haemophilia Acquired haemophilia is a rare haemorrhagic disease caused by production of anti-factor VIII antibodies and is sometimes associated with other autoimmune disorders, cancers lymphoproliferative syndromes and multiple transfusions during the postpartum period.
3B2Y	Other specified haemorrhagic diseases due to acquired coagulation factor defects
3B4Z	Coagulation defects, unspecified

Fibrinolytic defects (3B50-3B51)

A disease caused by determinants arising during the antenatal period, after birth or genetically inherited factors, affecting the fibrinolysis system which prevents blood clots from growing and becoming problematic. This disease is characterised by defects in the fibrinolysis system leading to coagulation of the blood. This disease may present with thrombosis.

3B50	Inherited fibrinolytic defects A disease caused by genetically inherited mutations affecting the fibrinolysis system which prevents blood clots from growing and becoming problematic. This disease is characterised by defects in the fibrinolysis system leading to coagulation of the blood. This disease may present with thrombosis. Coded Elsewhere: Hypoplasminogenaemia (DA0D.3)
3B50.0	Congenital alpha-2 antiplasmin deficiency
3B50.1	Congenital plasminogen activator inhibitor type 1 deficiency Congenital plasminogen activator inhibitor type 1 (PAI-1) deficiency is a disorder that causes premature lysis of haemostatic clots and a moderate bleeding syndrome. Spontaneous bleeding is rarely observed, whereas moderate haemorrhages of the knees, elbows, nose and gingiva are usually triggered by mild trauma. However, menstrual bleeding may be severe and a prolonged bleeding after surgery is common. The PAI-1 deficiency may be qualitative or quantitative, total or partial.
3B50.Y	Other specified inherited fibrinolytic defects

3B50.Z**Inherited fibrinolytic defects, unspecified****3B51****Acquired fibrinolytic defects**

A disease caused by determinates arising after birth, affecting the fibrinolysis system which prevents blood clots from growing and becoming problematic. This disease is characterised by defects in the Fibrinolysis system leading to coagulation of the blood. This disease may present with thrombosis.

3B60**Non-thrombocytopenic purpura**

A descriptive term for purpura caused by determinants other than low platelet count. This should be used for coding only when a more precise diagnosis is not available.

Exclusions:

- Antineutrophil cytoplasmic antibody-associated vasculitis (4A44.A)
- Antiphospholipid syndrome (4A45)
- Drug-associated immune complex vasculitis (4A85.03)
- Immune complex small vessel vasculitis (4A44.9)
- Leukocytoclastic vasculitis (4A44.B)
- Purpura or bruising due to vascular fragility (EE40.32)
- Thrombotic thrombocytopenic purpura (3B64.14)
- Traumatic purpura (EF31)

3B60.0**Hereditary vascular purpura****3B60.1****Acquired vascular purpura**

Purpura resulting from vascular factors rather than from abnormalities in the blood such as dysproteinaemias and disorders of platelets and coagulation.

Exclusions:

- Antineutrophil cytoplasmic antibody-associated vasculitis (4A44.A)
- Antiphospholipid syndrome (4A45)
- Capillaritis (EF40.0)
- Drug-associated immune complex vasculitis (4A85.03)
- IgA vasculitis (4A44.92)
- Leukocytoclastic vasculitis (4A44.B)
- Purpura or bruising due to vascular fragility (EE40.32)
- Thrombotic thrombocytopenic purpura (3B64.14)
- Traumatic purpura (EF31)

3B61**Thrombophilia**

A disease caused by determinants arising after birth or genetically inherited factors leading to abnormalities in blood. This disease is characterised by abnormality of blood coagulation that increases the risk of thrombosis, clots in blood vessels. This disease may present with deep vein thrombosis or pulmonary embolism. Confirmation is identification of abnormal blood coagulation in a blood sample.

3B61.0	Hereditary thrombophilia A disease caused by hereditary factors leading to abnormalities in blood. This disease is characterised by abnormality of blood coagulation that increases the risk of thrombosis, clots in blood vessels. This disease may present with deep vein thrombosis or pulmonary embolism. Confirmation is identification of abnormal blood coagulation in a blood sample. <i>Inclusions:</i> antithrombin deficiency
3B61.00	Hyperhomocysteinaemia A disease caused by deficiencies of vitamin B6, folic acid, or vitamin B12. Genetic defects in 5-MTHF reductase can consequently lead to hyperhomocysteinaemia. This disease is characterised by abnormally high level of homocysteine in the blood. This disease may present with cardiovascular disease, thrombosis, schizophrenia and osteoporosis. Confirmation is by identification of deficiency in a blood sample.
3B61.0Y	Other specified hereditary thrombophilia
3B61.1	Acquired thrombophilia A disease caused by determinants arising after birth. This disease is characterised by abnormality of blood coagulation that increases the risk of thrombosis, clots in blood vessels. This disease may present with deep vein thrombosis or pulmonary embolism. Confirmation is identification of abnormal blood coagulation in a blood sample. <i>Coded Elsewhere:</i> Antiphospholipid syndrome (4A45)
3B61.Y	Other specified thrombophilia
3B61.Z	Thrombophilia, unspecified
3B62	Qualitative platelet defects A disease caused by determinants arising after birth, during the antenatal period or genetically inherited factors. This disease is characterised by abnormalities in coagulation of the blood due to defective platelets. This condition may present with easy bruising, prolonged bleeding or, bleeding gums. Confirmation is by identification of decreased platelets in a blood sample. <i>Inclusions:</i> Thrombocytopathy <i>Exclusions:</i> Von Willebrand disease (3B12)
3B62.0	Inherited qualitative platelet defects A disease caused by genetically inherited mutations leading to abnormalities in platelets. This disease is characterised by abnormal platelet formation or function. Confirmation is by identification of mutations by genetic testing. <i>Coded Elsewhere:</i> Dense granule disease (3B62.3) Alpha-delta dense granule deficiency (3B62.4)

3B62.00	Alpha-granule diseases A condition caused by determinants arising after birth, in the antenatal period. This condition is characterised by defects in the alpha granules in platelets leading to abnormalities in coagulation mechanisms. This condition may present with prolonged bleeding, epistaxis, menorrhagia, easy bruising, anaemia, fatigue or shortness of breath. Confirmation is by identification of platelet defects in a blood sample.
3B62.01	Inherited giant platelet disorder A disease caused by genetically inherited mutations. This disease is characterised by abnormally large platelets, low platelet count and a bleeding tendency. Confirmation is by identification of mutations through genetic testing. Coded Elsewhere: MYH9 macrothrombocytopenia syndromes (3B64.01)
3B62.0Y	Other specified inherited qualitative platelet defects
3B62.0Z	Inherited qualitative platelet defects, unspecified
3B62.1	Bleeding diathesis due to thromboxane synthesis deficiency A disease caused by thromboxane synthesis deficiency. This disease is characterised by low levels of eicosanoids (lipids), abnormalities in coagulation leading to haemorrhaging. Confirmation is by identification of low levels of eicosanoids in a blood sample.
3B62.2	Isolated thrombocytopenia
3B62.3	Dense granule disease A condition caused by determinants arising after birth, in the antenatal period. This condition is characterised by defects in the dense granules in platelets leading to abnormalities in coagulation mechanisms. This condition may present with prolonged bleeding, epistaxis, menorrhagia, easy bruising, anaemia, fatigue or shortness of breath. Confirmation is by identification of platelet defects in a blood sample. Coded Elsewhere: Hermansky-Pudlak syndrome (EC23.20) Chédiak-Higashi syndrome (EC23.20)
3B62.4	Alpha-delta dense granule deficiency A condition caused by determinants arising after birth, in the antenatal period. This condition is characterised by defects in the alpha delta dense granules in platelets, leading to abnormalities in coagulation mechanisms. This condition may present with prolonged bleeding, epistaxis, menorrhagia, easy bruising, anaemia, fatigue or shortness of breath. Confirmation is by identification of low levels of alpha delta dense granules in a blood sample.
3B62.5	Haemophagocytic syndrome associated with infection This is an uncommon hematologic disorder that, typically, clinically manifests as fever, hepatosplenomegaly, lymphadenopathy, jaundice and rash, with laboratory findings of histiocytosis, and the pathologic finding of haemophagocytosis, infection-associated.
3B62.Y	Other specified qualitative platelet defects

3B62.Z Qualitative platelet defects, unspecified

3B63 Thrombocytosis

A disease caused by essential thrombocytosis or other myelo-proliferative disorders such as chronic myelogenous leukaemia, polycythaemia, myelofibrosis. This disease can also have secondary causes such as inflammation, surgery, hyposplenism, Splenectomy, asplenia, iron deficiency anaemia or haemorrhage. This disease is characterised by elevated platelet count in the blood. Confirmation is by identification of increased platelet count in a blood sample.

3B63.0 Congenital thrombocytosis

Familial thrombocytosis is a type of thrombocytosis, a sustained elevation of platelet numbers, which affects the platelet/megakaryocyte lineage and may create a tendency for thrombosis and haemorrhage but does not cause myeloproliferation.

Inclusions: Hereditary thrombocytosis

Exclusions: Essential thrombocythemia (3B63.1)

3B63.1 Acquired thrombocytosis

A chronic myeloproliferative neoplasm that involves primarily the megakaryocytic lineage. It is characterised by sustained thrombocytosis in the blood, increased numbers of large, mature megakaryocytes in the bone marrow, and episodes of thrombosis and/or haemorrhage. Progression to a post -essential thrombocythaemia myelofibrosis stage or transformation to acute myeloid leukaemia is rarely observed.

Inclusions: Idiopathic haemorrhagic thrombocythaemia

3B63.10 Secondary thrombocytosis

Coding Note: Code also the causing condition

3B63.1Y Other specified acquired thrombocytosis

3B63.1Z Acquired thrombocytosis, unspecified

3B63.Y Other specified thrombocytosis

3B63.Z Thrombocytosis, unspecified

3B64 Thrombocytopenia

This disease is characterised by decreased levels of platelets within the blood. This disease may present with increased bruising or haemorrhaging. Confirmation is by identification of decreased platelet count in a blood sample.

Coded Elsewhere: Isolated thrombocytopenia (3B62.2)

3B64.0 Congenital thrombocytopenia

A disease caused by determinants arising during the antenatal period leading to low platelet count. This disease is characterised by decreased levels of platelets within the blood. This disease may present with increased bruising or haemorrhaging. Confirmation is by identification of a decreased platelet count in a blood sample.

3B64.00	Congenital non-inherited thrombocytopenia Coded Elsewhere: Transient neonatal thrombocytopaenia (KA89)
3B64.01	Hereditary thrombocytopenia A disease caused by a genetically inherited mutation leading to decreased platelet count. This disease is characterised by decreased levels of platelets within the blood. This disease may present with increased bruising or haemorrhaging. Confirmation is by identification of decreased platelet count in a blood sample. Coded Elsewhere: Thrombocytopaenia - absent radius (LD2F.1Y) Familial platelet syndrome with predisposition to acute myelogenous leukaemia (3B62.3) Congenital thrombotic thrombocytopenic purpura due to ADAMTS-13 deficiency (3B64.14) Macrothrombocytopenia with mitral valve insufficiency (3B62.01)
3B64.0Z	Congenital thrombocytopenia, unspecified
3B64.1	Acquired thrombocytopenia A disease caused by determinants arising after birth, leading to low platelet count. This disease is characterised by low levels of platelets within the blood. This disease may present with increased bruising or haemorrhaging. Confirmation is by identification of decreased platelet count in a blood sample.
3B64.10	Immune thrombocytopenic purpura Immune thrombocytopenic purpura (or immune thrombocytopenia; ITP) is an autoimmune coagulation disorder characterised by isolated thrombocytopenia (a platelet count <100,000/microL), in the absence of any underlying disorder that may be associated with thrombocytopenia. Coded Elsewhere: Evans syndrome (3A20.5)
3B64.11	Secondary thrombocytopenic purpura This disease is characterised by a relative decrease in levels of platelets within the blood. This disease may present with increased bruising or haemorrhaging. Confirmation is by identification of decreased platelets in a blood sample.
Coding Note:	Code also the causing condition
3B64.12	Drug-induced thrombocytopenic purpura Thrombocytopenic purpura attributable to drug toxicity (e.g. cytotoxic chemotherapeutic or immunosuppressive agents) or to an idiosyncratic drug-associated allergic thrombocytopenia (e.g. quinine, thiazides).
3B64.13	Alloimmune thrombocytopenia A disease caused by determinants such as a blood transfusion that lead to an immune response to the foreign antigens. This disease is characterised by low levels of platelets in the body due to an immune reactive response to the foreign platelet antigens. This disease may present with increased bruising or haemorrhaging. Confirmation is by identification of decreased platelet count and presence of autoantibodies in a blood sample.

3B64.14 Thrombotic thrombocytopenic purpura
This condition is idiopathic. This condition is characterised by abnormality of blood coagulation causing extensive microscopic clots to form in the small blood vessels throughout the body resulting in low platelet count. This condition may present with seizures, hemiplegia, paresthesias, visual disturbance, and aphasia or anaemia. Confirmation is by identification of thromboses.

3B64.1Y Other specified acquired thrombocytopenia

3B64.Z **Thrombocytopenia, unspecified**

3B65 **Thrombotic microangiopathy, not elsewhere classified**
Thrombotic microangiopathies are microvascular occlusive disorders characterised by systemic or intrarenal aggregation of platelets, thrombocytopenia, and mechanical injury to erythrocytes. Thrombotic thrombocytopenic purpura (TTP) and haemolytic–uremic syndrome (HUS) represent a spectrum of thrombotic microangiopathies. In TTP, systemic microvascular aggregation of platelets causes ischemia in the brain and other organs. In HUS, platelet–fibrin thrombi predominantly occlude the renal circulation.

Coded Elsewhere: Thrombotic thrombocytopenic purpura (3B64.14)

Haemolytic uraemic syndrome (3A21.2)

Methylcobalamin deficiency type cbl G (5C50.B)

Hereditary haemolytic uraemic syndrome (3A10.Y)

3B6Y **Other specified coagulation defects, purpura or other haemorrhagic or related conditions**

3B6Z **Coagulation defects, purpura or other haemorrhagic or related conditions, unspecified**

Diseases of spleen (3B80-3B8Z)

3B80 **Congenital disorders of spleen**

Any condition caused by a failure of the spleen to correctly develop in the antenatal period.

Coded Elsewhere: Structural developmental anomalies of spleen (LB22)

3B80.0 **Splenomegaly in storage diseases**

A disease caused by storage diseases; genetically inherited metabolic disorders that result from defects in lysosomal, lipid or glycogen function, of the spleen. This disease is characterised by enlargement of the spleen. This disease may present with abdominal pain, chest pain, pallor, shortness of breath fatigue. Confirmation is through medical imaging.

3B81**Acquired disorders of spleen**

Any condition caused by determinants acquired after birth, leading to dysfunction of the spleen.

Coded Elsewhere: Injury of spleen (NB91.0)

Malignant neoplasms of the spleen (2C11.Z)

3B81.0**Tumour-like conditions of spleen**

Any condition caused by determinants acquired after birth, in the antenatal period or genetically inherited factors, leading to tumour-like conditions of the spleen. Confirmation is through medical imaging.

3B81.1**Postsurgical asplenia**

A disease caused by underlying diseases, splenectomy or splenic rupture from trauma. This disease is characterised by absence of normal spleen function. This disease may present with increased susceptibility to infection. Confirmation is through medical imaging.

3B81.2**Atrophy of spleen**

A disease caused by determinants arising after birth, during the antenatal period or by genetically inherited factors. This disease is characterised by partial or complete degradation of the spleen. This disease may present with increased susceptibility to infection. Confirmation is through medical imaging.

3B81.3**Nontraumatic laceration or rupture of spleen**

A disease caused by non-traumatically determinants such as infectious diseases, medical procedures such as colonoscopy, haematological diseases, medications, or pregnancy. This disease is characterised by laceration or rupturing of the spleen leading to lack of function. This disease may present with bleeding and increased susceptibility of infection. Confirmation is through medical imaging.

3B81.4**Splenosis**

A disease caused by determinants arising after birth such as physical trauma or splenectomy. This disease is characterised by autoimplantation one or more focal deposits of splenic tissue in various compartments of the body. Confirmation is through medical imaging.

3B81.5**Splenic cyst or pseudocyst****3B81.50****Pseudocyst of spleen**

A disease caused by determinants arising after birth, during the antenatal period or by genetically inherited factors. This disease is characterised by a noncancerous fluid-filled sac, pseudocysts are like cysts, but lack epithelial or endothelial cells. This disease is often asymptomatic but may present with abdominal pain, nausea and vomiting. Confirmation is through medical imaging.

3B81.51**Epithelial cyst of spleen****3B81.5Y****Other specified splenic cyst****3B81.5Z****Splenic cyst, unspecified**

3B81.6	Infarction of spleen A disease caused by determinants such as trauma, infection, or inherited factors leading to a shortage of oxygen in the spleen. This disease is characterised by death of spleen tissue and loss of function. Confirmation is by medical imaging. <i>Exclusions:</i> traumatic rupture of spleen (NB91.0) <i>Coded Elsewhere:</i> Neonatal haemorrhage originating in spleen (KA83.5)
3B81.7	Infection of spleen Any condition of the spleen, caused by an infection with a bacterial, viral, fungal, or parasitic source.
3B81.70	Acute septic splenitis <i>Inclusions:</i> septic spleen
3B81.71	Abscess of spleen This is a collection of pus (neutrophils) that has accumulated within a tissue because of an inflammatory process in response to either an infectious process (usually caused by bacteria or parasites) or other foreign materials (e.g., splinters, bullet wounds, or injecting needles), in the spleen.
3B81.7Y	Other specified infection of spleen
3B81.7Z	Infection of spleen, unspecified
3B81.8	Torsion of spleen A disease caused by abnormal development of splenic suspensory ligaments. This disease is characterised by twisting of the spleen leading to splenic infarction. This disease may present with abdominal pain. Confirmation is through medical imaging.
3B81.9	Fibrosis of spleen A disease caused by determinants arising after birth, during the antenatal period or by genetically inherited factors. This disease is characterised by formation of excess fibrous connective tissue leading to partial or complete degradation of the spleen. This disease may present with increased susceptibility to infection. Confirmation is through medical imaging.
Coding Note:	Code also the causing condition
3B81.A	Perisplenitis A disease caused by bacterial or viral infection, parasite infestation, or cysts. This disease is characterised by inflammation of the peritoneal surface of the spleen. This disease may present with abdominal pain, susceptibility to infection and enlargement of the spleen. Confirmation is by identification of infection in a blood sample.

3B81.B	Hypersplenism A disease caused by determinants such as cirrhosis, malaria, tuberculosis or inflammatory disorders leading overactive spleen function. This disease is characterised by the presence of an enlarged spleen. Confirmation is by identification through medical imaging. Exclusions: Splenomegaly, not elsewhere classified (ME10.01) congenital splenomegaly (LB22)
3B81.C	Chronic congestive splenomegaly A form of exaggerated spleen function characterised by splenic enlargement secondary to splenic vein thrombosis and/or portal hypertension
3B81.Y	Other specified acquired disorders of spleen
3B8Z	Diseases of spleen, unspecified
3C0Y	Other specified diseases of the blood or blood-forming organs
3C0Z	Diseases of the blood or blood-forming organs, unspecified

CHAPTER 04

Diseases of the immune system

This chapter has 45 four-character categories.

Code range starts with 4A00

Exclusions: Complications of pregnancy, childbirth and the puerperium (Chapter 18)

Neoplasms (Chapter 02)

Developmental anomalies (Chapter 20)

Coded Elsewhere: Organ specific autoimmune disorders

Symptoms, signs or clinical findings of blood, blood-forming organs, or the immune system (MA00-MA3Y)

This chapter contains the following top level blocks:

- Primary immunodeficiencies
- Nonorgan specific systemic autoimmune disorders
- Autoinflammatory disorders
- Allergic or hypersensitivity conditions
- Immune system disorders involving white cell lineages
- Certain disorders involving the immune system
- Organ specific autoimmune disorders

Primary immunodeficiencies (4A00-4A0Z)

4A00

Primary immunodeficiencies due to disorders of innate immunity

Coded Elsewhere: Constitutional neutropaenia (4B00.00)

4A00.0

Functional neutrophil defects

Inclusions: Congenital dysphagocytosis

Coded Elsewhere: Haemolytic anaemia due to glucose-6-phosphate dehydrogenase deficiency (3A10.00)

Papillon-Lefèvre syndrome (EC20.30)

4A00.00

Neutrophil immunodeficiency syndrome

Neutrophil immunodeficiency syndrome is a primary immunodeficiency characterised by neutrophilia with severe neutrophil dysfunction, leukocytosis, a predisposition to bacterial infections and poor wound healing, including an absence of pus in infected areas.

4A00.0Y

Other specified functional neutrophil defects

4A00.0Z	Functional neutrophil defects, unspecified
4A00.1	Defects in the complement system
	Exclusions: Atypical haemolytic uraemic syndrome (3A10) Paroxysmal nocturnal haemoglobinuria (3A21.0)
4A00.10	Immunodeficiency with an early component of complement deficiency
4A00.11	Immunodeficiency with a late component of complement deficiency
4A00.12	Immunodeficiency with factor B deficiency
4A00.13	Immunodeficiency with factor D anomaly Factor D deficiency is an autosomal recessive immunologic disorder characterised by increased susceptibility to bacterial infections, particularly <i>Neisseria</i> infections, due to a defect in the alternative complement pathway.
4A00.14	Hereditary angioedema Hereditary angioedema is caused in the majority of cases by genetically determined low absolute (type I) or functional (type II) levels of C1 inhibitor, a plasma proteinase inhibitor involved in regulation of complement activation. It is characterised clinically by recurrent subcutaneous and/or submucosal oedema and can result in life-threatening laryngeal obstruction. Involvement of the digestive tract commonly causes abdominal pain. This and the absence of accompanying urticarial weals or itch distinguish it from the common form of angioedema, which is part of the spectrum of urticaria.
4A00.15	Acquired angioedema Acquired angioedema is clinically similar to hereditary angioedema and is not associated with urticaria. It may be associated with a lymphoproliferative disorder (type I) or may be an isolated phenomenon due to an autoantibody directed against C1 inhibitor (type II).
4A00.1Y	Other specified defects in the complement system
4A00.1Z	Defects in the complement system, unspecified
4A00.2	Genetic susceptibility to particular pathogens
	Coded Elsewhere: Encephalitis due to herpes simplex virus (1F00.21) Chronic mucocutaneous candidosis (1F23.14)
4A00.3	Immunodeficiency with natural-killer cell deficiency
4A00.Y	Other specified primary immunodeficiencies due to disorders of innate immunity
4A00.Z	Primary immunodeficiencies due to disorders of innate immunity, unspecified
4A01	Primary immunodeficiencies due to disorders of adaptive immunity
4A01.0	Immunodeficiencies with predominantly antibody defects A disorder characterised by an inability to mount a normal immune response due to antibody (i.e. immunoglobulin) defects

4A01.00	Hereditary agammaglobulinaemia with profoundly reduced or absent B cells This refers to a hereditary type of primary immune deficiency disease characterised by a reduction in all types of gamma globulins, and rare X-linked genetic disorder that affects the body's ability to fight infection.
4A01.01	Immunodeficiencies with severe reduction in at least two serum immunoglobulin isotypes with normal or low numbers of B cells This refers to a nonfamilial type of primary immune deficiency disease characterised by a reduction in at least two serum immunoglobulin isotypes. Circulating B cells may be normal or low.
4A01.02	Specific antibody deficiency with normal immunoglobulin concentrations or normal number of B cells
4A01.03	Transient hypogammaglobulinaemia of infancy
4A01.04	Immunodeficiencies with isotype or light chain deficiencies with normal number of B cells
4A01.05	Immunodeficiencies with severe reduction in serum IgG or IgA with normal or elevated IgM and normal numbers of B-cells Coded Elsewhere: Hyper-IgM syndrome due to CD40 ligand deficiency (4A01.1Y) Hyper-IgM syndrome due to CD40 deficiency (4A01.1Y)
4A01.0Y	Other specified immunodeficiencies with predominantly antibody defects
4A01.0Z	Immunodeficiencies with predominantly antibody defects, unspecified
4A01.1	Combined immunodeficiencies Exclusions: autosomal recessive agammaglobulinaemia (Swiss type) (4A01.00) Coded Elsewhere: Laron syndrome with immunodeficiency (5A61.0)
4A01.10	Severe combined immunodeficiencies Severe combined immunodeficiency (SCID) comprises a group of rare monogenic primary immunodeficiency disorders characterised by a lack of functional peripheral T lymphocytes resulting in early-onset severe respiratory infections and failure to thrive.
4A01.11	Major histocompatibility complex class I deficiency
4A01.12	Major histocompatibility complex class II deficiency Immunodeficiency by defective expression of HLA class II is an autosomal recessive primary immune deficiency, manifesting by recurrent viral and bacterial infections, often leading to chronic diarrhoea and growth retardation.
4A01.1Y	Other specified combined immunodeficiencies
4A01.1Z	Combined immunodeficiencies, unspecified
4A01.2	Diseases of immune dysregulation

4A01.20	Immune dysregulation syndromes with hypopigmentation Coded Elsewhere: Hermansky-Pudlak syndrome (EC23.20) Chédiak-Higashi syndrome (EC23.20) Griscelli syndrome type 2 (4A01.23)
4A01.21	Immune dysregulation syndromes presenting primarily with autoimmunity Coded Elsewhere: Autoimmune polyendocrinopathy type 1 (5B00) Syndromic multisystem autoimmune disease due to ITCH deficiency (4A43.Y) Aicardi-Goutières syndrome (5C55.2) Spondylometaphyseal dysplasia with combined immunodeficiency (LD24.4)
4A01.22	Immune dysregulation syndromes presenting primarily with lymphoproliferation
4A01.23	Primary haemophagocytic lymphohistiocytosis A disease caused by determinants arising after birth, during the antenatal period or genetically inherited factors leading to uncontrolled proliferation of activated lymphocytes and macrophages. This disease is characterised by increased proliferation of morphologically benign lymphocytes and macrophages that secrete high amounts of inflammatory cytokines. This disease may present with fever, rash, jaundice, splenomegaly, lymphadenopathy, histiocytosis, haemophagocytosis, or cytopenia. Inclusions: Histiocytoses of mononuclear phagocytes Coded Elsewhere: Hermansky-Pudlak syndrome (EC23.20) Chédiak-Higashi syndrome (EC23.20)
4A01.2Y	Other specified diseases of immune dysregulation
4A01.2Z	Diseases of immune dysregulation, unspecified
4A01.3	Other well-defined immunodeficiency syndromes due to defects in adaptive immunity This refers to other defects in the highly specialized, systemic cells and processes that eliminate or prevent pathogen growth. Coded Elsewhere: Wiskott-Aldrich syndrome (3B62.0Y) Netherton syndrome (LD27.2) Dyskeratosis congenita (3A70.0)
4A01.30	Immunodeficiency due to defects of the thymus Coded Elsewhere: CATCH 22 phenotype (LD44.N0)
4A01.31	DNA repair defects other than combined T-cell or B-cell immunodeficiencies
4A01.32	Immuno-osseous dysplasia This is an autosomal recessive disorder with the diagnostic features of spondyloepiphyseal dysplasia, renal dysfunction, and T-cell immunodeficiency. Coded Elsewhere: Cartilage-hair hypoplasia (LD27.0Y)

- 4A01.33** Hepatic veno-occlusive disease - immunodeficiency syndrome
Hepatic veno-occlusive disease - immunodeficiency syndrome is characterised by the association of severe hypogammaglobulinemia, combined T and B cell immunodeficiency, absent lymph node germinal centres, absent tissue plasma cells and hepatic veno-occlusive disease.
- 4A01.34** Hyperimmunoglobulin E syndromes
- 4A01.Z** **Primary immunodeficiencies due to disorders of adaptive immunity, unspecified**
- 4A0Y** **Other specified primary immunodeficiencies**
- 4A0Z** **Primary immunodeficiencies, unspecified**
- 4A20** **Acquired immunodeficiencies**
Coded Elsewhere: Human immunodeficiency virus disease (1C60-1C62.Z)
Acquired neutropaenia (4B00.01)
- 4A20.0** **Adult-onset immunodeficiency**
Adults with disseminated mycobacterial infections and/or other AIDS-defining infections, often involving concomitant neutrophilic dermatoses. All patients have high titres of anti-interferon-gamma and normal CD4 T helper cell counts.
- 4A20.1** **Acquired immunodeficiency due to loss of immunoglobulin**
Acquired immunodeficiency due to loss of immunoglobulins (protein loss) may occur via the GI tract (protein losing enteropathy), via the kidney (nephrotic syndrome) or via the skin (in severe skin damage).
- 4A20.Y** **Other specified acquired immunodeficiencies**
- 4A20.Z** **Acquired immunodeficiencies, unspecified**

Nonorgan specific systemic autoimmune disorders (4A40-4A4Z)

Coded Elsewhere: Rheumatoid arthritis (FA20)

- 4A40** **Lupus erythematosus**
An autoimmune non-organ specific inflammatory disease characterised by the presence of antibodies to DNA, RNA and other components of the nucleus. It has a very variable clinical presentation and course ranging from an acute fulminant life-threatening disorder with involvement of heart, central nervous system and kidneys to an indolent chronic scarring skin disorder.
Coded Elsewhere: Subacute cutaneous lupus erythematosus (EB50)
Chronic cutaneous lupus erythematosus (EB51)
Neonatal lupus erythematosus (KA07.0)

4A40.0	Systemic lupus erythematosus Systemic lupus erythematosus (SLE) is a clinically multisystem disease, which is autoimmune in origin and is characterised by the presence of autoantibodies directed against nuclear antigens. Manifestations include rash, arthritis and fatigue, nephritis, neurological problems, anaemia and thrombocytopenia at the more severe end of the spectrum.
4A40.00	Systemic lupus erythematosus with skin involvement Systemic lupus erythematosus (SLE) involving the skin. This may present with a malar "butterfly" erythema or with extensive necrolysis of sun-exposed skin, particularly on the head, neck and upper torso. Coded Elsewhere: Immunobullous systemic lupus erythematosus (EB4Y)
4A40.0Y	Other specified systemic lupus erythematosus
4A40.0Z	Systemic lupus erythematosus, unspecified
4A40.1	Drug-induced lupus erythematosus Drug-induced lupus erythematosus is a syndrome in which positive antinuclear antibodies are associated with symptoms, such as fever, malaise, arthritis, intense arthralgia/myalgia, serositis, and/or rash. The syndrome appears during therapy with certain medications (e.g., procainamide, hydralazine, phenytoin) and tumour necrosis factor inhibitors. It occurs predominantly in Caucasians, has less female predilection than SLE, rarely involves kidneys or brain, is rarely associated with anti-dsDNA, is commonly associated with antibodies to histones, and usually resolves over several weeks after discontinuation of the offending medication.
4A40.Y	Other specified lupus erythematosus
4A40.Z	Lupus erythematosus, unspecified
4A41	Idiopathic inflammatory myopathy These comprise a diverse group of syndromes that have in common persistent muscle inflammation of unknown pathophysiology, resulting in damage that affects muscle function. The inflammatory muscle disease can either be acute or chronic in nature. Coded Elsewhere: Extraocular myositis (9C82.3)
4A41.0	Dermatomyositis Dermatomyositis is an inflammatory myopathy, showing progressive, symmetrical muscle weakness, low muscle endurance, and cutaneous manifestations such as Gottron's papules, heliotrope rash, shawl sign, V-sign and mechanic's hand. Internal organ manifestations such as interstitial pneumonia (pneumonitis) and myocarditis sometimes develop. The skin rash may precede the muscle symptoms and may be the only clinical sign of dermatomyositis in some patients (clinically, amyopathic dermatomyositis).

4A41.00	Adult dermatomyositis Adult dermatomyositis is a systemic inflammatory disorders affecting the skeletal muscles, the skin, and other organs. It is characterised by symmetric proximal muscle weakness, increased serum muscle enzymes, myopathic changes upon electromyography, typical histological findings on muscle biopsy, and typical dermatologic manifestations such as heliotrope rash or Gottron's papules. Exclusions: Myasthenia gravis or certain specified neuromuscular junction disorders (8C60-8C6Z)
4A41.01	Juvenile dermatomyositis Juvenile dermatomyositis is the early-onset form of dermatomyositis, a systemic autoimmune inflammatory muscle disorder, characterised by proximal muscle weakness, evocative skin lesion, and systemic manifestations.
4A41.0Z	Dermatomyositis, unspecified
4A41.1	Polymyositis Polymyositis is an inflammatory muscle disease of unknown aetiology occurring predominantly in adults and characterised clinically by proximal muscle weakness (shoulders, arms, thighs), often with associated myalgia. Involvement of pharyngeal and oesophageal muscles may result in dysphagia and a risk of aspiration pneumonia. Myocarditis with rhythm disturbances or cardiomyopathy is a rare but serious complication. Polymyositis may be associated with other autoimmune diseases, malignancy or viral infection. Although serum muscle enzyme concentrations and electromyography are usually abnormal, definitive diagnosis requires demonstration of characteristic histological changes, including muscle necrosis, muscle fibre regeneration and diffuse infiltration by CD8+ T lymphocytes, on muscle biopsy.
4A41.10	Juvenile polymyositis Juvenile polymyositis is a rare childhood idiopathic inflammatory myopathy. It is frequently misdiagnosed, as it lacks a unique clinical phenotype. Traditionally, it presents with weakness of the proximal muscles that evolves over weeks to months. The primary histologic features in are fibre size variability, scattered necrotic and regenerating fibres, and perivascular and endomysial cellular infiltrates. Exclusions: Systemic sclerosis (4A42) Overlap or undifferentiated nonorgan specific systemic autoimmune disease (4A43) Antiphospholipid syndrome (4A45) Vasculitis (4A44) Lupus erythematosus (4A40)

4A41.11	Paraneoplastic polymyositis Paraneoplastic is a rare cancer associated entity. It presents sub-, or acutely with proximal weakness, often including the neck flexors, dysphagia, rarely the respiratory muscles and the heart are involved. Sometimes muscle pain or myalgia occur. Myopathology shows a targeted, cell-mediated lymphocyte toxicity against muscle fibres in focal areas of inflammation within perimysial connective tissue and surrounding blood vessels. Muscle fibres may be destroyed by cytotoxic T cells. Non-Hodgkin's lymphoma, lung, and bladder carcinoma are the most frequently observed associated cancer types.
Coding Note:	Code also the causing condition
4A41.1Y	Other specified polymyositis
4A41.1Z	Polymyositis, unspecified
4A41.2	Inclusion body myopathy Inclusion body myopathy (IBM) is distinguished from polymyositis (PM) and dermatomyositis (DM) on the basis of clinical and histopathological features. A characteristic clinical phenotype is characterised by insidious onset of muscle weakness over months to years, muscle weakness localised predominantly in the thigh muscles and finger flexors, and resistance to glucocorticoid treatment. Typical histopathologic features include sarcoplasmic and nuclear inclusions and rimmed vacuoles. (Kelley's Textbook of Rheumatology, 6th Ed.)
4A41.20	Inflammatory inclusion body myositis Inclusion body myositis (IBM) is the most common idiopathic inflammatory myopathies after age 50. It typically presents with chronic insidious proximal leg and/or distal arm asymmetric muscle weakness leading to recurrent falls and loss of dexterity. Creatine kinase is up to 15 times elevated in IBM and needle electromyography mostly shows a chronic irritative myopathy. Muscle histopathology demonstrates endomysial inflammatory exudates surrounding and invading non-necrotic muscle fibres often times accompanied by rimmed vacuoles and protein deposits. Despite inflammatory muscle pathology, it likely that IBM is has a prominent degenerative component as supported by refractoriness to immunosuppressive therapy. Exclusions: Myasthenia gravis or certain specified neuromuscular junction disorders (8C60-8C6Z)
4A41.21	Noninflammatory inclusion body myopathy Noninflammatory inclusion body myopathy (IBM) is an idiopathic muscle disorder without inflammatory exudates and expression of class I major histocompatibility complex. Rimmed vacuoles and "IBM-like" filaments without inflammatory cells are described in muscle biopsy. Exclusions: Myasthenia gravis or certain specified neuromuscular junction disorders (8C60-8C6Z)
4A41.2Z	Inclusion body myopathy, unspecified
4A41.Y	Other specified idiopathic inflammatory myopathy

4A41.Z **Idiopathic inflammatory myopathy, unspecified**

4A42 **Systemic sclerosis**

Systemic sclerosis is a systemic disorder of the connective tissue; manifested by hardening and thickening of the skin, by abnormalities involving the microvasculature and larger vessels, and by fibrotic degenerative changes in various body organs including the heart, lungs, kidneys, and gastrointestinal tract. (Arthritis Rheum 1980;23:581-590)

Inclusions: Systemic scleroderma

Exclusions: Circumscribed scleroderma (EB61.0)

4A42.0 **Paediatric onset systemic sclerosis**

Systemic sclerosis arising before the age of 16. Involvement of internal organs is less common but arthritis and myositis are more common than in adults.

4A42.1 **Diffuse systemic sclerosis**

Diffuse cutaneous systemic sclerosis (dcSSc) is a subtype of Systemic Sclerosis (SSc) characterised by truncal and acral skin fibrosis with an early and significant incidence of diffuse involvement (interstitial lung disease, oliguric renal failure, diffuse gastrointestinal disease, and myocardial involvement).

4A42.2 **Limited systemic sclerosis**

Combination of calcinosis, Raynaud phenomenon, (o)esophageal dysfunction, sclerodactyly and telangiectasia.

4A42.Z **Systemic sclerosis, unspecified**

4A43 **Overlap or undifferentiated nonorgan specific systemic autoimmune disease**

Nonorgan specific systemic autoimmune diseases which do not fulfil the diagnostic criteria for any single recognised disease entity.

4A43.0 **IgG4 related disease**

IgG4 related syndrome (IgG4-related disease: IgG4-RD) is a clinical disease characterised by elevated serum IgG4 concentration and tumefaction or tissue infiltration by IgG4-positive plasma cells. The diagnostic criteria for IgG4 related syndrome have been proposed, and it may be present in a certain population of patients with a wide variety of diseases, including Mikulicz disease, autoimmune pancreatitis, hypophysitis, Riedel thyroiditis, interstitial pneumonitis, interstitial nephritis, prostatitis, lymphadenopathy, retroperitoneal fibrosis, inflammatory aortic aneurysm, and inflammatory pseudo tumour.

Coded Elsewhere: Primary cutaneous plasmacytosis (EK91.2)

Benign dermal lymphocytic or lymphoplasmacytic infiltrations
or proliferations (EE90-EE91)

Type 1 IgG4 related autoimmune pancreatitis (DC33)

4A43.1	Mikulicz disease Mikulicz disease is a disorder first reported by Johann von Mikulicz in 1892 and characterised by symmetrical swelling of the lachrymal, submandibular, and parotid glands, with massive infiltration of these glands by mononuclear cells. Serum autoantibodies, such as anti-Ro/SS-A, are usually negative and serum IgG4 concentration may be increased. Unlike Sjögren disease, IgG4-Mikulicz disease is characterised by the formation of lymphoid follicles, but shows lower levels of lymphocytic infiltration into salivary ducts, such that their structure remains intact.
4A43.2	Sjögren syndrome Sjögren syndrome is a slowly progressive, systemic inflammatory autoimmune disease affecting primarily the exocrine glands. Lymphocytic infiltrates replace functional epithelium, leading to oral and ocular dryness. Characteristic autoantibodies (e.g., anti-Ro/SS-A and/or anti-La/SS-B) are produced. The disorder can occur alone (it is then known as "primary-SS") or in association with another autoimmune disease (it is then known as "secondary-SS"). Coded Elsewhere: Keratoconjunctivitis sicca (9A79)
4A43.20	Primary Sjögren syndrome
4A43.21	Secondary Sjögren syndrome Secondary Sjögren syndrome is a progressive inflammatory autoimmune disease affecting the exocrine glands in the presence of other systemic autoimmune diseases, such as rheumatoid arthritis, systemic lupus erythematosus, and systemic sclerosis. Lymphocytic infiltrates replace functional epithelium, leading to oral and ocular dryness.
Coding Note:	Code also the causing condition
4A43.22	Paediatric onset Sjögren syndrome
4A43.2Y	Other specified sjögren syndrome
4A43.3	Mixed connective tissue disease Mixed connective tissue disease is an overlapping syndrome combining features of systemic lupus erythematosus, systemic sclerosis, and polymyositis with the presence of autoantibodies to U1-ribonucleoprotein. Raynaud's phenomenon is seen in nearly all patients and pulmonary arterial hypertension is the most common cause of death in MCTD patients.
4A43.4	Diffuse eosinophilic fasciitis Also called Shulman disease/diffuse fasciitis, diffuse eosinophilic fasciitis is a rare idiopathic disorder associated with induration of the skin (orange-peel sign) that generally develops rapidly. It is a dermal and hypodermal sclerosis associated with fibrotic thickening of the subcutaneous adipose lobular septa, superficial fascia, and perimysium. Full thickness excisional biopsy of skin lesions revealing fibrosis of the subcutaneous fascia is generally required for diagnosis. Onset follows unusual physical exertion and trauma, especially in males.
4A43.Y	Other specified overlap non-organ specific systemic autoimmune disease
4A43.Z	Undifferentiated non-organ specific systemic autoimmune disease

4A44**Vasculitis**

Vasculitides represent a heterogeneous group of diseases of multifactorial aetiology characterised by inflammatory lesions of vessels. These lesions consist of fibrinoid necrosis (necrotizing arteritis), giant cell infiltration without necrosis, immunoglobulins deposit or leukocytoclastic infiltration. The spectrum and severity of the systemic vasculitides is broad, from life or sight threatening fulminant disease to relatively minor skin disease.

Coded Elsewhere: Behçet disease (4A62)

Thrombotic microangiopathy, not elsewhere classified (3B65)

4A44.0**Rhizomelic pseudopolyarthritis****4A44.1****Aortic arch syndrome**

Arteritis, often granulomatous, predominantly affecting the aorta and/or its major branches. Onset usually in patients younger than 50.

4A44.2**Giant cell arteritis**

Arteritis, often granulomatous, usually affecting the aorta and/or its major branches, with a predilection for the branches of the carotid artery. Often involves the temporal artery. Onset usually in patients older than 50 and often associated with polymyalgia rheumatica.

4A44.3**Single organ vasculitis**

Vasculitis in arteries or veins of any size in a single organ that has no features that indicate that it is a limited expression of a systemic vasculitis. The involved organ and vessel type should be included in the name (e.g. cutaneous SVV, testicular arteritis, central nervous system vasculitis). Vasculitis distribution may be unifocal or multifocal (diffuse) within an organ. Some patients originally diagnosed with SOV will develop additional disease manifestations that warrant re-defining the case as one of the systemic vasculitides (e.g. cutaneous arteritis later becoming systemic polyarteritis nodosa, etc.). Chapel Hill Consensus Conference, 2011)

4A44.4**Polyarteritis nodosa**

Polyarteritis nodosa is an immunologically mediated systemic necrotising vasculitis affecting medium-sized vessels. In a few cases, the disease appears after viral infection but in the majority of cases there is no known triggering event. The clinical manifestations involve numerous organs and lead to a general alteration in the health status including rapid weight loss, paralysis of the peripheral nerves, renal disease, and digestive problems such as haemorrhages, perforation, appendicitis and pancreatitis. Arthralgia is almost always present and myalgia occurs in half of patients. Cardiac and cerebral anomalies (cephalalgia) are also reported, as well as ocular and genital (orchitis) manifestations.

- 4A44.5 Mucocutaneous lymph node syndrome**
Mucocutaneous lymph node syndrome (Kawasaki disease) is a globally distributed acute vasculitis of young children affecting medium to small calibre arteries and if untreated leads to coronary artery aneurysms in about a quarter of cases. Cardinal signs include cervical lymphadenopathy, conjunctival injection, rash (maculopapular, erythrodermic or erythema multiforme-like), strawberry tongue, oropharyngeal erythema, erythema and swelling of hands and feet and periungual desquamation. The cause is unknown but thought to be environmental, possibly from viral infection. A variant of the disease has been linked to SARS CoV-2 infection.
- Inclusions:** Kawasaki syndrome
- 4A44.6 Sneddon syndrome**
Sneddon syndrome associates livedo reticularis and neurological signs. Livedo is permanent, cyanotic, with no infiltration, and affects the limbs, trunk and sometimes the face. Neurological signs appear later and include cerebrovascular accidents, epilepsy, vertigo and more rarely a pseudobulbar syndrome, chorea, episodes of amnesia or transient amaurosis.
- 4A44.7 Primary angiitis of the central nervous system**
In primary angiitis of the central nervous system, vasculitis is limited to the central nervous system. Primary angiitis of the central nervous system is a very rare disease, and its manifestation can be mimicked by many other diseases. Patients with primary angiitis commonly show headache, waxing and waning altered mental status, and transient ischemic attack-like events. Diagnosis is often based on angiography, although brain biopsy remains the only definitive diagnostic test.
- 4A44.8 Thromboangiitis obliterans**
Thromboangiitis obliterans (TAO), or Buerger's disease, is a segmental occlusive inflammatory condition of arteries and veins, with thrombosis and recanalization of the affected vessels. It is a nonatherosclerotic inflammatory disease affecting small and medium sized arteries and veins of upper and lower extremities. TAO can be distinguished from other types of vasculitis based on its tendency to occur in young male subjects. The etiology and pathogenesis of TAO remains unknown; however, tobacco consumption plays a key role in the initiation and persistence of the disease.
- 4A44.9 Immune complex small vessel vasculitis**
Coded Elsewhere: Anti-glomerular basement membrane antibody mediated disease (MF85)
Susac syndrome (8A45.2Y)
- 4A44.90 Cryoglobulinaemic vasculitis**
Vasculitis with cryoglobulin immune deposits affecting small vessels (predominantly capillaries, venules, or arterioles) and associated with cryoglobulins in serum. Skin and glomeruli are often involved.